Evernorth Behavioral Health Administrative Guidelines

For contracted psychiatrists, psychologists, master's-level therapists, hospitals, and facility-based programs

March 2026

EVERNORTH HEALTH SERVICES

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EVERNORTH HEALTH SERVICES

Introduction

Who we are

Evernorth® Behavioral Health (Evernorth) is a subsidiary of The Cigna Group® and is the behavioral network for customers with Cigna Healthcare® plans. Evernorth also sells behavioral health solutions to employer groups that may not have Cigna Healthcare plans.

Evernorth provides behavioral care benefit management, employee assistance program services, and work/life program services to consumers through employers of all sizes in the United States, national and regional health maintenance organizations, Taft-Hartley trusts, and disability insurers. With headquarters in Bloomington, Minnesota, Evernorth operates four care management centers around the United States in support of a national network of psychiatrists, psychologists, and master-level therapists, clinics, and facilities.

Inside this guide

These guidelines highlight the programs and policies intended to help us work with you efficiently and effectively — for the sake of the people we serve together. We seek to build a strong, long-standing relationship with you and have developed these Administrative Guidelines to help make working with us easier, so you can spend more time providing quality care.

Your Participating Provider Agreement ("Provider Agreement") and these Administrative Guidelines describe the terms under which you agree to provide services to all patients. Terms include the reimbursement rates applicable to covered services provided to patients. However, the actual benefits payable for covered services provided to a patient in all cases are determined by the terms of the patient's benefit plan. In all circumstances, the parties shall comply with applicable law and the regulatory addendum contained herein.

Not all guidelines and program requirements may be outlined in this guide. Other guidelines and requirements or updates may be posted on the Evernorth® Provider website (**Provider.Evernorth.com**) or communicated through notifications delivered in person or by mail, email, phone, or fax.

Please note: These Administrative Guidelines were previously referred to as the "Medical Management Program." Any agreement or document that references the Medical Management Program, the provider guide, the provider manual, or the provider Administrative Guidelines shall be construed to reference this document. The information in this document should align with the terms of your Provider Agreement. If it does not, the terms of your Provider Agreement override what is stated here.

We manage mental health and substance use disorder services for benefit plans sponsored and/or administered, in whole or in part, in compliance with applicable laws, rules, and regulations, including, but not limited to, the Mental Health Parity and Addiction Equity Act, the Affordable Care Act, state parity laws, and other applicable state and federal laws and regulations.

Provider Newsroom

The **Provider Newsroom** offers easier access to the information you need to best support your practice.

You can find information on key topics, such as:

+ Policy updates.

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+ Health plan and network updates.
+ Clinical resources.
+ Digital tools and training.
+ Industry news.

To help ensure you do not miss important news, we also send periodic emails that highlight important updates.

Changes and updates

We may add, delete, or change policies and procedures, including those described in these Administrative Guidelines, at any time. When we make important changes that affect you, such as updates to policies or procedures, we will contact you by email, by mail, or through our **Provider Newsroom**. We review and may update these guidelines annually but also may make updates as needed. Updates to these guidelines will be shared through the **Provider Newsroom**.

Joining the network

If you are an out-of-network provider who is interested in joining the Evernorth® Behavioral Health (Evernorth) network, please refer to the **Join the Network page** on the Evernorth Provider website (**Provider.Evernorth.com**).

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Regulatory addenda and state-specific information

Enclosed at the end of these Administrative Guidelines are the state-specific regulations and contract provisions applicable to the provider and which govern the Provider Agreement. These provisions are incorporated into the Provider Agreement to the extent required by law and as specified in such addenda. The regulatory addenda will be frequently updated to be in compliance with applicable law.

Please use the table below to access each state regulatory addendum.

Alabama	Alaska	Arizona
Arkansas	California	Colorado
Connecticut	Delaware	District of Columbia
Florida	Georgia	Hawaii
Idaho	Illinois	Indiana
Iowa	Kansas	Kentucky
Louisiana	Maine	Maryland
Massachusetts	Michigan	Minnesota
Mississippi	Missouri	Montana
Nebraska	Nevada	New Hampshire
New Jersey	New Mexico	New York
North Carolina	North Dakota	Ohio
Oklahoma	Oregon	Pennsylvania
Rhode Island	South Carolina	South Dakota
Tennessee	Texas	U.S. Virgin Islands
Utah	Vermont	Virginia
Washington	West Virginia	Wisconsin
Wyoming		

Note: These requirements apply only to the extent required by applicable law and may not apply to patients covered under self-funded plans.

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Important contact information

Find the contact you need for information about your patients. Please note that call, claim, and service channels may differ based on the patient's ID card.

Action	Resource
Update your provider directory demographic information or notify us of errors or changes to the way you are currently listed in our provider directory, including: + Name. + Type/degree. + Specialty. + National Provider Identifier. + Clinic or facility affiliation. + Office email address. + Address. + Office phone number. + Whether you are accepting new patients.	Evernorth® provider resource page: **Health Care Provider Directory Changes**
Perform online transactions: + Verify patient eligibility and benefits and generate a benefit reference number. + Look up a procedure code to obtain coinsurance and maximums and determine if prior authorization is required. + Predict the total cost of service and patient liability for specific procedures. + Review medical or pharmacy coverage positions. + View the prescription drug list. + View sample ID cards. + Obtain a reference guide. + Submit preservice prior authorization appeals.	Evernorth Provider portal: **Provider.Evernorth.com**
Perform transactions using electronic data interchange: + Verify patient eligibility and coverage. + Inquire about patient coverage and covered services. + Check the status of a claim. + Request prior authorization for services. + Submit claims electronically. + Receive electronic remittance advice.	Vendor list: **Electronic Data Interchange Vendors**
Perform phone transactions: + Learn about electronic services. + Verify patient eligibility and coverage. + Check the status of a claim. + Request prior authorization for services.	Phone: **800.926.2273** **Note:** Customer service numbers are also included on the patient's ID card. Our hours of operation for routine business are Monday through Friday from 7:00 a.m. to 7:00 p.m. CT. Additionally, advocates and care managers are available 24 hours a day, seven days a week for clinical emergencies.
Submit a paper claim	Patient's ID card

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Action	Resource
Submit or inquire about an appeal or dispute	Phone: **800.926.2273** Fax: **877.815.4827** Mail: Evernorth Appeals PO Box 188064 Chattanooga, TN 37422
Inquire about provider credentialing	Phone: **800.926.2273**
Obtain other phone numbers and addresses	Patient's ID card

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Demographic information

We are committed to maintaining the most up-to-date, accurate, and complete information about providers in our network. Keeping your practice details current ensures that our customers can easily find and contact you.

We use your demographic information to:

+ Publish an online provider directory.
+ Send communications to providers.
+ Process claims.
+ Comply with state laws requiring accurate directory listings.
+ Determine network adequacy.

You can quickly validate your information by searching our online provider directory. You should notify us 90 days before any changes to your practice demographic information. This includes changes in service location, billing address, phone number, Taxpayer Identification Number (TIN), specialties, and new individual or organization National Provider Identifiers (NPIs). It is also important for you to update your status if you are no longer accepting new patients, as this information is included in the provider directory and relied upon by our customers.

Pursuant to Section 116 of The Consolidated Appropriations Act, 2021, group health plans must: *verify and update provider directory information not less frequently than once every 90 days; establish a procedure for the removal of a provider or facility with respect to which such plan or issuer has been unable to verify information during a period specified by the plan or issuers; and update the plan or issuer's database within two business days of such plan or issuer receiving updated information from such a provider or facility*.

In order to comply with Section 116 of The Consolidated Appropriations Act, 2021, we will contact all contracted providers every 90 days to validate directory information. It is the provider's responsibility to reply to that outreach by either confirming that their information is accurate or providing us with updated information. If a provider is unresponsive to all outreach attempts over the course of one calendar year, their information may be subject to suppression within our provider directory.

It is essential that you consistently identify yourself in written communications and claim submissions. Using abbreviations, variations of names, physician licensure, or TINs not listed in a Provider Agreement or not provided to us in advance of the change may result in delayed changes to the provider directory and incorrect claim payments.

How to submit demographic changes

To submit demographic changes, follow the directions outlined in the table below that best match your provider type.

Provider type	Ask	Action
Individual provider	Correct or update service address(es), NPI, office email address, appointment scheduling phone number, license, or whether you are accepting new patients.	Log in to the Evernorth® Provider portal (**Provider.Evernorth.com**) and go to My Practice > Directory Update. If you have not already registered for the portal, go to the **registration page**. On the portal, you can also access patient information, such as eligibility and benefits, view remittance reports, enroll in electronic funds transfer, and make directory profile updates. **OR**

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Provider type	Ask	Action
		Call Provider Services at **800.926.2273**. When prompted, say “health care professional” and then select 4.
	+ Correct or update name, clinic or facility affiliation(s), website, or TIN. + Add new provider or practice location to a currently contracted group. + Request other change types.	Call Provider Services at **800.926.2273**. When prompted, say “health care professional” and then select 4.
Clinic	Add, change, or delete service location information.	Submit the **Clinic Location Changes For Existing Clinic Contracts form** by email or fax.
Facility	+ Add, change, or delete service location information. + Add or remove services.	Call Provider Services at **800.926.2273**. When prompted, say “health care professional” and then select 4.

Note: You can also call Provider Services at **800.926.2273** with any questions. When prompted, say “health care professional” and then select 4.

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ID cards

Quick guide

We pack a lot of important information on our ID cards. Our Quick Guide to Cigna Healthcare® ID Cards helps define and clarify information that appears on most customer ID cards. We occasionally update this guide during the year. You can access the most current version by logging in to the Evernorth® Provider portal (**Provider.Evernorth.com**) and going to Resources > Using ID Cards.

Digital ID cards

We continue to leverage the power of technology and data to accelerate transitioning most of our customers to digital-only ID cards.[1] With providers at varying stages of readiness to accept digital ID cards, we are committed to making the transition as smooth as possible. For more information, visit our **dedicated digital ID card page** on the Evernorth® Provider website (**Provider.Evernorth.com**).

1. Some states have mandates that prohibit digital ID cards. Colorado, Texas, Minnesota, New York, Florida, and Georgia have certain exclusions. Customers residing in these states may continue to receive physical ID cards in the mail depending on their plan type.

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Benefit plan designs and features

We provide a variety of behavioral health services to customers with Cigna Healthcare® plans as well as to other plans nationwide.

Our benefit plan offerings include custom networks to help ensure our customers have access to quality behavioral health care that is appropriate and affordable. In alignment with our medical business and industry standards, the networks that support these plans may be refined by geography, specialty, quality metrics, and plan type (e.g., individual and family plans, health care exchange plans).

Provider participation in a particular network may vary depending on the customer's benefit design. As we introduce more of these custom networks, it will become increasingly important for providers to verify benefits and eligibility for their patients prior to rendering services.

Note: Notifications will be sent prior to changing a provider's participation status in a custom network.

The following table summarizes Cigna Healthcare benefit plan types.

Plan/benefit	Highlights
Cigna SureFit®	Cigna SureFit builds networks around local physician and hospital groups, providing our customers with access to personal, patient-centered care. + Primary care provider (PCP) selection is required, where permitted. + Referrals are not required for behavioral health services. + There is no out-of-network coverage, except in emergent or urgent care. + Providers are responsible for obtaining prior authorization for all in-network services, when required. Cost-share responsibilities and prior authorization requirements are shown on the customer's ID card.
Cigna Connect	Cigna Connect offers our customers access to the Connect Network, which is a narrow network of providers. + At enrollment, customers are required to select a PCP from our in-network provider listing to help coordinate care. + Referrals are not required for behavioral health services. + There is in-network coverage only. Only emergency care and special circumstances are covered when received from out-of-network providers; refer to the benefit plan for details. + Coinsurance or deductibles should not be collected at the time of service. + Providers are responsible for obtaining prior authorization for all in-network services, when required. + Providers must comply with Centers for Medicare & Medicaid Services requirements and meet specific standards for data submission and coding accuracy. For more information, see the risk adjustment section of this guide. Cost-share responsibilities and prior authorization requirements are shown on the customer's ID card.
Health Maintenance Organization (HMO) and Point of Service (POS)	HMO and POS plans offer our customers access to a local network of providers. + PCP selection is required at enrollment to help coordinate care.

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Plan/benefit	Highlights
	+ Referrals are not required for behavioral health services. + HMO plans offer in-network coverage only. + POS plans offer in- and out-of-network coverage. + Providers are responsible for obtaining prior authorization for all in-network services, when required. + Coinsurance or deductibles should not be collected at the time of service. Most payment responsibilities and prior authorization requirements are shown on the customer's ID card.
HMO Open Access and POS Open Access	HMO Open Access and POS Open Access plans offer our customers access to local providers and a variety of different benefit options. + PCP selection is not required. + Referrals are not required to see specialists. + Providers are responsible for obtaining prior authorization for all in-network services, when required. + HMO Open Access plans offer in-network coverage only. + POS Open Access plans offer in- and out-of-network coverage. In-network utilization is encouraged. + Coinsurance or deductibles should not be collected at the time of service. Cost-share responsibilities and prior authorization requirements are shown on the customer's ID card.
Network Open Access	Network Open Access plans allow for an array of cost-sharing options, including copays, coinsurance, and deductibles. + PCP selection is not required. + Referrals are not required to see specialists. + There is no out-of-network coverage, except in emergencies. + The provider is responsible for obtaining prior authorization for all in-network services, when required. Cost-share responsibilities and prior authorization requirements are shown on the customer's ID card.
LocalPlus® and LocalPlus In-Network (LocalPlusIN)	LocalPlus and LocalPlusIN plans offer our customers access to a local, focused network of providers, including doctors, specialists, and hospitals. + PCP selection is not required. + Referrals are not required to see specialists. + Providers are responsible for obtaining prior authorization for all in-network services, when required. + In areas where the LocalPlus network is not available, customers can temporarily access care through our Away from Home care feature, which provides coverage at the in-network benefit level. + If customers choose to access care from providers outside of the LocalPlus network (or outside the Away from Home Care feature when the LocalPlus network isn't available), they will likely pay more. Customers with the LocalPlusIN plan will pay the full cost of their care, except for emergency services as defined by their benefit plan.

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Plan/benefit	Highlights
	+ Customers have access to the Cigna Healthcare national network of laboratories, radiology offices, and dialysis centers as well as virtual care services. + Coinsurance and deductibles should not be collected at the time of service. Cost-share responsibilities and prior authorization requirements are shown on the customer's ID card. LocalPlus and LocalPlusIN plans are available in 34 markets in 24 states, including Alabama, Arizona, Arkansas, California, Colorado, Florida, Georgia, Illinois, Indiana, Kansas, Massachusetts, Maryland, Missouri, Mississippi, New Jersey, Nevada, New York, Oregon, Rhode Island, Tennessee, Texas, Utah, and Washington. As of January 1, 2025, LocalPlus includes a new network, LocalFlex, in Arizona. LocalPlus expansion into additional markets is planned for 2026.
Open Access Plus (OAP)	The Open Access Plus (OAP) plan offers our customers access to a large national network of providers. + PCP selection is not required. + Referrals are not required to see in-network specialists. + Services from out-of-network providers are covered at a reduced benefit level. + Providers are responsible for obtaining prior authorization for all in-network services, when required. + Coinsurance and deductibles should not be collected at the time of service. Most payment responsibilities and prior authorization requirements are shown on the customer's ID card.
Open Access Plus In-Network (OAPIN)	The OAPIN plan offers our customers access to a large national network of providers. + PCP selection is not required. + Referrals are not required to see in-network specialists. + There is no out-of-network coverage, except in emergencies. + Providers are responsible for obtaining prior authorization for all in-network services, when required. + Coinsurance and deductibles should not be collected at the time of service. Most payment responsibilities and prior authorization requirements are shown on the customer's ID card.
OAP New Jersey	The OAP New Jersey plan offers our customers access to providers in New Jersey only. + PCP selection is not required. + Referrals are not required to see in-network specialists. + There is no out-of-network coverage, except in emergencies. + Providers are responsible for obtaining prior authorization for all in-network services, when required.

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Plan/benefit	Highlights
	+ Coinsurance and deductibles should not be collected at the time of service. Most payment responsibilities and prior authorization requirements are shown on the customer's ID card.
Preferred Provider Organization (PPO) and Exclusive Provider Organization (EPO)	PPO and EPO plans offer our customers access to in-network providers across the country. **PPO** + Referrals are not required to see specialists. + Customers can access services from providers who do not participate in the network but will incur additional costs and be reimbursed at a lower coinsurance level. + Providers are responsible for filing the claim form and obtaining prior authorization for all in-network services, when required. + Coinsurance and deductibles should not be collected at the time of service. **EPO** + Referrals are not required to see specialists. + There is no out-of-network coverage, except in emergencies. + Providers are responsible for obtaining prior authorization for all in-network services, when required. + Coinsurance and deductibles should not be collected at the time of service. Most payment responsibilities and prior authorization requirements are shown on the customer's ID card.
Tiered benefits	Cigna Healthcare tiered benefits are available to our customers with OAP or PPO coverage. + Availability depends upon state regulatory approval for benefits and/or funding arrangement. + The tiered benefit has copays and coinsurance levels for covered services provided by Tier 1 providers that differ from those of other in-network providers. + The benefit is intended to encourage individuals with this plan feature to consider using a Tier 1 provider. + Tier 1 providers are determined through multiple criteria, including contractual requirements, business needs, access, and quality and cost-efficiency performance. + Cigna Care Designation (CCD) can be one of the considerations for inclusion in Tier 1 for the tiered benefit plan design in the markets where this benefit is available. However, it does not ensure inclusion. + The Tier 1 provider designation is available in 36 states and Washington, D.C., and distinguishes physicians in 21 specialties (three primary care and 18 other specialties) who participate in our network. **Physician profiles** We evaluate physician quality and cost-efficiency information by using a methodology consistent with national standards. Using this information, we

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Plan/benefit	Highlights
	are able to provide our customers with relevant information through physician quality and cost-efficiency displays and the CCD program. Providers who have earned CCD will be identified in our directory with the symbol, and providers with the Tier 1 status will have "Tier 1 Provider" displayed next to their name in our directory. **Note:** Provider reimbursement is unchanged whether or not an in-network provider has the designation under this program.

For more information about Cigna Healthcare benefit plans and products, log in to the Evernorth® Provider portal (**Provider.Evernorth.com**) and go to Resources > Medical Resources > Medical Plans and Products.

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Cigna Healthcare products

Products can vary by plan. This section applies for your patients with Cigna Healthcare® plans only.

Cigna Choice Fund

There are two Cigna Choice Fund® options: a health reimbursement account (HRA) and a health savings account (HSA). These plans package a health care fund account with a Preferred Provider Organization (PPO) or Open Access Plus (OAP) plan that has a deductible, coinsurance, and out-of-pocket maximum.

When claims are processed, you may be reimbursed directly from the patient's HRA or HSA account for their coinsurance and deductible (if funds are available). This reduces the need to collect funds from the patient at the point of service.

What you need to know

+ Preventive care visits and services are paid at 100 percent for most Choice Fund plans, with no patient cost-share applicable.
+ These plans typically do not include copays.
+ The behavioral copay is the specialist copay.
+ Most individuals with a Choice Fund plan have automatic claim forwarding (ACF). In these cases, the health account is automatically accessed to pay you directly (when funds are available). This helps to alleviate you from having to pursue the patient for any applicable coinsurance or deductible payments.
+ The amount that a patient owes is determined by the claim adjudication under the terms of the plan.
+ If the patient has ACF, collecting coinsurance and deductibles may cause an overpayment, resulting in a refund owed to the individual. Coinsurance and deductibles should not be collected at the time of service.

For more information, including information about ACF, please visit the **Cigna Choice Fund page** on the Cigna for Health Care Professionals website (**CignaforHCP.com**) or call Cigna Healthcare Provider Service at **800.88Cigna (882.4462)**.

Health care accounts

HSAs, HRAs, and flexible spending accounts (FSAs) are types of accounts patients can use to pay for eligible health care costs. Some HSAs, HRAs, and FSAs include a debit card, which allows the patient to easily use their funds to pay at the time of service. When a patient uses their debit card for their visits to an in-network provider, substantiating these claims helps to improve their experience and speed up how quickly you are paid.

If a transaction is not eligible per Internal Revenue Code regulation, the patient should be asked to provide a separate or additional form of payment. Additional information about eligible transactions is available at **Cigna.com/Expenses** or in **Publication 969 (2023), Health Savings Accounts and Other Tax-Favored Health Plans**. You can also call Cigna Healthcare Provider Services at **800.882.4462**.

Strategic alliances

Some of your patients may have a plan offered through a Cigna Healthcare® strategic alliance. This means Cigna Healthcare and another health plan jointly market benefit plans or share in the administration of the plan (e.g., Cigna Healthcare may perform claim repricing and other services). Patients with these plans can access in-network care through the alliance plan's network of providers in the alliance plan's select geographic area. Therefore, the Provider Agreement may or may not apply.

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Please refer to your patient's ID card to determine how to verify eligibility and benefits, obtain prior authorization, and submit claims.

Alliance	About
Priority Health	**Effective:** January 1, 2021 **Service areas:** Michigan's Lower Peninsula **Contract information:** Providers in Michigan's Lower Peninsula must be contracted through Priority Health to be considered in network for patients with a Priority Health ID card. Outside of this service area, your patients who have a Priority Health ID card have access to the Evernorth® Behavioral Health (Evernorth) network. Providers in Michigan's Lower Peninsula must be contracted through Evernorth to be considered in network for patients with Cigna Healthcare coverage. **Additional information:** You can contact Priority Health customer service at **800.942.4765** or visit **PriorityHealth.com**.
HealthPartners	**Effective:** January 1, 2007 **Service areas:** Minnesota, North Dakota, and Western Wisconsin **Contract information:** Providers in Minnesota, North Dakota, and Western Wisconsin must be contracted through HealthPartners® to be considered in network for patients with Cigna Healthcare coverage or a HealthPartners ID card. Outside of these states, your patients who have a HealthPartners ID card have access to the Evernorth network, except in South Dakota, Iowa, and Nebraska, and your patients with Cigna Healthcare coverage have access to the Evernorth network, including in South Dakota, Iowa, and Nebraska. Note: Some exceptions exist where the Provider Agreement supersedes the HealthPartners agreement for patients with Cigna Healthcare coverage. **Additional information:** Each entity manages their own employee assistance program network separately. For more information, refer to the **HealthPartners Quick Reference Guide**.
MVP® Health Care	**Effective:** July 1, 2007 **Service areas:** 46 counties in Upstate New York and Bradford County, Pennsylvania **Contract information:** Providers in Upstate New York counties and Bradford County, Pennsylvania must be contracted through MVP Health Care to be considered in network for patients with an MVP Health Care ID card. Outside of these counties, your patients with an MVP Health Care ID card have access to the Evernorth network, except in Vermont, Berkshire County in Massachusetts, and several counties in New York (Sullivan, Orange, Rockland, Westchester, Putnam, and Dutchess). Providers in Upstate New York counties and Bradford County, Pennsylvania must be contracted through Evernorth to be considered in network for patients with Cigna Healthcare coverage.

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Alliance	About
	Additional information: You can contact MVP Health Care at **888.687.6277**, visit **MVPHealthCare.com**, or access the **MVP Quick Reference Guide**.

Shared Administration Repricing

Cigna Healthcare® contracts with Taft Hartley trusts and Federal Employee Health Benefit plans to share the administration of self-funded plans. For these relationships, Cigna Healthcare provides access to the preferred provider organization, Open Access Plus, and LocalPlus® networks; performs inpatient medical management (and sometimes outpatient, depending on the employer group); and reprices claims according to negotiated rates. For some employer groups, Cigna Healthcare may also provide stop loss insurance, chronic condition management, and pharmacy benefits. Third-party administrators (TPAs) or employer groups are involved in the administration of these plans with respect to eligibility and claim payments on their own systems.

What you need to know

+ Cigna Healthcare requires TPAs to provide frequent eligibility information updates to help minimize late identification of individuals who are not covered.
+ Plan designs require in- and out-of-network benefit differentials to encourage individuals to use providers who participate in the Cigna Healthcare network.
+ Cigna Healthcare performs precontract checks to help ensure TPAs meet its standards for claim payment accuracy, payment turnaround time, and call statistics (e.g., average speed of answer). Adherence to these standards is contractually required.
+ Cigna Healthcare audits TPAs regularly to help ensure compliance with contract standards.
+ Cigna Healthcare network staff and provider service representatives are available to support you and facilitate resolution of any claim inquiries or issues.
+ Cigna Healthcare retains the authority to resolve differences regarding provider contract language and intent.

ID cards: ID cards contain the Cigna Healthcare logo and both paper and electronic claims submission addresses. (Note: Electronic claim submission is the most cost-effective method.) The Cigna Healthcare phone number for prior authorization and the TPA's phone number and address for eligibility, benefits, and claim status inquiry are also available on your patient's ID card.

Case management: All utilization reviews are provided through Cigna Healthcare and branded as CareAllies®, a division of The Cigna Group®. Employer groups may purchase review of outpatient services (e.g., ambulatory surgery, high-tech radiology).

Eligibility, benefits, and claim status and payment: For information related to these topics, please contact the TPA at the phone number and address listed on your patient's ID card.

Claim flow: Claims should be submitted directly to Cigna Healthcare using the Cigna Healthcare electronic payer ID 62308 or to the mailing address listed on your patient's ID card. Cigna Healthcare prices the claim based on the provider's contracted reimbursement rate and the results of its utilization review program. The priced claim is then forwarded to the Shared Administration Repricing employer group for payment based on the patient's eligibility and benefits. The Shared Administration Repricing employer group then remits payment following contractually agreed-upon turnaround requirements. Please contact the TPA directly to enroll with electronic remittance advice and electronic funds transfer.

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Clinical and contract-related appeals: Appeals of clinical denials should be submitted to Cigna Healthcare using the contact information supplied in the denial letter. Please submit appeals of application-of-contract rates directly to Cigna Healthcare, ATTN: Appeals, PO Box 188004, Chattanooga, Tennessee 37422.

Additional information: For additional information, please log in to the Evernorth® Provider portal (**Provider.Evernorth.com**) and go to Medical Resources > Medical Plans and Products > Shared Administration.

Payer solutions segment

Cigna Healthcare® contracts with third-party administrators (TPAs), selected insurers, and claim administrators (referred to collectively as "payers") to share the administration of self-funded and insured plans. For these relationships, Cigna Healthcare provides access to the preferred provider organization, Open Access Plus, and LocalPlus® networks; may perform medical management; and prices claims according to its negotiated rates. For some employer groups, Cigna Healthcare also provides stop loss insurance, chronic condition management, pharmacy benefits, and other products. Contracted payers maintain eligibility, administer benefits, and process claims for these accounts on their own systems.

What you need to know

+ Cigna Healthcare requires payers to provide frequent eligibility information updates to minimize late identification of individuals who are not covered.
+ Plan designs require in- and out-of-network benefit differentials to encourage individuals to use providers who are in the Cigna Healthcare network.
+ Cigna Healthcare performs a precontract review to help ensure payers meet its standards for claim payment accuracy, payment turnaround time, and call statistics (e.g., average speed of answer). Adherence to these standards is contractually required.
+ Cigna Healthcare meets regularly with payers to review service metrics and may audit payers to help ensure compliance with contract requirements standards. Cigna Healthcare also monitors service levels through routine metric reporting.
+ Individuals enrolled through these payers are "Participants" as defined by the Cigna Healthcare Provider Agreement. Additionally, Cigna Healthcare has a direct agreement with the employer groups or insurers responsible for funding claim payments.
+ Cigna Healthcare contracting staff and provider relations representatives are available to support providers with contracting questions. For claim-related inquiries, please contact the TPA listed on your patient's ID card.

Claim flow: Claims should be submitted directly to Cigna Healthcare using the Cigna Healthcare electronic payer ID 62308 or to the mailing address on your patient's ID card. Cigna Healthcare prices the claims based on its network contracted rates. The priced claim is then forwarded to the payer for payment based on the patient's eligibility and benefits. The payer then remits payment following the contractually agreed-upon turnaround requirements.

Eligibility, benefits, and claim status and payment: For information related to these topics, please contact the TPA at the phone number and address listed on your patient's ID card.

Clinical and contract-related appeals: Appeals of clinical denials should be submitted to Cigna Healthcare using the contact information supplied in the denial letter. Please submit appeals of application-of-contract rates directly to the address on your patient's ID card. Appeals must be submitted within 180 days of the initial denial. Unless otherwise specified in the Provider Agreement, contract-related appeals can be faxed to **877.804.1679** or mailed to Cigna Healthcare, PO Box 188062, Chattanooga, Tennessee 37422.

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EVERNORTH® HEALTH SERVICES

International Health

International Health is a segment of Cigna Healthcare® that offers solutions to globally mobile individuals and employers with a portion of their workforce that frequently spends extended time outside of the United States. These plans provide a whole-health service platform with a personalized, predictable, and simple customer experience that drives affordability of medical plans. As part of a global organization, customers have access to health care support in more than 200 countries and territories around the world.

What you need to know

+ International Health products are supported by Cigna Healthcare Preferred Provider Organization and Open Access Plus plans for services rendered in the United States.
+ Coverage for International Health customers is included in the Cigna Healthcare Provider Agreement for in-network providers in the United States.
+ In-network providers will be reimbursed at the same rate for services provided to international customers as they would for any other Cigna Healthcare customer.
+ There are three types of international customers in the United States:
    - **Returns** - Citizens of the United States living abroad. Many of these expatriates and their families will receive routine medical care during annual visits home. In addition, they will often return to the United States for the treatment of serious conditions.
    - **Inpatriates** - Foreign nationals who are living and working in the United States.
    - **Stay-behinds** - Some expatriates leave their families at home in the United States when they go on assignment overseas. These dependents (or "stay-behinds") have coverage through International Health.

Eligibility, benefits, and prior authorization: You can verify eligibility, benefits, and prior authorization requirements for many International Health customers by calling the number on the back of your patient's ID card.

Claims submission process: Form UB-O4 or Form CMS-1500 should be submitted via electronic data interchange using the payer ID 62308. **Note:** Contracted providers are required to request a copy of the International Health ID card and bill International Health directly. Any missing required information might result in a payment delay.

Clinical and payment-related appeals: Appeals for denied claims or prior authorization requests should be submitted using the contact information provided in the explanation of benefits and/or denial letter.

Reconciliation process: For issues with International Health claims, please call the phone number on the back of your patient's ID card (this number will differ from the standard U.S. customer service and provider service line).

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Medicare Advantage

In accordance with federal laws, the information under this section complements the Provider Agreement and applicable sections of a Medicare Advantage payer-specific manual.

Medicare managed care

Health Care Service Corporation (HCSC), under the HealthSpring℠ brand, administers behavioral health benefit services, including claims processing, customer service, medical management, and utilization management. Evernorth® Behavioral Health (Evernorth) administers behavioral health provider network services, including additions and terminations, contracting, credentialing, and fee negotiations.

For additional information, please visit the **provider page** and **provider manuals** on the **HealthSpring** website.

In select states, providers in the Evernorth network deliver services to patients enrolled in an health maintenance organization or preferred provider organization benefit agreement with Medicare Advantage organizations. The benefits supplied by Medicare Advantage organizations are a Medicare replacement product rather than a Medicare supplemental plan. A patient must be enrolled in Medicare and opt to have coverage by a Medicare Advantage organization. These patients are identified by their ID card.

Access to services

All services covered by Medicare must be provided in a manner consistent with professionally recognized standards of health care. Providers in the Evernorth® Behavioral Health (Evernorth) network must provide necessary covered services to patients or arrange for a covering provider 24 hours a day, seven days a week. Covering providers must satisfy Evernorth credentialing criteria and cannot seek to obtain reimbursement for which the provider already receives reimbursement from Evernorth.

Excluded providers

In accordance with 42 CFR 422.752, Evernorth® Behavioral Health and its downstream providers are barred from employing or contracting with individuals who are excluded from participation in Medicare under section 1128 or 1128A of the Social Security Act or with an entity that employs or contracts with such individuals for the provision of any of the following: health care, utilization review, medical social work, or administrative services.

Individuals or entities found to be in violation of this regulation may be subject to sanctions and civil money penalties, including, but not limited to, fines ranging from $10,000 to $100,000, suspension of enrollment of Medicare beneficiaries, and suspension of payments.

Compensation

1. Providers are to accept reimbursement from the Medicare Advantage plan in the amount set forth in and in accordance with the Provider Agreement, its Exhibit(s), and the terms of the patient's Medicare plan as full payment for covered services. The rates shall apply to all covered services. HealthSpring℠ Medicare shall notify the provider of the copay, deductible, or coinsurance, if any, which shall be charged to the patient pursuant to their coverage under their plan.

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Physician incentive plan

Physician incentive plan regulations require the disclosure of financial relationships between HealthSpring℠ Medicare or Evernorth® Behavioral Health (Evernorth) and the provider that could put the provider at significant risk. The Centers for Medicare & Medicaid Services seeks to ensure that they are aware of financial incentives for providers to withhold referrals for medical care. There is an elaborate process established to calculate significant financial risk. The process requires consideration of the number of patients involved and whether the financial arrangements involve agreements such as capitation, withholds, or bonuses. Providers are required to secure the Medicare Advantage plan's prior approval of any incentive arrangements relating to patients with HealthSpring Medicare coverage.

1. Prior to the execution of the Provider Agreement and throughout the term of the Provider Agreement, the provider shall submit to Evernorth and secure its prior approval of any provider incentive arrangements relating to patients and the covered services rendered. Evernorth has the right to disclose such arrangements if required to do so by applicable laws and regulations. The provider shall maintain at their sole expense any stop loss coverage required to be maintained by applicable law in connection with any such provider incentive arrangements and shall provide evidence of such coverage upon request.

2. Prior to the execution of the Provider Agreement, the provider shall secure approval from Evernorth regarding the percentage of the total covered services under the agreement, which may be 'referral services' as that term is defined under applicable laws and regulations. The provider shall not change the percentage of referral services without the prior written approval of Evernorth.

Obligations under federal funding

Payments received in connection with services rendered to patients with Medicare Advantage coverage are, in whole or in part, from federal funds. Recipients of such payments are subject to certain laws that are applicable to individuals and entities receiving federal funds, including Title VI of the Civil Rights Act of 1964, the Age Discrimination Act of 1975, the Americans with Disabilities Act, and all other applicable laws and rules. In addition, in order to comply with Centers for Medicare & Medicaid Services requirements for Medicare Advantage organizations, Evernorth® Behavioral Health and HealthSpring℠ Medicare must follow federal regulations identified in the Balanced Budget Act of 1997 and the Medicare Prescription Drug, Improvement, and Modernization Act of 2003. All written arrangements between a provider and downstream entities must comply with applicable Medicare laws and regulations. All providers must agree to comply with the Medicare Advantage organization's policies and procedures.

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Employee Assistance Professionals Association

The Employee Assistance Professionals Association (EAPA) defines an employee assistance program (EAP) as ". . . a workplace-based program designed to assist in the identification and resolution of productivity problems associated with employees impaired by personal concerns including, but not limited to alcohol, drug, legal, emotional, stress, and other personal concerns which may adversely affect employee job performance."

Key objectives of EAPs are to:

+ Support employers by offering assistance for employees to constructively manage personal problems, which may have a negative impact on job performance.
+ Work closely with employers' health care benefit programs and local community resources.
+ Accurately and quickly match employers' needs with appropriate resource assistance.

Employee assistance program services and models

Evernorth® Behavioral Health (Evernorth) employee assistance program (EAP) services are promoted as a resource to assist employers and employees and their dependents and household members ("customers") in identifying and resolving issues of daily life. The program offers a work/life service with life cycle information, consultations and referrals, and the opportunity for short-term counseling with professionals skilled in the assessment and treatment of a wide range of problems. The results for the sponsoring organization can be significant:

+ Higher employee productivity.
+ Prevention of potentially costly problems in the workplace.
+ The comfort of professional assistance in response to a critical incident.
+ The retention of good employees who appreciate their organization's efforts to maintain a healthier workforce.

Assessment and referral

The assessment and referral model includes one to three face-to-face or virtual sessions focusing on problem identification and resolution or referral to appropriate resources for problem resolution.

Most of our EAP customers have an assessment and referral model. This model provides sessions to formulate a customer's situation, communicate those findings to the customer, and make a referral into the customer's behavioral health benefit if ongoing care is necessary. Where clinically appropriate, a provider may refer the customer to themselves for continuing care.

Short-term counseling

Short-term counseling focuses on the resolution of the presenting problem.

Participation in short-term counseling plans has been increasing. This product includes more than three sessions and sometimes up to 10. The most common short-term counseling model elected includes five to eight sessions.

The goal of this product is to try and achieve problem resolution within the short-term counseling. If, after assessing, the provider determines that is not an appropriate goal, they should refer the customer to their behavioral health benefits.

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Stand-alone EAP services

For stand-alone EAP services, Evernorth is the customer's provider for only EAP services, not behavioral health care services. Stand-alone EAP services may be either assessment and referral or short-term counseling.

Integrated EAP services

For integrated EAP services, Evernorth is the customer's provider of both EAP and behavioral health care services. Integrated EAP services may be either assessment and referral or short-term counseling.

EAP exclusions

The Evernorth EAP benefit excludes coverage for:

+ Psychiatric evaluations/medication management.
+ Psychological testing.
+ Court-ordered treatment.
+ Workers' compensation/disability management/Family and Medical Leave Act (FMLA).
+ Fitness for duty/return-to-work determinations.

The EAP does not cover employment law, a nonclinical service. Coverage for employment law is excluded due to the dual nature of the EAP employer group (the individual employee [family member] and the employer who sponsors the program). Any legal information or advice given by a provider to an individual customer concerning employment law can have potentially detrimental consequences for the employer group. To avoid this conflict of interest, employment law services are excluded from the program.

While providing EAP services, providers shall refrain from discussing legal recourse as a potential action in resolving workplace concerns or disputes. Customers with concerns about workplace practices should be referred to their human resources department for further assistance.

Examples of excluded employment law questions or concerns are:

+ Workplace safety, accidents, injuries, or illnesses.
+ Coworker liability (including workplace assaults or threats).
+ Employee benefits issues/disputes or disputes concerning the agents of company-sponsored benefits or services.
+ Pension rights, employment termination, retirement questions, or disputes.
+ Employer-based civil rights violations (including workplace sexual harassment allegations).
+ All other alleged employer liability issues.

Eligibility for EAP services

Unlike behavioral health care services, eligibility for EAP services is not verified in most cases. Employees and their dependents and household members ("customers") are eligible for EAP services. Some employer groups extend EAP services to retired employees. There is no limit to the frequency with which eligible individuals can access EAP services for new problems or concerns.

Prepaid benefit
Because Evernorth EAP services are available at no cost, there are no out-of-pocket expenses or copays collected by providers. If a treatment referral is needed beyond EAP services, this should be coordinated based on the customer's

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health plan coverage. Customers should review such coverage with the provider prior to a referral in order to determine what expenses will be covered by their medical plan. Provider questions about coverage should be directed to Evernorth care management staff.

Barrier-free access
Eligible customers have access to Evernorth EAP services 24 hours a day, seven days a week through a toll-free phone number. Licensed behavioral clinicians in our care center can address emergencies or crisis situations immediately. During routine calls, customers speak with Evernorth personal advocates who will review their EAP benefit, discuss their personal concerns, and identify local providers for referral purposes.

Only in the event of a management/mandatory referral will an Evernorth staff member give the provider advanced notification of the EAP customer's name, the employer group, presenting problem, type of EAP benefit, and type of referral. If a customer would like to utilize their benefit, they must obtain an EAP code from Evernorth and provide it prior to their first counseling session. All EAP claims should be billed using Current Procedural Terminology (CPT®) code 99404.

Self-referring customers may choose to register for EAP services, select a provider, and receive a code for services at **myCigna.com** or **Well.Evernorth.com**. In this case, both the provider and customer will receive a letter confirming the approved service.

The appointment access standard for routine EAP cases is two business days. If the provider cannot offer an appointment within this time frame (when it is requested), the customer should be directed back to the Evernorth referral source.

If a customer schedules an appointment and fails to keep it, the provider should contact the customer to discuss their intentions. The provider is expected to report any customer concerns or complaints about satisfaction with EAP services to Evernorth at **800.926.2273.**

Types of referrals

Customers may contact the EAP as soon as they feel consultative assistance may be helpful. To prevent possible adverse impact on job performance, customers with personal concerns are encouraged to contact the EAP. Participation in the EAP is confidential within the limits of the law. Customers may access the EAP through self-referral, management referral, or continuation of employment/mandatory referral.

Self-referral
An EAP self-referral occurs when the customer contacts the EAP directly and voluntarily seeks assistance for a personal concern. The employer is given no information regarding the individual customer's contact.

Management referral
An EAP management referral involves circumstances where the customer has had job performance issues that have generated the referral by a manager or human resources contact.

Participation in the EAP should not jeopardize the customer's job security, promotional opportunities, etc. Conversely, participation in the EAP should not protect the customer from disciplinary action that may result from substandard job performance, conduct infractions, or a violation of company policy. Providers working with management referral cases are required to work with an assigned Evernorth EAP consultant who maintains communication with the referring manager. Providers are not to contact the referring manager and should direct inquiries to the Evernorth EAP consultant.

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Continuation of employment/mandatory referral
An EAP continuation of employment/mandatory referral is offered by a manager or human resources representative in lieu of termination. Common reasons for continuation of employment/mandatory referrals include, but are not limited to, substance use policy violations.

The release of information form will allow management to be informed of compliance with EAP attendance and recommendations. Providers who receive mandatory or continuation of employment referrals are required to work with an Evernorth EAP consultant who maintains communication with the referring company contact. It is extremely important that providers respond to the inquiries and requests for updates from the Evernorth EAP consultant as these cases involve job jeopardy for the customer. Additionally, the Evernorth EAP consultant will work collaboratively with the provider on cases where their provision of substance abuse professional or substance abuse expert services is requested.

Provider responsibilities related to management/mandatory referrals

Communication
Communication is critical to facilitating the effective resolution of the customer's issues. Our collaborative vision of program services involves the recognition and balancing of both the needs of the customer and their employer. An Evernorth EAP consultant will facilitate communication, when indicated, with the customer's employer.

Employers who request information from the provider should be directed to the Evernorth EAP consultant by calling their toll-free phone number. Direct communication between the provider and the employer must be reviewed and approved in advance by Evernorth.

Procedures
Providers are encouraged to remain aware that there is a "dual client" relationship in EAP work. The provider has both the organization/employer and the customer seeking services as the client. In instances where a concern about conflict of interest arises in this relationship, the provider is urged to utilize the Evernorth EAP consultant as a resource by calling **800.926.2273**.

The provider should meet with the customer to:

+ Complete a comprehensive assessment.
+ Formulate a plan to address the workplace performance issues and any areas that may be contributing to the problem.
+ Inform them of their specific recommendations and let them know that those recommendations will be reported back to the employer. No clinical or diagnostic information will be given to the employer.
+ Coordinate with their behavioral health benefits and/or appropriate community resources and provide them with the contact information for the referral resource if a referral is needed; obtain permission from them to communicate with the referral resource; and follow up with the referral resource to verify initial compliance.

The provider should contact the Evernorth EAP consultant with an initial update, which should include the following:

+ EAP dates of service.
+ Presenting problem.
+ Diagnostic impressions (chemical dependence, mental health, medications, risk of harm).
+ Information about how the workplace concerns are being addressed.
+ Confirmation on whether the customer is compliant with the EAP process.
+ Date of next EAP appointment.
+ Recommendations or referral (must include a name and contact number for the referral resource).

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Note: Do not communicate directly with the employer or complete any paperwork for the customer regarding return to work, disability, FMLA, fitness for duty, etc.

The provider should update the Evernorth EAP consultant regarding the customer's compliance:

+ Dates of subsequent EAP appointments.
+ The customer's progress/compliance with the EAP process.
+ If a referral was made, verification that the customer began attending the recommended treatment.
+ Date that the EAP case was closed and any recommendations for continued treatment.

The Evernorth EAP consultant obtains a signed release via the referring manager and provides the following information:

+ Dates of EAP appointments attended or not attended.
+ Date of next EAP appointment.
+ Any recommendations for services beyond the EAP. The information given to the employer is limited to the level of care, type of referral resource (e.g., inpatient, outpatient), and the name of the treating provider or facility.
+ The customer's demonstrated compliance with the initial recommendations.

Assessment and evaluation

During the assessment, the provider identifies the nature of the problem and a possible plan of action. Family, significant others, and friends may be included in this process, as appropriate. The **EAP Clinical Assessment Form** routinely focuses on the individual's level of functioning in the workplace, presence of any contributing stressors, and the need to develop additional coping skills that might enhance job performance and occupational satisfaction. Providers use information from the assessment to:

+ Help the customer clarify the nature of the presenting problem.
+ Identify underlying or collateral issues, including nonclinical contributing factors (e.g., legal, financial, child/eldercare needs).
+ Evaluate the level of problem/symptom severity.
+ Create an appropriate action plan in collaboration with the customer.

Evernorth encourages the provider to work with the customer to resolve their presenting problems within the available EAP visits whenever possible.

All EAP customers must receive the **Employee Assistance Program (EAP) Statement of Understanding Form** at the beginning of the initial session. Should the customer refuse to sign the form, the provider should make a note on the form and place it in the customer's file.

If the EAP customer reveals legally sensitive information regarding the workplace (e.g., sexual harassment, discrimination issues) or requests the provider to contact the workplace, the provider should notify an Evernorth EAP consultant by calling **800.926.2273** prior to taking any action. Under no circumstances should the provider suggest that the customer consult with or retain an attorney for the purpose of assessing the potential for legal action against the employer.

Disposition

To ensure customers receive appropriate services and value from the EAP program, Evernorth encourages providers to work with customers to resolve presenting problems within the context of the EAP. If the customer has only EAP benefits with Evernorth, the provider will coordinate referrals for ongoing care directly with the customer either through their behavioral health benefits or appropriate community resources.

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If Evernorth provides both the customer's EAP and behavioral health benefits, the provider can continue providing care if/when the following criteria are met:

+ Evernorth provides both the EAP and behavioral care benefits for the customer.
+ Continuation is clinically the most efficacious course of treatment.
+ It is the customer's request.
+ At least two other individual provider options are communicated.

If Evernorth does not manage the customer's mental health and substance use benefits or no benefits are available for the customer, the provider is expected to facilitate an appropriate transition to alternate resources. This may include referrals to community-based treatment programs or counseling agencies or interface with other health maintenance organizations. The provider is expected to secure an appropriate release of information form, contact the treatment provider or community resource to alert them to the referral, and confirm that the appropriate services can be provided.

Information, resources, and referrals are also available through the Evernorth EAP for legal needs, childcare and eldercare, pet care, Healthy Rewards®, and other work-life needs. The provider should direct the customer to call the Evernorth EAP for these services. In addition, providers are expected to maintain knowledge of community-based support groups and resources that may offer additional services (e.g., financial concerns, debt management, personal budgeting) and assist in the transition to these resources.

Follow-up and case closure
Approximately 45 days after a referral is made, an Evernorth coordinator contacts the customer, with prior permission, to assess whether their needs were met by the services that were provided.

To ascertain the disposition of a case, providers are required to contact the customer 10 to 14 days after the conclusion of the EAP episode of care to ensure that they have successfully connected with treatment or community resources for ongoing care or to confirm that no further assistance is needed.

Claims payment process
All EAP claims should be submitted with CPT code 99404. EAP claims submitted without the required CPT code will be denied or will not be processed.

Confidentiality
The Evernorth EAP's success and credibility hinges on the confidence that the EAP respects the customer's right to privacy and will protect the information they disclose within the parameters of the law. The EAP will maintain the confidentiality of customers and fully disclose conditions that limit confidentiality. Providers will not share information about involvement in the program without the customer's prior knowledge and written permission, except as required by law. Every effort is made to maintain strict confidentiality with self-referrals, management referrals, and continuation of employment/mandatory referrals.

Management consultation and education

The Evernorth EAP provides a number of consultative and educational tools to help managers recognize and address multiple workplace issues, including potential or existing substance use problems. Education, training, and reference guides are available to assist supervisors and human resource managers with problem recognition, intervention, EAP referrals, follow-up, and reintegration of employees into the workplace.

Evernorth EAP services provide management consultation by certified EAP professionals, on a per case basis, to support supervisors and human resource managers in developing immediate strategies to deal with performance issues, workplace behavior that may be indicative of potential alcohol and/or other drug problems, violence at work and home,

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workplace crises, critical incidents, and other sensitive situations. Management consultation is available 24 hours a day, seven days a week.

Providers are expected to collaborate with an Evernorth EAP consultant in servicing challenging management referral, continuation of employment/mandatory referral, and workplace trauma incidents.

Promotion

Evernorth believes clear communication about the EAP benefit at the time it is implemented and frequent reminders about the EAP enhances the visibility of the program and encourages utilization of available services. Evernorth offers several resources to help promote the EAP and provide additional avenues to make the availability of prevention, early intervention, consultation, and work/life services known to the organization and customers:

+ Communication materials, including brochures/quarterly awareness brochures, wallet cards, videos, guides for managers, newsletter articles, and posters on wellness, prevention, and personal growth.
+ Extensive library of wellness seminars, which are delivered on-site at employer locations or virtually by providers.
+ Employee orientation sessions, supervisory training, and health/benefit fairs.
+ EAP information through the Evernorth Provider website (**Provider.Evernorth.com**).

EAP specialty services

Evernorth EAP integrates provider specialty skills in the provision of critical incident assistance, substance abuse professional services, employee education and wellness seminars, management training, and management referrals. The Evernorth Provider Relations team facilitates the recruitment, orientation, and privileging of providers in these EAP specialty service areas.

To have the critical incident assistance specialty added to your profile, you must sign an attested specialty form confirming that you:

+ Have received formal training in Critical Incident Response (CIR).
+ Have delivered a minimum of four CIR services in the past two years.
+ Agree to make changes to your schedule to accommodate CIR requests within **two to 12 hours**.

To have the management referral specialty added to your profile, you must sign an attested specialty form confirming that you:

+ Have experience with customers who are required to access services.
+ Agree to assess a customer and develop a plan to address their issues that may be contributing to the workplace problem.
+ Are qualified and agree to perform a general substance use screening as part of your overall assessment.
+ Are familiar with local resources and agree to serve as an advocate for the customer to access the proper level of care.
+ Agree to follow up with referral resources to verify initial compliance with the recommended treatment.
+ Agree to follow up with the Evernorth EAP consultant within 24 hours of each appointment.

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Digital solutions

To help you make the most of your time, we provide convenient tools to reduce your administrative burden.

Tool	Summary
Evernorth® Provider website (**Provider.Evernorth.com**)	**Provider.Evernorth.com** offers publicly available information, including tools, policies and procedures, forms, education and training resources, and EAP resources. It also includes a portal that offers registered users access to secure patient information upon login, such as eligibility, benefits, and claim status. Access is managed by your website access manager.
Education, training tutorials, and videos	Our learning tools provide convenient access to material about our policies and procedures, electronic service capabilities, and other important information. To view or download and print these materials, go to the **Behavioral Education and Training page** on **Provider.Evernorth.com**.
Electronic data interchange (EDI)	EDI is a secure electronic connection that works with your practice management system and EDI vendor to submit claims and receive eligibility, claim status, and payment information.
Electronic funds transfer (EFT)	EFT offers a secure method for funds to be deposited directly into your bank account. Reimbursement payments are available the day the deposit is electronically transferred to your bank account.
ePrescribe	ePrescribing provides access to prescription eligibility, the drug list, and medication history for patients with pharmacy benefit coverage and the ability to send electronic prescriptions to the patient's pharmacy of choice, including Express Scripts® Pharmacy and Accredo® Specialty Pharmacy.
Interactive voice response (IVR)	The IVR phone system provides access to eligibility, benefit, and claim status information, prior authorization information, credentialing status, and more.

Evernorth Provider website

The Evernorth® Provider website (**Provider.Evernorth.com**) has been designed with you in mind — to fit your needs and the way you work. It includes a portal that you can log in to for secure access to patient and claims information and has features that will save you time.

Information	Action
Eligibility and benefits	+ Search for up to 10 patients at once. + Pull up benefits using a specific procedure code or view general benefits. + View real-time information about coinsurance, deductibles, and plan maximums. + Generate a benefit reference number and store the history for two years. + View your patient's ID card.
Claims information and electronic claim submission	+ Check claim status and details. + View payment information. + Upload requested documentation for pended claims. + Request a claim reconsideration or an appeal.

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Information	Action
	+ Access Claims 360, which provides an organizational view of your claims. + Use your Taxpayer Identification Number and filters to assist you in zeroing in on the claims you want to view.
Electronic funds transfer (EFT)	+ Enroll in EFT. + Check the status of your EFT enrollment. + Change EFT settings. + Change your report delivery preferences.
Remittance reports	+ Access your remittance reports the same day you receive your EFT. + Easily store and search payment information and share it with your office staff.

Registration

You can register for **Provider.Evernorth.com** in two ways:

+ Through your website access manager, who is responsible for adding or delegating new users and assigning the appropriate level of access immediately.
+ By going to the **registration page**. Please note that self-registering still requires vetting and confirmation from your website access manager and may take longer to complete.

Electronic data interchange

Electronic data interchange (EDI) allows patient information to be transferred between you and Cigna Healthcare® or Evernorth® Behavioral Health (Evernorth) in a standardized, secure way and makes it available right on your desktop.

You can use your existing EDI vendor, practice management software, or accounts receivable software to connect with our systems to:

+ Submit electronic claims to Cigna Healthcare (837), including coordination of benefits (COB) claims, and receive an electronic claim acknowledgment (277CA).
+ Receive payment information in the electronic remittance advice (835), including the amount paid and when the check or electronic funds transfer (EFT) was issued.
+ Submit electronic eligibility benefit inquiries (270/271) and track claims status (276/277).
    - Receive a real-time response in seconds.
    - Obtain benefit information.
    - Receive remaining health plan deductible and coinsurance amounts.
    - Obtain COB and shared administration or alliance information.
    - Obtain claims status and receive responses using the Health Insurance Portability and Accountability Act of 1996 standard health care claim status codes.
+ Submit electronic health service review/prior authorization requests (278).

For a list of EDI vendors and the transactions they support, visit **Cigna.com/EDIvendors**. For questions about transactions submitted through your EDI vendor, please contact the vendor directly.

Payer ID for submitting electronic claims

Payer ID	Claim type

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62308[1]	Medical, behavioral (including employee assistance program), and dental claims

1. Both primary and secondary COB claims can be submitted electronically to Evernorth or Cigna Healthcare.

You do not have to submit Medicare Part A and B Coordination of Benefits Agreement claims to Cigna Healthcare or Evernorth. The Medicare explanation of benefits or electronic remittance advice will show that those claims are forwarded to Cigna Healthcare or Evernorth as the secondary payer.

Interactive voice response

Our interactive voice response (IVR) system offers access to patient eligibility, benefit, and claims information. The IVR system allows you to:

+ Request information for multiple patients during a single phone call.
+ Hear the requested information or have it faxed to you.
+ Submit requests for prior authorization, referrals, and/or prescription authorizations.
+ Access detailed claims information, such as claim status, payee, check amounts, and when and where payments were sent.

To access the IVR system, call one of the following numbers:

+ Patients with Cigna Healthcare ID cards: **800.88Cigna (882.4462)**.
+ Patients with employee assistance program/behavioral health health maintenance organization **800.926.2273**.

User tips

+ During menu options, say “main menu” to go back to the main menu.
+ After accessing the self-service information (e.g., eligibility, benefits, and claim status), press “0” to speak with a provider service representative.

ePrescribe

ePrescribe is available to providers for patients covered by Cigna Healthcare® pharmacy benefit plans. ePrescribing provides access to prescription eligibility, the drug list, and medication history for patients and allows prescriptions to be sent electronically to a patient’s pharmacy of choice, including Express Scripts® Pharmacy and Accredo® Specialty Pharmacy. ePrescribing can be used during point of care and prescriptions can be sent before the patient leaves the office.

ePrescribing provides:

+ Significant patient safety advantages, including the ability to check for drug allergies or whether a prescription may interact with another medication.
+ Access to information that allows for review of medication efficacy and dosage adherence.
+ Access to the Cigna Healthcare drug list.
+ Administrative efficiencies by eliminating the need for handwritten prescriptions or those via phone or fax as well as subsequent calls to clarify handwritten prescriptions or renew a prescription.

Online training and resources

We offer online learning materials about our electronic capabilities, timely health care topics, and other important information. You can view or download the materials at your convenience.

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The **Behavioral Education and Training materials** are available on the Evernorth® Provider website (**Provider.Evernorth.com**).

Cultural competency and health equity

As the population of the United States continues to diversify, you may encounter patients with health beliefs and practices that are unfamiliar to you. This could affect your ability to establish trust, which may reduce adherence to treatment plans.

Gaining cultural competency skills, understanding health disparities, and the impact of nonmedical needs, such as social determinants of health, are integral aspects of delivering quality health care. This includes seeking to understand your patients' concept of health, intrinsic motivation, communication preferences, health care beliefs, and behaviors. This foundational knowledge enables you to customize treatment plans that align with your patients' social, cultural, and linguistic needs. This can help to build trust and rapport with your patients, increasing the likelihood of adherence and positive health outcomes.

We offer **Cultural Competency and Health Equity Resources** that can help you deliver culturally responsive care, creating an optimal experience for you, your staff, and your patients. These resources are available at no cost and include:

+ Articles, training, and provider and patient educational videos for specific subpopulations.
+ Comprehensive guide for identifying, documenting, and addressing social determinants of health within your practice.
+ Flyers to assist patients with accessing free or low-cost community resources.
+ Language assistance services, which includes discounts available to contracted providers.

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Credentialing and recredentialing

Credentialing for providers

Providers are credentialed before participating in the Evernorth® Behavioral Health (Evernorth) network and are recredentialed periodically thereafter to help ensure they continue to meet our qualifications for participation. Criteria for participation are determined by business needs and our credentialing policies and procedures, which are reviewed annually to reflect National Committee for Quality Assurance, local, and state standards.

Council for Affordable Quality Healthcare credentialing

After the initial onboarding process is complete, you will be prompted to complete the Council for Affordable Quality Healthcare® (CAQH®) application at **ProView.CAQH.org**. If you have questions about completing the application, contact the CAQH help desk at **888.599.1771** or **CAQH.UPDHelp@ACSGS.com**.

CAQH is a nonprofit alliance of managed care plans, physician-hospital organizations, and trade organizations. CAQH recognizes the need to simplify administrative requirements and allow you to focus on caring for patients. Improving processes for obtaining and managing data is a key factor in saving time. Working with health care delivery systems and various technical and software specialists, CAQH sponsors the Universal Provider Datasource initiative.

The CAQH Provider Data Portal, developed by managed care organizations with help from physicians, professional associations, and accreditation organizations, allows providers to complete one credentialing application by entering confidential information into a secure database that is shared, with your approval, with participating health plans and organizations. Providers complete the basic information only once and updates are made online or by fax. There is no charge to submit information to the CAQH credentialing database, and CAQH contacts providers regularly to ensure the information is complete. Some states mandate the use of the CAQH application, and we strongly encourages its use when submitting your application.

For more information about the CAQH Provider Data Portal, visit **ProView.CAQH.org**.

Credentialing process for providers

The credentialing process includes a review of the standard application and independent verification of certain documentation submitted. The information submitted must be accurate, current, and complete. You must not make any material misrepresentations in the information provided during your contractual relationship with Evernorth, including medical record information. In addition, you must continue to satisfy all requirements.

Credentialing requirements include, but are not limited to, the following:

+ A completed, signed, and dated application (dated within 180 days). Correction liquid must not be used in the signature area of paper applications. Applications with altered signatures will not be processed.
+ A completed, signed, and dated authorization and release form, if not included in the application form.
+ A completed, signed, and dated Provider Agreement (two originals), copy of a completed provider data sheet, copy of a completed W-9, and copy of Form CMS-1500 with box 33 completed (if not included on provider data sheet).
+ Documented work history in month/year format (within the last five years). Gaps exceeding six months must be explained in writing.
+ A current unrestricted license to practice medicine in the state where practicing.
+ A current unrestricted Drug Enforcement Agency (DEA) certificate (if applicable) for each state where the provider cares for patients.

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- A current unrestricted Controlled Dangerous Substances (CDS) certificate (if applicable).
- Board certification (if applicable) in a recognized specialty by the American Board of Medical Specialties or American Osteopathic Association.
- Unrestricted admitting privileges at at least one hospital, depending on the network in which you would like to participate and whether that hospital participates in that network. Exceptions may be granted (e.g., an applicant's specialty does not typically require admitting privileges, satisfactory alternative mechanism has been established as determined by Evernorth).
- Professional liability insurance with typical minimum coverage of $1 million per occurrence and $3 million aggregate unless a lower amount is mandated by the state where the provider cares for patients.
- Acceptable history of professional liability claim experience as determined by Evernorth.
- Completed professional liability information (with explanation of each case).
- Acceptable history of Medicare/Medicaid sanctions as determined by Evernorth.
- Acceptable responses to all questions on the credentialing application form as determined by Evernorth.
- Acceptable report from the National Practitioner Data Bank as determined by Evernorth.
- An acceptable history relative to all types of disciplinary action by any hospital and health care institution and any licensing, regulatory, or other professional organization (for each state where the provider cares for patients).

Provider rights

You have certain rights during the credentialing process, including the right to:

- Review the information submitted to support the credentialing application. Information from outside sources (e.g., licensing boards) will be made available for review.[1] You may exercise this right by contacting Provider Services at **800.926.2273** or by emailing **BehavioralProviderRecruitment@Evernorth.com**.
- Correct erroneous information.[1] When erroneous information is present, a representative from our Credentialing department will notify you in writing of the discrepancy. Corrections should be sent to the Credentialing department in writing within 15 business days at the location noted on the request. All responses are recorded with a date of receipt and maintained as part of your credentialing file.
- Receive the status of your credentialing or recredentialing application, upon request.[1] You may contact Provider Services at **800.926.2273** or email **BehavioralProviderRecruitment@Evernorth.com**.

The decision to accept or deny participation will be communicated in writing within 60 days of the decision.

All specific provision requirements for delegated credentialing are contained within the Evernorth Standards of Delegation document. Evernorth will evaluate certain exceptions to its credentialing criteria on a case-by-case basis.

1. References, recommendations, and other peer-review protected information will not be shared. All state and federal guidelines are also adhered to, where applicable.

Recredentialing process for providers

We recredential in-network providers once every three years or more often if required by state law. If you have not applied through the CAQH Provider Data Portal, you will be mailed a recredentialing letter approximately six months before your recredentialing date. The letter will direct you to complete the CAQH Provider Data Portal credentialing form.

If you already completed and updated the CAQH application and attestation and authorized Evernorth to receive current credentialing information, we will automatically have access to your application during the recredentialing process and

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will only contact you if needed. If you use a state-mandated form outside of CAQH, you must update any information that has changed, sign the attestation, and submit the application along with current supporting documents.

During the recredentialing process, completed applications are reviewed and certain new information is independently verified.

Recredentialing requirements include, but are not limited to, the following:

+ A completed, signed, and dated application (dated within 180 days). Correction liquid cannot be used in the signature area of paper applications. Applications with altered signatures will not be processed.
+ A completed, signed, and dated authorization and release form, if not included in the application form.
+ A current unrestricted license to practice medicine in the state.
+ Current DEA certificate number (if applicable).
+ Status of current board certification (if applicable).
+ A record of adequate education and board certification for any new specialty for which you request credentialing.
+ Adequate admitting privileges at at least one hospital, depending on the network in which you are requesting to participate and whether the hospital also participates in that network. Exceptions may be granted (e.g., a provider's specialty does not typically require admitting privileges, a satisfactory alternative mechanism has been established as determined by Evernorth).
+ Professional liability insurance with typical minimum coverage of $1 million per occurrence and $3 million aggregate unless a lower amount is mandated by the state where the provider cares for patients.
+ An acceptable history of professional liability claim experience as determined by Evernorth.
+ Completed professional liability form with an explanation of each case for paper applications.
+ A written explanation relevant to professional liability and practice review questions.
+ Acceptable history of Medicare/Medicaid sanctions as determined by Evernorth.
+ Acceptable results from the National Practitioner Data Bank as determined by Evernorth.
+ Acceptable responses to all questions on the credentialing application form as determined by Evernorth.
+ A signed, dated, and completed professional liability form.
+ A copy of current professional liability face sheet.
+ An acceptable history relative to all types of disciplinary action by any hospital and health care institution and any licensing, regulatory, or other professional organization (for each state where the provider cares to patients).

Notice of changes of licenses or accreditation
Providers are responsible for notifying Evernorth immediately of any changes to the information presented as part of the credentialing or recredentialing process. Failure to notify Evernorth of changes or to satisfy requirements may result in claims issues or your removal from the network.

Credentialing for facilities

Facilities are credentialed before participating in the Evernorth® Behavioral Health (Evernorth) network to help ensure they meet our quality standards for participation and to comply with accreditation requirements.

In-network facilities must maintain an ongoing quality improvement program that monitors and evaluates the quality and appropriateness of patient care, pursues improvement opportunities, and resolves problems. Accrediting organizations, such as The Joint Commission, the Association of Academic Health Centers, the Commission on Accreditation of Rehabilitation Facilities, the American Osteopathic Association, Community Health Accreditation Partner, and the Council on Accreditation, DNV, validate a quality improvement program. When accreditation, state

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Department of Health, or Medicare certification evidence is not available, we may perform a site visit and review of the hospital's or ancillary facility's quality improvement program.

Credentialing process for facilities

In accordance with our national credentialing requirements, facilities must apply for participation by completing a standard application and satisfactorily meeting the established criteria. Our credentialing and recredentialing policies and procedures are reviewed at least annually and revised as necessary, including revisions to reflect state and local quality assurance standards.

Facility credentialing requirements include, but are not limited to, the following:

+ A copy of an unrestricted state license or state operating certificate, as applicable.
+ A copy of a current accreditation letter or certificate.
+ If not accredited, a copy of the most recent Centers for Medicare & Medicaid Services evaluation or an on-site assessment completed by the state.
+ Proof of professional and general liability insurance that meets our minimum credentialing requirements.
    - **Hospital/residential setting:** $1 million per occurrence and $3 million aggregate.
    - **Partial hospitalization and intensive outpatient programs:** $1 million per occurrence and $1 million aggregate.
+ National Provider Identifier.
+ Any explanation requested on the application, including a list of malpractice settlements and judgments.
+ A copy of the quality management plan, if not accredited or Medicare and Medicaid certified.
+ A list of available services rendered by location, population, and type of care.
+ Absence of current sanctions from Medicaid or Medicare.

If an ancillary facility is not subject to state licensure requirements, our credentialing committee will determine if the facility meets remaining credentialing standards for participation in the network.

Recredentialing requirements for facilities

We recredential in-network facilities every three years or more frequently if required by applicable law. Our credentialing staff will confirm that the facility continues to be in good standing with state and federal regulatory bodies and, if applicable, is reviewed and approved by an accrediting body.

Facilities are responsible for notifying Evernorth immediately of any material changes to the information presented at the time of their prior credentialing or recredentialing cycle. Failure to notify Evernorth of changes or to satisfy requirements may result in termination from the network. Recredentialing and continued participation in the network are dependent upon the facility continuing to meet our credentialing and recredentialing standards.

Credentialing for ancillary providers

Note: This section is only relevant to organizations contracted under an ancillary provider agreement.

Ancillary providers need to complete our **Ancillary Provider Credentialing Attestation Form** and submit the completed attestation to **AncillaryProviders@Evernorth.com** on an annual basis.

Additionally, contracted ancillary providers must submit a monthly provider count by state for each state where they render services under an ancillary agreement. The information should be submitted to

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AncillaryProviders@Evernorth.com by the first day of every month. This process ensures Evernorth® Behavioral Health receives updated information concerning the composition and growth of providers rendering services under the ancillary agreement.

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Provider participation

In our role as a health service company, we contract with psychiatrists, nurse practitioners, physician assistants, psychologists, master-level clinicians, clinics, and facilities. In most situations, our customers expect to receive care from providers who participate in the Evernorth® Behavioral Health (Evernorth) network to maximize their benefits, even when their provider refers them elsewhere.

As part of your contract with Evernorth, you agree to refer your patients to other in-network providers, hospitals, and facilities. Naturally, there are some exceptions, for example, in an emergency or if services cannot be provided within the network. However, we have made significant investments in online tools, smartphone apps, and customer service to help our customers make informed decisions about their care and costs so they can "know before they owe." It is our expectation that you will partner with your patients to help them maximize their benefits by referring additional care to other in-network providers.

As an in-network provider, you must provide services with the same standard of care, skill, and diligence customarily used by similar providers in your community; follow the requirements of applicable law; and meet the standards of applicable accreditation organizations. All services that are provided within the scope of your practice or license must be provided on an in-network basis under the terms of your Provider Agreement, regardless of your physical location.

You are expected to provide services to patients in the same manner, in accordance with the same standards, and with the same time availability (i.e., wait time for an appointment) as offered to other patients. You must provide services to all patients consistent with the benefits covered in their benefit plan, without regard to race, ethnicity, national origin, religion, sex, age, mental or physical disability or medical condition, sexual orientation, claims experience, medical history, evidence of insurability (including conditions arising out of acts of domestic violence), genetic information, source of payment, or any other bases deemed unlawful under federal, state, or local law.

Further, you must meet our credentialing standards for training, licensure, and performance before joining the network. You will also be evaluated periodically to help ensure continued qualification. Performance requirements include providing quality services to patients and cooperating with our administrative, quality, and utilization management programs. We evaluate performance data for quality improvement activities, preferred status designation in our network, and reduced customer cost-sharing, as applicable.

Service standards and requirements

Your patients expect quality health care services. You can assist us in maintaining access to quality service by adhering to the following standards and requirements. Compliance with these standards may be monitored through site visits, medical record reviews, and patient surveys. For more detail, refer to the **Quality Management Program section**.

Acceptance and transfer of patients

You should not refuse or fail to provide services to any patient unless you are incapable of providing the necessary services. You are expected to provide services to patients in the same manner, in accordance with the same standards, and with the same time availability (i.e., wait time for an appointment) as provided to other patients.

Code of conduct

As an in-network provider, you acknowledge that you understand and will abide by the code of conduct. This includes maintaining professionalism with all interpersonal communication, including, but not limited to, voicemails, text messages, social media posts, emails, peer-to-peer discussions, patient interactions, faxes, and refraining from the use

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of profanity, intimidation, threats of physical harm, harassment, or disparaging or offensive comments. Providers who violate these standards may face consequences for their actions, including termination from the network.

Communication to patients of professional termination

If your participation with Evernorth® Behavioral Health (Evernorth) is terminated entirely or with respect to any of our benefit plan types, we will notify affected patients of the termination to the extent required by applicable law and applicable accrediting requirements. Such notification will occur before the effective date of the termination unless we do not receive sufficient advance notice. In this instance, we will notify affected patients to the extent required as soon as reasonably possible. Upon request, you are responsible for providing a listing of patients affected by your termination within seven business days of the date of the notice of termination.

In the event you decide to send any written communication to patients about your Evernorth participation or use our name in such manner, we reserve the right to review and approve the communication prior to release. You may not make any disparaging comments about Evernorth or its affiliates or any misrepresentation in communications about your participation during your contractual relationship with Evernorth. Refer to your Provider Agreement for more information.

In the event of suspension, additional referrals to the provider are halted, and, depending on the reason for suspension, we may reassign the provider's current patients. In those states where there are laws regulating the appeal process, the state law supersedes this procedure.

Provider specialties

We ask providers to identify their specialty areas of focus and the populations they treat, the modalities of treatment utilized, and optional information about themselves (e.g., nationality, language spoken, gender identity) so that we can help our customers with locating a provider to assist with their care.

Our staff utilizes a variety of means to ensure that the specialties listed in a provider's record are accurate and licensure is verified. The verification process will be completed during initial credentialing, when the provider requests to add a specialty to their profile, and during the recredentialing process. There are currently three different groups of specialties: self-reported specialties, attested specialties, and verified specialties.

A **self-reported specialty** is one that the provider can list as an area of practice based on their own selection. Self-reported specialties include general areas of behavioral health treatment for which providers have received training as a licensed behavioral health provider, such as depression, anxiety, couples counseling, anger management, grief/loss, LGBTQ+ health, domestic violence, and faith-based counseling.

An **attested specialty** is one that the provider can list as an area of practice based on education and experience, but it does not require additional documentation. Attested specialties include sexual disorders/offenders, autism, employee assistance program specialty services (critical incident response [CIR], certified employee assistance professional, employee education seminars, management referrals, and supervisor training), developmental disorders, dialectical behavior therapy, dual diagnosis, eating disorders, eye movement desensitization and reprocessing, exposure and response prevention therapy, maternal mental health, medical illness (e.g., cancer/oncology), neuropsychological testing, and pain management.

A **verified specialty** is one that the provider can substantiate their ability to provide specialty services by presenting documentation, such as specific licensure or certification. Verified specialties include addictionology, alcohol/substance use (certified), behavioral pediatricians, medication-assisted treatment (e.g., Suboxone® or Vivitrol®), medication management, substance abuse professional, and substance abuse expert.

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Providers must meet certain criteria for an attested or verified specialty to be added to their profile. If at any point Evernorth has been made aware that a provider no longer meets the criteria for an attested or verified specialty, the specialty will be removed from their profile. This will not affect the provider's network status or payment for services. However, we will only refer customers who need a specialty service to providers who have that specialty listed in their profile.

If you wish to add a verified or attested specialty to your profile, please submit the **Verified Specialty** or **Attested Specialty** form. To add specialties that are not included on these forms (e.g., self-reported specialties), please contact Provider Services at **800.926.2273**.

Urgent access

We are committed to providing excellent service and quality care to our customers. Our clinical operations and provider relations staff have worked together to better understand crisis stabilization services. As a result, we have defined crisis stabilization services to provide different access levels based on the customer's needs.

Crisis Stabilization Unit

A Crisis Stabilization Unit provides short-term care, typically one to seven days, for those experiencing a mental health emergency. The purpose of the unit is to prevent further decompensation that would result in an inpatient admission.

The use of a Crisis Stabilization Unit may apply in cases where one exists outside of an accredited hospital but where supervised and monitored services are available 24 hours a day, seven days a week. The facility setting for a crisis stabilization bed is within a unit that provides around-the-clock nursing or mental health staff supervision and continuous observation and control of behaviors to ensure the safety of the individual and others.

A Crisis Stabilization Unit provides:

+ Evaluation and intervention for individuals with acute symptoms of a mental health or substance use disorder when the clinical presentation does not immediately indicate the need for a higher level of care.
+ Supervised and monitored services 24 hours a day, seven days a week.
+ Psychiatric consultation.
+ Evaluation of family and social support systems that identify both opportunities and challenges and a plan to address the challenges.
+ Linkage and referrals to long-term services/community services.
+ An individualized treatment plan.
+ Medication management in some situations.
+ Individual and group and family counseling in some situations.
+ A complete medical evaluation and basic medical procedures as indicated.

Note: When medical services are not available on-site, the program must be able to ensure that the individual will be linked to appropriate treatment and providers within a reasonable time frame.

Crisis stabilization appointments

The crisis stabilization network consists of outpatient providers who are available for emergent referrals, offering active intervention within six hours for patients in need. Our crisis stabilization network is available 24 hours a day, seven days a week, and our clinical operations staff can easily identify crisis stabilization providers for referrals.

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During a crisis stabilization appointment, a licensed behavioral health care provider facilitates active treatment intervention and is able to assess, stabilize, and proactively identify the most appropriate level of care for an individual with an urgent or emergent need.

The goals of the intervention at this level of care are:

+ Reduction of acute symptoms due to a mental health or substance use disorder.
+ Reduction of potential for harm to self or others.
+ Identification and mobilization of available resources, including support networks.
+ Intervention focused on factors relevant to the crisis.

Appropriate interventions include assessment of support networks, identification and assessment of available services, mobilization of those services, and an estimate of the individual's ability to access services and participate in the treatment plan.

Note: This level of care is not appropriate for an individual who, by clinical presentation or history, requires the intensive structure of acute inpatient treatment for safety and stabilization.

Intermediate care

Intermediate care is a level of intervention that provides precautionary and preventive care to a patient who presents with a level of acuity that, if not addressed within 48 hours, could escalate to a higher level of care. The goal of our intermediate care network is to successfully link complex cases with providers who have the clinical expertise to manage specific diagnoses/problems for ongoing treatment. It is expected that the intervention be conducted by a licensed mental health provider who can assess, stabilize, and proactively identify the most appropriate level of care for the patient at that time. The intervention should include the following components:

+ A comprehensive psychiatric and medical history.
+ A description of the nature of the patient's impairments and the nature of any safety or risk issues (suicidal ideation, homicidal ideation).
+ A comprehensive evaluation of substance use or chemical dependency issues.
+ A psychosocial evaluation, including a systems analysis of the patient's family and support network and identification of any barriers for a successful treatment plan.
+ Documentation of any current treatment providers and a description of the services provided and medications, if any (including names, dosages, and frequencies).

Providers offering intermediate care will:

+ Be able to intensify treatment plans as needed for their patients to prevent unnecessary hospitalization as clinically indicated.
+ Develop short-term, evidence-based treatment plans incorporating a systems approach.
+ Include effective collaboration between Evernorth® Behavioral Health (Evernorth), psychiatrists, primary care providers, and other community resources in their treatment plan prior to requesting a higher level of care.

If the acuity of the patient's condition does not allow for prior authorization of coverage, contact Evernorth as soon as possible. Please be prepared to provide the following information to our clinical staff:

+ Patient's name, age, and ID number.
+ History, diagnosis, indications, and nature of the immediate crisis.
+ Alternative treatment provided or considered.

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+ Treatment goals, estimated length of stay, and discharge plans.

Meet-and-greet appointments

A meet-and-greet appointment is a predischarge visit conducted by an in-network non-MD provider for the purpose of coordinating and scheduling the ambulatory follow-up appointment for two to seven days after discharge. The provider will enter the facility as a visitor; therefore, they do not need to be credentialed with the facility. Please note that the meet-and-greet service is usually performed during visiting hours, but scheduling may vary by facility.

A meet-and-greet appointment is a service initiated by Evernorth® Behavioral Health (Evernorth) for which prior authorization must be obtained from an Evernorth care manager or care coordinator. Claims should be submitted with Current Procedural Terminology code 99499 (unlisted evaluation/management services). The claim will be processed as long as there is a comment in the inpatient authorization and the provider includes a written description of the service provided (e.g., “predischarge consultation,” “meet and greet”). Without a written description, the claim payment will be denied due to lack of information. A copay for this service depends on the patient’s benefit plan.

Behavioral virtual services

Offering virtual services to patients gives you the potential to broaden the scope of your practice, reach patients in a larger geographical area, expand access to behavioral health care, and reduce your time on the road. Virtual therapy, medication management, and intensive outpatient programs are typically available to patients nationwide and related claims will be administered in accordance with the patient’s benefit plan.

Providers should utilize the Current Procedural Terminology code that reflects the services rendered and add modifier 95 to indicate that it was a virtual service. This will not change the reimbursement; however, Evernorth® Behavioral Health (Evernorth) needs to monitor how all services are delivered.

Providers must meet all state requirements to provide virtual behavioral services, including any licenses and certifications. Providers must also comply with their state laws about the use of audio or video-based technology and be aware of relevant in-person and virtual practice guidelines. Providers are expected to follow all regulatory and licensure requirements related to their scope of practice and any limitations on the use of specific technologies and prescribing practices. They must also abide by state board and specialty training requirements.

Providers who meet the telehealth specialty requirements may deliver services virtually with no additional credentialing. To provide virtual sessions through Evernorth, please attest to meeting the designated specialty requirements on the **Attested Specialty Form**. Upon receipt and approval by Evernorth of the completed form, “telehealth” will be added as a specialty to your profile.

Forms

You can use the following forms to add various specialties and services to your profile.

+ **Attestation for Participation**
+ **Attested Specialty Form**
+ **Verified Specialty Form**

Appointment access standards

Access to behavioral health care is a critical measure of our mission to deliver high-quality, cost-effective health care for our customers.

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Providers should make every effort to see patients within the time frames listed below. **Note:** State and/or federal requirements will override these standards, when applicable. For state-specific access-to-care requirements, please see the regulatory addenda at the end of this document.

+ **Post-discharge from inpatient care:** within seven calendar days.
+ **Life-threatening emergency:** immediate.
+ **Non-life-threatening emergency:** within six hours.
+ **Urgent care:** as soon as possible but not to exceed 48 hours.
+ **Initial visit for routine care:** within 10 business days.
+ **Follow-up visit for routine care:** within 30 calendar days.
+ **Initial visit for employee assistance program participant:** within two business days.
+ **Office wait time:** 15 minutes or less.

Providers who are unable to meet the appointment time frames should immediately refer their patients to our **provider directory** or contact Evernorth® Behavioral Health through the mental health and substance use phone number on the patient's ID card for alternative referral. (Please refer to the **urgent access section**.)

Phone accessibility and response

There must be a publicized phone number for patients to call, and calls must be answered promptly by a properly trained person to respond to those of a routine, urgent, or emergent nature.

When it is necessary to place callers on hold, callers should be asked if they can hold and should only be placed on hold after giving an affirmative response. Callers who do not wish to hold should have their calls handled as appropriate. If the phone is answered by an answering machine, the message must give emergency instructions that clearly explain how to get urgent or emergency care and, when appropriate, how to contact another provider who is on call for triage and screening services or, if needed, to give urgent or emergency care.

General referrals to emergency department settings for all access standards other than non-life-threatening and life-threatening emergencies are not considered to be evidence of appropriate crisis coverage. It is also expected that your outward greeting includes your message response time frame (e.g., all calls will be returned within 24 to 48 hours).

Professional services

All services must be provided or overseen by licensed or certified professional personnel and at facilities that comply with generally accepted professional and ethical standards, community standards, state and federal law, and accreditation organization standards.

We allow providers to bill for professional services, which are rendered by supervisees, as permitted by state licensing regulations. If you engage in supervisor (or supervisee) billing, the fully credentialed provider should be listed on the claim as the rendering provider. This practice does not replace the need for fully licensed providers to follow Evernorth® Behavioral Health credentialing standards.

Cooperation with programs

We are committed to promoting access to quality services for your patients. To support this commitment, we require your cooperation with our programs, including administrative (e.g., claim appeals), wellness, and medical management programs.

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Cooperation in establishing and implementing policies and programs to comply with regulatory, contractual, or certification requirements of the Centers for Medicare & Medicaid Services (CMS), the National Committee for Quality Assurance, Healthcare Effectiveness Data and Information Set (HEDIS®),[1] and any other applicable accreditation organization is equally important.

1. HEDIS® is a registered trademark of the National Committee for Quality Assurance (NCQA).

Risk adjustment program

Cigna Healthcare® Individual & Family Plans (IFPs) must comply with the Affordable Care Act's (ACA's) risk adjustment program, which aims to prevent adverse selection and promote an equitable environment in the individual health care marketplace. Risk adjustment is a government program associated with the ACA and administered by CMS.

CMS requires payers with IFPs to report patient diagnoses annually via claims and through medical record diagnoses abstraction (coding). Providers may be asked to provide patient medical records in support of the risk adjustment program or in response to the mandatory Risk Adjustment Data Validation (RADV) audit, which occurs every year. RADV is an audit of Cigna Healthcare–reported data only.

If you receive a request to submit a patient medical record as part of a risk adjustment request or RADV audit, please note the following:

+ Requests are not an audit of provider work, claims, or financial agreements.
+ Subjective therapeutic notes are not required. Sensitive data can be omitted, and, in many cases, a basic Subjective, Objective, Assessment and Plan note (minus the subjective section) will suffice. Please ensure the diagnoses and treatment plan are authenticated with the provider's signature, credentials, and date.
+ Requests for medical records may be made two times annually and will recur annually.

If you have any questions about the risk adjustment program or medical record review process, please visit the **Individual & Family Plans page** or contact the IFP Risk Adjustment Operations team at **CignaHealthcareHCPEducation@CignaHealthcare.com**.

Referrals

Occasionally, it may be necessary for a provider to refer a patient outside of their practice. A provider should search the provider directory or contact an Evernorth® Behavioral Health (Evernorth) case manager for referral to an appropriate in-network provider.

Patients whose benefit plans are administered by Evernorth generally expect that when they choose to seek care from an in-network provider, charges for all related care will be processed at the in-network benefit level. For this to occur, the patient must be referred to in-network providers, including other providers, laboratories, or facilities. When a patient is referred to an out-of-network provider, the patient may incur unexpected financial liability. Patients whose plans include out-of-network benefits can use these benefits for services covered under their plan; however, in doing so, these patients will generally incur higher out-of-pocket costs. To ensure that patients are making informed choices when accessing care, the referring provider must fully disclose the financial effect of referrals to in- and out-of-network providers under their benefit plan, including the referring provider's financial interests, if any.

Medication

Nonprescribing providers who believe medication should be considered can search the provider directory or contact an Evernorth® Behavioral Health (Evernorth) case manager for referral to an appropriate Evernorth provider. The provider can offer the patient the name and phone number of an appropriate prescriber. The patient may then contact the

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prescriber for an appointment. When medication is prescribed, patients should have prescriptions filled at an authorized pharmacy.

Patient billing

Copay: A copay is a fixed dollar amount that a patient pays per service. Copay amounts are printed on the patient's ID card and should be collected at the time of service.

Coinsurance and deductible: A deductible is the amount a patient pays for eligible services before their health plan begins to share in the cost of covered services. Coinsurance is the percentage of covered charges that the patient pays for after they have satisfied their deductible. For patients with plans that have a deductible or coinsurance, you should submit claims to the payer or its designee and receive an explanation of payment indicating the patient responsibility before billing patients. Coinsurance and deductible amounts should not be collected at the time of service.

Most patients with a Cigna Choice Fund® plan have automatic claim forwarding (ACF) enabled so the deductible and coinsurance amounts they owe are paid directly out of their health care account. After claim processing, if funds are available, Cigna Healthcare® automatically sends payment to you on behalf of the patient, usually along with the Cigna Healthcare portion of the payment. ACF is chosen by the employer and is currently active for most of our customers with Choice Fund plans.

Please note for patients with Choice Fund plans, the ACF feature is chosen by the patient and is now referred to as AutoPay by our vendor, HSA Bank. Most of the time, ACF is not a chosen option like it is with our health reimbursement account plans.

Fee-forgiving/waiver of copay, coinsurance, or deductible: Most benefit plans exclude from the patient's coverage those charges for which they are not obligated to pay. Therefore, if a patient is not obligated to pay a charge, any claim for reimbursement for any part of that charge under such a contract or benefit plan is generally not covered. It is our view that "fee-forgiving" on any particular claim, or any portion thereof, could constitute fraud and may subject a provider to civil and criminal liability.

Patient incentives: Providers shall not directly or indirectly establish, arrange, encourage, participate in, or offer any patient incentives. Providers include hospitals, ancillary services, health care facilities, individual and group practitioners, and all other entities delivering covered health care services to patients.

Patient incentive means any arrangement by a provider that:

1. Reduces or satisfies a patient's cost-sharing obligations (including, but not limited to, waiving copay, deductibles, or coinsurance or offering cash price discounts or prompt pay discounts).
2. Pays on behalf of or reimburses a patient for any portion of the patient's costs for coverage (e.g., insurance premiums) under a policy or plan insured or administered by Evernorth Behavioral Health or an affiliate.
3. Provides a patient with any form of material or financial incentive (other than the reimbursement terms under the Provider Agreement) to receive covered services from the provider or their affiliates.

In the event of noncompliance with this provision:

1. We may terminate the provider's participation agreement, as noncompliance is a "material breach" of the agreement.
2. The provider shall not be entitled to reimbursement under their agreement with respect to covered services provided to a patient in connection with a patient incentive.
3. We may take such other action appropriate to enforce this provision.

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Denied payment and patient liability

You cannot bill patients for covered services or for services for which payment was denied due to your failure to comply with your Provider Agreement, these Administrative Guidelines, and/or other program requirements, including utilization management requirements and timely filing requirements.

Self-pay agreements

There may be times when having a patient sign a self-pay agreement is appropriate, such as when services are not covered by a patient's benefit plan. Services not covered by Evernorth® Behavioral Health (Evernorth) include, but are not limited to:

+ Late appointment cancellations.
+ Court-ordered treatment that is outside the scope of routine outpatient care and is determined by Evernorth to be not medically necessary.
+ Missed employee assistance program appointments.
+ Services for which the patient elects not to use their benefit plan.

The provider must obtain the patient's written approval in the form of a self-pay agreement, including a full financial disclosure, for any services that the plan denied or that are not covered services in advance of services being rendered. The approved **Self-Pay Agreement** is available in the **Forms Center** on the Evernorth Provider website (**Provider.Evernorth.com**).

The provider may use a self-pay agreement of their own design; however, all data elements as described in the self-pay agreement must be contained therein. Self-pay agreements signed by the patient either at the time of admission to the facility or at the start of outpatient treatment that reference the possibility of a self-pay arrangement in the future will not be accepted as proof of a self-pay agreement. In these circumstances, the patient must be financially held harmless as per the terms of the Provider Agreement.

The self-pay agreement must include statements acknowledging the patient is aware of the Evernorth appeal process and has elected not to appeal at that time and is aware the agreement only applies to the specific level of care or services they are requesting and that authorization must be obtained or another self-pay agreement must be signed if they move to a different level of care.

The self-pay agreement is in effect only from the date the patient signs it until it is rescinded; the agreement may never be retroactive.

Provider concerns related to administrative processes

We resolve concerns related to administrative issues separate from the administrative or clinical treatment denial and appeal process. For administrative concerns, please contact Evernorth as follows:

+ **Appeals of administrative denials:** Call the phone number listed in the denial letter.
+ **Claims:** Call Provider Services at **800.926.2273**, unless you are a provider practicing in California. Providers in California should refer to the **Contact Information (Appendix A) page**.
+ **Fee schedule–related issues/contract questions:** Call Provider Services at **800.926.2273** or the appropriate center listed on the **Contact Information (Appendix A) page**.

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Provider concerns related to patient reviews

If you would like to reply to an online patient review, request an appeal to one, or report one as being inappropriate or inaccurate, you can log in to the Evernorth® Provider portal (**Provider.Evernorth.com**) and go to Resources > Behavioral Administrative Guidelines or call the Patient Reviews team at **888.535.0873**, Monday through Friday from 8:00 a.m. to 5:00 p.m. PT.

A member of our team will investigate the concern and determine whether the flag is warranted. This process generally takes between one and three business days. If the review is accepted as inappropriate or inaccurate, it will be removed immediately. If it is determined to be appropriate and accurate, the review will remain in the directory and the provider will be given the opportunity to respond to it directly.

There may be instances when a review comment is suppressed, but the overall recommendation remains valid. For example, if profanity is used, the entire comment will be removed but the rest of the review will be published.

Providers who continue to be dissatisfied may pursue arbitration as outlined in the Dispute Resolution Procedure section in the Provider Agreement.

Resolving patient complaints

If a patient complains to Evernorth® Behavioral Health (Evernorth) about some aspect of a provider's care, the provider is required to participate in the internal Evernorth complaint resolution process. An Evernorth provider relations representative will outreach the provider via phone or email to address the concern. Once the concern has been discussed, the provider relations representative will either inform the provider that the concern has been closed or that there is further action that needs to be taken to resolve the issue. The action will be outlined in a written communication. If, for any reason, the concern is not addressed in a timely manner or resolved, the provider relations representative may take further action as outlined in the Provider Agreement.

If a patient complains to the provider about an administrative issue, the patient should be directed to call the phone number listed on the patient's ID card. For patients residing in California, the appropriate number is listed on the **Contact Information (Appendix A) page**.

Confidentiality

We maintain strict policies to protect confidential information. As an in-network provider, you are responsible for maintaining the confidentiality of patient information in all settings in accordance with federal and state laws. Written policies and procedures should be established that include the designation, maintenance, release, and control of access to confidential records.

If you have questions or comments about plan policies, call Provider Services at **800.926.2273**.

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Eligibility

Determining eligibility

It is important to determine patient eligibility prior to rendering services. We recommend verifying a patient's eligibility prior to their appointment date. Patients are responsible for presenting their ID card or enrollment form (if they are awaiting receipt of an ID card) as proof of coverage.

Eligibility verification

In addition to viewing your patient's ID card, you should verify eligibility by:

+ Logging in to the Evernorth® Provider portal (**Provider.Evernorth.com**) and going to Patients > Search Patients.
+ Submitting an eligibility benefit inquiry (270/271) through your electronic data interchange (EDI) vendor.
+ Using our interactive voice response system.
+ Contacting Provider Services at **800.926.2273**.

When verifying eligibility and benefit information through the portal or your EDI vendor, you can receive:

+ Eligibility status (active, inactive, not covered).
+ Coverage effective and term dates.
+ Patient insurance and plan types (e.g., Preferred Provider Organization, Health Maintenance Organization [HMO], or Cigna Choice Fund® health reimbursement account).
+ Plan-level copay, coinsurance, deductible, and accumulator amounts.
+ Benefit-specific copay, coinsurance, and deductible amounts.
+ An indicator of different benefits for in network and out of network.
+ The HMO code, network ID, and line of business (e.g., 018, VA085, Flex) for patients covered by managed care plans.
+ Personal Health Solutions, Personal Health Solutions Plus, Health Matters Care Management Complete, and Health Matters Care Management Preferred medical management identification.
+ Coordination of benefits information (Medicare Part A, Medicare Part B, or other).
+ Primary care provider, if one has been selected.
+ Visit limits, including remaining visits.
+ Benefits that are part of a tiered or client-specific network.

You can also use **Provider.Evernorth.com** to view important information about a patient's eligibility, including:

+ A copy of your patient's ID card.
+ Subscriber information.
+ Initial coverage date, current coverage date, and plan renewal information.
+ Employer details, plan funding type, and if there is other insurance.
+ Information about prior coverage, if it has terminated.
+ Benefits by procedure code or general benefits. **Note:** You can save the details of your searches for up to two years by generating a benefit reference number.
+ Detailed information on benefit types, including medical and dental preventive care, vision, maternity, infertility, allergy injections, well-child care, and inpatient and outpatient psychiatric services.
+ Available services, including chiropractic care, durable medical equipment, external prosthetics, home health care, short-term rehabilitation, and magnetic resonance imaging.

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In addition, you also have access to the “Am I in-network for this patient?” tool on **Provider.Evernorth.com** to verify if you are in network for a patient and understand what level of benefits will apply.

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Case management program

Our case management program philosophy is to marshal resources and advocate for your patients, with a goal of returning patients to the highest possible level of functioning as soon as clinically indicated.

The Clinical Case Management team works with treating providers to determine whether a procedure, treatment, or service meets medical necessity criteria and to provide information about the patient's coverage.

Our case managers focus on the whole person so that they can have the greatest positive impact on the clinical outcome, ensuring, through utilization management activities, that continued authorization is based on the appropriateness of care provided. The case manager also closely collaborates with our triage clinicians, care coordinators, and personal advocates, who are all an important part of the patient's care. Designated case management interventions are designed to add value to each case, and the case manager manages each case with varying levels of appropriate clinical intensity.

Prior authorization protocol

Our prior authorization program helps you determine if a patient's care will be covered under their benefit plan.

In an effort to support accurate coverage determinations and access to quality care for your patients, we continually review our prior authorization process and requirements and may make changes to the requirements, as needed.

Utilization management – responsibility for prior authorization

We require that providers obtain prior authorization prior to performing a service for patients whose benefit plans require prior authorization. Our **Prior Authorization and Billing Resource Guide** outlines prior authorization requirements and billing code suggestions; however, it does not replace checking a patient's eligibility and benefits.

Prior authorization determinations are based upon the patient's eligibility, the specific terms of the applicable benefit plan, internal or external clinical coverage guidelines, and the patient's particular circumstances. Failure to obtain prior authorization may result in nonpayment.

Utilization management – prior authorization for admissions

We require prior authorization for all inpatient and residential admissions for all of our utilization management models. Partial and intensive outpatient programs may require prior authorization depending upon the plan.

We review certain levels of care to establish medical necessity, confirm that the proposed length of stay is appropriate, and determine if the requested services are covered benefits. The submission of accurate and complete clinical information with the prior authorization request is essential for timely processing.

The following information is typically required for prior authorization:

+ Patient's name and ID number.
+ Patient's date of birth.
+ Diagnosis, including International Classification of Diseases, 10th Revision, Clinical Modification (ICD-10-CM).
+ Requesting or referring provider.
+ Servicing provider, vendor, or facility.
+ Pertinent medical history and justification for service.

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+ Date of admission or start date.
+ Anticipated length of stay for facility stays.
+ Date of request.
+ Additional insurance coverage (if applicable).
+ Place of service and level of care (inpatient and outpatient).
+ Preliminary discharge plan, such as next level of care or provider.

Coverage policies and medical necessity criteria are available on the **Coverage Policies page** on the Evernorth® Provider website (**Provider.Evernorth.com**). **Note:** Certain policies require a **Provider.Evernorth.com** login. If you are not yet a registered user, please **register** to access all of our coverage, reimbursement, and administrative policies.

Emergency admissions

If the acuity of the patient's condition does not allow for prior authorization, contact us as soon as possible. Please be prepared to provide the following information to our staff:

+ Patient's name, age, and ID number.
+ History, diagnosis, indications, and nature of the immediate crisis.
+ Alternative treatment provided or considered.
+ Treatment goals, estimated length of stay, and discharge plans.

If the clinical indicators for hospitalization are unclear based on prudent layperson guidelines, **MCG Behavioral Health Guidelines**, **Level of Care Utilization System (LOCUS) criteria** for ages 19 and older, **Child and Adolescent Level of Care/Service Intensity Utilization System (CALOCUS-CASII) criteria** for ages 6 through 18, or **American Society of Addiction Medicine (ASAM) Criteria®**, we may request additional information, consult a physician reviewer, or send the clinical indicators to a physician reviewer for review with the facility or provider. If coverage for hospitalization is then authorized, we can assist with facilitating the admission, or, if the patient has already been admitted, a case manager will conduct regular, ongoing reviews with the facility staff.

Extenuating circumstances

Extenuating circumstances are factors beyond the control of the rendering provider or facility that make it impractical to obtain or validate the existence of a prior authorization prior to rendering the service (e.g., natural disaster or incorrect insurance information). When a request for prior authorization is received retrospectively due to extenuating circumstances, the provider or facility must submit appropriate medical records, supporting information, and an explanation of the extraordinary circumstances responsible for the delay or failure to obtain prior authorization.

For example, in circumstances where the patient submitted the wrong insurance information, the provider or facility should submit documentation that shows the patient submitted the wrong insurance information (e.g., a copy of the patient's insurance card, note in office records). The denial decision will be upheld if the provider or facility only submits a medical record and not the explanation.

As a reminder, under the terms of your Provider Agreement, you cannot bill your patients for covered services that are denied due to failure to obtain prior authorization.

General considerations – prior authorization

Prior authorization is neither a guarantee of payment nor a guarantee that billed codes will not be considered incidental or mutually exclusive to other billed services. Coverage is subject to the terms of a patient's benefit plan and eligibility

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on the date of service. Our **Prior Authorization and Billing Resource Guide** outlines prior authorization requirements and billing code suggestions.

We (or our designees) make coverage determinations in accordance with the time frames required under applicable law. You must supply all information requested within the time frames specified for us to make a prior authorization determination. Failure to provide information within the time frames requested may result in nonpayment.

A number is assigned to approved prior authorization requests.

Our Coverage Policy Unit is responsible for the development of internal clinical guidelines as well as for the proper use of externally developed guidelines (e.g., **MCG Behavioral Health Guidelines**, **LOCUS criteria**, **CALOCUS-CASII criteria**, **The ASAM Criteria**). Our utilization management staff or delegates use these guidelines to assess the medical necessity of a treatment or procedure, determine coverage for an appropriate inpatient length of stay, and make other clinically based coverage decisions.

Coverage for services is reviewed on a case-by-case basis. In the absence of a controlling federal or state coverage mandate, benefits are ultimately determined by the terms of the individual's benefit plan document (i.e., a group service agreement, evidence of coverage, certificate of coverage, summary plan description, or similar document).

However, to facilitate accurate and consistent coverage determinations, we maintain certain collateral source information and product-specific tools that aid our staff in applying the terms of a benefit plan document to a particular benefit request.

Copies of the clinical coverage guidelines and other resources used in making coverage determinations are available on the **Coverage Policies page** on **Provider.Evernorth.com**.

Retrospective prior authorization

We may consider retrospective prior authorization requests of inpatient services. Retrospective requests are submitted after the patient has already received care and been discharged. Prior authorization is available in real time for inpatient services when requests are submitted up to four calendar days since admission or on the last covered day, though additional time may be allowed. Requests outside of the above-mentioned time frame will be managed by a care manager for a more detailed assessment and coverage decision.

Provider offices are encouraged to supply as much information as possible when requesting a retrospective prior authorization request so we can perform a thorough review.

Autism spectrum disorder and applied behavior analysis

Prior authorization is typically required for applied behavior analysis (ABA). If you are treating a patient with autism spectrum disorder, please call the number on the back of their ID card to confirm eligibility and benefits. Benefit coverage varies by plan and state mandates. In most cases, your request will be referred to the Autism Utilization Management team for review and to discuss the patient's treatment plan. The patient's parent/legal guardian may also be asked to participate in the Autism Coaching and Support Program.

Please use the resources listed below based on your need:

+ To learn more about our team and how to make a request for ABA services, refer to our **Autism Information and Resources page**.
+ For your fee schedule and a listing of autism spectrum disorder–related services eligible for reimbursement, refer to Exhibit A in your Provider Agreement.

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+ For information about Evernorth specialty networks, including autism assessment and treatment, refer to our **Specialty Networks and Forms (Appendix F) page**.

Testing

Generally, clinical review for psychological testing for covered diagnoses will not be required. There may be certain situations where a review will be required, such as for a specific patient/account benefit plan. Testing related to custody evaluations, rehabilitation, vocational counseling, or school evaluations are generally not covered. Neuropsychological testing typically falls under medical coverage. Prior to rendering services, please call the number on the back of your patient's ID card to confirm benefits and eligibility.

Buprenorphine treatment

Buprenorphine treatment is a modality for outpatient office-based medication-assisted treatment (MAT) of opioid use disorders. We have case managers who can partner with individuals on MAT to enhance adherence. We consider outpatient opioid treatment with buprenorphine to be potentially eligible for coverage. Both the induction phase of treatment and ongoing medication management are considered to be routine services and do not typically require prior authorization.

Pass-through billing

Please note that services billed as pass-through billing are not reimbursable as covered services. Pass-through billing occurs when an entity bills for a service that is not actually performed by that billing entity. These tests may not be billed to Evernorth or any Evernorth affiliate, payer affiliate, payer, or patient.

Coaching and support programs

We offer several coaching and support programs to help individuals initiate and engage in behavioral treatment. The six core programs are substance use, opioid and pain management, autism, parent and families, eating disorders, and intensive behavioral health case management.

Our case managers, specially trained in coaching individuals with mental health and substance use disorders, utilize a motivational interviewing approach focused on decreasing both internal and external barriers that may prevent an individual from initiating or engaging in treatment or recovery activities. This approach allows each program to meet the individual's unique needs.

In addition to the six core programs, we also offer a few other programs to support your patients.

+ **Community support program:** This program addresses social determinants of health (SDOH) and assists those who are struggling to meet basic needs or who are experiencing other disparities. It works in partnership with medical and behavioral health teams across the enterprise to identify and address those needs and help remove barriers to connecting to helpful resources.
+ **Caregiver program:** This program supports individuals caring for an aging loved one or an adult or child living with a chronic medical or behavioral condition. It helps individuals manage the day-to-day challenges of caregiving by helping them access resources, navigate care options, and promote self-care.
+ **Changing Lives by Integrating Mind & Body® (CLIMB):** This program provides individual and group coaching services to patients who are struggling to cope when living with a chronic physical or emotional condition. It uses a cognitive behavioral approach based on identifying and improving thinking patterns that contribute to unhealthy behavior and negative emotions.

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Eligibility for these programs is dependent upon a patient's plan.

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Claims and compensation

Timely and accurate reimbursement is important to you and us. We have a number of customer service and claim centers throughout the country that are responsible for processing claims. For some patients, a third party, in accordance with our standards, may process claims. The customer service number and claim center mailing address are displayed on the patient's ID card. We encourage you to check the patient's ID card at each visit for the most current information.

You may perform the following claim-related tasks through the Evernorth® Provider portal (**Provider.Evernorth.com**):

+ View claims status (paid, denied, or pended).
+ View service line details, including amount not covered, coinsurance, patient responsibility, and service line remark codes.
+ View payment information, including amount paid, check number, date issued, payment method, and date of service.
+ Upload supporting documentation for pended claims. You can attach up to six files at a time (64MB maximum per file), and the files must be in .bmp, .gif, .jpeg, .pdf, .png, or .tif format.
+ Submit a claim reconsideration or appeal request and check the status of the request.
    - An online questionnaire simplifies the submission process and provides step-by-step guidance.
    - You can attach supporting documentation (up to 10 files at a time, 64MB maximum per file) and enter notes for your request.
    - Each submission will include a reference number.

Claims submission

You can help improve claims processing accuracy and timeliness by following our guidelines.

+ Be consistent with your demographic information when identifying yourself in claims submissions. If you need to change the way you submit claims, refer to the **demographic information section**.
+ Avoid using abbreviations or variations of names or doing business as names with combinations of your licensure numbers, National Provider Identifiers (NPIs), and Taxpayer Identification Numbers (TINs) not listed in your Provider Agreement. This can delay or result in incorrect claim payments.
+ Notify us in advance of changes to your information.

We require all claims to be submitted for processing, including, but not limited to, those paid in full at the time of service and those for services that are not covered. This information is used for other program outreach and reporting purposes.

Electronic claims submission

We strongly encourage you to submit your claims electronically. Doing so can help you save time and money and improve claims processing accuracy.

+ Allows you to send primary and secondary coordination of benefits (COB) claims quickly, reduce paperwork, and eliminate printing and mailing expenses.
+ Decreases the chance of transcription errors or missing data.
+ Tracks claims received electronically, which are automatically archived before processing.
+ Eliminates the need to submit claims to multiple locations.
+ Saves time on resubmissions (incomplete or invalid claims can be reviewed and corrected online).

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+ Provides confirmation of an accepted or rejected claim.

Payer ID for submitting electronic claims

Payer ID	Claim type
62308[1]	Medical, behavioral (including employee assistance program), and dental claims

1. Both primary and secondary COB claims can be submitted electronically to Evernorth Behavioral Health or Cigna Healthcare.

You do not have to submit Medicare Part A and B Coordination of Benefits Agreement claims to Cigna Healthcare® or Evernorth® Behavioral Health (Evernorth). The Medicare explanation of benefits or electronic remittance advice will show that those claims are forwarded to Cigna Healthcare or Evernorth as the secondary payer.

Paper claims submission

We strongly encourage you to submit claims electronically to save time and money. However, if you need to file a paper claim, use one of these claim forms:

+ **Form UB-O4** if you are contracted as a facility on an institutional agreement or are contracted on an ancillary agreement and have a per diem–based fee schedule.
+ **Form CMS-1500** if you are contracted on a provider services agreement, contracted on an ancillary agreement and have a Current Procedural Terminology fee schedule, or contracted on a clinic services agreement.

In instances where you must submit a paper claim, we will scan, sort, and store the claim electronically to reduce manual keying errors and improve response time. Follow these guidelines when completing and submitting paper claims:

+ If using a super bill or form other than a UB-04 or CMS-1500, ensure the form has the same information fields listed in the **definition of a complete claim section**.
+ Include your NPI on the claim.
+ Make sure all appropriate claim form fields are completed; use black ink when handwriting information.
+ Refer to the patient's ID card for the correct claims submission address.
+ Include the patient's ID number on all claim attachments and correspondence.
+ Type or print clearly on the form; cursive writing is not recommended.

If submitting a replacement or corrected claim, please do not stamp "Corrected Claim" on the claim form. Corrected claims should follow the National Uniform Billing Committee's guidelines, which are listed below:

+ **CMS-1500:** Field 22 should include a '7' or '8' to indicate a replacement or voided claim.
+ **UB-04:** The Claim Frequency Type Code should be submitted with a '7' or '8' in Form Locator 4 as well as the tracking number.

Definition of a complete claim

We define a complete claim as including the following information at a minimum in the CMS-1500:

+ Patient's ID number, name, address, and date of birth.
+ Subscriber's name and address.
+ Release of information.
+ Assignment of benefits.
+ Diagnosis codes.
+ Date of service.
+ Place of service.
+ Current Procedural Terminology or Healthcare Common Procedure Coding System codes and modifier.
+ Diagnosis pointer.

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- Billed charge.
- Units.
- TIN or Social Security number.
- Total charge.
- Rendering provider's name, NPI, and license type.
- Billing provider's name, address, and NPI.

We define a complete claim as including the following information at a minimum in the UB-04:

- Billing facility's name and address.
- Bill type.
- TIN.
- Date range.
- Patient's name, address, and date of birth.
- Revenue code.
- Date of service.
- Units.
- Billed charge.
- Total charge.
- Assignment of benefits.
- Facility's NPI.
- Subscriber's name.
- Patient's ID number.
- Diagnosis codes.
- Admitting diagnosis code.

Additional claims information needed

Sometimes it is necessary to include additional information to support a claim or make a benefit determination. Supplemental documentation should be included or sent as soon as possible after requested to avoid delays in claims processing.

Requests for supplemental claim information are sent to the address we have on file for you in our demographic database, which could potentially be a lockbox for claims payments. Please make sure we have the most current and correct mailing address for you so that you receive supplemental claims information and requests as well as other correspondence from us in a timely manner.

The table below outlines some of the claim categories that require supplemental information. A complete, up-to-date listing of our **clean claim requirements** is available on the Evernorth® Provider website (**Provider.Evernorth.com**). The requirement to provide supplemental claim information is subject to applicable law, and applicable law will control in the event of a conflict.

Claim category	Additional information
Air ambulance	Narrative/transport notes
Anesthesia	Specific time
Billing appropriateness	Itemized bill/clinical records or notes
Coordination of benefits (COB)	Copy of the primary payer's explanation of payment when Evernorth® Behavioral Health (Evernorth) is secondary (for paper claims) **Note:** Evernorth payer ID, 62308, can receive COB claims electronically. Please contact your vendor for information on how to submit COB claims electronically.
Cosmetic or potentially cosmetic procedures	+ Operative report + Office notes and treatment plan + History and physical + Photographs (if available) + Height/weight + Operative report and treatment results (if already performed) + Visual field-testing results for blepharoplasty

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Claim category	Additional information
Diagnosis related group clinical review	Clinical records or notes
Drugs - injectable	Healthcare Common Procedure Coding System (HCPCS) codes or National Drug Codes (NDCs) **Note:** When an individual's health plan requires prior authorization, we require a valid NDC number,[1] NDC units, an NDC unit qualifier, and a HCPCS code in three claim scenarios: + Not otherwise classified HCPCS codes (e.g., J3490, J3590, J9999). + HCPCS codes with utilization management controls (e.g., J0725, J1830, J2941). + Newly launched HCPCS codes without an average sale price.
Experimental, investigational, or unproven procedures	Operative or physician notes or other clinical information
High-dollar claims	Itemized bill
Home health care	+ Office notes and treatment plan + All visit notes, complete history, and physical + Infusion drug report, if applicable
Modifiers:[2] + 22 - Increased procedural services + 25 - Significant, separately identifiable evaluation and management service by the same physician on the same day of the procedure or other service + 59 - Distinct procedural service + 62 - Two surgeons + 66 - Surgical team + Others may also require additional information	Operative, office, or physician notes or other clinical information (for a select few modifier 25 and a select set of codes for modifier 59; National Correct Coding Initiative pairs require documentation with the initial professional claim [Form CMS-1500] submission) **Note:** Claims should continue to be submitted electronically, even if supporting documentation is required. Indicate in the PWK (Claim Supplemental Information) segment of Loop 2300 of the electronic claim that the documentation will be sent through another channel.
Morbid obesity	+ Complete history and physical + Proposed treatment plan, including any surgical procedures + Measures tried previously and patient's response
Predeterminations	+ Office notes and treatment plan + Complete history and physical + Photographs, if applicable + Pertinent diagnostic study results
Provider stop loss (facility only)	Itemization by date of service and revenue code (depending on the type of stop loss provision)
Unexpected place of service (e.g., office services performed in an ambulatory surgical center)	Operative or physician notes or other clinical information
Unlisted Current Procedural Terminology (CPT®) or HCPCS codes (e.g., CPT codes ending in "99," such as CPT code 64999 - unlisted procedure, nervous system) and unidentifiable services	+ A clear description of the service, device, or procedure provided (If the unlisted code is submitted for a drug, provide the name, dosage, a valid NDC number,[1] NDC units, HCPCS units, and medical necessity for the drug; if the unlisted code is for a surgical service, provide the operative report.) + Reference to whether the service, device, or procedure was provided separately from any other service, device, or procedure rendered + Information to establish medical necessity for the service, device, or procedure + Detailed description of the approved radiology procedure, if applicable + Laboratory or pathology report pointing out the specific test used, if applicable

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1. If a valid NDC number is not provided, the claim may be denied.
2. Modifier 25 and Modifier 59 policies are available on **Provider.Evernorth.com**. Certain policies require a **Provider.Evernorth.com** login. If you are not a registered user, please **register** to access all of our coverage, reimbursement, and administrative policies.

Claims filing deadline

Claims should be filed as soon as possible to promote prompt payment. We will only consider claims submitted within 90 days of the date of service or as otherwise defined in your Provider Agreement and the exceptions noted below.

For services rendered on consecutive days, such as for a hospital confinement, the filing limit will be counted from the last date of service. The following are exceptions to the 90-day time limit:

+ Applicable state law provides for a longer timely filing limit in which case that time limit will apply.
+ COB (90-day filing limit is applied based on the primary payer's processing date as stated on an explanation of benefits [EOB] or explanation of payment [EOP]).
+ Medicare (90-day filing limit is applied based on the primary payer's processing date as stated on an EOB or EOP).
+ Medicare secondary payer (three years).
+ Medicaid (three years).
+ Resubmission of a claim originally filed in a timely manner, returned with new or additional information as requested by us (90-day filing limit is reset to the date of the request for more information).
+ Services provided to patients through arrangements with third-party vendors (filing limit is applied based on third-party requirements, which may be more or less than 90 days).
+ Extenuating circumstances (e.g., catastrophic events).

Claims inquiry and follow-up

You can inquire about the status of your claims in several ways. Each of the options outlined below allow you to access details of processed claims.

+ Submit an electronic data interchange (EDI) claim status inquiry (276/277) through your EDI vendor.
+ Log in to **Provider.Evernorth.com**.
+ Call the customer service number on the patient's ID card or on the EOP to access the automated interactive voice response system or to speak with a customer service representative.

When inquiring about a claim, have the following information available:

+ Provider's name, National Provider Identifier (NPI), and Taxpayer Identification Number (TIN) and the state they practice in.
+ Patient's name, date of birth, and ID.
+ Subscriber's name
+ Date and description of service.
+ Amount of claim.
+ Date claim was submitted.

For inquiries submitted via **Provider.Evernorth.com** or through your EDI vendor, you will receive the following for each claim:

+ Status using the standard Health Insurance Portability and Accountability Act of 1996 claims status and claims status category codes.
+ Claim number.

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+ Total charge and paid amounts.
+ Processed date.
+ Payment date, method (check or electronic funds transfer), and check number.
+ Status history (available for two years).

Claims payment policies and procedures

Claims from in-network providers are subject to our claims payment policies and procedures. These policies are the guidelines for calculating payment of claims and include our standard claim code auditing methodology, review of charges to service provided, and procedures for claim adjudication.

Standard claims coding/bundling methodology

If you have questions about our standard claims coding, bundling methodology, or payment policies or how specific types of billing codes will be processed, you can log in to **Provider.Evernorth.com** and go to Resources > Reimbursement and Payment Policies > Claim Editing Policies and Procedures.

Patient liability collection limitations

Copay: A copay is a fixed dollar amount that a patient pays per service. Copay amounts are printed on the patient's ID card and should be collected at the time of service.

Coinsurance and deductible: A deductible is the amount a patient pays for eligible services before their health plan begins to share in the cost of covered services. Coinsurance is the percentage of covered charges that the patient pays for after they have satisfied their deductible. For patients with plans that have a deductible or coinsurance, you should submit claims to the payer or its designee and receive an explanation of payment (EOP) indicating the patient responsibility before billing patients. Coinsurance and deductible amounts should not be collected at the time of service.

Most patients with a Cigna Choice Fund® plan have automatic claim forwarding (ACF) enabled so the deductible and coinsurance amounts they owe are paid directly out of their health care account. After claim processing, if funds are available, Cigna Healthcare® automatically sends payment to you on behalf of the patient, usually along with the Cigna Healthcare portion of the payment. ACF is chosen by the employer and is currently active for most of our customers with Choice Fund plans.

Please note for patients with Choice Fund plans, the ACF feature is chosen by the patient and is now referred to as AutoPay by our vendor, HSA Bank. Most of the time, ACF is not a chosen option like it is with our health reimbursement account plans.

Fee-forgiving/waiver of copay, coinsurance, or deductible: Most benefit plans exclude from the patient's coverage those charges for which they are not obligated to pay. Therefore, if a patient is not obligated to pay a charge, any claim for reimbursement for any part of that charge under such a contract or benefit plan is generally not covered. It is our view that "fee-forgiving" on any particular claim, or any portion thereof, could constitute fraud and may subject a provider to civil and criminal liability.

Denied payment and patient liability: You cannot bill patients for covered services or for services for which payment was denied due to your failure to comply with your Provider Agreement, these Administrative Guidelines, and/or other program requirements, including utilization management requirements and timely filing requirements.

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Coordination of benefits

Patients may be covered by more than one health benefit plan. In some cases, payment may be the primary responsibility of another payer. Billing multiple health benefit plans to obtain payment is called coordination of benefits (COB). You should assist us to maximize recoveries under COB and bill services to the responsible primary payer. After receiving a payment or denial notice from the primary payer, you should submit the COB claim electronically to Cigna Healthcare® or Evernorth® Behavioral Health (Evernorth). However, if you submit COB claims on paper, then a copy of the primary payer's explanation of payment (EOP) is required.

The Cigna Healthcare/Evernorth payer ID, 62308, can receive COB claims electronically; please contact your vendor for information on how to submit these claims. For more information about electronic claims, go to the **claims submission section**.

Cigna Healthcare or Evernorth as primary payer

When the Cigna Healthcare or Evernorth plan is the primary payer, payment is made in accordance with your Provider Agreement without regard to the secondary plan. After receiving payment from Cigna Healthcare or Evernorth, you should submit the COB claim to the secondary plan.

Cigna Healthcare or Evernorth as secondary payer

When the Cigna Healthcare or Evernorth plan is the secondary payer, you should first submit the claim to the primary payer. After receiving a payment or denial notice from the primary payer, you should submit the claim to Cigna Healthcare or Evernorth, along with a copy of the primary payer's EOP. Paper copies are not required if you submit Health Insurance Portability and Accountability Act of 1996–compliant COB content electronically through an electronic data interchange claims submission.

We participate in Medicare Coordination of Benefits Agreement, also known as Medicare Crossover, for individuals whose coverage is made available through Medicare Part A and B. This eliminates the need for you to submit Medicare COB claims to Cigna Healthcare or Evernorth. The Medicare explanation of benefits or electronic remittance advice will show that those claims were forwarded to Cigna Healthcare or Evernorth as the secondary payer.

The Cigna Healthcare or Evernorth payment as the secondary payer, when added to the amount payable from other sources under the applicable COB rules, will be no greater than the payment for covered services under your Provider Agreement and is subject to the terms and conditions of the patient's health benefit plan and applicable state and federal law. Use of applicable COB provisions may result in a payment from Cigna Healthcare or Evernorth, when added to the amount payable from other sources, that is less than 100 percent of your payment for covered services under your Provider Agreement.

When Medicare is the primary payer and the Cigna Healthcare or Evernorth plan is the secondary payer, applicable Medicare billing rules (including Medicare COB rules) will apply to your reimbursement. The financial responsibility of the Cigna Healthcare or Evernorth plan as a secondary payer under Medicare COB rules is limited to the patient's financial liability (i.e., the applicable Medicare copay, coinsurance, and/or deductible) after application of the Medicare-approved amount. The Medicare payment plus the patient liability amounts constitutes payment in full, and you are prohibited from collecting any monies in excess of this amount.

Order of benefit determination

We follow the National Association of Insurance Commissioners guidelines about the industry standard of order of benefit determination subject to applicable law and the terms of the benefit plan.

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Determining primacy on a patient/spouse

The plan that covers a person as an employee, subscriber, or retiree is always considered the primary payer over a plan that covers the person as a spouse or dependent. If a plan subscriber has two employers and has group health insurance coverage through both, the plan for the company that the subscriber has worked for longer is considered primary.

If a person has coverage under a state or federal continuation plan and is covered under another group health insurance plan, the plan covering the person as an employee, subscriber, or retiree (or as that person's dependent) is primary and the continuation coverage is secondary.

Determining primacy on a dependent child

Dependent children of parents who are married and living together follow the "birthday rule." The plan of the parent whose birthday falls earlier in the calendar year is primary to the plan of the parent whose birthday falls later in the year. Only the month and day of birth are relevant; the birth year is not taken into consideration. If both parents have the same birthday, the parent with the plan that has been in effect longer is primary.

Dependent children of parents who are divorced, separated, or not living together follow the "custodial rule." If a court decree states that one of the parents is responsible for the dependent child's health care coverage, that parent's plan is primary, followed by the plan of the other parent. If a court decree awards joint custody without specifying which parent is responsible for providing health insurance coverage, the birthday rule is followed.

If there is no court decree allocating responsibility for the dependent's health coverage, the order of benefit determination under the custodial rule is as follows:

1. The plan of the custodial parent.
2. The plan of the custodial parent's spouse, if applicable.
3. The plan of the noncustodial parent.
4. The plan of the noncustodial parent's spouse, if applicable.

Determining primacy with Medicare

For Medicare beneficiaries, federal law or regulation determines the order of benefit determination, which may differ from the rules described above. The group health plan that covers Medicare beneficiaries, aged 65 or older, through active employment (theirs or that of their spouse), and where the employer has 20 or more employees is the primary payer. The group health plan that covers Medicare beneficiaries, under age 65, through active employment and where the employer has 100 or more employees is the primary payer.

The group health plan is primary for Medicare beneficiaries who have end-stage renal disease during the first 30 months of their Medicare eligibility.

Workers' compensation

You must submit a potential workers' compensation claim to the applicable workers' compensation carrier for review before submitting the claim to us. If the workers' compensation carrier denies the claim, a copy of the denial must be included with the claim submission. If the workers' compensation denial is not received with the claim, payment for services will be denied unless state law specifically prohibits a denial on these grounds.

Part of the post-review process may include a vendor contacting the patient for information about the case. If it is determined that we have made a medical payment on a valid workers' compensation case, we will require a full refund.

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The vendor will provide information about that process. In this case, you should then resubmit the claim to the workers' compensation carrier responsible for payment.

Subrogation and reimbursement requirements

Subrogation may apply if a patient is injured in an accident of any type and someone else is responsible for the injury. If you treat a patient with a subrogation claim, your contract, as well as these Administrative Guidelines, will apply to the same extent that they apply to any other patient. Appropriate authorizations must be obtained to help ensure payment. Additionally, please note that claims should be submitted to us.

Other billing guidelines

Overpayment recovery

When an overpayment has been identified, the plan will obtain a refund in one of two ways:

1. **Offsetting payments from future claims:**
   In states where permitted, when the plan identifies that an overpayment has been made on a patient's claim, the payment will be reversed, leaving a negative balance in the provider's account. Providers are requested to repay overpayments directly to the plan as outlined in the overpayment notice sent once the overpayment is identified. Based on banking arrangements for specific employer groups, as well as state mandates, some claims may offset prior to the plan receiving the requested refund. When this occurs, any refund received from the provider is returned to the provider with a letter explaining the offset. Until overpayment is resolved, payment on additional claims may be suspended. For inquiries about the overpayment process, please call the number on the patient's ID card.

   For self-funded plans and states that allow offset: If the overpayment on the provider's file has not offset within 30 days, an overpayment letter is sent to the provider requesting the refund. If the plan does not receive the refund within 60 days, a second refund request letter is sent advising that if the plan does not receive the refund within the next 30 days, a third notice will be sent. If the refund is not received within the next 30 days, the plan will again attempt to deduct (offset) the negative balance from future payments to be made to the provider. If at any time we receive the refund from the provider and the overpayment has already been offset, the plan will return the check to the provider with a letter advising that the overpayment has been offset.
2. **Requesting a refund from the provider who rendered the services:**
   Not all overpayments can be set up to offset; therefore, a provider may be advised when an overpayment has been identified and they will be expected to promptly refund any overpaid amount to the address enclosed in the refund request. Our standard recovery method is by refund check. Failure to comply with recovery efforts may result in the plan initiating the dispute resolution process set forth in the Provider Agreement. We reserve the right to reduce future reimbursement amounts to recover previous overpayments subject to all statutory and contractual requirements. If a provider identifies an overpayment, an unsolicited refund can be sent back to the plan via the address on the explanation of payment (EOP).

EOP

An EOP accompanies all claims payments. It itemizes the services processed or considered for payment as well as payment information, such as copays, deductibles, patient responsibility amounts, contracted discounts, payment amounts, and date(s) of service. We use a standard format for payment explanations, combining the check and claim detail information in a single document. This consolidated format is called the "Check/EOP."

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You must be a registered user of the Evernorth® Provider portal (**Provider.Evernorth.com**) and have the remittance reports entitlement to access the "Check/EOP" format.

Electronic funds transfer and electronic remittance advice

We offer electronic funds transfer (EFT) and electronic remittance advice (ERA). When you use EFT and ERA together, you can access your funds and complete your accounts receivable posting faster.

EFT

EFT is our standard payment method for provider reimbursement. It securely deposits fee-for-service and capitated payments directly into your bank account.

EFT benefits

By enrolling in EFT, you can:

+ Eliminate paper check mail delivery and handling.
+ Access funds on the same day of the deposit.
+ Increase efficiency and improve cash flow.
+ View online remittance reports for each deposit. (See additional information about online remittance reports below.)
+ Easily reconcile payments using a single remittance tracking number. This number is listed in the EFT report your bank sends to you (field 3 of record 7), in the "Reference Identification Field" (or TRN02) on your ERA, and on the first page of your online remittance report.

EFT enrollment

You can enroll in EFT by logging in to the Evernorth® Provider portal (**Provider.Evernorth.com**) and going to My Practice > Enroll in Electronic Funds Transfer (EFT) Options. Once you have completed the electronic enrollment form, if needed, we will send a "pre-note" transaction to your bank to verify all the banking information is correct.

You can check the status of your EFT application by logging in to **Provider.Evernorth.com** and going to My Practice > Manage EFT Settings. If you are not yet registered for the portal, go to the **registration page** on **Provider.Evernorth.com**. The enrollment process typically takes four to six weeks.

The following are EFT enrollment guidelines:

+ For savings account deposits, verify that your bank will support EFT.
+ If you use more than one Taxpayer Identification Number (TIN), you must complete a separate enrollment for each TIN.
+ If your TIN, National Provider Identifier (NPI), billing address, or bank account changes, you must submit a change request by logging in to **Provider.Evernorth.com** and going to My Practice > Manage EFT Settings.

Payment bulking options

You can choose to have your payments grouped or bulked in either of two ways:

+ TIN and payment address - By electing TIN bulking, all claims will be grouped into a single payment based on TIN and payment address.

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- NPI - By electing NPI bulking, all claims will be grouped into a single payment for each billing provider's NPI from the submitted claim.
    - The ERA or remittance report will be bulked by TIN or NPI, depending on your payment bulking preference with your electronic data interchange (EDI) vendor.
    - You can elect a separate bank account for each billing provider's NPI.
    - To have your payments bulked or grouped based on your NPI, update your payment bulking preferences on **Provider.Evernorth.com**. You must log in and go to My Practice > Manage EFT Settings to make updates.

Online remittance reports

If you are enrolled in EFT, you can access remittance reports that explain your processed claims, such as direct deposit reports (DDARs) or checkless explanations of payment (EOPs). The remittance reports search tool allows you to:

- View your remittance reports the same day you receive your EFT.
- Easily reconcile payments using a single remittance tracking number. This number is listed in the EFT report your bank sends to you (field 3 of record 7), in the "Reference Identification Field" (or TRN02) on your ERA, and on the first page of your online remittance report.
- Look up a remittance report using several options:
    - Deposit amount: Search for a specific deposit amount or deposits made within a specific date range.
    - Patient information: Search for a specific patient by entering their ID and date of birth; their ID and first name and the subscriber's name; or their date of birth and first name and the subscriber's last name).
    - Claim/reference number (the Evernorth-assigned claim/reference number located on your EOP, claim remittance advice, DDAR, and provider explanation report). **Note:** If you conduct a claim inquiry search, you can also view the remittance report for that specific claim by clicking the remittance report link at the bottom of the page.
    - Remittance tracking number (the number Evernorth assigns to your EFT; this number is included on the EFT file to the bank).
- View each claim within the deposit, including the paid amount and patient responsibility.
- Search within the remittance report for specific patients or claims.

If you are already registered for **Provider.Evernorth.com** and have access to claims status inquiry, you automatically have access to online remittance reports.

If you have not yet registered for the portal, go to the **registration page** on **Provider.Evernorth.com**. Once you complete the registration information and it has been validated, you also need to enroll in EFT (if you have not already). Once your EFT is effective, you can access your remittance reports online.

Primary administrators: If you have staff that will need access to online remittance reports, log in to **Provider.Evernorth.com** and go to My Practice > Modify Existing Users/Add New Users.

ERA

To help reduce your payment cycle, we offer ERA (an 835). ERA is the Health Insurance Portability and Accountability Act of 1996–compliant detailed explanation of how a submitted health care claim was processed. The ERA may be automatically loaded into your accounts receivable system, which can help:

- Reduce costs and save time.
- Reduce posting errors.

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+ Shorten the payment cycle.

We will provide the following information needed to reconcile your payments:

+ The patient account number you submitted on the claim.
+ The charge amount, paid amount, and patient responsibility for the claim.
+ The charge amount and paid amount for each service line, except for claims that may be paid at a claim level (e.g., diagnosis related group claims).
+ The amount and explanation of adjustments between the charged amount and the paid amount.
+ The allowed amount for each service line.
+ Adjustments not related to a specific claim (e.g., late payment interest or refund acknowledgments).
+ The NPI submitted on your claim(s), which is included in the Provider Summary (TS3) field.

ERA enrollment

To enroll in ERA, you should notify your EDI vendor that you would like to enroll in ERA and provide them with the appropriate enrollment information. If you use more than one TIN, you should complete separate enrollment information for each TIN.

Your EDI vendor will send the completed enrollment information to us for processing. We will finalize your registration within 10 business days of receiving it. Then, you may begin receiving ERA on your next payment cycle.

For a list of EDI vendors and the transactions they support, visit **Cigna.com/EDIVendors**.

Posting payments and adjustments

In addition to posting applicable payments, you are required to make contractual adjustments to reconcile a patient's account based upon the contractual or negotiated rate and as noted on the EOP. Contractual adjustments are reflected on the EOP, ERA, or other plan remittance or payment statements.

Applicable rate

The rates detailed in your Provider Agreement extend to services performed on a patient, including services covered under the patient's in-network benefits.

For facilities, if a covered service is not listed in the Provider Agreement, the default rate will apply. The default rate should only be a temporary solution while the agreement is being amended to apply to that location.

If medical necessity is required and services have been denied, a provider can utilize the self-pay agreement at the time of the denial. As an in-network provider, it is our expectation that the rate for the service be the same as if the care was approved. However, if you provide services not covered under the patient's plan, the provider may charge their billed charge, but the expectation is that the rate for the service be aligned with the contracted rates.

Rates and changes to coverage

For facilities only

If a patient is receiving inpatient treatment when their coverage status changes, the hospital's reimbursement for covered services will be prorated based on the total number of days of the entire length of stay that the patient had coverage.

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Clinical claim review program

The clinical claim review program evaluates claims for accuracy and appropriateness before a benefit payment determination is made. As part of this program, an experienced team of professionals, including nurses and physicians, reviews billing and coding to confirm accurate reporting; examines submitted documentation; reviews submitted claims against applicable coverage, payment, and reimbursement policies; and verifies coverage for services under the terms of the patient's benefit plan.

Compensation

We compensate providers in a manner intended to emphasize preventive care, promote quality of care, and ensure the most appropriate use of health care services. We reinforce this philosophy through utilization management decisions by our medical directors, physician advisors, and case management staff. Our employees are encouraged to promote appropriate utilization of health care services. The same criterion applies for staff eligible to receive additional payments based on their performance. Our employees and consultants receive no financial incentives or rewards to deny coverage of medically necessary care. We offer no incentives for utilization management decision-makers for underutilization of care.

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Resolving payment questions

You can take the steps outlined below prior to providing nonemergency treatment or services to a patient as well as prior to submitting the claim for reimbursement to help avoid unnecessary claim processing delays or denials and minimize the need to pursue the dispute resolution process.

Prior to providing services, do one of the following:

+ Log in to the Evernorth® Provider portal (**Provider.Evernorth.com**).
    - Verify the patient's benefits.
    - Confirm the Current Procedural Terminology (CPT®) or revenue code is covered under the patient's plan.
    - Review coverage policies.
    - Determine if prior authorization is required for outpatient services and, if it is, request it.
+ Call the number listed on the patient's ID card.

Prior to filing a claim:

+ Ensure either your billing staff or vendor includes all critical information needed for the claim to be expeditiously processed. Items to include are:
    - Patient's name, date of birth, address, gender, and age.
    - ID number on your patient's ID card.
    - Itemized bill.
    - Description of the treatment or service (CPT or Healthcare Common Procedure Coding System code).
    - Diagnosis code.
    - Specific charge for each service.
    - Anesthesia time in hours and minutes.
    - Medicare or other insurance explanation of payment, if Evernorth or Cigna Healthcare is the secondary payer.
    - Rendering provider's name, address, Taxpayer Identification Number, and National Provider Identifier (if applicable).
    - Description of the service and any clinical notes to support the need for an unlisted code if billing an unlisted procedure code.
+ Include modifiers on the claim if they are needed to describe the service performed. If billing telehealth services, please use the following guidance:
    - Modifier 95 in Field 24-D to specify telehealth.
    - 02 for Place of Service in Field 24-B.

Reimbursement

When you receive the explanation of payment (EOP) or electronic remittance advice (ERA), review it carefully to understand our reimbursement decision. If you do not understand the reasons provided on the EOP or ERA or the decision is different from what you expected, please call the number on the back of your patient's ID card.

If it is determined that we made a claim processing error, customer service will ensure the claim is corrected and no additional action is required by you. If it is determined that there was an omission or incorrect information was submitted (e.g., missing field or missing modifier), you will be asked to submit a corrected claim with the appropriate resubmission codes. The claim will be reevaluated with this new information.

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Dispute resolution

Provider payment appeals

Unless your Provider Agreement or state law provides for a different process, the processes in this section apply whenever you have a dispute with Evernorth® Behavioral Health (Evernorth) about a payment, including disputes over the amount that you believe you should have been paid and if you think you were not paid in a timely manner.

Before you start the appeals process described below, please review the claim and your Provider Agreement to confirm there is an issue. If you believe there is one, please call Provider Services at **800.926.2273** to try to resolve the issue first. Many issues can be resolved quickly by providing requested or additional information.

When you call, please have the patient's name, date of service, and treating provider's name and Taxpayer Identification Number available.

If we state the claim has been processed correctly but you disagree, your next step is to file an appeal with Evernorth (or one of our delegates as noted below). Fee schedule or reimbursement terms for multiple patients may not require individual appeals.

You can initiate our appeal process through a written request. The appeals process aims to resolve contractual disputes about post-service payment denials (or partial denials) and other payment disputes. If the issue is not resolved to your satisfaction, you may request dispute resolution, including arbitration, as the final resolution step.

Disputes about the performance or interpretation of the Provider Agreement will first be resolved in accordance with the applicable internal dispute resolution (appeals) process outlined in these Administrative Guidelines. If the dispute is not resolved through that process, follow the dispute resolution provisions in your Provider Agreement. The standard dispute resolution process provides that either party may request, in writing, that the parties attempt in good faith to resolve the dispute promptly by negotiation between designated representatives of the parties who have authority to settle the dispute. If the matter is not resolved within 60 days of a party's written request for negotiation, either party may initiate arbitration by providing written notice to the other party.

Unless applicable state law provides otherwise, you may not institute arbitration until the provider has completed the internal appeals process.

Appeals

All appeals are to be initiated in writing within 180 calendar days of the date of the initial payment or denial decision. If the appeal relates to a payment that we adjusted, the appeal is to be initiated within 180 calendar days from the date of the last payment adjustment.

For additional information on claim appeals policies and procedures, visit the **Clinical Reimbursement Policies and Payment Policies page** on the Evernorth® Provider website (**Provider.Evernorth.com**).

Appeal requests may not be submitted using any automation software or systems, including, but not limited to, automation via electronic faxes or any other submission mediums. For us to accept an appeal for review, each individual appeal must be submitted by a person regardless of submission medium. This requirement applies whether the appeal request is submitted by you or a third-party billing service on your behalf. If we do not accept an appeal request because it was submitted using automation, we will notify you that you need to resubmit the appeal request correctly.

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Appeal types and filing instructions

Administrative denials
Administrative denials may be issued for several reasons, including exhausted benefits for continued stay requests only, services not covered under the patient's benefit plan, lack of prior authorization for services, or benefits exhausted for the contract year. Patients or providers on their behalf are entitled to appeal administrative denials.

An appeals coordinator from the Evernorth Behavioral Health Central Appeals Unit handles the appeal review of an administrative denial. Appeal options can be discussed with case management staff or the Evernorth Behavioral Health Central Appeals Unit.

Contract, fee schedule, and multiple patient disputes
Fee schedule adjustments and reimbursement disputes may not require individual appeals. They may be quickly resolved through submitting an adjustment request via the reconsideration feature on **Provider.Evernorth.com**. You must log in to access this feature.

Medical necessity
For medical necessity denials or inpatient facility denials related to level of care, length of stay, or delayed treatment days, you should include the complete facility record (e.g., physician orders, progress notes, patient's medical history and physical exam results, consultations, results of diagnostic testing, operative reports, and discharge summary).

If your dispute involves an issue about the medical necessity of a service or procedure in addition to a pricing accuracy concern, a clinician will review the nonpricing part of your appeal. If your dispute relates to a benefit determination issue in addition to a pricing accuracy issue, the applicable plan's benefits will be reviewed, and our response will refer to those benefits.

Most appeals are resolved within 60 calendar days of receipt and notification of our decision will be made to the requesting provider within 75 days unless state law requires a different time frame.

Untimely claim submissions
For any claims denial decisions related to untimely claims submission (failure to submit a claim within 90 days of the date of service), you should submit justification and supporting documentation for the delay with your appeal request. Acceptable documentation includes the electronic data interchange transmission report or evidence that a claim was submitted due to coordination of benefits with another carrier.

If you are disputing the timeliness of your payment, you should include documentation showing the date you submitted the claim and any communications with us relating to the claim.

For any documentation required under this section, you are responsible for securing the information from any vendors that you might use.

If, after you follow this process, we determine that the initial decision was correct and will be upheld, an appeal denial letter will be sent to you explaining the decision and outlining any additional appeal rights. An appeal determination that overturns the initial decision will be communicated through the EOP with the reprocessed claim.

Additional payment appeal options
If you are still not satisfied after completing the internal appeal process, you may request dispute resolution, including arbitration. This is a binding, final resolution for the respective claim, unless otherwise specified in your Provider Agreement or state addendum.

The process for arbitration may be specified in your Provider Agreement. If it is not specified in your Provider Agreement and is not prohibited by state law, the process in the following two paragraphs will apply.

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If the dispute is not resolved through the appeal processes described above, either party can initiate arbitration by providing written notice to the other. The appeal processes must be followed in their entirety before initiating arbitration. If one of the parties initiates arbitration, the proceeding will be held in the jurisdiction of the provider's domicile. The parties will jointly appoint a mutually acceptable arbitrator. If the parties are unable to agree upon such an arbitrator within 30 days after one of the parties has notified the other of the desire to submit a dispute for arbitration, then the parties will prepare a Request for a Dispute Resolution List and submit it to the American Health Lawyers Association Alternative Dispute Resolution Service (AHLA ADR Service), along with the appropriate administration fee. Under the Code of Ethics and Rules of Procedure developed by the AHLA ADR Service, the parties will be sent a list of 10 arbitrators along with a background and experience description, references, and fee schedule for each. The 10 arbitrators will be chosen by the AHLA ADR Service based on their experience in the area of the dispute, their geographic location, and other criteria as indicated on the request form. The parties will review the qualifications of the 10 suggested arbitrators and rank them in order of preference from one to nine. Each party has the right to strike one of the names from the list. The person with the lowest total will be appointed to resolve the case.

Each party will assume its own attorney's fees and all of its costs of arbitration; however, the compensation and expenses of the arbitrator along with any administrative fees or costs will be borne equally by the parties. Arbitration is the exclusive remedy for the resolution of disputes under the parties' agreement. The decisions of the arbitrator will be final, conclusive, and binding, and no action at law or in equity may be instituted by the parties other than to enforce the award of the arbitrator. The parties intend this alternative dispute resolution procedure to be a private undertaking and agree that an arbitration conducted under this provision will not be consolidated with an arbitration involving other physicians or third parties and that the arbitrator will be without power to conduct an arbitration on a class basis. Judgment upon the award rendered by the arbitrator may be entered in any court of competent jurisdiction.

Determinations for hospital and facility appeals
Unless prohibited by state law, if a hospital or facility fails to request an appeal review or arbitration of the hospital's or facility's payment or termination dispute within the applicable time frames, our last determination about the dispute will be binding. The hospital or facility should not bill the patient for payments that are denied on the basis that the hospital or facility failed to submit the request for review or arbitration within the required time frames.

Provider termination appeals
On occasion, we deem it necessary to terminate a Provider Agreement. Appeal rights are offered to providers terminated due to quality of care or quality of service and to those terminated for failure to meet our credentialing requirements in states that mandate appeal rights be offered.

To initiate a review of a termination, submit the following information in writing within 30 calendar days of the date of the termination notice: a completed provider termination appeal letter indicating the reason for the appeal, a copy of the original termination notice, and supporting documentation for reconsideration.

Coverage denials

For clinical cases under review resulting in an adverse determination (coverage denial) following the peer-to-peer review, a review of the decision is available with a physician not previously involved in the case. The review is with an Evernorth–contracted, board-certified psychiatrist or doctoral-level psychologist and may be done on an expedited basis, if the situation is deemed urgent and the patient is still in that level of care, or on a standard basis, where more information, such as the medical record or a summary of treatment, may be made available. An appeal must be submitted within 180 calendar days from the claim denial unless determined otherwise by plan or state regulation. We strongly encourage the submission of clinical information prior to the peer-to-peer review or appeal process.

In some circumstances, out-of-network providers requesting a standard appeal must submit a valid Appointments of Representative form signed by the patient in order for the standard appeal request to be considered valid. Please call the number on the back of your patient's ID card to confirm whether this applies to your standard appeal request.

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The standard appeal is filed through the Evernorth Behavioral Health Central Appeals Unit. It is important to note that appeals committees are for the purpose of resolving patient issues. Payment disputes where the patient is held harmless are not eligible for review by the Evernorth Behavioral Health Central Appeals Unit unless the provider, with a patient's written authorization, requests an appeal on behalf of a patient.

A patient, the patient's delegate, or a provider on behalf of a patient who is dissatisfied with the outcome of the appeals determination may file an appeal by following the health plan's or, in some instances, the state's external appeals process. A second-level appeal must be submitted within 60 days from the receipt of the first-level appeal decision letter. Many states offer an expedited process if the patient feels the situation is urgent and the patient is still in that level of care. The case manager assigned to the case can provide information regarding the extent of appeals available.

If an appeal subsequently overturns an earlier decision, we will implement the appeal decision or process the authorization or claim for payment of services. Decisions are communicated in writing with all adverse determinations and contain the following information:

+ The specific guidelines on which the determination is based, including **MCG Behavioral Health Guidelines**, **LOCUS criteria**, **CALOCUS-CASII criteria**, **The ASAM Criteria**, or the plan coverage policy.
+ The facts and evidence considered.
+ The clinical rationale for the determination.

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National ancillaries: laboratories

Laboratory services are the most commonly used ancillary services by behavioral providers.

We currently contract with numerous local and national laboratories, including Laboratory Corporation of America® (LapCorp®) and Quest Diagnostics, Inc. (Quest) as well as other reference, pathology, genetic, and esoteric laboratories, to provide access to quality services at in-network, cost-effective rates.

By referring your patients to a laboratory that participates in our network, you can help ensure your patients maximize their benefits while limiting their out-of-pocket expenses.

LabCorp

+ Phone: **888.LABCORP**
+ Website: **LabCorp.com**

Quest

+ Phone: **866.MyQuest**
+ Website: **QuestDiagnostics.com**

A complete list of in-network laboratories is available in the **Evernorth provider directory**.

If your patient does not have a Cigna Healthcare® plan, please call the number on the back of their ID card to identify in-network laboratories.

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Patient information

Patients receive an ID card that includes an ID number, designated copay information, coinsurance and deductible amounts, and the name of their primary care provider (PCP), if applicable. The ID card does not guarantee eligibility or coverage.

There are several options to verify benefits and eligibility for your patients with Cigna Healthcare® plans:

+ Log in to the Evernorth® Provider portal (**Provider.Evernorth.com**) and go to Patients > Search Patients. If you are not registered for the portal, go to the **registration page** on **Provider.Evernorth.com**.
+ Submit an eligibility benefit inquiry (270/271) through your electronic data interchange vendor.
+ Call the number on the patient's ID card.
+ Call Provider Services at **800.926.2273**.

Each time a patient visits your office, you should review their ID card.

We make no representations or guarantees about the number of patients referred to a provider. We also reserve the right to direct patients to select in-network providers and to influence their choice of an in-network provider.

Alternate member identifier

To help protect the privacy of patients and prevent identity theft, we have phased out the use of Social Security numbers (SSNs) as the patient identifier. You should use the identifier on the patient's ID card to submit claims and inquire about eligibility or claims status. We may continue to accept claims and inquiries submitted with either the alternate member identifier (AMI) or the subscriber's SSN for patients without an AMI.

Note: Many of the identifiers begin with U0 (zero). In some cases, when entering the ID number, the capital letter 'O' is being input instead of the number "0" (zero). If your claim submissions are rejected for "invalid ID," check that you have entered the correct identifier – U0 (zero), rather than UO (capital letter O).

In addition, you may submit the subscriber ID with or without the subscriber relationship suffix shown on the patient's ID card (e.g., U01234567 *01*).

Verification options

For information on a patient's benefit plan, including copay, coinsurance, or deductible amounts:

+ Review the patient's ID card.
+ Submit an eligibility and benefit (270/271) inquiry through an electronic data interchange vendor that securely transmits data to us.
+ Log in to the secure Evernorth® Provider portal (**Provider.Evernorth.com**) and go to Patients > Search Patients or call Provider Services at **800.926.2273**.

Concerns or complaints

A patient should contact the number on the back of their ID card if they have a concern or complaint about benefit plan administration, coverage, or exclusions or the quality of service or care received. We will make an attempt to resolve the problem during the first phone call. If a patient is not satisfied with the response, they may follow the processes for submitting a complaint outlined in their benefit plan document. The process may include an Evernorth® Behavioral

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Health (Evernorth) representative contacting a provider to obtain information that may help in the resolution of the concern or complaint. This also provides an opportunity for the provider to respond to the concern or complaint.

Provider cooperation

A patient may ask for your assistance with an appeal. We encourage you to assist the patient by providing all relevant clinical records or a statement on their behalf.

We may contact you during the review and investigation of a patient's concern, complaint, appeal, or audit to request information, such as treatment records, or written statements. If treatment records are requested, you should include history, assessments, and clinical notes. You are required to cooperate and assist with the resolution and appeals process within the time periods requested to help ensure a full and fair review. This also helps ensure we are compliant with applicable laws.

Either a patient or an Evernorth representative may ask for your assistance with an appeal, quality-of-care, or quality-of-service complaint. To best address or resolve the patient's concern or appeal, we encourage timely submission of all relevant requested information.

If you believe an accelerated time frame is needed and it meets the expedited criteria, you may request an expedited appeal on behalf of the patient. An expedited review is available when:

+ The treating provider, with knowledge of the patient's behavioral condition, believes the standard time frame for processing the appeal request may jeopardize the patient's life, health, or ability to regain maximum functionality or would subject the patient to severe pain that cannot be adequately managed without the care or treatment that is the subject of the appeal.
+ There is an admission or continuing inpatient hospital stay for a patient who has received emergency services but has not been discharged from a facility.

You can contact the plan at the number on the patient's ID card to initiate the expedited appeal process and obtain filing instructions. If faxing your request, please include "expedited" in the reference field of the fax cover sheet as well as the appeal itself.

Health Insurance Portability and Accountability Act of 1996

The Health Insurance Portability and Accountability Act of 1996 (HIPAA) ensures the portability of insurance coverage to protect patients from "prior condition" limits due to changes in employment or coverage.

The HIPAA administrative simplification provisions include regulations about privacy, standard code sets and transactions, security, and unique health identifiers. They were designed to safeguard a patient's protected health information (PHI), standardize the transmission of certain common transactions between health care entities, and standardize the medical codes used in those transactions. These standardization rules help reduce health care administrative costs.

We are committed to maintaining the confidentiality of PHI and have policies and procedures to protect oral, written, and electronic PHI. Our **Notice of Privacy Practices** describes how we use and disclose PHI and advises patients of their rights under federal and state laws.

We acknowledge that certain providers may be subject to the confidentiality requirements of 42 C.F.R. Part 2 as well as certain state laws. Providers may be required to obtain certain consents and authorizations from patients in order to provide information to us. We rely on the provider subject to such laws to obtain all necessary consents and authorizations to submit information to us.

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Security regulations

The HIPAA standards for the security of electronic health information specify a series of administrative, technical, and physical security procedures for covered entities to use to ensure the confidentiality, integrity, and availability of electronic PHI.

National Provider Identifier

The National Provider Identifier (NPI) is a unique identification number for use in standard health care transactions. It is a number issued to providers and covered entities that transmit standard Health Insurance Portability and Accountability Act of 1996 (HIPAA) electronic transactions (e.g., electronic claims and claims status inquiries). Providers and covered entities may apply for NPIs through the National Plan and Provider Enumeration System established by the Centers for Medicare & Medicaid Services for this purpose.

+ Type 1 NPIs are assigned to individual providers (e.g., psychiatrists, nurse practitioners, psychologists, master-level therapists).
+ Type 2 NPIs are assigned to organizations (e.g., hospitals, home health agencies, clinics, nursing homes, residential treatment centers, laboratories, ambulance companies, group practices, pharmacies).

The NPI fulfills a requirement of HIPAA and must be used by health plans, providers, and health care electronic data interchange (EDI) vendors in HIPAA standard electronic transactions. The NPI is intended to:

+ Replace other identifiers previously used by providers and assigned by payers (e.g., Unique Physician Identification Number, Medicare or Medicaid numbers).
+ Establish a national standard and unique identifier for all providers.
+ Simplify health care system administration.
+ Encourage the electronic transmission of health care information.

Health plans accept the NPI on standard HIPAA transactions as outlined below. This approach should not be confused with any guidance specific to Medicare claims requirements.

837 electronic claims

+ The “Billing Provider” Taxpayer Identification Number (TIN) and NPI are required. Any additional provider identification on the claim, such as the “Rendering Provider” or “Referring Provider,” must include the name and NPI when submitted.
+ An organization may have more than one organization or type 2 NPI. You should use the most appropriate organizational NPI as the primary identifier when submitting the “Billing Provider” on claims. The TIN must be submitted as the secondary provider identifier. This TIN is the number used on the Internal Revenue Service Form 1099, which is either Employer Identification Number (EIN) for organizations or the Social Security number (SSN) for individuals; both an EIN and SSN should not be included concurrently. Other identifiers, such as a Medicare provider number, are considered “legacy” identifiers and should not be included.
+ Submission of the “Billing Provider” TIN on the electronic claim is a HIPAA requirement. The National EDI Transaction Set Implementation Guide specifically states: “If 'code XX - NPI' is used, then either the Employer’s Identification Number or the Social Security Number of the provider must be carried in the REF in this loop. The number sent is the one which is used on the 1099.”
+ Under HIPAA Accredited Standards Committee X12 5010 standards, “Pay to Provider” information is limited to an alternate address only. No additional identifiers, neither TIN nor NPI, are permitted. The “Pay to Provider” address is only needed if it is different from that of the “Billing Provider.”

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- We will reject electronic claims received without an NPI unless the submitter is ineligible to receive an NPI. If you are not eligible to receive an NPI, notify us by updating your demographics.
- As with any change to your billing process, if you or your organization plan to change the way claims are submitted because of your NPI implementation or enumeration, please notify us of this change. One example would be an organization that has enumerated multiple NPI subparts and will start to bill using the "new" subpart providers.

835 electronic remittance advice

Since October 2013:

- For claims paid by check or electronic funds transfer (EFT) with TIN bulking, we group the claims within the 835 remittance by the "Billing Provider" NPI submitted on the original claim(s). A Provider Summary (TS3) field is added to the 835 and includes the "Billing Provider" NPI to help providers easily reconcile their payments.
- For claims paid by EFT with NPI bulking, we send a separate 835 for each NPI with the "Billing Provider" NPI returned as the "Payee" NPI. A Provider Summary (TS3) field is also added to the 835 and will include the "Billing Provider" NPI to help providers easily reconcile their payments.
- The NPI for the "Rendering Provider" is included in the 835 regardless of bulking preference if the "Rendering Provider" NPI was submitted on the 837 electronic claim.

Real-time request transactions (270, 276, 278)

- All eligibility benefit inquiries (270) should be submitted with either a type 1 (individual) or type 2 (organizational) NPI. We will also accept a 270 submitted with a TIN.
- For professional or dental claims status inquiries (276), the "Billing Provider" or "Rendering Provider" NPI from the submitted claim should be used to inquire about the claim status.
- For institutional claim status inquiries (276), the "Billing Provider" NPI from the submitted claim should be used to inquire about the claim status.
- For all claim types, we will continue to accept claims status inquiries (276) using the TIN from the submitted claim.
- Health Care Services Review — Request for Review (278) transactions should include the NPI or TIN to identify any providers included in the request.
- Providers should contact their EDI vendor for details about the submission of NPI on these transactions.

Patient rights and responsibilities

We support informing patients of their rights and responsibilities related to the provision of care and services. As an in-network provider, you should:

- Facilitate the patient's awareness of their rights and responsibilities at their first appointment. This can be accomplished by providing a copy of the statement to the patient or displaying the document. You can copy the information that is on the **Participant Rights and Responsibilities (Appendix C) page** for this purpose.
- Offer to help the patient get more information about any of the items in the statement.
- Explain to the patient how to access services, including outside of normal business hours.
- Discuss the services available to the patient and possible charges for those services.

Note: Certain states have specific patient notification requirements applicable to mental health or substance use services or to human immunodeficiency virus status. The patient rights and responsibilities statement does not necessarily meet all state-specific requirements. State laws vary by patient, access to records, and duty to warn. Therefore, an addendum with required language must be attached to the patient rights and responsibilities statement to comply with your state-specific regulations. Please consult your legal advisor for guidance.

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Patients with the employee assistance program should be provided with the **Employee Assistance Program (EAP) Statement of Understanding Form**.

We are committed to maintaining and protecting the confidentiality of a patient's personal and sensitive information. To better understand our handling of personal health information, refer to the **Collection, Use and Disclosure of Protected Health Information (Appendix C+) page** on the Evernorth® Provider website (**Provider.Evernorth.com**).

Customer rights and responsibilities statement

The following statement of customer rights and responsibilities is given or made available to our customers.

You have the right to:

+ Receive coverage for the benefits and treatment available under your health benefit plan when you need it and in a way that respects your privacy and dignity.
+ Receive information on how to access websites or customer service via toll-free phone and fax numbers.
+ Receive language interpretation and text telephone (TTY) service upon request.
+ Receive the understandable information you need about your health benefit plan, including information about services that are covered and not covered and any costs that you will be responsible for paying.
+ Obtain understandable information about Evernorth® Behavioral Health (Evernorth) programs and services, including the qualifications of staff that support Evernorth wellness and similar programs and any contractual relationships related to such programs.
+ Have access to current information on in-network providers and places you can receive care and information about a particular health care provider's training and practice.
+ Select a primary care provider (PCP) for yourself and each covered member of your family and change your PCP for any reason. However, many benefit plans do not require that you select a PCP.
+ Have your personal identifiable data and medical information kept confidential by Evernorth and your health care provider; know who has access to your information; and know the procedures used to ensure security, privacy, and confidentiality. Evernorth honors the confidentiality of its customers' information and adheres to all federal and state regulations about confidentiality and the release of personal health information.
+ Participate with your health care provider in health decisions and have them give you information about your condition and your treatment options, regardless of coverage or cost. You have the right to receive this information in terms and language you understand.
+ Learn about any care you receive. You should be asked for your consent for all care, unless there is an emergency and your life and health are in serious danger.
+ Refuse medical or behavioral health care. If you refuse care, your health care provider should tell you what might happen. We urge you to discuss your concerns about care with your health care provider. Your health care provider will give you advice, but you will have the final decision.
+ Be advised of who is available to assist you with any special Evernorth programs or services you may receive and who can assist you with any requests to change or unenroll from programs or services offered by or through Evernorth.
+ Be heard. Our complaint-handling process is designed to hear and act on your complaint or concern about Evernorth or the quality of care you receive from health care providers and the various places you receive care in our network; provide a courteous, prompt response; and guide you through our grievance process if you do not agree with our decision. Evernorth strives to resolve your complaint on initial contact and in a manner that is consistent with your applicable benefit plan. Language interpretation and TTY services are available for complaint and appeal processes.
+ Be treated with respect and recognition of your dignity and right to privacy.

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- Know and make recommendations about our policies that affect your rights and responsibilities. If you have recommendations or concerns, please call customer service at the toll-free number on your ID card.
- Request and receive information regarding how to appeal a utilization management decision.
- Receive utilization management determinations from quality professionals who do not receive financial incentives based on utilization management decisions.

You have the responsibility to:

- Review and understand the information you receive about your health benefit plan. Please call customer service when you have questions or concerns.
- Understand how to obtain services and supplies that are covered under your plan, including any emergency services needed outside of normal business hours or when you are away from your usual place of residence or work, by using the indicated number on your ID card or by accessing online resources.
- Show your ID card before you receive care.
- Schedule a new patient appointment with any in-network health care provider, build a comfortable relationship with your health care provider, ask questions about things you do not understand, and follow your health care provider's advice. You should understand that your condition may not improve and may even get worse if you do not follow your health care provider's advice.
- Understand your health condition and work with your health care provider to develop treatment goals that you both agree on and to follow the treatment plan and instructions.
- Provide honest, complete information to us and the health care providers caring for you.
- Know what medicine you take, why you take it, and how to take it.
- Pay all copays, deductibles, and coinsurance for which you are responsible at the time service is rendered or when they are due.
- Keep scheduled appointments or notify the health care provider's office ahead of time if you are going to be late or miss an appointment.
- Pay all charges for missed appointments and for services that are not covered by your plan.
- Voice your opinions, concerns, or complaints to customer service or your health care provider.
- Notify your plan administrator and treating health care provider as soon as possible about any changes in family size, address, phone number, or status with your health benefit plan or if you decide to unenroll from our programs and services.

Statement on confidentiality of alcohol and drug use records

Our staff and providers in our network will not identify a patient as involved in alcohol or substance use treatment to others outside the treatment program, unless one of the following applies:

- The patient consents in writing.
- The disclosure is allowed by court order.
- The disclosure is made to medical personnel in a medical emergency or to qualified personnel for research, audit, or program evaluation.
- The disclosure is made to a primary care provider to coordinate care when required by a health plan and the patient consents verbally or in writing.
- The patient commits or threatens to commit a crime at the treatment program or against any person who works for the program.
- There is suspected child abuse or neglect or a danger to the patient or others when reporting is permitted or required under state laws to appropriate state or local authorities.

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Prescription drug program

For psychiatrists and prescribers (e.g., nurse practitioners) only

A patient's medical plan offers a prescription drug benefit program where administered pharmacy benefits generally require that the patient fills prescription medications from the plan's in-network pharmacies or through the plan's home delivery pharmacy. Medications are dispensed per prescription order or refilled in quantities normally prescribed up to a 30-day supply or as defined by the plan, the U.S. Food and Drug Administration, or applicable law. Up to a 90-day supply of maintenance medication may be dispensed through the home delivery pharmacy or obtained from designated retail pharmacies if the patient's benefit plan allows for a 90-day supply at retail.

Brand-name medications are often substituted with generic equivalents when available. Patients who choose to fill a brand-name medication rather than its generic equivalent may be subject to higher out-of-pocket costs.

Prescription drug lists

For physicians and other providers only

For patients with Cigna Healthcare® plans, the prescription drug list is a list of generic and brand-name medications. All medications are approved by the U.S. Food and Drug Administration (FDA). You may access the prescription drug list by logging in to the Evernorth® Provider portal (**Provider.Evernorth.com**) and going to Resources > Drug List or by calling the number on the back of the patient's ID card.

Cigna Healthcare regularly updates the drug list to reflect any changes to covered prescription medications.[1] Changes are typically made on January 1 and July 1 of each year to ensure customers have access to safe, effective, and lower-cost medications. As a result of these changes, certain medications may change tiers or certain high-cost medications may no longer be covered. Cigna Healthcare encourages the use of lower-cost generic or preferred-brand alternatives, when appropriate.

In addition, Cigna Healthcare may not cover medications that are newly approved by the FDA for the first six months after approval to allow time for a thorough clinical review. This includes medications, medical supplies, or devices that are covered under standard pharmacy benefit plans. If currently available medications/therapies are not appropriate, providers can request coverage through a medical necessity review process during this interim review period.

1. The coverage status of a prescription medication may change periodically for various reasons. For example, a prescription medication may be removed from the market, a new prescription medication in the same therapeutic class as a prescription medication may become available, or other market events may occur. Market events, such as an increase in acquisition cost of a prescription medication, may affect the coverage status of a prescription medication. As a result of coverage changes, benefit plans may require customers to pay more or less for the prescription medication, obtain the prescription medication from a certain pharmacy), or try another covered prescription medication first.

Medications requiring prior authorization (medical necessity request process)

In-network prescribers are responsible for following the Cigna Healthcare® prescription drug list. You are required to contact the pharmacy service center to request prior authorization if any of the following apply:

+ A generic or preferred medication should not be prescribed in your medical judgment for a patient.
+ The patient has a closed-formulary benefit plan.
+ The medication is not covered on the Cigna Healthcare drug list.
+ The prescribed medication requires prior approval of coverage.

When your treatment plan includes certain prescription drugs, your patient's pharmacy benefit may require that you request prior authorization. You can speed up this process by requesting it electronically.

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Electronic prior authorization can save you time by eliminating the forms, faxes, and calls associated with manual requests and can provide you with faster coverage determinations. As an added benefit, your patients will have faster access to their drug therapy, which may improve medication adherence.

You have several options for submitting prior authorization requests:

+ **Electronically (preferred):** Visit the **CoverMyMeds®** website or use Surescripts® via your electronic health record or electronic medical record system.
+ **Fax:** Send to **800.390.9745** (pharmacy benefit) or **855.840.1678** (medical benefit).
+ **Phone:** Call **800.88Cigna (882.4462)**.

Information fields must be complete and legible on the submitted request. Incomplete forms will be denied or returned for illegible or missing information. Requests can be marked as urgent for an expedited review. Urgent is defined as: waiting for a decision could jeopardize the patient's life, health, or ability to regain maximum function or the patient is in the office/facility now for infusion/injection.

Prior authorization forms are available on the Evernorth® Provider website (**Provider.Evernorth.com**).

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Quality Management program

Our Quality Management program supports our goal of continuous improvement in the quality of care and services delivered to your patients. We are accredited by the National Committee for Quality Assurance (NCQA) as a managed behavioral health organization and by URAC® as a health utilization management organization.

Additionally, the Quality Management program is designed to fulfill managed care organization standards and support health plan customers in their accreditation through NCQA and URAC.

We value the input and involvement of providers in our Quality Management program, and we communicate program results annually in the **Provider Newsroom**. If you would like to receive more information, become more involved, or offer feedback and suggestions concerning the Quality Management program, please contact Provider Services at **800.926.2273**.

Creating opportunities for quality improvement

We review and monitor data about many aspects of our operations to assess performance and create improvements in patient care and service. Examples of data that may be collected and measured quarterly or annually include:

+ Provider satisfaction surveys.
+ Patient satisfaction surveys.
+ Provider treatment record audits.
+ Complaint and appeal analysis and trending data.
+ Utilization data.
+ Compliance with our appointment access, provider and provider geographical availability, and phone access standards.
+ Healthcare Effectiveness Data and Information Set metrics.
+ Follow-up visits after hospitalization. (**Note:** Typically, all discharged patients should be seen within seven working days of discharge.)

Coordination of behavioral health and medical care

It is our policy that behavioral health services must be closely coordinated with general medical care and between behavioral health providers when care is delivered simultaneously by more than one provider, program, or facility. To enable the sharing of information between providers as treatment proceeds, we suggest that providers request a required release of information signature from the patient during the treatment process, even if the patient initially declines permission for communication between providers. For the coordination of medical and behavioral care especially, this policy reflects our understanding of the complex interrelationship between emotional and physical factors and our appreciation for the fact that psychiatric problems often complicate or present as medical illnesses. Conversely, medical problems may present with psychiatric symptoms.

A **sample release form** and a **draft letter** for structured communication with primary care providers (PCPs) is available on the Evernorth® Provider website (**Provider.Evernorth.com**).

For each consenting patient in simultaneous medical and behavioral health treatment, we expect providers to:

+ Obtain and document the PCP's name, address, phone number, and fax number (if available).
+ Obtain and document a release of information, as required by law, to exchange information between the provider and PCP. If the patient refuses to sign a release form, the reason for refusal should be documented.

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+ Exchange appropriate clinical information directly with the PCP in an effective and timely manner throughout the treatment regimen. With proper consent, facilities may contact the PCP upon admission and discharge. At a minimum, communication should occur:
    - After the initial assessment, to include diagnosis, medication, initial treatment plan, and diagnostic tests recommended or ordered.
    - When a patient is not compliant with treatment recommendations.
    - When a patient's condition is unstable.
    - When there are clinically significant changes in a patient's condition or level of care, including, but not limited to, inpatient, partial hospitalization, intensive outpatient treatment, comorbid medical and behavioral conditions, or new or substantial medication changes.
    - At the completion of behavioral treatment.
+ Document verbal and written communication with the PCP.

We monitor a provider's compliance with coordinating behavioral health and medical care through medical record reviews and care management reviews as part of the recredentialing process and the Quality Management program.

Health promotion/preventive health services

We encourage providers to educate their patients about their diagnosis, treatment information, and suitable health promotion strategies. This may include providing literature, suggesting books, or informing the patient of other educational resources and self-help groups. As appropriate, the provider should recommend the use of preventive strategies that may include relapse prevention, stress management, wellness programs, and lifestyle changes. In addition, providers should document their efforts in the patient's medical record.

We offer consumer-oriented educational materials on a variety of topics, including attention deficit hyperactivity disorder (ADHD). The intent of our ADHD materials is to educate and empower caregivers, reduce the impact of problems commonly associated with the disorder, and encourage early detection of ADHD in siblings.

Educational information and tools are mailed to parents and guardians, when requested, to improve their understanding of and their ability to manage their child's condition and to encourage coordination and consistency of response across medical, behavioral, educational, family, and other social settings.

Initial and annual screening for depression, stress, and anxiety is now routine within Cigna Healthcare® disease management programs, encouraging the identification and treatment of behavioral health disorders that may occur as a coexisting condition with a medical disease. Those who screen positive are provided with intensive care management and ongoing standardized protocols for assessment and intervention are applied until the behavioral health condition remits. Where consent is provided, additional materials and support are made available to the patient's medical provider or a referral for treatment with a qualified behavioral health provider can be arranged.

See the **Behavioral Health Resources and Education Center** for the latest information on these services or contact us at **800.926.2273**.

Educational opportunities

You are encouraged to participate in educational programs focusing on clinical practice issues pertinent to service delivery systems.

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Medical necessity criteria

MCG Behavioral Health Guidelines

On November 27, 2020, Cigna terminated use of its Standards and Guidelines/Medical Necessity Criteria for Treatment of Mental Health Disorders and transitioned to the **MCG Behavioral Health Guidelines**. For regulatory and reference purposes, applicable versions of Cigna's former Standards and Guidelines/Medical Necessity Criteria for Treatment of Mental Health Disorders above will be used throughout the duration of cases that were submitted to Cigna prior to November 27, 2020. These cases include:

+ Prior authorization requests.
+ Continued care requests/concurrent reviews.
+ Appeals where the Cigna Healthcare document is used in initial determinations.

We use the **MCG Behavioral Health Guidelines** for guidance in conducting mental health level of care medical necessity reviews for health plan business unless contractual requirements or federal or state law requires the use of other clinical criteria. The MCG Behavioral Health Guidelines have wide acceptance as an evidence-based standard for mental health care; are developed and maintained in compliance with state and federal regulations, including mental health parity laws; and are informed by consideration of guidance issued by at least 10 professional organizations.

LOCUS and CALOCUS-CASII criteria

We adopted the **Level of Care Utilization System (LOCUS) criteria** for ages 19 and older and the **Child and Adolescent Level of Care/Service Intensity Utilization System (CALOCUS-CASII) criteria** for ages 6 through 18 in compliance with California Senate Bill 855 and New York State Office of Mental Health's Part 514 Regulations.

LOCUS and CALOCUS-CASII criteria are accepted as evidence-based standards for mental health care. These criteria are used to conduct mental health level of care medical necessity reviews for commercial health plans in California and New York, unless federal or state law requires the use of other specifically identified clinical criteria. These guidelines complement our suite of existing evidence-based criteria to support your clinical judgment and decision-making processes.

Additional resources

In addition to the MCG Behavioral Health Guidelines, LOCUS criteria, and CALOCUS-CASII criteria, we use the following resources to make medical necessity determinations when appropriate:

+ **The ASAM Criteria®** (developed by the American Society of Addiction Medicine; used for guidance in conducting medical necessity reviews of substance use disorder levels of care).
+ **Evernorth® Behavioral Health and Cigna Healthcare® coverage policies** (for autism spectrum disorders, pervasive developmental disorders, and transcranial magnetic stimulation).
+ **Evernorth Behavioral Health Prior Authorization and Billing Resource Guide**.
+ State and federal regulations and licensing standards. (**Note:** We comply with state-specific laws, as written.)

Clinical practice guidelines

We review and adopt clinical practice guidelines from professional societies and other recognized sources, such as the American Psychiatric Association, the American Academy of Pediatrics, and the National Institute on Alcohol Abuse and Alcoholism. Our care management staff use these guidelines in their work with providers to guide decisions about the

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appropriate treatment for common behavioral disorders. The conclusions expressed within these guidelines are based on scientific, evidence-based research. More information about the **clinical practice guidelines** is available on the Evernorth® Provider website (**Provider.Evernorth.com**).

Through the Quality Management program, we monitor whether treatment is consistent with selected guidelines that we have adopted. Feedback, suggestions, and input regarding clinical practice guidelines are always welcome and can be provided to your provider relations representative or to Provider Services at **800.926.2273**.

Clinical screening tools and treatment support toolkits

We have assembled a number of validated screening and assessment tools and, where necessary, obtained permissions from copyright holders for distribution to providers in our network. These tools are available on the **Clinical Practice Tools page** on the Evernorth® Provider website (**Provider.Evernorth.com**).

Additionally, patient and provider toolkits containing educational and treatment support tools are available on the **Primary Physician Tool page** on **Provider.Evernorth.com**.

Integration with chronic condition management

Our chronic condition management program is a whole-person solution focused on helping your patients who have one or more chronic conditions. The program uses an integrated multidisciplinary approach, supported by evidence-based medical guidelines and influential behavioral techniques, to help patients fully manage their personal health. This includes adhering to medication regimens, understanding and managing risk factors, and maintaining up-to-date screenings.

For more information, go to the **Chronic Condition Management page** on the Evernorth® Provider website (**Provider.Evernorth.com**) or contact us at **855.246.1873**.

Treatment records

We believe well-documented treatment records, whether electronic or paper, facilitate communication, coordination, and continuity of care and promote the efficiency and effectiveness of treatment. The provider is responsible for maintaining an adequate clinical record for each patient and furnishing us with clinical data as necessary for utilization review or quality management. Our recordkeeping standards require the patient's name and ID number on each page in the record. Treatment record entries should be legible, signed with the clinician's name and credentials in ink, dated, and maintained in a consistent chronological order within each file. Records should be easily and readily retrievable in a secure environment that protects patient confidentiality.

Treatment record review standards are based on state mandates, when applicable, and Centers for Medicare & Medicaid (CMS) requirements about the treatment of patients with Medicare coverage. In states where there are laws regulating the recordkeeping process, these laws shall prevail if greater than our minimum standards; if not, our standards shall apply.

Documentation in the provider's treatment record for a patient should include, but is not limited to:

+ Key demographic data.
+ The presenting problem (reason for visit).
+ A full psychological and medical history.
+ A mental status evaluation, including assessment of suicidal and homicidal ideation.
+ International Classification of Diseases, 10th Revision, Clinical Modification diagnosis.

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+ Treatment plan with measurable goals.
+ All diagnostic and treatment services ordered or provided, directly or through referral.
+ Coordination of care information, including evidence of coordination with the primary care providers (PCPs) and other clinicians involved in the patient's care and the patient's consent for release of information.
+ CMS requirements, including whether advanced directives were executed and whether the PCP has a copy of the advanced directives.

We also encourage you to document attempts to coordinate care with a patient's PCP and any discussions in which you encourage a patient without a PCP to select a PCP. Our **Behavioral Treatment Record Review Tool** can be used as a resource to assist in treatment record review.

To assess compliance with our record standards, we audit the recordkeeping practices of select high-volume providers. These providers are near recredentialing and have seen patients in the prior 12 months. We send a letter to these providers soliciting blinded copies of three clinical records. We use the audit results to give providers feedback (particularly when results are below the 80 percent performance goal) and drive organizational quality improvement.

Quality treatment recordkeeping supports treatment outcomes through improved treatment planning, the monitoring of patient progress toward goals, and improved communication in the case management process. It also may reduce risk management difficulties for providers by providing a record of the treatment progress along with **documentation of informed consent**, the patient's understanding of their rights and responsibilities, and the patient's understanding of the treatment plan.

Clinical and quality improvement studies

This information pertains to physicians and other providers.

Clinical and quality improvement studies help evaluate quality and appropriateness of care provided to patients. Topics for evaluation and special studies are chosen based on relevant demographics and epidemiological characteristics of patients. Clinical studies review issues such as preventive and chronic care/Healthcare Effectiveness Data and Information Set measures against preventive care guidelines and compliance with treatment standards for depression and other behavioral health diagnoses. Scientifically based criteria, which we adopted from nationally recognized organizations, are used for specific conditions. Population-based assessment is conducted whenever appropriate, supplemented by focused medical record review or patient surveys. Data are collected, reviewed, and analyzed for trends and opportunities for improvement.

National Quality Forum

The National Quality Forum (NQF) was established to facilitate a national collaboration to improve health and health care quality through measurement. They strive to achieve this mission by convening key public- and private-sector leaders to establish national priorities and goals to achieve health care that is safe, effective, patient-centered, timely, efficient, and equitable. NQF-endorsed measures are considered the gold standard for health care quality measurement in the United States. Expert committees that are comprised of various stakeholders, including patients, providers, and payers, evaluate measures for NQF endorsement. The federal government and many private sector entities use NQF-endorsed measures above all others because of the rigor and consensus process behind them. We encourage all providers to become familiar with the endorsed measures to promote public accountability and quality improvement. Many of the measures are used in our evaluation process for hospitals and providers. More information is available at on the **NQF website**.

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Centers of Excellence

When patients need behavioral health care, we encourage providers to refer them to facilities with the Evernorth® Behavioral Centers of Excellence (COE) designation. These facilities have met certain measures for patient outcomes and cost-efficiency based on the methodology outlined below.

The patient outcomes score is a quality measure of a facility's relative effectiveness in treating a mental health, substance use, or eating disorder. The measures are evaluated at the national level to ensure consistency in care standards across all facilities.

Facilities that offer inpatient and/or residential levels of care are assessed on the quality measures listed below:

+ Seven-day ambulatory follow-up.
+ Sustained follow-up.
+ Readmission rates.
+ Emergency department admissions.

The cost-efficiency score is a measure of a facility's average cost to treat a particular mental health, substance use, or eating disorder. The cost metrics are assessed at the regional level to account for geographic variations in pricing structures and reimbursement rates and are based on a facility's Evernorth fee schedule and average length-of-stay data. It does not include provider fees or outpatient services. The score reflects the rates that a facility charges as well as the average time spent in the facility for the specific treatment. A variety of factors, including geographic cost differences, may affect the overall score.

A facility can receive a score of up to three stars (*) for both patient outcomes and cost-efficiency measures for each evaluated mental health, substance use, or eating disorder level of care. Those that attain at least five stars (three stars for patient outcomes and two stars for cost-efficiency or three stars for cost-efficiency and two stars for patient outcomes) receive the COE designation. There are four COE designations available for inpatient/residential and partial hospitalization program/intensive outpatient program facilities: mental health, substance use disorders, child and adolescent mental health, and eating disorders.

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Resources

Our resource library provides access to important forms and information:

+ **Contact Information (Appendix A)**
+ **Claims Information (Appendix B)**
+ **Participant Rights and Responsibilities (Appendix C)**
+ **Collection, Use and Disclosure of Protected Health Information (Appendix C+)**
+ **Health Care Provider Forms (Appendix D)**
+ **Employee Assistance Program (EAP) Forms (Appendix E)**
+ **Specialty Networks and Forms (Appendix F)**
+ **Primary Care Physician (PCP) Communication Information**
+ **Standards for Delegation of Credentialing Activities**

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ALABAMA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Alabama regarding provider contracts with providers rendering health care services in the State of Alabama. To the extent that such Alabama laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Alabama laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

I

Payor may only retroactively deny, adjust or seek recoupment for reimbursement made to Provider (a) during the twelve (12) month period after the date Payor paid such claim; or (b) the expiration of the same period of time that Provider is required to submit claims pursuant to Provider's contract, whichever date occurs first. Except that Payor may retroactively deny, adjust or recoup reimbursement for Covered Services subject to coordination of benefits with another carrier, the Alabama Medicaid Agency, or to Medicare claims, except the Medicare+Choice plan, during the eighteen (18) month period after the date Payor paid such claim.

(a.) The above provision does not apply if Payor retroactively denies, adjusts or recoups reimbursement to Provider because: (1) the information submitted to Payor was fraudulent; or (2) the claim submitted to Payor was a duplicate claim.

(b.) Payor must give Provider notice specifying the reason for retroactively denying, adjusting or recouping reimbursement. Notice may be in paper or electronic format, but Provider must agree to accept notice by electronic media.

(c.) If Provider disputes the retroactive denial, adjustment, or recoupment on all or a portion of a claim, Provider must notify Payor within thirty (30) days after receipt of notice.

(d.) If the retroactive denial, adjustment or recoupment deals with a medical necessity determination, level of service determination, coding error, or billing irregularity, retroactive denial, adjustment or recoupment must be reconciled to specific claim.

II

The following is provided in accordance with Act No. 2015-320 (Code of Alabama § 27-1-17.1).

IF A COVERED HEALTH CARE PROVIDER REQUESTS PAYMENT UNDER A HEALTH INSURANCE PLAN FROM A HEALTH INSURER OR ITS CONTRACTED VENDOR OR A REGIONAL CARE ORGANIZATION BE MADE USING ACH ELECTRONIC FUNDS TRANSFER, THAT REQUEST MUST BE HONORED. FURTHERMORE, SUCH A REQUEST

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MAY NOT BE USED TO DELAY OR REJECT A TRANSACTION, OR ATTEMPT TO ADVERSELY AFFECT THE COVERED HEALTH CARE PROVIDER.

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ALASKA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as the "Provider") to comply with legislative and regulatory requirements of the State of Alaska regarding provider contracts with providers rendering health care services in the State of Alaska. To the extent that such Alaska laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Alaska laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) Emergency Services. The term "Emergency Services" means health care services that are provided by a hospital or other emergency facility after the sudden onset of a medical condition that manifests itself by symptoms of sufficient severity, including severe pain, that the absence of immediate medical attention would reasonably be expected by a prudent person who possesses an average knowledge of health and medicine to result in the placing of the Participant's health in serious jeopardy, or a serious impairment to bodily functions, or, a serious dysfunction of a bodily organ or part.

(2) Clean Claim. The term "Clean Claim" means a claim that does not have a defect, impropriety or circumstance requiring special treatment that precludes timely payment on the claim.

(3) Termination. To the extent that the Agreement contains a provision allowing discretionary termination of the Agreement, such provision shall apply equitably to both Provider and Evernorth.

(4) Dispute Resolution. In the event of a dispute between Evernorth and Provider, a fair, prompt and mutual dispute resolution process shall be used. Provider and Evernorth will hold an initial meeting at which Provider and Evernorth are present or are represented by individuals with authority regarding the matters in dispute. The meeting shall be held within ten (10) working days after Evernorth receives written notice of the dispute or gives written notice to Provider, unless Provider and Evernorth agree in writing to a different schedule. If, within thirty (30) days following the initial meeting, Evernorth and Provider have not resolved the dispute, the dispute shall be submitted to mediation directed by a mediator who is mutually agreeable to Evernorth and Provider and who is not regularly under contract to or employed by either Evernorth or Provider. Each party shall bear its proportionate share of the cost of mediation, including the mediator fees. If, after a period of sixty (60) days following commencement of mediation, Evernorth and Provider are unable to resolve the dispute, either party may seek other relief allowed by law. Evernorth and Provider agree to negotiate in good faith at the initial meeting and in mediation. Prior to the initiation of the mutual dispute resolution process set forth herein, at the Provider's discretion, and to the extent permitted by Alaska Stat. § 21.07.010, Evernorth and Provider may attempt to resolve payment disputes in accordance with the payment dispute resolution procedures set forth in the Medical Management Program.

(5) Communication with Participants. In accordance with Alaska Stat. § 21.07.010(a)(5), Evernorth shall not penalize Provider or terminate the Agreement because Provider acts as an advocate of a Participant in seeking appropriate, medically necessary health care services. Evernorth shall not interfere with Provider's ability to openly communicate with a Participant about all appropriate diagnostic testing and treatment

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options.

(6) Financial Inducements or Incentives. In accordance with Alaska Stat. § 21.07.010(b)(1), this Agreement shall not be interpreted to contain direct financial incentives to Provider for withholding Covered Services that are medically necessary. Nothing herein, however, shall be construed to prohibit incentives to Provider for efficient management of the utilization and cost of Covered Services.

(7) Product Participation and Compensation. Nothing in this Agreement shall be interpreted to require Provider to contract for all products currently offered or that may be offered in the future by Evernorth. Nothing in this Agreement shall be interpreted to require Provider to accept from Evernorth or Payor the same rate of compensation for Covered Services rendered as that which Provider has contracted for with another managed care entity.

(8) Indemnification. In accordance with Alaska Stat. § 21.07.010(c), nothing in this Agreement shall require Provider to indemnify or hold harmless Evernorth for the acts or conduct of Evernorth.

(9) Effect of Termination and Liabilities After Termination. If a Participant is pregnant or being actively treated by Provider on the date of termination of this Agreement, the Participant may continue to receive Covered Services from Provider as described herein and this Agreement shall remain in force with respect to the continuing treatment. The Participant shall be treated for the purposes of benefit determination or claim payment as if Provider were still under agreement with Evernorth. However, treatment is required to continue only while the Participant's coverage remains in effect and until the end of the medically necessary treatment for the condition, disease, illness or injury, if the Participant has a terminal (life expectancy of less than one (1) year) condition, disease, illness or injury or for the period that is the longest of the following: the end of the current plan year; up to ninety (90) days after the termination date, if the event triggering the right to continuing treatment is part of an ongoing course of treatment; or, through completion of postpartum care, if the Participant is pregnant on the date of termination.

(10) Claims and Overpayment Recovery. To the extent required by applicable law, Evernorth and Provider shall comply with the provisions of Alaska Stat. § 21.36.495, as may be amended from time to time. The recovery of overpayments, if any, shall be conducted in accordance with Alaska Stat. § 21.36.125, as may be amended from time to time, and Bulletin B 07-06 as applicable.

(11) Covered Services. Upon request, Evernorth shall make available to Provider information that identifies Covered Services.

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ARIZONA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Arizona regarding provider contracts with providers rendering health care services in the State of Arizona. To the extent that such Arizona laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context indicates otherwise. For the avoidance of doubt, to the extent such Arizona laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

1. The Compensation section of the Agreement is amended to add the following subsections:

    a. In the case of a non-Medicare HMO plan offered by Evernorth or an insured Plan offered by an Evernorth affiliate, and except in cases of fraud, Evernorth will adjust or request adjustment of the payment of a claim within one (1) year after the date Evernorth has paid the claim, unless the parties agree to a mutually applicable longer period of time.

    b. The following applies in the event that Provider believes Provider has been underpaid for a Covered Service. In the case of a non-Medicare HMO plan offered by Evernorth or an insured Plan offered by an Evernorth affiliate, and except in cases of fraud, Provider must submit a written request for an appeal or adjustment with Evernorth or its designee within one (1) year from the date of Evernorth's payment or explanation of payment, unless the parties agree to a mutually applicable longer period of time. In the case of a self- insured Plan administered by an Evernorth affiliate, Provider must submit a written request for an appeal or adjustment with Evernorth or its designee within one hundred eighty (180) days from the date of Payor's payment or explanation of payment. All requests for appeal or adjustment must be submitted in accordance with Evernorth's provider payment appeal process set forth in the Medical Management Program. Requests for appeals or adjustments submitted after the date specified may be denied for payment, and Provider will not be permitted to bill Evernorth, Payor or the Participant for those services for which payment was denied.

2. In the event of Evernorth's insolvency, Provider shall continue to provide Covered Services to Participants covered under an HMO Plan at the same rates and subject to the same terms and conditions established in the Agreement, until the earliest of the following:

    a. A determination by the court that Evernorth is either unable to pay, or unable to provide adequate assurance that it will be able to pay, Provider's claims for Covered Services that were rendered to Participants after Evernorth is declared insolvent.

    b. A determination by the court that continuation of the Agreement would constitute undue hardship to Provider.

    c. A determination by the court that Evernorth has satisfied its obligations to all Participants under

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its HMO Plans.

3. Credentialing: Evernorth is part of the Council for Affordable Quality Healthcare (CAQH) for credentialing and recredentialing network physicians. Evernorth strongly encourages the use of the CAQH Universal Provider DataSource when submitting Provider's application. CAQH does comply with Arizona state application requirements.

4. Appointments and Scheduling Guidelines:

   a. Specialist Access Standards:

      - Urgent or emergent care – call 911
      - Urgent or emergent care that does not require 911 – immediate or same day appointment
      - Non-Urgent or emergent care – within one (1) week (seven (7) days)
      - Elective care (routine) – within thirty (30) days
      - High index of suspicion of malignancy – Less than seven (7) days

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ARKANSAS REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Arkansas regarding provider contracts with providers rendering health care services in the State of Arkansas. To the extent that such Arkansas laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Arkansas laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

I

(1) Pursuant to the requirements of A.C.A. § 23-63-1801 et seq.:

(a) Except in cases of fraud committed by Provider, Payor may only exercise recoupment for reimbursement from Provider during the eighteen (18) month period after the date that Payor paid the claim submitted by Provider.

(a.1) Provider may submit a corrected claim for up to six (6) months after recoupment for services that were actually provided but billed in error without intent to defraud.

(b) If Payor exercises recoupment from Provider under this section, Payor shall give Provider a written or electronic statement specifying the basis for the retroactive denial and the statement shall contain, at a minimum, the information required by subsection (e) below.

(c) If Payor determines that payment was made for services not covered under Participant's Plan, Payor shall give written notice to Provider of its intent to exercise recoupment and may:
(1) Request a refund from Provider; or
(2) Make a recoupment of the payment from Provider in accordance with subsection (e).

(d) Notwithstanding subsection (a) above, if Payor or an agent contracted to provide eligibility verification verifies that an individual is a Participant and if Provider provides health care services to the individual in reliance on such verification, Payor may not thereafter retroactively deny a claim on the basis that the individual is not a Participant unless such retroactive denial occurs within one hundred twenty (120) days of the date that Payor paid the claim; otherwise Payor is barred from making such recoupment unless there was fraud by Provider.

(e) If Payor chooses to recoup from Provider amounts previously paid under a retroactively denied claim pursuant to subsections (a) or (c), Payor shall provide Provider written documentation that specifies:

(1) The amount of the recoupment;

(2) The person's name to whom the recoupment applies;

(3) Patient identification number;

(4) Date or dates of service;

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(5) The health care service or services on which the recoupment is based;

(6) The pending claims being recouped or that future claims will be recouped; and

(7) Specific reason for the recoupment.

(2) Upon termination of the Agreement, Provider may, at the option of Participant, continue to provide Covered Services for a current episode of an acute condition for which a Participant was under Provider's care at the time of such termination so long as Participant retains eligibility under a Plan, until the earlier of completion of such services, or the expiration of ninety (90) days. During the period of continued care, Provider shall be deemed to be a Participating Provider for purposes of reimbursement, utilization management and quality of care. Provider shall be compensated for Covered Services provided to any such Participant in accordance with the compensation arrangements of the terminated Agreement until ninety (90) days following termination. Provider has no obligation under the Agreement to provide services to individuals who cease to be Participants.

(3) The Agreement may permit network rental arrangements which allow Evernorth to sell, lease, assign, convey, or otherwise grant access to Provider's health care services, discounted rates, or fees established in the Agreement.

(4) To the extent required by A.C.A. § 23-99-1305, as may be amended from time to time:

(a) "Material Amendment" shall mean, as set forth in A.C.A. § 23-99-1302 as may be amended from time to time: a change in the Agreement that results in a decrease in fees, payments, or reimbursement to Provider; a change in the payment methodology for determining fees, payments or reimbursement to Provider; a new or revised coding guideline; a new or revised payment rule; or, a change of procedures that may reasonably be expected to significantly increase Provider's administrative expenses.

(b) Evernorth shall provide in writing any Material Amendment to the Agreement at least ninety (90) days before the effective date of the Material Amendment.

(5) Notwithstanding any provision in the Agreement to the contrary, the Agreement shall open for renegotiation and revision at least one (1) time every three (3) years; neither party is required to terminate the Agreement in order to open the Agreement for renegotiation of the terms.

II

(1) To the extent required by A.C.A. § 23-99-1306, nothing in the Agreement or in any exhibit, schedule, or addendum to the Agreement, shall be construed to directly or indirectly prohibit Provider from entering into a healthcare contract with another contracting entity or another healthcare provider. Subject to the severability provision of the Agreement, if any provision may be so construed, such provision shall be null and void and the Agreement shall be construed so as not to contain such provision.

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CALIFORNIA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc., Evernorth Behavioral HealthCare of California, Inc, Evernorth Group Life Insurance Company (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of California regarding provider contracts with providers rendering health care services in the State of California. To the extent that such California laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such California laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

I

1. Language Assistance Program (LAP) Requirements. The following provision is required by (i) CA Health and Safety Code Section 1367.04 and Title 28 California Code of Regulations Section 1300.67.04 with respect to services rendered by Provider to Participants covered by Evernorth Behavioral HealthCare of California, Inc. and (ii) CA Insurance Code Section 10133.8 and Title 10 California Code of Regulations Section 2538.3 with respect to services rendered by Provider to Participants insured by Evernorth Health and Life Insurance Company ("CHLIC").

   (a) Evernorth shall establish and maintain an ongoing language assistance program to ensure Limited English Proficient ("LEP") Participants have appropriate access to language assistance while accessing health care services as required by the LAP laws referenced above. Provider shall cooperate and comply, as applicable, with Evernorth's Language Assistance Program; however, Evernorth shall maintain ongoing administrative and financial responsibility for implementing and operating on an ongoing basis the Language Assistance Program for Participants. The term "Limited English Proficient" shall have the same meaning as set forth in the LAP laws cited above.

2. Provider Directory Requirements:

On July 1, 2016, California Senate Bill 137 became effective for managed care and insured benefit plans. Its purpose is to improve the accuracy of provider directories, as well as support future requirements that will become effective next year for content standardization and new search criteria.

The new law requires that for the directories for these plans, all health plan insurers:

- Develop and implement procedures to periodically contact contracted providers to validate the accuracy of their information displayed, and keep this information current. Provider verification may be required several times per year, depending on contracting arrangements.
- Display certain fields of information, including the National Provider Identifier (NPI) number, California license number(s), an office email address, non-English languages you may speak, and if you have a qualified medical interpreter on staff.
- Display all contracted providers.

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The following provision is required by (i) CA Health and Safety Code Section 1367.27 with respect to services rendered by Provider to Participants covered by Evernorth HealthCare of California, Inc. and (ii) CA Insurance Code Section 10133.15 with respect to services rendered by Provider to Participants insured by CHLIC.

(A) Provider shall inform Evernorth within five (5) business days when either of the following occurs:
(1) Provider is not accepting new Participants;
(2) If Provider had previously not accepted new Participants, Provider is currently accepting new Participants.
(B) If Provider is not accepting new Participants and is contacted by an Evernorth Participant or potential customer; provider shall direct the individual to:
(1) Evernorth Customer Service for assistance in finding another provider.

(2) the California Department of Managed Care or the Department of Insurance if Provider's accepting new patients status in the provider directory is inaccurate, or does not reflect the information most recently provided to Evernorth. Provider will refer the Participant or potential customer to the department with jurisdiction over the Participant's or potential customer's plan to report any inaccuracy with the provider directory or directories.

Pursuant to the obligations established by (i) CA Health and Safety Code Section 1367.27 with respect to services rendered by Provider to Participants covered by Evernorth HealthCare of California, Inc. and (ii) CA Insurance Code Section 10133.15 with respect to services rendered by Provider to Participants insured by CHLIC, the following shall apply:
(A) Providers will have thirty (30) business days to respond to all notices sent to them by Evernorth or a group or entity on behalf of Evernorth, and either confirm their directory information is current and accurate, or otherwise, update their directory information. Provider verification may be required several times per year, depending on contracting arrangements.
(B) If Provider is not a hospital:

- Providers that do not respond to directory verification notices, or providers who respond with partial or inaccurate information that cannot be verified by Evernorth will receive a notice that if a response is not received within ten (10) business days, they will be suppressed from showing in on-line and printed directories at their next update.
- Providers shall be suppressed from the online and printed directories at the next required update after the ten (10) day notice period. Providers shall not be suppressed if they respond before the end of the ten (10) day notice period.
- Providers will be restored in the on-line and printed directories once a full and accurate response is received and verified in accordance with Evernorth policies and requirements for updating directory errors and information.
- To the extent permitted by applicable laws, Evernorth may terminate this Agreement for a pattern or repeated failure of Provider to alert Evernorth to a change in the information required by applicable laws to be in the directory or directories.

3. Pursuant to the obligations established by (i) CA Health and Safety Code Section 1374.14 with respect to covered services rendered by Provider to Participants covered by Evernorth HealthCare of California, Inc. and (ii) CA Insurance Code Section 10123.855 with respect to Covered Services rendered by Provider to Participants insured by CHLIC, the following shall apply:
(a) If Provider is not a hospital:
    - Evernorth shall reimburse Provider for the diagnosis, consultation, or treatment of a Participant appropriately delivered through covered telehealth services on the same basis as and to the same extent that Evernorth is responsible for reimbursement for

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the same Covered Service through in-person diagnosis, consultation or treatment.

II

1. California Department of Insurance Requirements for Contracts between Network Providers and Insurers. The following provisions are required by Title 10 California Code of Regulations Section 2240.4 and shall apply to health care services rendered by the Provider with respect to Participants insured by CHLIC:

   (a) Provider shall not make any additional charges to Participants for rendering Covered Services except as provided for in the contract between CHLIC and the Participant (or the employer group under which the Participant is entitled to health coverage).

   (b) The parties acknowledge that Provider's primary consideration shall be the quality of the health care services rendered to the Participants.

   (c) Provider shall not discriminate against any Participant in the provision of Covered Services on the basis of sex, marital status, sexual orientation, race, color, religion, ancestry, national origin, disability, health status, health insurance coverage, utilization of medical or mental health services or supplies, or other unlawful basis including without limitation, the filing by such Participant of any complaint, grievance, or legal action against a provider.

   (d) The Agreement including this Addendum and any exhibits or documents referenced therein contains all the terms and conditions agreed upon by the parties pertaining to the rendering of Covered Services by the Provider to Participants.

III

1. Evernorth Healthcare of California, Inc. is a prepaid comprehensive health care service plan, licensed in the State of California under the Knox-Keene Health Care Services Plan Act of 1975, as amended, to provide or arrange for the provision of health care services to Participants of various prepaid health programs or plans operated by Evernorth Healthcare of California, Inc. that are subject to the requirements of Chapter 2.2 of Division 2 of the Code and Chapter 1 of Title 28 of the California Code of Regulations; any provision required to be in the contract by either of the above shall bind the Plan whether or not provided in the contract.

2. Changes to Agreement applicable to Evernorth HealthCare of California, Inc. The following provisions apply with respect to services rendered by Provider to Participants covered by Evernorth HealthCare of California, Inc. and shall amend the corresponding terms in the base Agreement. The underscored language reflects the changes made to the corresponding term in the base Agreement as required by the California Department of Managed Health Care.

   Emergency Medical Condition - Means a medical condition manifesting itself by acute symptoms of sufficient severity (including active labor and severe pain) such that a Participant could reasonably expect the absence of immediate medical attention to result in (i) serious jeopardy to the health of the individual or, in the case of a pregnant woman, the health of the woman or her unborn child, (ii) serious impairment to bodily functions, or (iii) serious dysfunction of any bodily organ or part.

   Emergency Services - Means those Covered Services that are (i) required by a Participant for the evaluation or stabilization of an Emergency Medical Condition, and (ii)

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furnished by a health care provider qualified to furnish Emergency Services. Where applicable, Emergency Services also means an
additional screening, examination, and evaluation by an appropriate provider to determine if a psychiatric emergency medical condition exists, and the care and treatment necessary to relieve or eliminate the psychiatric emergency medical condition, within the capability of the facility.

Records.
Provider shall maintain medical records and documents relating to Participants as may be required by applicable law. but at any rate for at least two (2) years. Medical records of Participants and any other records containing individually identifiable information relating to Participants will be regarded as confidential, and Provider and Evernorth shall comply with applicable federal and state law regarding such records. Provider will obtain Participants' consent to or authorization for the disclosure of private and medical record information for any disclosures required under this Agreement if required by law. Upon request, Provider will provide Evernorth with a copy of Participants' medical records and other records maintained by Provider relating to Participants for purposes of conducting quality assurance and peer review, case management and utilization reviews, credentialing, payment adjudication and processing, resolving Participant grievances and appeals and other activities reasonably necessary for the proper administration of the Plans.

These records shall be provided to Evernorth at no charge and within the timeframes requested by Evernorth and will also be made available during normal business hours for inspection by Evernorth, Evernorth's designee, accreditation organizations, or to any governmental agency that requires access to these records. This provision survives the termination of this Agreement.

Compensation.
Payments for Covered Services will be the lesser of the billed charge or the applicable fee under Exhibit A, subject to the Administrative Guidelines and minus any applicable Copayments, Coinsurance and Deductibles. The rates in this Agreement will be payment in full by Payor for all services furnished to Participants under this Agreement. Provider shall submit claims for Covered Services at the location identified by Evernorth and in the manner and format specified in this Agreement and the Administrative Guidelines. Claims for Covered Services must be submitted within ninety (90) days of the date of service or, if Payor is the secondary payor, within ninety (90) days of the date of the explanation of payment from the primary payor. Claims received after this ninety (90) day period may be denied, unless Provider submits evidence of good cause for the delay. Provider shall not bill Evernorth, the Payor or the Participant for those denied services.

- The following requirements are effective through December 31, 2025

  Non HMO claims must be acknowledged within 15 business days after receipt of the claim unless the claim is paid within that time. Once acknowledged, a claim must be paid, pended or denied within 30 business days after receipt.

- HMO [and the in-network portion of Network/Open Access POS] claims must be acknowledged within 15 business days after receipt of the claim unless the claim is paid within that time. Claims must be paid, contested or denied within 45 business days* after receipt. . If any claim is pended or denied, the insurer must notify the claimant of the

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portion of the claim being pended or denied and the reasons for doing so.

- For specialized health plans that contract with an HMO (e.g. CBH-CA), claims must be acknowledged within 15 business days after receipt of the claim unless the claim is paid within that time. Once acknowledged, a claim must be paid, pended or denied within 30 business days after receipt.

- These requirements are extended to any delegated claims processing entities.

- Non HMO Plans: Claims not processed within the specified timeframe will be subject to late payment interest at the rate of 10% per year.

- HMO Plans (including CBH): Claims that are not processed within the specified timeframe will be subject to late payment interest at the rate of 15% per year. If late payment interest is not paid with the applicable claim, or within the 5 business day grace period after a claim is paid, an additional $10 penalty per claim must be paid.

The following requirements are effective starting January 1, 2026:

- A health plan or insurer (including specialized health plans and insurers) must reimburse a complete claim or portion thereof, whether in or out of state, as soon as practicable within 30 calendar days after receipt of a claim. If the plan/insurer denies or contests any part of the claim, the plan/insurer must notify the claimant in writing as soon as practicable within 30 calendar days after receipt of the claim.

- If a complete claim is not reimbursed within 30 calendar days after the plan/insurer's receipt of the claim (including its receipt of additional information necessary to reconsider a contested claim), interest accrues at a rate of 15% per year starting the first calendar day after the 30-calendar-day period. A plan/insurer must automatically include in its payment of the claim all interest that has accrued without requiring the claimant to request interest. A plan/insurer that fails to comply with this requirement also must pay the claimant a fee the greater of either $15 or 10% of the accrued interest.

- For HMO plans, automatically means payment of the interest due to the provider within 5 working days of the payment of the claim.

Amendment.
Except as provided hereafter, amendments of material terms of this Agreement shall be agreed to in advance in writing by Evernorth and Provider. Evernorth shall provide at least forty-five (45) business days' notice to Provider of its intent to change a material term of this Agreement unless the parties mutually agree to waive the notice. In the event that state or federal law or regulation or any accreditation requirements of an accreditation organization, should change, alter or modify the present services, levels of payments to Evernorth, standards of eligibility of Participants, or any operations of Evernorth, such that the terms, benefits and conditions of this Agreement must be changed accordingly, then upon notice from Evernorth, this Agreement shall be deemed to be automatically amended to conform to the requirements (including notice requirements if less than forty-five (45) business days) of such state or federal law or regulation, and Provider shall continue to perform Covered Services under this Agreement as modified.

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The parties acknowledge that Evernorth may find it necessary to amend the Administrative Guidelines from time to time that will impact Provider. Evernorth shall notify Provider in writing of material revisions to the Administrative Guidelines, and such material revisions shall be deemed approved by Provider if Provider does not notify Evernorth of Provider's disapproval within forty-five (45) business days of receipt of notice of such material revisions. Provider's approval of such amendments shall not be unreasonably withheld, conditioned or delayed. If Provider disapproves of the change within the specified timeframe and the parties cannot reach agreement to the change in the Administrative Guidelines, Provider shall have the right to terminate the Agreement prior to the implementation of the change.

3. Provider shall report to Evernorth all surcharge and copayments paid by Participant directly to Provider in accordance with California Health and Safety Code §1385. Surcharges are prohibited for Covered Services. If Evernorth learns that Provider is charging Participants a surcharge or a fee for sums owed by Evernorth for Covered Services, Evernorth shall take appropriate action against Provider.

4. Within each service area of the Plan, basic health care services shall be provided in a manner which provides continuity of care including the availability of primary care physicians who will be responsible for coordinating the provision of health care services to Participants.

IV

1. Office Hours and Accessibility: Primary care physicians and specialists must be available to provide services or arrange for services to be provided to Participants twenty-four (24) hours per day every day of the year. There must be a publicized telephone number for Participants to call and telephone calls must be answered promptly. Participants must be informed of the length of time they may expect to wait for a return call from a provider. Staff who answers calls must be trained in the appropriate response to medical calls of a routine, urgent or emergent nature, specifically in the types of questions to ask a caller. However, under no circumstances shall unlicensed staff attempt to assess, evaluate, advise or make any decisions regarding the condition of a Participant or determine when a Participant needs to be seen by a licensed medical professional. A provider's telephone answering machine and/or an answering service must clearly explain how Participants can get urgent or emergency care, and when it's appropriate, how they can contact another provider who's agreed to be on-call for triage and screening services or, if needed, give urgent or emergency care.

2. Access: Participants must have access to appropriate medical care within a reasonable length of time, appropriate to the individual Participant's health care needs.

3. Appointments and Scheduling Guidelines: Providers should ensure Participants have access to timely appointments and scheduling. The following guidelines represent both the Plan's and the state of California's requirements for appointment accessibility.

    a. Life threatening emergency or high risk cases should have access to immediate appointments, appropriate emergency room authorization or direction to dial 911.

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b. Urgent cases not requiring prior authorization should have access to appointments within forty-eight (48) hours of the appointment request.
c. If services must be prior authorized, appointments should be offered concurrent with the prior authorization request and the appointment should be provided within ninety-six (96) hours of the appointment request.
d. Non-urgent, symptomatic or routine appointments with a specialty care physician should be scheduled within 15 business days of the appointment request or prior appointment.
e. Non-urgent, symptomatic or routine appointments with a mental health or substance use disorder provider (non-physician) should be scheduled within 10 days of the appointment request or prior appointment.
f. Follow-up care with a mental health or substance use disorder provider (non-physician) should be scheduled within 10 business days of the prior appointment.
g. Non-urgent appointments for ancillary services for the diagnosis or treatment of injury, illness, or other health condition should be scheduled within fifteen (15) business days of the appointment request.
h. Average waiting time in the office should not exceed thirty (30) minutes.
i. When it is necessary for a provider or a Participant to reschedule an appointment, the appointment shall be promptly rescheduled in a manner that is appropriate for the Participant's health care needs, and ensures continuity of care consistent with good professional practice.
j. California Department of Insurance requires that basic health care services (excluding emergency health care services) are available at least forty (40) hours per week, except for weeks including holidays. Services should be available until at least 10:00 p.m. at least one (1) day per week or for at least four (4) hours each Saturday, except for Saturdays falling on holidays (§ 2240.1. Adequacy and Accessibility of Provider Services, Insurance Code 106(b)).

A prompt investigation will be required should Evernorth's monitoring efforts identify potential deficiencies. Such communication will be made in writing. A corrective action plan (CAP) will be required if deficiencies are confirmed. The CAP request will be made in writing and will include a description of the identified deficiency, the rational for the CAP request and the name and telephone number of the person authorized to respond to questions or concerns.

4. Customer Billing: Provider should not collect or attempt to collect or maintain any legal action against a Participant for any sums owed by Evernorth as per applicable law. Provider's agreement with Evernorth shall include the following language as required by California Health and Safety Code § 1379: contracted providers in California "shall not bill, charge, collect a deposit from, seek compensation, remuneration or reimbursement from, or have any recourse against Participants or persons other than the applicable Payor for Covered Services or for any amounts denied or not paid under this Agreement due to Provider's failure to comply with the requirements of Evernorth's or its designee's Utilization Management Program or other Administrative Guidelines, or failure to file a timely claim or appeal. This provision does not prohibit collection of any applicable Copayments, Coinsurance and Deductibles.

5. Administrative Guidelines; Precertification Protocol

   a. Utilization Management – Precertification of Inpatient Admissions
      i. Evernorth requires precertification for all planned inpatient non-obstetrical admissions for all of Evernorth's medical management models.
      ii. Evernorth reviews certain procedures to establish medical necessity, confirm that the proposed length of stay is appropriate, and determine if the requested services are covered benefits.

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iii. The following information is available on the Evernorth for Health Care Professionals website, **Provider.Evernorth.com**.
iv. Note: Precertification requirements may vary based upon the delegated relationships between Evernorth and medical groups or hospitals. For some managed care programs, medical services may require prior authorization from both a delegated entity and Evernorth (e.g. inpatient stays for managed care patients affiliated with delegated entities). Other services may require prior authorization from only the delegated entity, or from Evernorth alone. Please contact Evernorth for clarification on specific precertification requirements for HMO, Network and POS plans.

b. Post-Service Pre-Pay Medical Necessity List – review to determine which services will be reviewed for coverage retrospectively. Evernorth also recommends Provider review Evernorth's coverage criteria by accessing Evernorth's Coverage Policies on **Provider.Evernorth.com** > Resources > Reimbursement and Payment Policies. Services that are noted as not being medically necessary or experimental, investigational or unproven, may not be covered under either PHS or PHS+ plans.

6. Out-of-Network Referrals: Provider must refer patients to, or arrange for the provision of Covered Services by, Participating Providers except in the case of emergency or as otherwise required by law, or in the event that Participating Providers in the geographic area are unable to provide Medically Necessary services. Where Provider does refer patients to a non-Participating Provider, Provider must use Evernorth's Out-of-Network Referral Disclosure form, requesting a written agreement from the patient that states the patient knows the service may not be covered, and that if the patient has out-of-network benefits, that the patient may have higher out-of-pocket costs. Additionally, Provider must indicate to the patient whether or not Provider has a financial interest in the non-Participating Provider or facility. A copy of the form must be given to the patient and the original must be placed in the patient's medical file.

7. Non-Covered Services: When Provider determines that a service is not a covered benefit prior to the delivery of the service, Provider may request a written agreement from the patient that states the patient knows the service is not covered and agrees to pay directly for the specific service. If Provider does not secure a written agreement for specified services from a patient receiving non-covered services, Provider may not charge the patient directly for the service.

8. Payment received is less than expected: In the event that Evernorth or a delegated payer, such as a medical group, pays Provider less than expected on a claim for Covered Services, the patient is not liable for any amount Provider believes is owed to Provider from Evernorth or the delegated payer. If Provider thinks a claim for Covered Services was not paid correctly, please submit an appeal through Evernorth's claim appeal process (**Provider.Evernorth.com** > Resources > Reimbursement and Payment Policies > Claim Appeals Policies and Procedures). Explain in the appeal why Provider thinks Provider's claim was not paid correctly and include additional detail as appropriate.
   a. Provider's Evernorth Agreement: The Agreement with Evernorth includes the following language as required by California Health and Safety Code § 1379: contracted providers in California "shall not bill, charge, collect a deposit from, seek compensation, remuneration or reimbursement from, or have any recourse against Participants or persons other than the applicable Payer for Covered Services or for any amounts denied or not paid under this Agreement due to Provider's failure to comply with the requirements of Evernorth's or its designee's utilization management program or the Administrative Guidelines, or failure to file a timely claim or appeal. This provision does not prohibit collection of any applicable Copayments, Coinsurance and Deductibles."

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In an effort to support access to quality care for Plan Participants, Evernorth continually reviews its precertification process and requirements. Updates include additions and removals based on the Evernorth review process, as well as new Current Procedural Terminology (CPT®) and Healthcare Common Procedure Coding System (HCPCS) codes that require precertification. Evernorth may make additional changes to the precertification requirements, as needed.

9. Maternal Mental Health Program:

   California Assembly Bill (AB) 2193 requires:
   1) Health plans to develop a maternal mental health program, and
   2) Obstetricians (including OBGYNs, nurse midwives, and family practice physicians who deliver babies) to screen pregnant women for maternal mental health disorders including but not limited to postpartum depression in accordance to sound clinical practice guidelines and using validated screening tools.

   Evernorth offers a high-risk pregnancy case management program that provides customers with support in managing treatment and accessing behavioral health care. To learn how patients with Evernorth coverage can enroll in this program, Provider may call Evernorth or direct them to call Evernorth at 1.800.615.2906. Patients with Evernorth coverage may also seek treatment for these disorders directly and learn about qualified in-network behavioral health providers by visiting **Well.Evernorth.com** or calling the telephone number on the back of their ID card.

10. Autism Behavioral Health Treatment Requests:
    Patients enrolled in any Evernorth plan type that have behavioral health benefits through Evernorth should submit requests for autism behavioral treatment through the Evernorth Behavioral Health autism prior-authorization team. A mental health coordinator will verify benefits and review the request against the requirements noted in California SB 964 (2011). Review requests may be submitted by the Participant, the treating physician or the delegated medical group by contacting Evernorth at 1.800.Evernorth24 (1.800.244.6224). Select the prior authorization prompt, then the Evernorth Behavioral Health prompt.

    For Participants that belong to a delegated capitated medical group, Evernorth maintains financial risk for behavioral health services, including ABA therapy.

    For more information on Assembly Bill 1000 and referral requirements, please visit the California Legislative Information homepage at **http://leginfo.legislature.ca.gov/faces/home.xhtml#** > Bill Information > Bill Number AB 1000.

11. Claims and Compensation:
    a. California Specific Prior Authorization Guarantee:
       Providers who wish to pursue payment for services which Evernorth provided written authorization and verification of eligibility for a Participant who subsequently is determined not to be eligible, do not bill the member and instead the following steps should be taken:
       • Use best efforts to collect the charges directly from the responsible party including: another insurance company, the government, a self-insured employer, or the participant.
       • If the responsible party is a Participant and the Provider receives no payment for at least three (3) billing cycles, request reimbursement by submitting a written dispute/appeal as per the Dispute Resolution process outlined in this Addendum. Evernorth will provide compensation for the Covered Services rendered at the rate set forth in the contract, less

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applicable Copayments, Coinsurance and Deductibles.

- The dispute should include the Evernorth authorization and either the notes from your Accounts Receivable (AR) system indicating that the ineligible Participant was billed three (3) cycles, and if applicable that the other insurance was billed and payment wasn't received, or a copy of the last bill sent to the Participant and if applicable a (EOP) received from the other insurance carrier.

File the appeal for reimbursement either three hundred sixty-five (365) days from the Evernorth issued denial (standard California dispute submission timeframe) or one hundred twenty (120) days from the last billing cycle, whichever is more, for the request to be considered.

12. Uniform Prior Authorization Drug Form Requirements:

    California Senate Bill 866 requires the use of a uniform prior authorization drug form by all insurers and plans effective April 1, 2014. Provider can complete the standardized form on the Evernorth for Health Care Professionals website at **Provider.Evernorth.com** > Resources > Forms Center > State Specific Forms > California General Medication Prior Authorization Form.

    To prevent inadvertent delays related to Evernorth's request for additional information, please provide as much detail as possible about the request. Provider can review Evernorth's pharmacy coverage policies (**Provider.Evernorth.com** > Resources > Coverage Policies > Pharmacy A-Z Index) and attach any relevant chart notes and lab results that will facilitate the review.

    If Provider has questions about California Senate Bill 866 and the use of prior authorization drug forms, call Evernorth's pharmacy service center at the number on Provider's patient's ID card, and press the prompt for provider and pharmacy services.
    For more information on Senate Bill 866 and referral requirements, please visit the California Legislative Information home page at: **http://leginfo.legislature.ca.gov/faces/home.xhtml#** > Bill Information > Bill Number SB 866

13. Prohibition Against Participant Billing:
    Contracted providers should not bill the Evernorth Plan Participant for payments that are denied on the basis that the provider failed to submit the request for review or arbitration within the required time frames, nor shall contracted provider bill participants for amounts greater than their contracted rates (other than applicable Copayments/Coinsurance).

14. Participant Information:
    a. Interpretation Services for Limited English Speaking Patients
        i. Participants who reside in the state of California have the right to an interpreter when receiving treatment and services. Providers must follow applicable state and federal regulations to deliver language services through trained and qualified interpreters. Evernorth offers contracted providers access to free telephonic interpretation through a qualified language service vendor 24 hours a day/7 days a week. To engage an interpreter once the participant is ready to receive services, please call 1.800.806.2059. Provider will need the participant's Evernorth ID number, date of birth and your Tax ID number to confirm eligibility and access interpretation services. It is not necessary to arrange for these services in advance.
        ii. If a participant prefers to use a family member or friend to provide interpretation services, after s/he has been told that a trained interpreter is available free of charge, the participant's refusal to use the trained interpreter shall be documented in the participant's medical record, when in a provider setting, or the participant's

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administrative file (call tracking record) in the Customer Services setting.

b. Participant Grievance Process
    i. If participants have complaints or appeals about administration, coverage or exclusions in their benefit plan, or service or care received, they can contact Evernorth Customer Service for plan participants using the number on their ID card. Participants may also complete and submit our standard grievance form to file their complaint or appeal. Evernorth's California Medical Grievance Form and California Grievance Brochure (outlining submission instructions) are available on our website at well.**Evernorth.com** > Forms Center, or at the following link: **https://well.evernorth.com/app/public/evernorth-in-california**. To view and print this information, scroll down to the California Specific Forms section of the webpage.
    ii. In accordance with state law, copies of the California Grievance Form must be made available to Evernorth participants at all affiliated provider offices. Hard copies of these forms are available on our website in Evernorth's threshold languages (English, Spanish and Traditional Chinese) to accommodate our limited English proficient (LEP) customers. Copies should be distributed annually to all affiliated providers offices with instructions to supply the form to any patient with Evernorth coverage who communicates dissatisfaction with the services or care received, a utilization management decision or a claim denial.
    iii. It is Evernorth's policy to provide all participants who are filing a grievance and who are in need of language assistance, with such assistance upon request. This assistance is not limited to those who speak certain non-English languages. Evernorth participants may request assistance by calling Evernorth Customer Service using the telephone number on the back of their ID cards.
    iv. Once a complaint or appeal is received by Evernorth, Evernorth's investigation process may include outreach by an Evernorth representative to the administrative or claim office or to a specific provider in order to obtain information that will aid in the resolution of the concern or complaint; and to provide an opportunity for the provider to respond to the concern or complaint.
    v. Evernorth provides information, on our website, to participants on how to contact their health plan, file a complaint, obtain assistance from the DMHC and seek an Independent Medical Review (IMR). IMR forms are available in non-English languages, including English, Spanish and Traditional Chinese, on the DMHC website at **dmhc.ca.gov** > File a Complaint > Submit an Independent Medical Review > Complaint Form. Hard copies may be requested by submitting a written request to: Department of Managed Health Care, Attention: HMO Help Notices, 980 9th Street, Suite 500, Sacramento, CA 95814.

15. Legally Protected Health Care Activity:
    a. Evernorth will not release medical information related to a person seeking or obtaining gender-affirming healthcare or gender-affirming mental health care or any person or entity allowing a child to receive gender-affirming care in response to any subpoena or request. Evernorth affirms it is conducting outreach to its contracted delegates and has an established oversight plan including any audit mechanisms to ensure compliance with the requirements.
    b. Evernorth affirms the plan and its contracted entities will not release medical information to persons or entities who have requested that information and who are authorized by law to receive that information, pursuant to Civil Code section 56.10(c), if the information is related to an individual seeking or obtaining gender-affirming health care or gender-affirming mental health care or to a person or entity allowing a child to receive gender-affirming health care or gender-affirming mental health care, and the information is being requested pursuant to another state's law that authorizes a person to bring a civil or criminal action against a person or

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entity that provides, seeks, obtains, or receives gender-affirming health care or gender-affirming mental health care or who allows a child to receive gender-affirming health care or gender-affirming mental health care. Evernorth affirms it is conducting outreach to its contracted delegates and has an established oversight plan including any audit mechanisms to ensure compliance with the requirements.

c. Evernorth will not cooperate with any inquiry or investigation by, or providing medical information to, any individual, agency, or department from another state or, to the extent permitted by federal law, to a federal law enforcement agency that would identify any individual and that is related to an individual seeking or obtaining gender-affirming healthcare or mental healthcare that is lawful under California laws. Evernorth affirms it is conducting outreach to its contracted delegates and has an established oversight plan including any audit mechanisms to ensure compliance with the requirements.

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COLORADO REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between EVERNORTH Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Colorado regarding provider contracts with providers rendering health care services in the State of Colorado. To the extent that such Colorado laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Colorado laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1)(A) The definition for Emergency Services, if any, shall comply with Colorado laws and regulations to the extent applicable.

(1)(B) "Participating Provider" shall mean, to the extent necessary to comply with applicable laws and regulations, a hospital, physician or group of physicians, or any other health care practitioner or entity, either within or outside of Colorado, that has a direct or indirect contractual arrangement with Evernorth to provide Covered Services with regard to the Plan covering the Participant.

(2) Provider shall receive payments for Covered Services as set forth in the Agreement. Colorado law prohibits the use of financial disincentives or the withholding of full compensation because of the number or type of referrals made by Provider to Participating Providers in accordance with applicable utilization management requirements concerning the provision of Covered Services to Participants.

(2)(A) Evernorth shall, as required by Section 10-16-121.3, Colorado Revised Statutes, as may be amended from time to time, offer at least one method of payment to Group or Represented Provider that does not require an associated fee charged to Group or Represented Provider. Method of payment offers may be communicated in the provider reference manual, in Administrative Guidelines, or in other written materials provided by Evernorth, or at a website identified by Evernorth. Evernorth shall not restrict the method or form of payment so that the only acceptable payment method is a credit card payment.

(3) Pursuant to the requirements of Section 10-16-704 (4.5), Colorado Revised Statutes, to the extent applicable:

With respect to services reimbursed on a fee-for-service basis, if Provider believes Provider has been underpaid for a a Covered Service, Provider must submit a written request for an appeal or adjustment with Evernorth or its designee within twelve (12) months after the date of the original payment or explanation of benefits.

With respect to services reimbursed on a fee-for-service basis, Payor may only retroactively adjust reimbursement made to Provider during the twelve (12) month period after the date of the original explanation of benefits.

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Adjustments to claims related to coordination of benefits with federally funded health benefit plans, including Medicare and Medicaid, shall be made within thirty-six (36) months after the date of service.

(4) Neither Provider nor Evernorth is prohibited from protesting or expressing disagreement with a medical decision, medical policy or medical practice of Provider or Evernorth.

(5) If the Participant's primary care physician, in consultation with the Participant and his/her participating specialist determines that the Participant needs ongoing care from such specialist, the Participant may receive a referral for Medically Necessary treatment to such participating specialist ("Standing Referral"). A time period for the Standing Referral of up to one (1) year (or longer if authorized by Evernorth) shall be determined by the primary care physician, in consultation with the participating specialist. The Participant's primary care physician shall record the reason, diagnosis or treatment plan necessitating the Standing Referral. The participating specialist shall refer the Participant back to the primary care physician for primary care. The participating specialist shall be compensated for Covered Services rendered to any Participant in accordance with his/her agreement with Evernorth.

(6) Evernorth may not take an adverse action, as defined by applicable state laws or regulations, against Provider because: a) Provider expresses disagreement with Evernorth's decision to deny or limit benefits to a Participant or assists the Participant to seek reconsideration of Evernorth's decision; or b) Provider discusses with a current, former or prospective patient any aspect of the patient's medical condition, any proposed treatments or treatment alternatives, whether covered by Evernorth or not, policy provisions of a plan or Provider's personal recommendation regarding selection of a health plan based on Provider's personal knowledge of the health needs of such patients.

(6)(A) Evernorth may not take an adverse action, as defined by applicable state laws or regulations, against Provider because Provider, acting on good faith: communicates with a public official or other person concerning public policy issues related to health care items or services; files a complaint, makes a report, or comments to an appropriate governmental body regarding actions, policies, or practices of Evernorth that Provider believes might negatively affect the quality of, or access to, patient care; provides testimony, evidence, opinion, or any other public activity in any forum concerning a violation or possible violation of any provision of C.R.S.A. § 10-16-121; reports what Provider believes to be a violation of law to an appropriate authority, or; participates in any investigation into a violation or possible violation of any provision of C.R.S.A. § 10-16-121.

(6)(B) Evernorth may not take an adverse action, as defined by applicable state laws or regulations, against Group or Represented Provider or subjecting Group or Represented Provider to financial disincentives based solely on Group or Represented Provider's provision of, or assistance in the provision of, a legally protected health-care activity, as defined by applicable state laws or regulations, in this state, so long as the care provided did not violate state law.

(6) (C) Evernorth may not take an adverse action, as defined by applicable state laws or regulations, against Provider or provide financial incentives or subject Provider to financial disincentives based solely on a patient satisfaction survey or other method of obtaining patient feedback relating to the patient's satisfaction with pain treatment.

(7) In the event the Agreement is terminated due to expiration or nonrenewal, or the Benefit Plan is terminated or changed, resulting in a loss of benefits with respect to Provider, the provisions of

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Section 10-16-705(4), as amended by 2022 CO SB 1284 (Laws of 2022 ch. 446), of the Colorado Statutes shall apply.

(8) If Provider is not a hospital, Agreements for less than two (2) years in duration may be terminated without cause by Evernorth or Provider with ninety (90) days advance written notice to the other party. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement by Provider or Evernorth, such longer notification period will apply.

(9) If Provider is a hospital, Evernorth and Provider shall provide at least sixty (60) days written notice to each other before terminating the Agreement without cause. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement by Provider or Evernorth, such longer notification period will apply.

(10) If Provider is not a hospital, the following provisions shall apply:

(a) Agreements for two (2) or more years in duration may be terminated without cause in accordance with the terms set forth in the Agreement.

(b) Evernorth can terminate Provider's participation immediately (or upon such longer notice required by applicable law, if any) if such Provider no longer maintains the licenses required to perform his/her duties under the Agreement, Provider is disciplined by any licensing, regulatory, accreditation organization, or any other professional organization with jurisdiction over Provider, or if Provider no longer satisfies Evernorth's credentialing requirements.

(c) Evernorth or Provider can terminate the Agreement if the other becomes insolvent.

(d) Any termination notice must be in writing and sent by United States mail, postage prepaid, to Evernorth to the address provided pursuant to the Notice section of the Agreement. Evernorth may also notify Provider by sending an electronic notice with automatic receipt verification to Provider's e-mail address. Either party can change the address for notices by giving written notice of the change to the other party in the manner just described.

(e) Payment terms shall not survive the termination of the Agreement except as required by law or as agreed upon by Provider.

(f) Evernorth shall provide Provider with at least ninety (90) days written notice of the effective date of a Material Change to the Agreement. Such notice will be conspicuously entitled "NOTICE OF MATERIAL CHANGE TO CONTRACT."

"Material Change" means a change to an Agreement that: a) decreases the provider's payment or compensation; b) changes the administrative procedures in a way that may reasonably be expected to significantly increase the providers administrative expense; c) replaces the maximum allowable cost list used with a new and different maximum allowable cost list by a person or entity for reimbursement of generic prescription drugs; or d) adds a new category of coverage.

A Material Change does not include: a) a decrease in payment or compensation resulting solely from a change in a published fee schedule upon which the payment or compensation is based and the date of applicability is clearly identified in the Agreement; b) a decrease in payment or compensation resulting from a change in an Agreement for pharmacy services such as a change in a fee schedule

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based on average wholesale price or maximum allowable cost; c) a decrease in payment or compensation that was anticipated under the terms of the Agreement, if the amount and date of applicability of the decrease is clearly identified in the Agreement; d) an administrative change that may significantly increase the provider's administrative expense, the specific applicability of which is clearly identified in the contract; e) changes to an existing prior authorization, precertification, notification or referral program that do not substantially increase the provider's administrative expense; or changes to an edit program or to specific edits.

If Provider objects in writing to the material change within fifteen (15) days and there is no resolution of the objection, Evernorth or Provider may terminate the Agreement upon written notice to the other party but no later than sixty (60) days prior to the effective date of the Material Change.

If Provider does not object to the Material Change within fifteen (15) days, the change shall be effective as specified in the notice.

If the Material Change is the addition of a new category of coverage and Provider objects within fifteen (15) days, the Material Change shall not be effective and Evernorth may not terminate Provider for this reason.

Notwithstanding anything in this section, Evernorth may modify the Agreement by operation of state or federal law or regulation and Evernorth may make such notification to Provider by any reasonable means.

(11) Provider shall, consistent with state and federal statutes and regulations, share medical record information with providers treating or who have treated the same health maintenance organization plan Participant which facilitates the continuity of health care services.

(12) Intermediary Contracts. If Provider is an Intermediary as defined by C.R.S.A. § 10-16- 102(40) and 3 Colo. Code of Regs. § 4.2-15(IV)(B), or any other applicable law or regulation, as may be amended from time to time, Provider as an Intermediary agrees to the following:

(a) If contracted to perform utilization management, utilization review, provider credentialing, administration of health insurance benefits, setting or negotiation of reimbursement rates, payment to providers, network development, disease management programs, or any other program subject to Section 10-16-705(6.5) C.R.S., Intermediary shall comply with the same standards, guidelines, medical policies, and benefit terms as Evernorth.

(b) If contracted to perform utilization management, utilization review, provider credentialing, administration of health insurance benefits, setting or negotiation of reimbursement rates, payment to providers, network development, disease management programs, or any other program subject to Section 10-16-705(10.5)(a) C.R.S., Intermediary shall indicate the name of Intermediary and the company for which it is conducting the work when making any payment to a health care provider on behalf of Evernorth.

(c) Intermediary will comply, and shall require Subcontracted Providers to comply, with all of the applicable requirements of Section 10-16-705, C.R.S.

(d) Evernorth is responsible for ensuring that Subcontracted Providers have the capacity and legal authority to furnish Covered Services.

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(e) Evernorth has the right to approve or disapprove participation status of Subcontracted Providers in its own or a contracted network for the purpose of delivering Covered Services to its Participants.

(f) Intermediary shall provide Evernorth with copies of Subcontracted Providers' contracts in accordance with applicable law and Evernorth shall maintain copies of all such contracts.

(g) As applicable, Intermediary shall transmit utilization documentation and claims paid documentation to Evernorth. Evernorth shall monitor the timeliness and appropriateness of payments made to providers and health care services rendered to Participants.

(h) As applicable, Intermediary shall maintain books, records, financial information, and documentation of services provided to Participants at the Intermediary's place of business in the State of Colorado.

(i) Intermediary agrees to allow the Commissioner of the Division of Insurance for the State of Colorado access to the Intermediary's books, records, financial information and any documentation of services provided to Participants as necessary to determine compliance with the law.

(j) Evernorth shall have the right, in the event of Intermediary's insolvency, to require the assignment to Evernorth of the provisions of a Subcontracted Provider's contract addressing the provider's obligations to furnish Covered Services.

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CONNECTICUT REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Connecticut regarding provider contracts with providers rendering health care services in the State of Connecticut. To the extent that such Connecticut laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Connecticut laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) The definition for Emergency Services, if any, shall comply with Connecticut laws and regulations to the extent applicable.

(1.1) "Material Change" or "Material Adverse Change" shall mean a change that could reasonably be expected to have a material adverse impact on the aggregate level of payment by Evernorth or Payor to Provider for Covered Services under this Agreement, or on Provider's administration of their services.

(1.2) "Timely Notice" shall mean the timeframes established by the parties for advance written notice of a Material Change as set forth in this Addendum.

(2) Provider, in utilizing laboratories or testing facilities for Participants, shall utilize laboratories or testing facilities that are Participating Providers or notify the Participant if Provider intend to utilize a laboratory or testing facility that is not a Participating Provider.

(3) Evernorth and/or Represented Providers shall provide not less than 90 days' written notice to each other of any intent to terminate the Agreement prior to the proposed date of termination or, in the case of a nonrenewal, from the end of the Agreement period. Group and Represented Providers shall, within thirty (30) days of receiving or issuing a written notice under this section, provide to Evernorth a list of its patients who are Participants who have been treated by Group and Represented Providers within the last twelve (12) months. Evernorth shall make a good faith effort to provide written notice in accordance with applicable law to all Participants who are patients being treated on a regular basis by Group and Represented Providers.

(3.1) Evernorth shall administer Participant requests for continuity of care in accordance with applicable state laws and regulations, including but not limited to C.G.S.A. § 38a- 472f(g)(2)(B).

(3.2) The parties shall, if subject to § 38a-472f(g)(1)(C), continue to abide by the terms of the Agreement, including reimbursement terms for all health care services and provisions provided under the Agreement, for a period of 60 days from the date of nonrenewal or termination of the Agreement by either party, except where the parties agree in writing to the termination of the Agreement and

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provide Participant notices applicable under state laws and regulations.

(4) (A) Provider hereby agree that in no event, including, but not limited to, nonpayment by Payor, Evernorth or an intermediary, Evernorth's insolvency or the insolvency of an intermediary, or breach of the Agreement shall Provider bill, charge, collect a deposit from, seek compensation, remuneration, or reimbursement from, or have any recourse against a Participant or person acting on the Participant's behalf, other than Payor, Evernorth or an intermediary, for services provided pursuant to the Agreement. The Agreement and this provision shall not prohibit collection of Coinsurance, Deductibles, Copayments or cost-sharing amounts, or costs for non-covered services delivered on a fee-for-service basis, which have not otherwise been paid by a primary or secondary carrier in accordance with regulatory standards for coordination of benefits, from Participants in accordance with the terms of the Participant's Plan.

(A)(1) The Agreement does not prohibit Provider and a Participant from agreeing to continue services solely at the expense of Participant as long as Provider has clearly informed Participant that such services are not covered or are no longer covered. Except as provided herein, the Agreement does not prohibit Provider from pursuing any available legal remedy.

(B) Provider agree, in the event of Evernorth's insolvency, to continue to provide Covered Services to Participants for the duration of the period for which premiums on behalf of the Participant were paid to Evernorth, or until the Participant's discharge from inpatient facilities, whichever time is greater.

(B)(1) In the event of Evernorth's or an intermediary's insolvency or other cessation of operations, Provider shall deliver Covered Services to Participants without requesting payment from a Participant other than a Coinsurance, Copayment, Deductible or other out-of-pocket expense for such services until the earlier of the termination of the Participant's coverage under the network plan, including any extension of coverage provided under the contract terms or applicable state or federal law for covered persons who are in an active course of treatment, as set forth in applicable state law, or are totally disabled, or (ii) the date the Agreement would have terminated if Evernorth or an intermediary had remained in operation, including any extension of coverage required under applicable state or federal law for covered persons who are in an active course of treatment or are totally disabled.

(C) Notwithstanding any other provision in the Agreement, nothing in the Agreement shall be construed to modify the rights and benefits contained in the Participant's Plan.

(D) Provider may not bill a Participant for Covered Services, except for cost-sharing amounts, where Evernorth denies payment because Provider have failed to comply with the terms or conditions of the Agreement.

(E) Provider further agrees (i) that the provisions of paragraphs (A), (B), (C) and (D) of this section shall survive termination of the Agreement regardless of the cause giving rise to termination and shall be construed to be for the benefit of the Participant, and (ii) that this provision supersedes any oral or written contrary agreement now existing or hereafter entered into between Provider and Participant, or persons acting on Participant's behalf.

(F) If Provider contracts with other providers or facilities who agree to provide Covered Services to Participants with the expectation of receiving payment directly or indirectly from Payor, such providers or facilities shall agree to abide by the provisions of paragraphs (A), (B), (C), (D) and (E)

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of this section.

(5) Pursuant to Connecticut law, it is an unfair trade practice for Provider to request payment from a Participant, other than Coinsurance, Copayment, Deductible or other out-of-pocket expense for Covered Services or Emergency Services, or facility fees or surprise bills as defined by applicable state laws, or to report to a credit reporting agency a Participant's failure to pay a bill for such services when Evernorth has primary responsibility for payment of such services, fees or bills.

(6) If Provider is not a hospital, Evernorth may make Material Changes to Provider's fee schedule once annually upon at least ninety (90) days advance written notice. Upon receipt, Provider may accept the changes, or terminate the Agreement by giving at least sixty (60) days advance written notice. Evernorth may make Material Changes to Provider's fee schedule at any time as permitted under applicable law subject to at least thirty (30) days advance written notice of the changes.

(6.1) Evernorth shall give Provider at least ninety days' advance written notice of any change to the provisions of the Agreement or other documents incorporated by reference in the Agreement, and any change to the Administrative Guidelines, provider manuals and policies incorporated by reference in the Agreement, that will result in a Material Change to the Agreement or the procedures that Provider must follow pursuant to the Agreement.

(6.2) Evernorth shall afford Provider the right to appeal any proposed change to the provisions, other documents, Administrative Guidelines, provider manuals or policies incorporated by reference in the Agreement.

(7) If Provider is not a hospital, Evernorth shall not cancel, deny or demand the return of full or partial payment for an authorized Covered Service due to administrative or eligibility errors more than eighteen (18) months after the receipt of a clean claim except as permitted under applicable law. Evernorth shall provide at least thirty (30) days advance written notice of any cancellation, denial or demand.

(8) If included in the Agreement, a "Most Favored Nation" provision prohibited by C.G.S.A. § 38a-479b(c) is hereby deleted in its entirety.

(9) In addition to the terms of the Agreement establishing requirements for records, Provider shall make health records available to appropriate state and federal authorities involved in assessing the quality of care provided to, or investigating grievances or complaints of, covered persons. Provider shall comply with applicable state and federal laws related to the confidentiality of medical and health records and a covered person's right to view, obtain copies of or amend such covered person's medical and health records.

(10) Subrogation/Worker's Compensation: Subrogation applies if the patient with Evernorth coverage is injured in an accident of any type and someone else is responsible for the injury. Examples may include motor vehicle accidents, dog bites, slips and falls, or the results of an act by a third party. State law may prohibit recovery of subrogation claims in certain circumstances.

(11) Precertification Protocol: Precertification of coverage is not required for Emergency Services. However, Emergency Services resulting in a hospital admission should be reported on the day of admission to the patient's primary care physician where applicable and to Evernorth if Evernorth has weekend and after-hours coverage or within twenty-four (24) hours or the next business day for remaining locations.

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(12) Referrals: Child Early Intervention services do not require a referral for a specialist to provide services to assess or provided as part of an individualized family services plan (IFSP).

(13) Retroactive Adjustments and Denials:

(A) Advance Notice to Deny, Cancel, Recover Payment:

a. The organization shall give at least thirty (30) days' advance notice to a health care professional by mail, electronic mail or facsimile of the organization's cancellation, denial or demand for the return of full or partial payment.
b. When the exception is for return of full or partial payment from a health care professional, the notice shall disclose (i) the amount that is demanded to be returned, (ii) the claim that is the subject of such demand, and (iii) the basis on which such return is being demanded.

(B) Health Care Professional Dispute Rights, Responsibilities & Time Lines

a. No later than thirty (30) days after the receipt of the notice, a health care professional may appeal such cancellation, denial or demand in accordance with the procedures provided by such organization. Any demand for the return of full or partial payment shall be stayed during the pendency of such appeal.
b. If there is no appeal or an appeal is denied, the health care professional may resubmit an adjusted claim to the organization not later than thirty (30) days after the receipt of the notice or the denial of the appeal, whichever is applicable.
c. If a return of payment was demanded, the disputed claim shall not be resubmitted.
d. A health care professional shall have one (1) year after the date of the written notice to identify any other appropriate insurance coverage applicable on the date of service and to file a claim with the organization, regardless of the organization's timely filing requirements.

(C) Exceptions to 18-Month Time Period

a. The law allows organizations to cancel, deny payment recovery requirement more than eighteen (18) months after receipt of a clean claim in these exception situations:
    i. The organization has documented basis to believe that such claim was submitted fraudulently by such health care professional;
    ii. The health care professional did not bill appropriately for such claim based on the documentation or evidence of what medical service was actually provided;
    iii. The organization has paid the health care professional for such claim more than once;
    iv. The organization paid a claim that should have been or was paid by a federal or state program; or
    v. The health care professional received payment for such claim from a different insurer, payor or administrator through coordination of benefits or subrogation, or due to coverage under an automobile insurance or workers' compensation policy (in which case, the health care professional shall have one (1) year after the date of the cancellation, denial or return of full or partial payment to resubmit an adjusted secondary payor claim with the organization on a secondary payor basis, regardless of such organization's timely filing requirements).

(14) Other Important Information: The applicable contracted rate includes all Medically Necessary services that hospitals customarily provide to outpatients and specifically excludes those services, which, in accordance with community standards, are considered office-based procedures, or services.

(15) Emergency Room, Ambulatory Surgery and Observation Admissions: If a patient with Evernorth coverage is admitted as an inpatient from hospital's emergency, outpatient, rapid treatment or

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observation or ambulatory surgery facility, all such facility charges are included in and are part of the compensation for inpatient services. If a patient with Evernorth coverage is admitted to a hospital's ambulatory surgical facility from the hospital's emergency area, all such emergency charges are included in and are part of the applicable outpatient surgery rate. If a patient with Evernorth coverage is admitted to the observation unit from the hospital's emergency area, all such emergency charges are included in and are part of the applicable observation rate. If a patient with Evernorth coverage is admitted to the observation area from the ambulatory or outpatient surgery facility or the cardiac catheterization/angioplasty laboratory, all such observation unit charges are included in and part of the applicable ambulatory or outpatient surgery rate or cardiac catheterization or angioplasty rate. Under no circumstances shall the cost of Medically Necessary rapid treatment or observation care exceed the cost of an inpatient medical per diem. If the primary reason for admission from the ambulatory surgery facility is associated with the hospital's scheduling or administrative procedures, the ambulatory surgery rate will apply.

(16) Provider Responsibility: The provider is responsible for notifying a covered customer of their financial obligations for non-covered services, if possible, prior to the delivery of the non-covered health services.

(17) If Provider participates in a Evernorth tiered network, as that term is defined in applicable state law, Evernorth shall provide to Provider, upon request, Provider's calculated score and related data, as available, and a description of the standards used for selecting and tiering Provider, including: definitions and specifications of measures used to develop such standards and measure performance, with delineation of any inclusions or exclusions under each measure; a defined time period of not less than one year to measure performance based on such standard; and, a summary of the grievance process for Provider to appeal the results of Evernorth's tiering decisions and performance measures.

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DISTRICT OF COLUMBIA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the District of Columbia regarding provider contracts with providers rendering health care services in the District of Columbia. To the extent that such District of Columbia laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such District of Columbia laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

A. (1) Payor may only retroactively deny reimbursement to Provider:

(a) For services subject to coordination of benefits with another health insurer during the eighteen (18) month period after the date that the Payor paid the Provider; or

(b) Except as provided in paragraph A. (1) (a) of this subsection, during the six (6) month period after the date that the Payor paid the Provider.

(b)(i) A Payor that retroactively denies reimbursement to a Provider under subsection A. (1) (a) shall provide the Provider with a written statement specifying the basis for the retroactive denial. If the retroactive denial of reimbursement results from coordination of benefits, the written statement shall provide the name and address of the entity acknowledging responsibility for payment of the denied claim.

(2) This subsection shall not apply if a Payor retroactively denies reimbursement to a Provider because:

(a) The information submitted to the Payor was fraudulent;

(b) The information submitted to the Payor was improperly coded and the Payor has provided to the Provider sufficient information regarding the coding guidelines used by the Payor at least thirty (30) days prior to the date the services subject to the retroactive denial were rendered; or

(c) The claim submitted to the Payor was a duplicate claim.

(3) Information submitted to the Payor may be considered to be improperly coded under paragraph A. (1) (a) of this subsection if the information submitted to the Payor by the Provider:

(a) Uses codes that do not conform with the coding guidelines used by the Payor applicable as of the date that services were rendered; or

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(b) Does not otherwise conform with the contractual obligations of the Provider to the Payor applicable as of the date that services were rendered.

(c) If a Payor retroactively denies reimbursement for services as a result of Coordination of Benefits, the Provider shall have one hundred eighty (180) days after the date of denial, unless the Payor permits a longer time period, to submit a claim for reimbursement for the service to the health insurer responsible for payment.

(d) A Payor that retroactively denies reimbursement to a Provider under this section shall provide the Provider with a written statement specifying the basis for the retroactive denial.

(e) This section shall not apply to an adjustment to reimbursement made as part of an annual contracted reconciliation of a risk-sharing arrangement.

(4) In the event the Agreement is terminated by either party, Evernorth shall give reasonable advance notice of such termination to those Participants whom Provider is currently treating and who are affected by the termination.

(5) Except in cases where termination of the Agreement was due to failure to meet quality standards of care or fraud, Provider shall continue to provide Covered Services for specific conditions for which a Participant was under Provider's care at the time of such termination as follows. When Medically Necessary, Participants with serious illness undergoing a course of treatment or who are in the second trimester of a pregnancy shall be permitted to continue to receive Medically Necessary Covered Services, with respect to the course of treatment or for such pregnancy, for ninety (90) days from the date of the notice of termination. Provider shall be compensated for Covered Services provided to any such Participant in accordance with the compensation arrangements under the Agreement. Participants shall not be liable to Provider for any amounts owed for Covered Services provided during the period of continued care other than Copayments, Deductibles or Coinsurance billed in accordance with the terms of a Plan. Provider has no obligation under the Agreement to continue to provide services to individuals who cease to be Participants.

(6) To the extent required by D.C. Code § 31-3132, Evernorth shall allow Provider a minimum of one hundred eighty (180) days from the date a Covered Service is rendered or the date of inpatient discharge to submit a claim for reimbursement of services.

(7) Provider hereby agrees that in no event, including, but not limited to, non-payment by Payor or entity with access to this Agreement by virtue of a contract with Payor for any reason, including a determination that the services furnished were not Medically Necessary, Payor's insolvency, Provider's failure to submit claims within the time period specified or breach of this Agreement, will Provider bill, charge, collect a deposit from, seek compensation, remuneration or reimbursement from, or have any recourse against Members or persons other than Payor for Covered Services furnished pursuant to this Agreement. This provision will not prohibit collection of applicable copayments, coinsurance or deductibles billed in accordance with the terms of Corporation's agreements with Members.

Provider further agrees that this provision will survive the termination of this Agreement regardless of the cause giving rise to such termination and will be construed to be for the benefit of

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Members. Finally, this provision supersedes any oral or written agreement to the contrary now existing or hereafter entered between Provider and Members or persons acting on their behalf.

Any modifications, additions, or deletions to the provisions of this hold harmless clause will become effective on a date no earlier than thirty (30) days after the Commissioner has received written notice of such proposed changes."

(8) In the event of a health carrier or intermediary insolvency or other cessation of operations, the participating Provider's obligation to deliver covered health care services to covered persons without balanced billing will continue until the earlier of:
(i) the termination of the covered person's coverage under the network plan, including any extension of coverage provided under the contract terms or applicable District or federal law for covered persons who are in an active course of treatment or totally disabled; or (ii) the date the contract between the carrier and the Provider would have terminated if the health carrier or intermediary had remained in operation, including any required extension for covered persons who are in an active course of treatment.

II

Intermediary Contracts. If Group is an intermediary as defined by CDCR 26-A4799, Group as an intermediary agrees to the following:

1. In accordance with applicable laws and regulations, Evernorth shall have the right to disapprove the participation status of a Represented Provider in Group's contracted network for the purpose of delivering Covered Services to Evernorth's Participants.
2. In addition to the negotiated requirements of the Agreement, Group shall allow access to all Group's health care subcontracts and shall allow Evernorth to make copies to facilitate regulatory review, including requests for those subcontracts made by the Commissioner.
3. If applicable under the terms of the Agreement, Group shall transmit utilization documentation and claims paid documentation to Evernorth as set forth in the Agreement. Evernorth shall monitor the timeliness and appropriateness of payments made to Represented Providers and health care services received by Participants.
4. Group shall allow the Commissioner access to Group's books, records, financial information and any documentation of services provided to Participants, as necessary to determine compliance with applicable laws and regulations.
5. In accordance with applicable laws and regulations, Group shall maintain the books, records, financial information, and any documentation of services provided at its principal place of business in the District of Columbia and preserve them for seven years.
6. In accordance with applicable laws and regulations, Evernorth shall have the right, in the event of Group's insolvency, to require the assignment to Evernorth of the provisions of Represented Providers contracts addressing the obligation to furnish Covered Services; Evernorth shall then be obligated to pay subcontracted Represented Providers for furnishing Covered Services under the same terms and condition as Group prior to the insolvency.

7. In accordance with applicable laws and regulations, Group and Represented Providers shall comply with all applicable requirements of § 4705 of Chapter 47; to the extent Evernorth delegates its responsibilities to Group, Evernorth shall retain full responsibility for Group's compliance with applicable laws and regulations.

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(9) The provisions of Sections (7) and (8) of this District of Columbia Regulatory Addendum shall (i) be construed in favor of the covered person, (ii) survive the termination of the contract regardless of the reason for the termination, including the insolvency of the health carrier, and (iii) supersede any oral or written contrary agreement between a Provider and a covered person or a covered person's authorized representative that is contrary to or inconsistent with the provisions.

(10) Unless otherwise set forth in the Agreement, Hospital shall provide such written disclosure or notice to a Participant, as required by CDCR 26-A4704.21, as may be amended from time to time, for non-emergency services at the time of authorization of services, or within ten days of an appointment for in-patient or outpatient services, or at the time of a non-emergency admission at Hospital, acknowledging that the facility is a participating provider of Participant's network plan and disclosing that certain providers at Hospital may not be participating providers but may be performing services for Participant. The disclosure or notice shall state that Participant may be subject to higher cost-sharing pursuant to the plan summary of benefits and coverage, including balance billing, if the Covered Services are performed by an out-of-network provider at Hospital, and that information regarding how much the health plan will pay for Covered Services performed by out-of-network providers is available upon request. The disclosure or notice also shall inform a Participant, or their authorized representative, of the participating providers available to provide the Covered Services.

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DELAWARE REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Delaware regarding provider contracts with providers rendering health care services in the State of Delaware. To the extent that such state laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Delaware laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

I

In the event that Evernorth's final decision regarding reimbursement for an individual claim, procedure or service does not authorize reimbursement of Provider's claim in its entirety, Evernorth shall give Provider written notice of Provider's right to arbitration. Provider shall attempt to resolve disputes informally with Evernorth before requesting arbitration pursuant to this provision. The arbitrator may dismiss an arbitration petition without prejudice, if the arbitrator finds that Provider has not attempted to resolve the matter informally.

Petition for Arbitration: Provider or an authorized representative may request review of Evernorth's final reimbursement decision through arbitration by delivering a Petition for Arbitration to the Department of Insurance so that it is received by the Department no later than sixty (60) days after the date of mailing of Evernorth's final reimbursement decision. The Department shall make available, by mail and on its web site, a standardized form for a Petition of Arbitration. Provider or an authorized representative must deliver to the Department an original and three (3) copies of the Petition for Arbitration. At the time of delivering the Petition of Arbitration to the Department, Provider or an authorized representative must also: send a copy of the Petition to Evernorth by certified mail, return receipt requested; deliver to the Department a Proof of Service confirming that a copy of the Petition has been sent to Evernorth by certified mail, return receipt requested; and deliver to the Department a non-refundable filing fee. The fee shall be $50 for claims of $1,000 or less, in all other cases the fee shall be $100. The Department may refuse to accept any Petition that is not timely filed or does not otherwise meet the criteria for arbitration, including the disputes described in 18 Del. C. Section 333(j)(1) – (3).

Response to Petition for Arbitration: Within twenty (20) days of receipt of the Petition, Evernorth must deliver to the Department an original and three (3) copies of a Response with supporting documents or other evidence attached. At the time of delivering the Response to the Department, Evernorth must also: send a copy of the Response and supporting documentation to Provider or an authorized representative by first class U.S. mail, postage prepaid; and deliver to the Department a Proof of Service confirming that a copy of the Response was mailed to Provider or an authorized representative. The Department may return any non-conforming Response to Evernorth. If Evernorth fails to deliver a Response to the Department in a timely fashion, the Department, after verifying proper service, and with written notice to the parties, may assign the matter to the next scheduled Arbitrator for summary disposition. The Arbitrator may determine the matter in the nature of a default judgment after establishing that the Petition is properly supported and was properly served on Evernorth. The

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Arbitrator may allow the re-opening of the matter to prevent a manifest injustice. A request for re-opening must be made no later than seven (7) days after notice of the default judgment.

Summary Dismissal of Petition by the Department: If the Department determines that the subject of the Petition is not appropriate for arbitration or is meritless on its face, the Department may summarily dismiss the Petition and provide notice of such dismissal to the parties.

Appointment of Arbitrator: Upon receipt of a proper Response, the Department shall assign an Arbitrator who shall schedule the matter for a hearing so that the Arbitrator can render a written decision within forty-five (45) days of the delivery to the Department of the Petition for Arbitration. The Arbitrator shall be of suitable background and experience to decide the matter in dispute and shall not be affiliated with any of the parties or with the patient whose care is at issue in the dispute.

Arbitration Hearing: The Arbitrator shall give notice of the arbitration hearing date to the parties at least ten (10) days prior to the hearing. The parties are not required to appear and may rely on the papers delivered to the Department. The arbitration hearing is to be limited, to the maximum extent possible, to each party being given the opportunity to explain their view of the previously submitted evidence and to answer questions by the Arbitrator. If the Arbitrator allows any brief testimony, the Arbitrator shall allow brief cross-examination or other response by the opposing party. The Delaware Uniform Rules of Evidence will be used for general guidance but will not be strictly applied. Because the testimony may involve evidence relating to personal health information that is confidential and protected by state or federal laws from public disclosure, the arbitration hearing shall be closed. The Arbitrator may contact, with the parties' consent, individuals or entities identified in the papers by telephone in or outside of the parties' presence for information to resolve the matter. The Arbitrator is to consider the matter based on the submission of the parties and information otherwise obtained by the Arbitrator in accordance with this regulation. The Arbitrator shall not consider any matter not contained in the original or supplemental submissions of the parties that has not been provided to the opposing party with at least five (5) days' notice, except claims of a continuing nature that are set out in the filed papers.

Arbitrator's Written Decision: The Arbitrator shall render a decision and mail a copy of the decision to the parties within forty-five (45) days of the filing of the Petition. The Arbitrator's decision is binding upon the parties, except as provided in 18 Del. C. Section 333(f).

II

1. Payments. Provider shall submit a claim for reimbursement for a Covered Service within one hundred eighty (180) days of the date the Covered Service was rendered.

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FLORIDA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Florida regarding provider contracts with providers rendering health care services in the State of Florida. To the extent that such Florida laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Florida laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(A) (1) The "Dispute Resolution Procedure" provision of the Agreement is amended to add the following optional dispute resolution process for the resolution of disputes relating to the payment of provider claims:

After the exhaustion of the Agreement's internal process to resolve disputes relating to the payment of provider claims, but before the initiation of arbitration to resolve such disputes, either party may initiate the Agency for Health Care Administration dispute resolution process, to the extent that such process applies to the dispute, by providing written notice to the other party. If the Agency for Health Care Administration dispute resolution process does not apply to the dispute, and therefore such dispute resolution process does not occur, either party may initiate arbitration by providing written notice to the other party.

(2) Notwithstanding the time period for correcting underpayments as provided in the Agreement or the Medical Management Program, all claims for underpayment from a provider licensed under chapter 458, chapter 459, chapter 460, chapter 461, or chapter 466 of the Florida Statutes must be submitted to Evernorth or its designee within twelve (12) months after Payor's payment of the claim.

(3) The Compensation provision of the Agreement is amended to add the following:

(a) All claims for overpayment submitted to a provider licensed under chapter 458, chapter 459, chapter 460, chapter 461, or chapter 466 of the Florida Statutes must be submitted within twelve (12) months after the Payor's payment of the claim, except that claims for overpayment may be sought beyond that time from providers convicted of fraud.

(b) Notwithstanding the twelve (12) month period provided in subsection (a) above, all claims for overpayment submitted to a provider not included in subsection (a) above must be submitted within thirty (30) months after the Payor's payment of the claim, except that claims for overpayment may be sought beyond that time from providers convicted of fraud.

(B) With respect to Covered Services rendered to Participants covered under an HMO Plan:

(1) The definition for **Emergency Services**, if any, shall comply with Florida laws and regulations to the extent applicable.

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(2) To the extent applicable and/or not otherwise preempted by federal law, the parties shall comply with Section 641.3155 of the Florida Statutes. To the extent required by Section 641.3155 of the Florida Statutes, Provider shall submit all claims for payment by mail or by electronic transfer within six (6) months after the date of discharge for inpatient services or the date of service for outpatient services.

(3) Evernorth or Provider may not terminate the Agreement unless the terminating party provides the other party with a written reason for such termination, which may include termination for business reasons of the terminating party. Termination of the Agreement by Provider, for any reason, shall be upon sixty (60) days' prior written notice to Evernorth and the State of Florida Office of Insurance Regulation. Nonpayment for goods or services rendered by Provider is not a valid reason for avoiding such sixty (60) day advance notice of termination. Evernorth will provide sixty (60) days' advance written notice to Group and the State of Florida Office of Insurance Regulation before terminating the Agreement, except in the case in which Participants' health is subject to imminent danger or Provider's ability to practice medicine is effectively impaired by an action of the Board of Medicine or other governmental agency. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement by Provider or Evernorth, such longer notification period will apply.

(4) In accordance with Section 641.234 of the Florida Insurance Laws, the Agreement may be terminated by an order of the State of Florida Office of Insurance Regulation, if it determines that the compensation paid by Evernorth under the Agreement is unreasonably high.

(5) If the Agreement is terminated for any reason other than for cause, Evernorth and Provider shall allow Participants for whom treatment was active to continue coverage and care when Medically Necessary, through completion of treatment of a condition for which the Participant was receiving care at the time of termination so long as Participant retains eligibility under a Plan, until the Participant selects another treating provider, or during the next open enrollment period offered by the organization, whichever is longer, but not longer than six (6) months after termination of the Agreement. Each party to the terminated contract shall allow a Participant who has initiated a course of prenatal care, regardless of the trimester in which care was initiated, to continue care and coverage until completion of postpartum care. This does not prevent Provider from refusing to continue to provide care to a subscriber who is abusive, noncompliant, or in arrears in payments for services provided. For care continued under this provision, Evernorth and Provider shall continue to be bound by the terms of the Agreement. Changes made within thirty (30) days before termination of a contract are effective only if agreed to by both parties.

(6) Provider shall prominently post a consumer assistance notice to Participants in Provider's reception area. The notice must include the address and toll-free number of the Agency for Health Care Administration, the Subscriber Assistance Program, and the State of Florida Office of Insurance Regulation. The notice shall also state that the toll-free number for the applicable Member Service Center shall be provided upon request.

(C) With respect to Covered Services rendered to Participants covered under a non-HMO Plan which is insured by Evernorth or an Evernorth affiliate:

To the extent applicable and/or not otherwise preempted by federal law, the parties shall comply with Section 627.6131 of the Florida Statutes.

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(D) Risk Management Program

(1) Participating providers must report all adverse incidents involving Evernorth HMO participants to the Evernorth HealthCare of Florida Risk Management Department. This responsibility is in accordance with F.S. 641.55(1) (d) and the Agency for Health Care Administration's Interpretive Guidelines for Internal Risk Management Programs. Florida law requires HMOs to develop and implement an incident reporting system based upon the affirmative duty of all providers to report injuries and adverse incidents to the health plan Risk Manager.

(2) Evernorth defines an adverse incident as "any outcome or event that is inconsistent with the routine care of a patient." Examples of adverse incidents include medication errors, treatment errors, unexpected death, and delays in treatment or diagnosis.

(3) Provider should report an adverse incident immediately by calling the Risk Management Hot Line at 813.637.1248 to leave a detailed message about the adverse incident. Provider may also submit a summary of the incident to the Risk Management department by fax to 860.687.4317, or send an email to **FloridaRiskManagement@Cigna.com**. Provider should to include Provider's full name, provider identification number and a telephone number where Evernorth can reach Provider for further information. Strict confidentiality is maintained at all times.

(4) For questions or concerns about the program or Provider's responsibilities, Evernorth encourages Provider to contact Evernorth's Risk Management department at 860.902.4943.

(E) Appointments Wait Time Standards

(1) In accordance with F.S. 641.485, health care services should be accessible with reasonable promptness, and at minimum meet the following for Evernorth HMO participants:

(a)Emergent life-threatening cases must be seen immediately, 24 hours a day 7 days a week.

(b)Urgent cases must be seen within 24 hours of the appointment request.

(c)Routine symptomatic cases must be seen within two weeks of the appointment request.

(d)Routine non-symptomatic cases must be seen as soon as possible.

(2) Participants with appointments should have a professional evaluation within one hour of the scheduled appointment time. If a delay is unavoidable, the participant should be informed and provided an alternative.

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GEORGIA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as the "Provider") to comply with legislative and regulatory requirements of the State of Georgia regarding provider contracts with providers rendering health care services in the State of Georgia. To the extent that such Georgia laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Georgia laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) It is the intent of the parties to this Agreement to ensure quality services that meet all uniform treatment standards required by Georgia law and any provision herein which may be inconsistent with that intent shall be void.

(2) The definition for **Emergency Services**, if any, shall comply with Georgia laws and regulations to the extent applicable.

(3) The following definitions are added to the Agreement:

Physician - Means an individual who is qualified to practice medicine under the applicable state law or partnership or professional association of such persons and is a licensed Doctor of Medicine or Doctor of Osteopathy.

Primary Care Physician - Means a Physician engaged in general practice, family practice, internal medicine or pediatrics who, through an agreement with Evernorth, provides basic health services to and arranges specialized services for those Participants who select him or her as their Primary Care Physician.

(4) Pursuant to Georgia Code Section 33-20A-62 to the extent applicable and/or not otherwise preempted by federal law:

(i) Evernorth, or any agent thereof, shall not conduct a post-payment audit or impose a retroactive denial of payment of any claim by Provider relating to the provision of health care services to Participants unless:

1. Evernorth, or any agent thereof has provided to Provider written notice of the intent to conduct such an audit or impose such a retroactive denial of payment of such claim, or any part thereof, and has provided in such notice the specific claim and the specific reason for the audit or retroactive denial of payment;
2. Not more than twelve (12) months have elapsed since the last date of service or discharge covered by the claim prior to the delivery to Provider of such written notice; and
3. Any such audit or retroactive denial of payment was completed and notice provided to

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Provider of any payment or refund due within the following time periods, whichever is applicable: (a) if the claim was submitted within ninety (90) days of the last date of service or discharge covered by the claim, within eighteen (18) months of the last date of service or discharge covered by such claim; or (b) if the claim was submitted more than ninety (90) days after the last date of service or discharge covered by the claim, the earlier of eighteen (18) months after Provider's initial submission of such claim or twenty-four (24) months after the date of service.

(ii) Evernorth, or any agent thereof, shall not be required to respond to Provider's request for additional payment or to adjust any previously paid claim or any part thereof following a final payment unless:

1. Provider makes a request in writing to Evernorth, or any agent thereof, specifically identifying the previously paid claim, or any part thereof, and provides the specific reason for additional payment; and
2. The written request for additional payment or adjustment was submitted within the following time periods, whichever is applicable: (a) if the claim was submitted within ninety (90) days of the last date of service or discharge covered by the claim, the earlier of twelve (12) months after the date both Provider and Evernorth, or any agent thereof, agree that all payments relative to the claim have been made and all appeals of such determinations have been made or waived by Provider or twenty-four (24) months after the date of service or discharge; or (b) if the claim was submitted more than ninety (90) days after the last date of service or discharge covered by the claim, the earlier of six (6) months after the date both Provider and Evernorth, or any agent thereof, agree that all payments relative to the claim have been made and all appeals of such determinations have been made or waived by Provider or twenty-four (24) months after the date of service or discharge.

(iii) A Participant who is not billed by Provider, or agent thereof, within forty-five (45) days of the date that Provider, or agent, knew that further payment was due as a result of a post-payment audit, retroactive denial, or rejected request to adjust a previously paid claim shall be relieved of any and all legal obligations to respond to a request for additional payment.

(iv) Notwithstanding any other provision to the contrary, when precertification has been obtained for a service, Evernorth, or any agent thereof, shall be prohibited from contesting, requesting payment, or reopening such claim, or any portion thereof, except to the extent Evernorth is not liable for the payment under Georgia Code Section 33- 20A-7.1.

(5) Physician providers only: In the event of termination of the Agreement by either party, the provisions of Georgia Code Section 33-20A-61 concerning continued services and rights and obligations upon termination of the Agreement.

(6) If included in the Agreement, a "Most Favored Nation" provision or any other provision prohibited by Georgia laws and regulations is hereby deleted in its entirety.

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HAWAII REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Hawaii regarding provider contracts with providers rendering health care services in the State of Hawaii. To the extent that such Hawaii laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Hawaii laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

I

1. Definitions. Unless otherwise defined in the Agreement, the following terms shall have the meaning set forth below. The definition for Emergency Services, if any, shall comply with state laws and regulations to the extent applicable.

1.1 Material Change or Material Adverse Change shall mean a change that could reasonably be expected to have a material adverse impact on the aggregate level of payment by Evernorth or Payor to Provider for Covered Services under this Agreement, or on Provider's administration of their services.

1.2 Timely Notice shall mean the timeframe or timeframes established by the parties for prior written notice of an amendment to the Agreement as set forth in the Amendment provision, or any other provisions of the Agreement governing changes or amendments to the Agreement.

2. In addition to the negotiated terms of the Agreement establishing limits on billing Participants, the following shall apply in accordance with applicable laws and regulations:

A. Provider agrees that in no event, including but not limited to nonpayment by Evernorth or its intermediary, insolvency of Evernorth, or breach of this Agreement, shall

Provider bill, charge, collect a deposit from, seek compensation, remuneration or reimbursement from, or have any recourse against a Participant or a person other than Evernorth or its intermediary acting on behalf of Participant for Covered Services provided pursuant to this Agreement. This provision does not prohibit Provider from collecting Coinsurance, Deductibles or Copayments, as specifically provided in the evidence of coverage, or fees for non-covered services delivered on a fee-for-service basis to Participants. Nor does this provision prohibit Provider and Participant from agreeing to continue services solely at the expense of the Participant, as long as Provider has clearly informed Participant that Evernorth may not cover or continue to cover a specific service or services. Except as provided in the Agreement, this provision does not prohibit Provider from pursuing any available legal remedy.

A.1 Provider shall obtain an executed agreement of financial responsibility prior to the time services are rendered for non-covered services or services that do not meet the criteria in section 432E-1.4, Hawaii Revised Statutes, before billing or collecting from a Participant or person acting on behalf of a Participant.

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B. In the event of Evernorth's or its intermediary's insolvency or other cessation of operations, Provider's obligation to deliver Covered Services to Participants without balance billing will continue until the earlier of: the termination of the Participant's coverage under the Plan, including any extension of coverage provided under the contract terms or applicable state or federal law for Participants who are in an active course of treatment or totally disabled; or, the date the Agreement, including any required extension for Participants in an active course of treatment, would have been terminated if Evernorth or its intermediary had remained in operation.

C. Subsections A, A.1 and B shall be construed in favor of the Participant, shall survive the termination of the contract regardless of the reason for termination, including Evernorth's insolvency, and shall supersede any oral or written contrary agreement between Provider and Participant or the representative of a Participant if the contrary agreement is inconsistent with the hold harmless and continuation of covered services provisions required by subsections A, A.1 and B or applicable laws and regulations.

3. In no event shall Provider collect or attempt to collect from a Participant any money owed to Provider by Evernorth.

4. Nothing in the Agreement shall be construed to offer an inducement to Provider to provide less than Medically Necessary services to Participants. Nothing in the Agreement shall be construed to prohibit Provider from discussing treatment options with Participants irrespective of Evernorth's position on the treatment options, or from advocating on behalf of Participants within the utilization review or grievance or appeals processes used by Evernorth or its delegate or in accordance with any rights or remedies available under applicable state or federal law.

5. Nothing in the Agreement shall be construed to penalize Provider because Provider, in good faith, reports to state or federal authorities any act or practice by Evernorth that jeopardizes patient health or welfare.

6. Evernorth and Provider shall provide at least sixty (60) days written notice to each other before terminating the Agreement without cause. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement by Provider, such longer notification period will apply. When Provider either gives or receives notice of termination, Provider shall supply a list of those Participants that are patients of the Provider.

7. Except as set forth in the negotiated and agreed upon terms of the Agreement, the rights and responsibilities under the Agreement shall not be assigned or delegated by either party without prior written consent of the other party.

8. In addition to the negotiated terms of the Agreement establishing requirements for records, Provider shall make health records available to appropriate state and federal authorities involved in assessing the quality of care or investigating the grievances or complaints of Participants, and to comply with the applicable state and federal laws related to the confidentiality of medical or health records and Participant's right to see, obtain copies of, or amend the Participant's medical and health records.

9. The provisions in the Agreement shall be construed to avoid conflict with the provisions contained in the Plan, or in applicable laws and regulations.

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10 To the extent required by section 432E-10, Hawaii Revised Statutes, Provider agrees to comply with requests for any information necessary for Evernorth to comply with the plan performance measurement and data reporting, including information relevant to effectiveness, appropriateness, access and availability of care, Participant satisfaction, utilization, and any other information as required to comply with applicable laws and regulations.

II

This section shall apply only if Provider is an intermediary as defined by applicable state laws and regulations including but not limited to section 431:26-101, Hawaii Revised Statutes, as may be amended from time to time.

1. Provider shall ensure that all contracts entered into with participating providers comply with all the requirements of applicable laws and regulations including but not limited to section 431:26-104, Hawaii Revised Statutes.

2. Nothing in the Agreement shall be construed to delegate or assign to Provider Evernorth's statutory responsibility to monitor the offering of Covered Services to Participants.

3. In accordance with applicable laws and regulations, Evernorth shall have the right to approve or disapprove the participation status of a subcontracted provider in Evernorth's or Provider's contracted network for the purpose of delivering Covered Services to Evernorth's Participants.

4. In accordance with applicable laws and regulations, Evernorth shall have the right, in the event of Provider's insolvency, to require the assignment to Evernorth of the provisions of Provider's contract addressing the Provider's obligation to furnish Covered Services.

5. In addition to the negotiated terms of the Agreement, Evernorth shall have access to all Provider's subcontracts, and shall have the right to make copies to facilitate regulatory review, upon twenty (20) days prior written notice from Evernorth.

6. In addition to the negotiated requirements of the Agreement, Provider shall maintain the books, records, financial information and documentation of services provided to Participants at its principal place of business in the state and shall preserve them for the time period required by applicable law in a manner that facilitates regulatory review.

7. Provider shall allow the insurance commissioner access to Provider's books, records, financial information and any documentation of services provided to Participants, as necessary to determine compliance with applicable laws and regulations.

8. If applicable under the terms of the Agreement, Provider shall transmit utilization documentation and claims paid documentation to Evernorth as set forth in the Agreement. Evernorth shall monitor the timeliness and appropriateness of payments made to providers and health care services received by Participants.

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ADDENDUM TO PROVIDER AGREEMENT FOR THE STATE OF IDAHO

The provisions set forth in this Addendum are being added to the Agreement with the provider named in the Agreement (hereafter referred as "Provider") to comply with legislative and regulatory requirements of the State of Idaho regarding provider contracts with providers rendering health care services in the State of Idaho. To the extent that such Idaho laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of the Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans.

(1) Emergency Services. The term "Emergency" means the sudden onset of a medical condition that manifests itself by symptoms of sufficient severity including, but not limited to, severe pain, such that the absence of immediate medical attention could reasonably be expected by a prudent person who possesses an average knowledge of health and medicine, to result in placing a person's health in serious jeopardy, serious impairment to bodily functions, or serious dysfunction of any bodily organ.

(2) Termination. In the event Provider breaches any term of the Agreement, Evernorth shall provide written notice to Provider setting forth any breach of the Agreement for which Evernorth proposes that the Agreement be terminated or not renewed. Evernorth shall provide a period of thirty (30) days for Provider to cure such breach prior to termination or nonrenewal of the Agreement. Evernorth may terminate this Agreement immediately without a period of time to cure any breach if Evernorth determines that the breach is willful, an act of fraud, or poses an immediate danger to the public health or safety.

(3) Communication with Participants. Nothing in the Agreement shall be construed to permit Evernorth to refuse to contract with Provider or compensate Provider for Covered Services solely because Provider has in good faith communicated with any current, former, or prospective Participants regarding the provisions, terms, or requirements of Evernorth's products as they relate the Participant's needs, including the provision of non- covered services at the Participant's election, and Provider's participation in Evernorth's provider network. Evernorth shall not terminate or otherwise penalize Provider who is practicing in conformance with community standards for advocating on behalf of a Participant.

(4) Indemnification. Nothing in this Agreement shall be construed to require Provider to indemnify or hold Evernorth harmless unless Evernorth also agrees to indemnify and hold harmless Provider under comparable circumstances.

(5) Information Referred to or Adopted by the Agreement. Evernorth shall provide upon Provider's request and within a reasonable time any documents referred to or adopted by reference in the Agreement. Information that is proprietary, constitutes a trade secret, or is a confidential personnel record shall be excluded from disclosure under this provision.

(6) Financial Inducements or Incentives. To the extent required by Idaho Code § 41- 3928, nothing in the Agreement shall be construed to constitute an incentive plan that includes a specific payment made, in any type or form, to the provider as an inducement to deny, reduce, limit, or delay specific, medically necessary, and appropriate services covered by the health benefit plan and provided with respect to a specific Participant or group of Participants with similar medical conditions.

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(7) Collection from Participants and Participant Non-Liability. In accordance with Idaho Administrative Code § 18.01.26.004, Provider shall not bill or charge any Participant for the difference between the amount specified in the fee schedule for a Covered Service and the amount normally charged by Provider for the particular service. This provision shall not preclude Provider from collecting the specified coinsurance, copayment, or applicable deductible under the Participant's health benefit plan.

(8) Adjustment of Fee Schedule Amounts. Nothing in the Agreement shall be construed to require Provider to accept the unnegotiated adjustment of or to enter into negotiations to reduce the Provider's contractual reimbursement rate to equal the lowest reimbursement rate Provider has agreed to charge any other payor; or to require Provider to disclose to Evernorth its contractual reimbursement rates with other payors.

(9) Compensation and Claims Submission. To the extent required by Applicable Law, Evernorth or payor and Provider shall comply with the provisions of Chapter 56 of Title 41 of the Idaho Code regarding the administration and payment of claims, including but not limited to Idaho Code § 41-5602.

(10) Binding Effect and Assignment. To the extent required by Applicable Law, including but not limited to Idaho Code § 41-1847, if Evernorth assigns the Agreement as permitted herein, Evernorth shall send prompt written or electronic notice to Provider in accordance with the notice provisions of the Agreement. Such notice shall include the name and principal business address of each assignee.

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ILLINOIS REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between EVERNORTH Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Illinois regarding provider contracts with providers rendering health care services in the State of Illinois. To the extent that such Illinois laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Illinois laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(A) "Emergency Services" shall mean, unless otherwise defined by applicable Illinois laws and regulations, transportation services, including but not limited to ambulance services, and covered inpatient and outpatient hospital services furnished by a provider qualified to furnish those services that are needed to evaluate or stabilize an emergency medical condition. "Emergency medical condition" shall mean, unless otherwise defined by applicable Illinois laws and regulations, a medical condition manifesting itself by acute symptoms of sufficient severity, including, but not limited to, severe pain, such that a prudent layperson, who possesses an average knowledge of health and medicine, could reasonably expect the absence of immediate medical attention to result in placing the health of the individual or, with respect to a pregnant woman, the health of the woman or her unborn child in serious jeopardy, or serious impairment to bodily functions, or serious dysfunction of any bodily organ or part.

(B) Appointment Wait Time Standard

(1) For outpatient treatment of mental, emotional, nervous or substance use disorders providers should ensure Participants have access to timely appointments and scheduling.
   a. Participants should be seen for initial appointments within ten business days ofthe appointment request.
   b. Participants should be seen for a repeat or follow-up appointment within 20 daysof the appointment request.

(C) With respect to Covered Services rendered to Participants covered under an HMO Plan:

(1) Provider shall, upon request of Participant, provide Participant the following: (a) information related to the provider's educational background, experience, training, specialty, and board certification, if applicable; (b) the names of licensed facilities on the provider panel where the provider presently has privileges for the treatment, illness or procedure that is the subject of the request; and (c) information regarding the provider's participation in continuing education programs and compliance with any licensure, certification, or registration requirements, if applicable.

(2) a. Evernorth must give Provider at least sixty (60) days' notice of nonrenewal or termination of the Agreement. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement by Evernorth, the longer notification period will apply.

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b. Provider must give Evernorth at least sixty (60) days' notice for termination of the Agreement for cause and at least ninety (90) days' notice by Provider for termination of the Agreement without cause. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement by Provider, the longer notification period will apply.

(3) Evernorth shall not retaliate against Provider if Provider advocates for appropriate health care services for Participants. To advocate for medically appropriate health care services means to appeal a decision to deny payment for health care services pursuant to the reasonable grievance or appeal procedure.

(3.1) Evernorth shall not prohibit Provider from discussing any specific or all treatment options with Participants irrespective of Evernorth's position on those treatment options or from advocating on behalf of Participants within the utilization review, grievance, or appeals processes established by Evernorth in accordance with any rights or remedies available under applicable state or federal law.

(4) All nurse and other ancillary and paramedic personnel shall maintain all necessary professional credentials, including but not limited to appropriate licenses, certifications, accreditations and other similar approvals required by applicable local, state and federal laws and regulations.

(5) Providers must notify the network plan electronically or in writing within 10 business days of any changes to the information listed in the provider directory. For the information regarding whether a provider is accepting new patients, the provider must give notice to the issuer within 20 business days of deciding to cease accepting new patients covered by the plan if the new patient limitation is expected to last 40 business days or longer.

(6) a. Upon termination of the Agreement by Provider, or upon termination of the Agreement by Evernorth, if Evernorth terminates the Agreement for reason(s) other than termination in situations involving imminent harm to a patient or a final disciplinary action by a state licensing board, Provider shall, at the Participant's option, continue to provide Covered Services to the Participant for up to ninety (90) days following the date of the written notice of Provider's termination, or if the Participant is in the third trimester of pregnancy, throughout the term of the Participant's pregnancy, including post-partum care directly related to the pregnancy. During the transitional period under this section, Provider shall agree: (1) to continue to accept reimbursement at the rates applicable prior to the start of the transitional period; (b) to adhere to the Plan's quality assurance requirements and provide the necessary medical information related to such care; and (c) to otherwise adhere to the Plan's policies and procedures, including but not limited to procedures regarding referrals and obtaining preauthorizations for treatment.

a. Participants shall not be liable to Provider for any amounts owed for Covered Services provided during the period of continued care other than Copayments, Deductibles or Coinsurance billed in accordance with the terms of a Plan.

b. Provider has no obligation under the Agreement to continue to provide Covered Services to individuals who cease to be Participants.

(7) Provider shall give Evernorth at least fifteen (15) days advance written notice of cancellation, modification or termination of general or professional liability insurance.

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(8) The "Charges to Participants" provision is amended to add the following Participant hold harmless requirements:

a. the provision shall also apply to Provider's assignees or subcontractors;

b. the Participant, persons acting on Participant's behalf (other than Payor) and the employer or group contract holder shall be third party beneficiaries of the provision; and

c. the provision supersedes any oral or written agreement now existing or hereafter entered into between Provider and Participant, persons acting on Participant's behalf (other than Payor) and the employer or group contract holder.

(9) Evernorth or Payor shall not request recoupment or withhold an offset from future payments twelve (12) months or more after the original payment was made, except in cases in which: a court, government administrative agency, other tribunal, or independent third-party arbitrator makes or has made a formal finding or fraud or material representation; Evernorth or Payor is acting as a plan administrator for the Comprehensive Health Insurance Plan under the Comprehensive Health Insurance Plan Act; or, Provider has already been paid in full by another payor, third party, or worker's compensation insurer.

(10) To the extent that Provider is subject to the requirements of IAC 50, Section 2051.290, as may be amended from time to time:

a. Provider shall maintain and make medical records available as required by IAC 50, Section 2015.290 c) 1);
b. the rights and responsibilities under the Agreement cannot be sold, leased, assigned, assumed or otherwise delegated by either party except as provided by IAC 50, Section 2015.290 h);
c. Provider shall not differentiate or discriminate in the treatment of any Participant because of race, color, national origin, ancestry, religion, sex, marital status, sexual orientation, age, health status, veteran's status, handicap or source of payment, or participation in the preferred provider program, ethnicity, sexual preference, disability or as otherwise required by IAC 50, Section 2015.290 j);
d. Evernorth will give Provider access to benefit information concerning the type, scope and duration of benefits to which a Participant is entitled as required by IAC 50, Section 2015.290 o) and the terms of the Agreement.

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INDIANA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Indiana regarding provider contracts with providers rendering health care services in the State of Indiana. To the extent that such Indiana laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Indiana laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(A) (1) If included in the Agreement, a "Most Favored Nation" provision or any other provision prohibited by Indiana Code Section 27-8-11-9 (Insurers) or Section 27-13- 15-4 (Health Maintenance Organizations) is hereby deleted in its entirety.

(2) The definition for **Emergency Services**, if any, shall comply with Indiana laws and regulations to the extent applicable.

(3) (a) A Payor may not, more than two (2) years after the date on which an overpayment on a claim was made to Provider by the Payor:

(1) request that Provider repay the overpayment; or
(2) adjust a subsequent claim filed by the Provider as a method of obtaining reimbursement of the overpayment from the Provider.

(b) A Payor may not be required to correct a payment error to Provider more than two (2) years after the date on which a payment on a claim was made to Provider by the Payor.

(c) This section does not apply in cases of fraud by Provider, the Participant, or the Payor with respect to the claim on which the overpayment or underpayment was made.

(3.1) If either Group or Evernorth intends to terminate the Agreement, the terminating party must, in accordance with Indiana Code Section 27-1-37-9, as may be amended from time to time, and subject to provisions governing the term of the Agreement, provide not less than 90 days written notice before the Agreement terminates. Notwithstanding the foregoing, to the extent that the Agreement provides for, or applicable law requires a longer notification period with respect to termination of the Agreement by Group or Evernorth, such longer notification period will apply.

(4) With respect to those providers defined in Indiana Code Section 27-1-37.1-4, Evernorth shall give written notice to Provider of any amendment to the Agreement not less than forty-five (45) days before the proposed effective date of the amendment. Provider may terminate the Agreement without penalty by informing Evernorth that Provider does not accept the amendment. Written notice of termination must be given to Evernorth not later than fifteen (15) days after Provider receives notice of the amendment. Termination of the Agreement is effective ninety (90) days

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after the date Evernorth receives written notice that Provider does not accept the amendment or, on an earlier date if the parties mutually agree. This section does not apply to amendments that are required to comply with a state or federal law and does not apply to the termination of the Agreement under Section 3.1 of this addendum.

(5) The Agreement may permit network rental arrangements which allow Evernorth to lease, rent, or otherwise grant access to a Participating Provider's health care services to a third party, and the third party accessing the health care contract is:

(a) a payer or third-party administrator or another entity that administers claims on behalf of the payer;

(b) a preferred provider organization or preferred provider network, including a physician-hospital organization; or

(c) an entity engaged in the electronic claims transport between Evernorth and the payer.

(B) With respect to Covered Services rendered to Participants covered under an HMO Plan:

(1) If Provider terminates the Agreement, Provider shall give Evernorth not less than ninety (90) days' prior written notice of the termination. If Provider renders thirty (30) percent or more of the services required by Evernorth's commercial HMO Participants, Provider shall give Evernorth not less than one hundred twenty (120) days' prior written notice of termination. If Evernorth terminates the Agreement, Evernorth shall give Provider not less than ninety (90) days prior written notice of the termination. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement by Provider, such longer notification period will apply.

(2) (a) Upon termination of the Agreement by Provider, or upon termination of the Agreement by Evernorth, if Evernorth terminates the Agreement for reasons other than due to a quality of care issue, Provider shall, upon the request of the Participant, continue to provide Covered Services to the Participant until the earlier of the following: (1) sixty (60) days following termination of the Agreement; (2) if the Participant is in the third trimester of pregnancy, throughout the term of the Participant's pregnancy; or (3) if Provider is a hospital, the day the Participant is released from inpatient status at the hospital. During the continuation period under this section, Provider: (1) shall continue to accept the terms and conditions of the Agreement, together with the applicable Coinsurance, Copayments or Deductibles, as payment in full; and (2) is prohibited from billing a Participant for any amounts in excess of the Participant's applicable Coinsurance, Copayments or Deductibles.

(b) Provider has no obligation under the Agreement to provide continued services to individuals who cease to be Participants.

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IOWA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as the "Provider") to comply with legislative and regulatory requirements of the State of Iowa regarding provider contracts with providers rendering health care services in the State of Iowa. To the extent that such Iowa laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Iowa laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) The definition for Emergency Services, if any, shall comply with Iowa laws and regulations to the extent applicable.

(2) A. Upon termination of the Agreement by Provider, or upon termination of the Agreement by Evernorth, if Evernorth does not terminate the Agreement for cause, Provider shall continue to provide Covered Services to Participants undergoing a specified course of treatment for a terminal illness or a related condition for a period of up to ninety (90) days from the date of Provider's termination, or if the Participant is in the second or third trimester of pregnancy, throughout the term of the Participant's pregnancy including postpartum care related to the childbirth and delivery. During the continuation period under this section: (1) the parties shall be bound by the terms and conditions of the Agreement; (2) Provider shall be compensated for Covered Services provided to any such Participant in accordance with the compensation arrangements under the Agreement; and (3) Provider is prohibited from billing Participants for any amounts in excess of the Participant's applicable Coinsurance, Copayments or Deductibles.

B. Provider has no obligation under the Agreement to continue to provide Covered Services to individuals who cease to be Participants.

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KANSAS REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred as "Provider") to comply with legislative and regulatory requirements of the State of Kansas regarding provider contracts with providers rendering health care services in the State of Kansas. To the extent that such Kansas laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Kansas laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(A) In the event that Payor erroneously pays a claim providing payment to which Provider is not entitled, Payor shall not initiate a request for reimbursement or refund of the erroneous payment, or in any other way seek to recoup the erroneous payment, unless such action is initiated within eighteen (18) months after the end of the month in which the erroneous payment was made. In cases of fraud, such action may be initiated within the applicable statute of limitations pursuant to K.S.A. 60-513, and amendments thereto.

(B) With respect to Covered Services rendered to Participants covered under an HMO Plan:

Upon termination of the Agreement, Provider shall continue to provide Covered Services to Participants for a period of up to ninety (90) days in those cases where the continuation of such care is Medically Necessary and in accordance with the dictates of medical prudence and where the Participant has special circumstances such as a disability, a life threatening illness or is in the third trimester of pregnancy. Provider shall be compensated for Covered Services provided to any such Participant in accordance with the compensation arrangements under the Agreement until ninety (90) days following termination and thereafter compensation for continued services authorized by Evernorth shall be as mutually agreed. Provider has no obligation under the Agreement to provide services to individuals who cease to be Participants.

(C) Dispute Resolution Process: Kansas requires a single level appeals process for providers submitting on behalf of patients with Evernorth coverage. See the dispute resolution provisions in the Agreement and the Medical Management Program.

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KENTUCKY REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Kentucky regarding provider contracts with providers rendering health care services in the State of Kentucky. To the extent that such Kentucky laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Kentucky laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) The definition for Emergency Services, if any, shall comply with Kentucky laws and regulations to the extent applicable.

(2) If Provider enters into any subcontract agreement with another provider to provide their licensed health care services to Participants, where the subcontracted provider will bill Evernorth or Participant directly for the subcontracted services, the subcontract agreement must meet all the requirements of KRS 304.17A-527 and shall be filed with the Commissioner of Insurance.

(3) (a) Evernorth shall, upon request of Provider, provide or make available, when contracting or renewing an existing contract with Provider, the payment or fee schedules or other information sufficient to enable Provider to determine the manner and amount of payments under the contract for Provider's services prior to the final execution or renewal of the contract and shall provide any change in such schedules at least ninety (90) days prior to the effective date of amendment.

(b) Any change to payment or fee schedules applicable to Provider shall be made available to Provider at least ninety (90) days prior to the effective date of the amendment. This subsection (b) shall not apply to changes in standard codes and guidelines developed by the American Medical Association or a similar organization.

(4) Pursuant to KRS 304.17A-235 this section (4) is applicable to agreements with Participating Providers as defined below:

(a) Definitions:

"Material Change" means a change to an Agreement, the occurrence and timing of which is not clearly identified in the Agreement, that decreases the health care provider's payment or compensation or changes the administrative procedures in a way that may reasonably be expected to significantly increase the provider's administrative expense, and includes any changes to provider network requirements, or inclusion in any new or modified insurance products.

"Participating Provider" means a Provider that has entered into an agreement with Evernorth to provide health care services.

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(b) Evernorth shall provide Participating Provider with at least ninety (90) days written notice of a Material Change to the Agreement. The notice shall provide the proposed effective date of the change and include a description of the Material Change, and such notices and disclosures as required by applicable laws and regulations, including but not limited to KRS 304.17A-235, as may be amended from time to time, in the manner and format as prescribed by applicable laws and regulations.

For changes to an existing prior authorization, precertification, notification, or referral program, or changes to an edit program or specific edits, Evernorth shall provide notice of the change to the Participating Provider at least fifteen (15) days prior to the change.

(5) Except in cases of fraud, Payor may only retroactively deny reimbursement to Provider during the twenty-four (24) month period after the date Payor paid the claim submitted by Provider. Payor shall not be required to correct a claim payment error, if Provider's request for a claim payment correction is filed more than twenty-four (24) months after the date that Provider received payment for the claim from Payor.

(6) (a) Pursuant to KRS 304.17A-643, if the Agreement between Evernorth and Provider is terminated for reasons other than a quality of care issue or fraud, Provider may request, with the concurrence of the Participant or authorized person, to continue treatment for Participants in special circumstances. "Special circumstances" includes a circumstance in which a Participant has a disability, a congenital condition, a life-threatening illness, or is past the 24th week of pregnancy where the disruption of the Participant's continuity of care could cause medical harm. With respect to those Participants who retain eligibility under a Plan and who are in an active course of treatment for special circumstances, Provider shall continue to provide Covered Services in accordance with the terms of this Agreement: (a) for a period up to nine (9) months in the case of a Participant who at the time of the termination has been diagnosed with a terminal illness; (b) if a Participant is beyond the 24th week of pregnancy, for a period that extends through the delivery of the child, immediate post-partum care, and examination within the first six (6) weeks following delivery; or (c) for a period up to ninety (90) days after the effective date of termination for all other Participants in an active course of treatment for special circumstances. Provider shall be compensated for such Covered Services in accordance with the compensation arrangements under the Agreement.

(b) Pursuant to KRS 304.17A-527 (1)(b), if the Agreement between Evernorth and Provider is terminated for any reason, other than a quality of care issue or fraud, Provider shall continue to provide services and Payor shall continue to reimburse Provider in accordance with the compensation arrangements under the Agreement until Participant is discharged from an inpatient facility, or the active course of treatment is completed, whichever is greater, and in the case of a pregnant woman, services shall continue to be provided through the end of the post-partum period if the pregnant woman is in her fourth (4th) or later month of pregnancy at the time the Agreement is terminated.

(c) This provision survives termination of this Agreement, is intended to be for the benefit of Participants, and supersedes any oral or written agreement to the contrary now existing or hereafter entered into between you and a Participant or persons acting on the Participant's behalf.

(7) Limitations on Billing Participants. Pursuant to KRS 304.17A-527 (1)(a), Provider shall not under any circumstances, including nonpayment of moneys due Provider by Payor, insolvency of Payor, or

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breach of the Agreement bill, charge, collect a deposit from, seek compensation, remuneration or reimbursement from, or have any recourse against Participants or any person acting on their behalf for Covered Services or for any amounts denied or not paid under this Agreement. This provision does not prohibit collection of any applicable Copayments, Coinsurance and Deductibles. This provision survives termination of this Agreement, regardless of the cause giving rise to such termination and shall be construed to be for the benefit of Participants and supersedes any oral or written agreement to the contrary now existing or hereafter entered into between Provider and a Participant or persons acting on the Participant's behalf. Modifications to this section will become effective no earlier than the date permitted by applicable law.

(8) How This Agreement Can Be Terminated.

(a) If Provider is not a hospital: Provider can terminate this Agreement at any time by providing at least sixty (60) days advance written notice. Either Provider or Evernorth can terminate this Agreement immediately if the other becomes insolvent. Evernorth can also terminate the Agreement immediately (or upon such longer notice required by applicable law, if any) if Provider no longer maintains the licenses required to perform his/her/its duties under this Agreement, the Provider is disciplined by any licensing, regulatory, accreditation organization, or any other professional organization with jurisdiction over the Provider, or if the Provider no longer satisfies Evernorth's credentialing requirements.
(b) If Provider is a hospital: Provider may initiate the termination of this Agreement by providing six (6) months written notice to the other party prior to any anniversary of the Effective Date (the "Notice Period") provided however, that no such termination shall be effective prior to the expiration of the initial term. During the Notice Period, Evernorth may cease marketing efforts for Provider and discontinue referral of Participants to Provider.
(c) Any termination of this Agreement as a result of a professional review action will comply with the standards of 42 U. S. C. § 11112.

(9) The section of the Agreement entitled Compensation is amended to add the following sentence to conform to applicable state laws:

Notwithstanding any provision to the contrary set forth in the Compensation section of the Agreement, or any similar provision in the Agreement, or a rate exhibit, to the extent required by KRS 304.17C-085 as may be amended from time to time, the rates in the Agreement will be payment in full for all Covered Services furnished to Participants under the Agreement.

(10) The section of the Agreement entitled Compensation is amended to add the following sentence to conform to applicable state laws:

Notwithstanding any provision to the contrary set forth in the Compensation section of the Agreement, or any similar provision in the Agreement, or a rate exhibit, to the extent required by KRS 304.17C-085 as may be amended from time to time, the rates in this Agreement apply to all Covered Services provided to Participants in the Plan types covered by this Agreement, including services covered under a Participant's in or out-of-network benefits and whether the Payor or Participant is financially responsible for payment.

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LOUISIANA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Louisiana regarding provider contracts with providers rendering health care services in the State of Louisiana. To the extent that such Louisiana laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall apply. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Louisiana laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

A. Upon termination of the Agreement:

1. In the event a Participant has been diagnosed as being in a high-risk pregnancy or is past the 24th week of pregnancy, the Participant shall be allowed to continue receiving Covered Services, subject to the consent of the treating Provider, through delivery and postpartum care related to the pregnancy and delivery.

2. In the event a Participant has been diagnosed with a life-threatening illness, the Participant shall be allowed to continue receiving Covered Services, subject to the consent of the treating Provider, until the course of treatment is completed, not to exceed three (3) months from the effective date of such termination.

3. Provider shall be compensated for such Covered Services in accordance with the compensation arrangements under the Agreement. The contractual requirements for Provider to comply with Evernorth's utilization management and quality management policies and procedures shall remain in effect for the applicable period specified in subsections 1. and 2. above.

B. The above provisions do not apply if:

1. The reason for termination of the Agreement is due to suspension, revocation, or applicable restriction of the Provider's license to practice in Louisiana by the Louisiana State Board of Medical Examiners, or for another documented reason related to quality of care.

2. The Participant chooses to change Provider.

3. The Participant moves out of the geographic service area of the Provider.

4. The Participant requires only routine monitoring for a chronic condition but is not in an acute phase of the condition.

C. Evernorth Prepayment Programs: To the extent permitted by applicable state law, prepayment programs described herein will be conducted in accordance with applicable state laws (including Regulation 49) and the terms of the Agreement.

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MAINE REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Maine regarding provider contracts with providers rendering health care services in the State of Maine. To the extent that such Maine laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Maine laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

A. (1) The definition for **Emergency Services**, if any, shall comply with Maine laws and regulations to the extent applicable.

(2) Payor may not impose on Provider any retrospective denial of a previously paid claim or any part of that previously paid claim unless Payor has provided the reason for the retrospective denial in writing to Provider and the time that has elapsed since the date of payment of the previously paid claim does not exceed 12 months.

(2.1) The retrospective denial of a previously paid claim may be permitted from the 12 months from the date of payment until no later than 36 months from the date of payment for the following reasons only: (1) the claim payment was incorrectly made because Provider or Participant was already paid in full for the health care services identified in the claim; (2) the health care services identified in the claim were not delivered by Provider; (3) the claim payment is the subject of adjustment with another insurer, administrator or payor; or, (4) the claim payment is the subject of legal action.

(2.2) The retrospective denial of a previously paid claim may be permitted beyond 12 months from the date of payment for the following reasons only: (1) the claim was submitted fraudulently; or, (2) the claim payment was for services covered by Title XVII, Title XIX or Title XXI of the Social Security Act.

(2.3) For purposes of this provision, "retrospective denial of a previously paid claim" means any attempt by Payor to retroactively collect payments already made to Provider with respect to a claim by requiring repayment of such payments, reducing other payments currently owed to Provider, withholding or setting off against future payments or reducing or affecting the reimbursement rates for future claim payments to Provider in any other manner.
Provider has six (6) months from the date of notification under this provision to determine whether Participant has other appropriate insurance that was in effect on the date of service.
Notwithstanding the terms of the Agreement, Evernorth shall allow for the submission of a claim that was previously denied by another insurer because of Participant's transfer or termination of coverage.

(3) a. Any modification, addition, or deletion to the provision of the Agreement relating to limitations on

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billing participants shall become effective upon the review and approval of the Maine Bureau of Insurance.

b. Evernorth shall notify Provider of a proposed amendment to the Agreement at least sixty (60) days prior to the amendment's proposed effective date and may file such notice of a proposed amendment to a provider agreement only 4 times per calendar year on January 1st, April 1st, July 1st, and October 1st, except that, at any time, Cigna may file a notice of a proposed amendment in response to a requirement of the state or federal government or due to a change in current procedural terminology codes used by the American Medical Association. If an amendment that has substantial impact on the rights and obligations of Provider is made to a manual, policy or procedure document referenced in the Agreement, such as material changes to fee schedules or material changes to procedural coding rules specified in the manual, policy or procedure document, Evernorth shall give sixty (60) days notice to Provider. If the change is to a reimbursement policy and the estimated aggregate change to participating provider reimbursement as a result of the change is more than $500,000 per year, the notice must include Cigna's good faith estimate of the total annual financial impact of the amendment on the aggregate amount of reimbursement payments made by Cigna to all providers within the state with whom Cigna has a provider agreement. After the sixty (60) day notice period has expired, the amendment to a manual, policy or procedure document becomes effective and binding on both Evernorth and Provider subject to any applicable termination provisions in the Agreement, except that Evernorth and Provider may mutually agree to waive the sixty (60) day notice requirement. This subsection may not be construed to limit the ability of Evernorth and Provider to mutually agree to the proposed change at any time after Provider has received notice of the proposed amendment. Evernorth may notify Provider of a proposed amendment by way of an attachment to an electronic communication, and shall indicate in the subject line that notice of an amendment is included in the communication; as part of the notice, Evernorth shall provide a copy of the revised provider agreement, manual, policy or procedure document without changes being noted and a copy of the revised provider agreement, manual, policy or procedure document with changes being noted by underling added language and by striking through deleted language.

(4) a. Evernorth may not terminate or non-renew the Agreement, unless Evernorth provides Provider with a written explanation prior to the termination or nonrenewal of the reasons for the proposed termination or nonrenewal and provides an opportunity for a review or hearing. Termination or nonrenewal may not be effective earlier than sixty (60) days from Provider's receipt of the notice of termination or nonrenewal. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement by Evernorth, such longer notification period will apply.

b. Evernorth will notify an enrollee of the termination of the provider's contract at least 60 days in advance of the date of termination. When circumstances related to the termination render such notice impossible, the carrier must provide affected enrollees as much notice as is reasonably possible. The notice given to the enrollee must include instructions on obtaining an alternate provider and must offer the carrier's assistance with obtaining an alternate provider and ensuring that there is no inappropriate disruption in the enrollee's ongoing treatment.

c. Notice and Hearing. If Evernorth should choose to terminate the Agreement, Evernorth will notify Provider of this decision in writing. The notice will include the reason(s) for the termination including reference to the evidence or documentation leading to the decision and a notice of Provider's right to request a hearing or review, at Provider's discretion. If Provider should desire a

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hearing with regard to the termination of the Agreement, Provider must notify Evernorth in writing within thirty (30) days of Provider's receipt of the notice of termination. A hearing will be held within thirty (30) days after receipt of the request by Evernorth. The hearing shall be conducted by a panel of at least three (3) people appointed by Evernorth, at least one-third of which shall be clinical peers of Provider. The panel shall render a decision in a timely manner and shall notify Provider of the decision in writing which will include one of the following resolutions: (a) unconditional reinstatement; (b) provisional reinstatement subject to certain conditions as set forth by Evernorth; or (c) termination. Termination will be effective no earlier than sixty (60) days after Provider's receipt of the panel's decision or until the termination date of the Agreement, whichever is earlier. If Provider is unsatisfied with the panel's decision, Provider may appeal the decision further pursuant to the Dispute Resolution procedures specified in the Agreement and Medical Management Program.

d. The requirements set forth in this provision do not apply in cases involving imminent harm to patient care, a final determination of fraud by a governmental agency or a final disciplinary action by a state licensing board or other governmental agency that impairs Provider's ability to practice.

(5) a. Upon termination of the Agreement, except in cases involving imminent harm to patient care, a final determination of fraud by a governmental agency, or a final disciplinary action by a state licensing board or other governmental agency that impairs Provider's ability to practice, Provider shall continue to provide Covered Services, for those Participants who retain eligibility under a Plan and are in active treatment under Provider's care at the time of such termination, for a transitional period of sixty (60) days from the date of notice to the Participant of Provider's termination, or if the Participant is in the second or third trimester of pregnancy at the time of the termination of the Agreement, and Provider is treating the Participant during the pregnancy, the transitional period shall extend through the provision of postpartum care directly related to the pregnancy.

b. During the transitional period under this provision, Provider shall: (a) continue to accept reimbursement at the rates applicable prior to the start of the transitional period as payment in full and shall not impose cost-sharing with respect to the Participant in an amount that would exceed the cost-sharing that would have been imposed had the Agreement not been terminated; (b) adhere to Evernorth's quality assurance requirements and provide the necessary medical information related to such care; and (c) otherwise adhere to Evernorth's policies and procedures, including but not limited to procedures regarding referrals and prior authorizations and providing services pursuant to any treatment plan approved by Evernorth.

c. Provider has no obligation under the Agreement to provide services to individuals who cease to be Participants.

(6) In addition to the Participant billing obligations set forth in the Agreement, in accordance with 24-A MRSA § 4303(8-A), a Participant is not liable to Provider for any sums owed by Payor, and Provider may not collect or attempt to collect any amount from Participant beyond the amount permitted by the Agreement or the managed care plan, notwithstanding Payor's failure to pay, insolvency, or any other breach of the Agreement.

(7) If included in the Agreement, a "Most Favored Nation" provision prohibited by 24-A MRSA § 4303(15) is hereby deleted in its entirety.

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(8) Benefit Design/Coverage Decisions: Evernorth, Evernorth's designee or the Payor will be solely responsible for the Plan design and for interpreting the terms of and making final coverage determinations under a Plan so long as such interpretation is consistent with 24A MRSA 4303(11).

(9) Healthplan Quality Committee for Maine: As a health care professional in Maine, Provider has the opportunity to provide input on Evernorth policies, serve on Evernorth's Healthplan quality committee and volunteer to participate in focus groups and surveys. Should Provider wish to provide feedback to the Quality Management Department about Evernorth's published health care professional satisfaction and preventive health reports or receive more information about the Evernorth Quality Management Program, the annual program evaluation or other chronic case or preventive health measures, please call 800.591.9407.

(10) Nothing in the Agreement shall be construed to prohibit Provider from providing a Participant, in accordance with applicable laws and regulations, with the option of paying Provider's discounted cash price as that term is defined in federal regulations.

(11) Provider must give Participant written notice at least 14 days before filing a standard appeal on the Participant's behalf, and the filing must include an attestation that timely notice of intent to file an appeal has been provided. Provider must notify Participant in writing within 7 days after the appeal is filed that the appeal has been filed, and attest to Evernorth that timely notice of filing has been provided. If Provider subsequently withdraws the appeal, written notice of the withdrawal must be provided to Participant and attested to Evernorth within 7 days after the appeal is withdrawn.

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MARYLAND REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Maryland regarding provider contracts with providers rendering health care services in the State of Maryland. To the extent that such Maryland laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Maryland laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) The definition for **Emergency Services**, if any, shall comply with Maryland laws and regulations to the extent applicable.

(2) The following definition of **Experimental Medical Care** is added to the Agreement:

"Experimental Medical Care" means medical, surgical or other health care procedures and treatments which are experimental or investigational, as determined by Evernorth's Medical Director in accordance with consensus derived from peer review medical and scientific literature and the practice of the national medical community, including (i) any procedures or treatments which are not recognized as conforming to accepted medical practice; (ii) any procedures or treatments in which the scientific assessment of the technique, or its application for a particular condition, has not been completed or its effectiveness has not been established; and (iii) any procedures or treatments for which the required approval of a governmental agency has not been granted at the time the services are rendered. Covered Services do not include Experimental Medical Care.

However, to the extent applicable under Section 15-827 of the Insurance Article, Covered Services include the patient cost to a Participant in a clinical trial, as a result of:

(a) treatment provided for a life-threatening condition; or
(b) prevention, early detection, and treatment studies on cancer.

Coverage shall be required if:

(1) The treatment is being provided or the studies are being conducted in a Phase I, Phase II, Phase III, or Phase IV clinical trial for cancer; or the treatment is being provided in a Phase I, Phase II, Phase III, or Phase IV clinical trial for any other life-threatening condition.
(2) The treatment is being provided in a clinical trial approved by:
    (i) one of the National Institutes of Health;
    (ii) a National Institutes of Health cooperative group or a National Institutes of Health center;
    (iii) the FDA in the form of an investigational new drug application;
    (iv) the federal Department of Veterans Affairs; or

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(v) an institutional review board of an institution in the state which has a multiple project assurance contract approved by the Office of Protection from Research Risks of the National Institutions of Health.

(3) The facility and personnel providing the treatment are capable of doing so by virtue of their experience, training, and volume of patients treated to maintain expertise.

(4) There is no clearly superior noninvestigational treatment alternative.

(5) The available clinical or preclinical data provide a reasonable expectation that the treatment will be at least as effective as the noninvestigational alternative.

Covered Services include the patient cost incurred for drugs and devices that have been approved for sale by the FDA whether or not the FDA has approved the drug or device for use in treating the patient's particular condition, to the extent that the drugs or devices are not paid for by the manufacturer, distributor, or provider of that drug or device.

An entity seeking coverage for treatment in a clinical trial approved by an institutional review board under subsection (2)(v) above shall post electronically and keep up-to-date a list of the clinical trials meeting the requirements of this section. The list shall include for each clinical trial: (a) the phase for which the trial is approved; (b) the entity approving the trial; (c) whether the trial is for treatment of cancer or another life- threatening disease and, if not cancer, the particular disease; and the estimated number of patients in the trial.

(3) Provider has the right to elect not to serve on a provider panel for workers' compensation services.

(4) If Provider is a hospital, where precertification is not required for a hospital admission, Provider must notify Evernorth or its designee within twenty-four (24) hours after the admission as prescribed in the Medical Management Program, unless in the case of an emergency inpatient admission, the Participant's medical condition prevents Provider from determining Participant's insurance status and any applicable emergency admission notification requirements.

(5) To the extent required by § 15-1005 of the Maryland Insurance laws, Provider shall submit claims on the appropriate claim form for all Covered Services within one hundred and eighty (180) days of the date those services are rendered. Claims received after this one hundred and eighty (180) day period may be denied for payment.

Pursuant to Maryland law, to the extent applicable, within thirty (30) days after Payor's receipt of a claim Payor shall pay any undisputed fee-for-service amounts owing under the Agreement in accordance with this provision. If Payor contests the claim, denies all or part of the claim, or needs additional information to adjudicate the claim, Payor shall send a notice of receipt and status of the claim that states: (i) that Payor refuses to reimburse all or part of the claim and the reason for the refusal; (ii) that the legitimacy of the claim or the appropriate amount of reimbursement is in dispute and additional information is necessary to determine if all or part of the claim will be reimbursed and what specific additional information is necessary; or (iii) that the claim is not clean and the specific additional information necessary for the claim to be considered a clean claim. "Clean claim" means a claim for reimbursement as defined in regulations adopted by the Commissioner under Section 15-1003 of the Annotated Code of Maryland, as may be changed from time to time.

If additional information is requested by Payor, Payor shall pay or deny the claim, or portion of the claim, within thirty (30) days after receipt of the required additional information. If Payor fails to comply with the requirements of this provision, Payor shall pay interest on the amount of the claim

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that remains unpaid thirty (30) days after the claim is received by Payor. The applicable monthly interest rate shall be: 1.5% for claims paid from the 31st through the 60th day; 2% for claims paid from the 61st through the 120th day; and 2.5% after the 120th day. Any interest owing under the Agreement shall be paid without the necessity of filing an additional claim for such interest.

(6) Pursuant to Maryland law, to the extent applicable, Payor may only retroactively deny reimbursement made to Provider during the six (6) month period after the date Payor paid such claim. Notwithstanding the foregoing, Payor may retroactively deny reimbursement for services that are subject to coordination of benefits with another carrier, the Maryland Medical Assistance Program, or the Medicare program during the eighteen (18) month period after the date Payor paid such claim. The term "reimbursement" includes any applicable capitation payments made to Provider.

The requirements of this provision do not apply if Payor retroactively denies reimbursement to Provider because: (1) the information submitted to the Payor was fraudulent; (2) the information submitted to Payor was improperly coded and Payor provided sufficient information regarding the coding guidelines used by Payor at least thirty (30) days prior to the date the services subject to the retroactive denial were rendered; or (3) the claim submitted to Payor was a duplicate claim.

(7) Termination of the Agreement, by either party, shall be upon at least ninety (90) days' prior written notice by the terminating party, unless said termination is for reasons related to fraud, patient abuse, incompetency, or loss of licensure status. To the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement, that notification period will apply.

(8) In the event that Provider terminates the Agreement, Provider shall continue to furnish health care services to Participants for whom Provider was responsible for the delivery of health care services prior to the notice of termination for at least ninety (90) days after the date of the notice of termination.

(9) Evernorth may not prohibit Provider from discussing with or communicating to Participant information that is necessary for the delivery of health care services, including:

(a) communications that relate to treatment alternatives:
(b) communications that are necessary or appropriate to maintain the provider- patient relationship while the patient is under Provider's care;
(c) communications that relate to Participant's right to appeal a coverage determination with which Provider or Participant does not agree; and
opinions and the basis of an opinion about public policy issues.

(10) Disclosure of Carriers. Pursuant to Section 15-112.2 of the Insurance Article, a list of carriers shall be available to Provider at **Provider.Evernorth.com**.

(11) Services After Hours. Pursuant to Section 15-136 of the Insurance Article, Evernorth or Payor shall, as specified in the Agreement or an Exhibit to the Agreement, pay a bonus to a primary care provider when rendering services, pursuant to contracts or policies that provide hospital, medical or surgical benefits to individuals or groups on an expense-incurred basis, in the office between the hours of 6 p.m. and 8 a.m. or on weekends or national holidays.

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(12) The following guidelines represent both Evernorth's and the state of Maryland's requirements for appointment accessibility per Maryland Code Reg 31.10.44.06.

(a) Urgent care: Within 72 hours. This includes:
- Inpatient urgent care services for mental health services;
- Inpatient urgent care services for substance use disorder services;
- Outpatient urgent care services for mental health services; and
- Outpatient urgent care services for substance use disorder services.

(b) Routine Primary Care: Within 15 calendar days.
(c) Preventative care/well visits: Within 30 calendar days.
(d) Non-urgent specialty care services: Within 30 calendar days.
(e) Non-urgent mental health care services: Within 10 calendar days.
(f) Non-urgent substance use disorder care services: Within 10 calendar days.

(13) Nurse Practitioners in Maryland: Evernorth contracts with nurse practitioners in Evernorth's network in Maryland who meet credentialing requirements and who are practicing primary care medicine. Complete an application with CAQH or call Evernorth at 800.88Evernorth (882.4462) to begin the application process.

(14) Electronic Health Records: In 2009, Maryland became the first state to require that certain state-regulated payers, including Evernorth, provide incentives to select providers to promote the adoption of electronic health records (EHRs) with the passage of House Bill 706 Electronic Health Records – Regulation and Reimbursement. For more information, visit: **http://mhcc.maryland.gov/mhcc/pages/hit/hit_ehr/hit_ehr_state_incentive.aspx**.

(15) Patient Centered Medical Home Program: Evernorth is participating in the Maryland Multi-Payer Patient Centered Medical Home Pilot (MMPP).

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MASSACHUSETTS REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Massachusetts regarding provider contracts with providers rendering health care services in the State of Massachusetts. To the extent that such Massachusetts laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Massachusetts laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) "Emergency Medical Condition" is a medical condition, whether physical or mental, manifesting itself by symptoms of sufficient severity, including severe pain, that the absence of prompt medical attention could reasonably be expected by a prudent layperson who possesses an average knowledge of health and medicine, to result in placing the health of an insured or another person in serious jeopardy, serious impairment to body function, or serious dysfunction of any body organ or part, or, with respect to a pregnant woman, as further defined in § 1867(e)(1)(B) of the Social Security Act, 42 U.S.C. § 1395dd(e)(1)(B).

(2) "Medical Necessity" or "Medically Necessary" is defined as, health care services that are consistent with generally accepted principles of professional medical practice as determined by whether:

(a) the service is the most appropriate available supply or level of service for the insured in question considering potential benefits and harms to the individual;

(b) is known to be effective, based on scientific evidence, professional standards and expert opinion, in improving health outcomes; or

(c) for services and interventions not in widespread use, is based on scientific evidence.

(3) If Evernorth contracts with another entity to perform some or all of the functions governed by the requirements of M.G.L. c. 176O, Evernorth shall be responsible for ensuring compliance by said entity with the applicable provisions of Massachusetts law. Any failure by said entity to meet such requirements shall be the responsibility of Evernorth to remedy and shall subject Evernorth to any and all enforcement actions, including financial penalties authorized under M.G.L. c. 176O.

(4) Notwithstanding anything to the contrary set forth in the Agreement, the following shall be applicable to the Compensation section of the Agreement:

(a) Within forty-five (45) days after Payor's receipt of Provider's completed claim, Payor shall: (i) pay for any fee-for-service amounts owing under this Agreement for such health care services provided; (ii) notify Provider in writing of the reason or reasons for nonpayment or (iii) notify Provider in writing of what additional information or documentation is necessary to complete the claim for reimbursement.

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(b) If Payor fails to comply with the requirements of (a) above, Payor shall pay 1½ % interest per month, not to exceed 18% per year, accruing beginning forty-five (45) days after the Payor's receipt of request for reimbursement. Interest payments shall not apply if the claim for the Covered Service was submitted fraudulently or negligently or Provider was already paid for the Covered Service.

(5) Provider shall comply with Evernorth's requirements for utilization review, quality management and improvement, credentialing and the delivery of preventive health services.

(6) Provider shall provide ninety (90) days advance written notification to Evernorth should Provider plan to implement a policy to charge a fee to Participants as a condition to be part of Provider's panel for care.

(7) Evernorth shall not refuse to contract with or compensate Provider for Covered Services solely because Provider has in good faith communicated with or advocated on behalf of one or more prospective, current or former patients regarding the provisions, terms or requirements of a Plan as they relate to the needs of Provider's patients, or communicated to one or more prospective, current or former patients with respect to the method by which Provider is compensated for Covered Services. Nothing in this provision shall be construed to preclude Evernorth from requiring Provider to hold confidential specific compensation terms.

(8) In the event of termination of the Agreement by either party, the written notice of termination must include the reason(s) for such termination. The parties do not have the right to terminate the Agreement without providing a reason for such termination, as neither party has the right to terminate the Agreement without cause.

(9) Pursuant to the requirements of M.G.L. c. 176O, Section 15 to the extent applicable to the Agreement:

(a) Evernorth shall notify a Participant at least thirty (30) days before the disenrollment of such Participant's primary care physician and shall permit such Participant to continue to be covered for health services, consistent with the terms of the Plan, by such primary care physician for at least thirty (30) days after said primary care physician is disenrolled, other than disenrollment for quality-related reasons or for fraud. Such notice shall also include a description of the procedure for choosing an alternate primary care physician.

(b) Evernorth shall allow any female Participant who is in her second or third trimester of pregnancy and whose provider in connection with her pregnancy is involuntarily disenrolled, other than disenrollment for quality-related reasons, or for fraud, to continue treatment with said provider, consistent with the terms of the Plan, for the period up to and including the Participant's first postpartum visit.

(c) Evernorth shall allow any Participant who is terminally ill and whose provider in connection with said illness is involuntarily disenrolled, other than disenrollment for quality-related reasons, or for fraud, to continue treatment with said provider, consistent with the terms of the Plan, until the Participant's death.

(d) During the period of continued treatment by a provider under subsections (a), (b) and (c), of this provision, Provider must agree: (1) to accept reimbursement from Payor at the rates applicable prior to the notice of disenrollment as payment in full and not to impose cost sharing with respect to the Participant in an amount that would exceed the cost sharing that could have been imposed if

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Provider had not been disenrolled; (2) to adhere to the quality assurance standards of Evernorth and to provide Evernorth with necessary medical information related to the care provided; and (3) to adhere to Evernorth's policies and procedures, including procedures regarding referrals, obtaining prior authorization and providing services pursuant to a treatment plan, if any, approved by Evernorth. Nothing in this provision shall be construed to require the coverage of benefits that would not have been covered if Provider remained a Participating Provider.

(10) Evernorth shall notify Provider in writing of modifications in provider compensation, modifications in Covered Services, or modifications in Evernorth's procedures, documents or requirements, including those associated with utilization review, quality management and improvement, credentialing and preventive health services, that have a substantial impact on the rights or responsibilities of Provider, and the effective date of the modifications. The notice shall be provided sixty (60) days before the effective date of such modification.

(11) Provider is not required to indemnify Evernorth for any expenses and liabilities, including, without limitation, judgments, settlements, attorneys' fees, court costs and any associated charges, incurred in connection with any claim or action brought against Evernorth based on Evernorth's management decisions, utilization review provisions or other policies, guidelines or actions.

(12) In the event of Payor's insolvency, Provider shall look solely to Payor for compensation for Covered Services, except for Copayments, Deductibles or Coinsurance. Under no circumstances shall Provider directly or indirectly make any charges or claims, other than for Copayments, Deductibles or Coinsurance, against any Participants or their representatives. This provision shall survive the termination of the Agreement for services rendered prior to the termination, regardless of the cause of the termination.

(13) Pursuant to the requirements of M.G.L. c. 176O § 9A and 211 CMR 152.00 to the extent applicable to the Agreement:

(a) Provider shall have the right to opt-out of any new select network, limited, regional or tiered network health benefit plan introduced by Evernorth at least sixty (60) days before the plan is submitted for regulatory approval.

(b) Nothing in the Agreement shall be construed to: guarantee Provider a right to participate in any Evernorth select network or tiered network plan; require or permit either party to alter or terminate the Agreement, in whole or in part, to affect parity with an agreement with other carriers or health care providers based on Evernorth's decision to introduce or modify a select network plan or tiered network plan; require Evernorth to place all members of Provider in the same tier of a tiered network plan; or require Evernorth to include all members of Provider in a select network plan on an all-or- nothing basis. Any supplemental payment required or permitted under the Agreement shall not be enforceable unless each payment has been publicly disclosed as a condition of state accreditation. To the extent the Agreement contains any provision prohibited by this section, the duties and obligations of the provision shall apply only to health benefit plans not subject to M.G.L. c. 176O § 9A and 211 CMR 152.00.

(c) Evernorth shall notify Provider in writing at least sixty (60) days before the effective date of any modification to the process used to classify participating providers by benefit tier, the timelines used to make and implement reclassification decisions by benefit tier, the information collected from participating providers, and the criteria used to make classifications.

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(d) Provider shall have the right to receive notification of classification to a benefit tier, an explanation of the past experience and other criteria used to make classification decisions, and to appeal classification decisions and receive an appeal decision prior to the new classification being made available.

(e) Evernorth shall notify participating providers about health benefit plans that use networks subject to 211 CMR 152.00 as set forth in the Medical Management Program.

(14) Provider shall use Telehealth technology that conforms to applicable federal and state health information privacy and security standards as well as standards for informed consent.

(15) Provider shall, in addition to the provider directory requirements found in the Administrative Guidelines, promptly advise Evernorth when their availability to see new patients changes, including when implementing a waitlist; Evernorth shall make changes to reflect changes to Provider availability within two business days of receiving notice of a change in status.

(16) Evernorth shall, should the Agreement extend to behavioral health service providers as defined by applicable law or any entity that manages or administers mental health or substance use disorder benefits for Evernorth and behavioral health service providers, establish a base fee schedule, as defined by applicable law, for evaluation and management services for such providers that is not lower than the base fee schedule used for evaluation and management services for primary care providers, as defined by applicable law, of the same or similar licensure type in the same geographic region.

(17) With respect to the 48 hour notification requirement 211 CMR 52.14 (d):

(a) insureds have the opportunity to obtain health care services for an Emergency Medical Condition, including the option of calling the local pre-hospital emergency medical service system by dialing the emergency telephone access number 911, or its local equivalent, whenever the insured is confronted with an Emergency Medical Condition which in the judgment of a prudent layperson would require pre-hospital emergency services;

(b) no insured shall in any way be discouraged from using the local pre-hospital emergency medical service system, the 911 telephone number, or the local equivalent;

(c) no insured will be denied coverage for medical and transportation expenses incurred as a result of such Emergency Medical Condition; and

(d) if the carrier requires an insured to contact either the carrier or its designee or the primary care provider of the insured within forty-eight (48) hours of receiving Emergency Services, that notification already given to the carrier, designee or primary care provider by the attending emergency provider shall satisfy that requirement.

(18) With respect to Medical Management:

(a) Evernorth shall make an initial determination regarding a proposed admission, procedure or service that requires such a determination within two (2) business days of obtaining all necessary

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information. For the purposes of this section, "necessary information" shall include the result of any face-to-face clinical evaluation or second opinion that may be required. In the case of a determination to approve an admission, procedure or service, Evernorth shall notify the health care professional by telephone within twenty-four (24) hours, and shall provide written or electronic confirmation of the telephone notification to the Participant and the health care professional with two (2) business days thereafter. In the case of an adverse determination, Evernorth shall notify the health care professional by telephone within twenty-four (24) hours, and shall provide written or electronic confirmation of the telephone notification to the Participant and the health care professional within one (1) business day thereafter.

(b) Evernorth shall make a continued stay review determination within one (1) business day of obtaining all necessary information. In the case of a determination to approve an extended stay or additional services, Evernorth shall notify the health care professional by telephone within one (1) business day, and shall provide written or electronic confirmation to the Participant and the health care professional within one (1) business day thereafter. A written or electronic notification shall include the number of extended days or the next review date, the new total number of days or services approved, and the date of admission or initiation of services. In the case of an adverse determination, Evernorth shall notify the health care professional by telephone within twenty-four (24) hours, and shall provide written or electronic notification to the Participant and the health care professional within one (1) business day thereafter. The service shall be continued without liability to the Participant until the Participant has been notified of the determination.

(c) Evernorth shall make a post-acute care determination by the next business day following receipt of all necessary information to establish medical necessity of the requested service; provided, however, that no new admission may occur until the applicable pre-admission screening and resident review required pursuant to 42 CFR 483 is complete.

If the calendar day immediately following the date of submission of the completed request is not an Evernorth business day, and Evernorth cannot otherwise make a determination by the next calendar day, and the receiving post-acute care facility or agency is both open to new admissions and has indicated that said facility or agency will accept the enrollee, then prior authorization must be waived and Evernorth must provide coverage and may begin its concurrent review of the admission on the next business day.

If the calendar day immediately following the date of submission of the completed request is not a business day, Evernorth may approve or deny the prior authorization request within 1 calendar day or the prior authorization is waived. If prior authorization is waived, concurrent review is permitted on the next business day.

Evernorth is not permitted to retrospectively deny coverage for services to an enrollee admitted to a post-acute care facility or transitioned to a post-acute care agency after a waiver of prior authorization unless the claim was a result of fraud, waste or abuse.

An adverse determination of a prior authorization request pursuant to this section may be appealed by an enrollee or the enrollee's provider and such appeal, in the case of an enrollee of a commercial payer, shall be subject to the expedited grievance process pursuant to clause (iv) of subsection (b) of section 13 of chapter 176O of the General Laws.

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In the case of non-emergency transportation between an acute care hospital and a post-acute care facility, payers must approve or deny a request for prior authorization according to the same process outlined above and once authorization has been granted, it will be valid for not less than 7 calendar days following approval.

Nothing in this section shall be construed to require a payer to reimburse for services that are not a covered benefit.

(d) The written notification of an adverse determination shall include a substantive clinical justification therefore that is consistent with generally accepted principles of professional medical practice, and shall, at a minimum: (a) identify the specific information upon which the adverse determination was based; (b) discuss the patient's presenting symptoms or condition, diagnosis and treatment interventions and the specific reasons such medical evidence fails to meet the relevant medical review criteria; (c) specify any alternative treatment option offered by Evernorth, if any; (d) reference and include applicable clinical practice guidelines and review criteria; and (e) include a clear, concise and complete description of Evernorth's formal internal grievance process and procedures for obtaining external review pursuant to 105 CMR 128.400.

(e) Evernorth shall give the health care professional treating a Participant an opportunity to seek reconsideration of an adverse determination from a clinical peer reviewer in any case involving an initial determination or a concurrent review determination. The reconsideration process shall occur within one (1) business day of the receipt of the request and shall be conducted between the health care professional rendering the service and clinical peer reviewer or a clinical peer designated by the clinical peer reviewer, if said reviewer cannot be available within one (1) business day. If the adverse determination is not reversed by the reconsideration process, the Participant, or the health care professional on behalf of the Participant, may pursue the grievance process established pursuant to 105 CMR 128.000. . The reconsideration process allowed pursuant to 211 CMR 52.08(6) shall not be a prerequisite to the internal grievance process or an expedited appeal required by 105 CMR 128.000.

(19) With respect to Quality Management:

(a) Evernorth shall conduct an annual survey of customers to assess satisfaction with access to specialist services, ancillary services, hospitalization services, durable medical equipment and other covered services. The survey shall compare the actual satisfaction of customers with projected measures of their satisfaction. If incentive plans are utilized, Evernorth shall establish mechanisms for monitoring the satisfaction, quality of care and utilization compared with projected utilization of health care services by customers.

(20) With respect to Specialing Services:

(a) Per Massachusetts Division of Insurance Bulletin 2018-01, Evernorth Behavioral Health will provide necessary authorization for medically necessary Special Services ("Specialing"). Specialing is medically necessary additional services or support that has been identified by an inpatient facility as needed to enable the facility to admit a patient (i.e., 1:1 staff member/patient ratio, payment to convert a double occupancy room to a single, extra staff). Specialing can be requested before or during a psychiatric admission by contacting Evernorth Behavioral Health's Utilization Management department. Evernorth does not dictate the billing codes used for Specialing Services. Services will be paid in accordance with the provider agreement, unless a single-case

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agreement is necessary.

(21) With respect to the Behavioral Health Treatment and Referral Platform (BH TRP)

(a) The Massachusetts Executive Office of Health and Human Services (EOHHS), in partnership with PointClickCare, provides a technology platform to support inpatient psychiatric (IP Psych) treatment, referral, and admission processes for patients in Emergency Departments (Eds) and medical/surgical floors in the Commonwealth who need IP Psych level of care (LOC).

(b) All acute hospitals (with an ED and/or IP Psych units) and freestanding IP Psych Facilities in the Commonwealth are required to use the BH TRP as specified in the Expedited Psychiatric Inpatient Admissions (EPIA) protocol.

(22) Covered Practitioners in Massachusetts: A carrier that requires the designation of a primary care physician shall provide Participants with an opportunity to select a participating nurse practitioner as a primary care physician or to change their primary care physician to a participating nurse practitioner at any time during their coverage period. Evernorth accepts applications from licensed Nurse Practitioners who wish to be contracted as a primary care practitioner and appear in Evernorth's directory. Additionally, MA law requires carriers recognize Physician Assistants (PAs) as participating providers and if a carrier requires the designation of a primary care practitioner, it shall provide customers with an opportunity to select a participating Physician Assistant as a primary care practitioner. A participating Physician Assistant practicing within the scope of their license, including all regulations requiring collaboration with or supervision by a physician under section 9E of chapter 112, shall be considered qualified within the carrier's definition of primary care provider to an insured. To request an application or for more information, contact Evernorth at 800.88Evernorth. (882.4462).

(23) Pain Management: In compliance with Massachusetts statute ALM GL ch. 175, § 47KK and Bulletin 2019-06, Evernorth has identified possible alternative pain management services and medications that may be covered for Provider's Evernorth patients. Listed below are some of Evernorth's non-medication services and medications that may be alternative treatments to opioids for treating pain. These are typically covered under Evernorth's plans, but Provider will need to check coverage for each individual patient as coverage will vary based on the customer's plan.

(a) Non-Medication Treatment Alternatives for Pain:

- Acupuncture
- Chiropractic
- Physical therapy
- Occupational therapy
- Physician medicine/rehab
- Cognitive behavioral therapy
- Nutrition counseling
- Osteopathic manipulation medicine
- Interventional pain management nerve block
- Spine surgery
- TENS unit

(b) Non-Opioid Medication Alternatives for Pain:

- NSAIDs
- Topical analgesics
- COX-2 inhibitors
- Skeletal muscle relaxants
- Antidepressants

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- Anticonvulsants
- Corticosteroids

Provider can find more information about these and other possible alternatives to treating pain at Evernorth's "help with pain" website (**https://www.Evernorth.com/helpwithpain/**) or by reviewing Evernorth's coverage policies (**https://www.Evernorth.com/health-care-providers/coverage-and-claims/policies/**).

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MICHIGAN REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Michigan regarding provider contracts with providers rendering health care services in the State of Michigan. To the extent that such Michigan laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Michigan laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) Claims for Covered Services must be submitted within one (1) year of the date of service or the date of discharge from the facility.

(2) With respect to a Provider who is a physician:

If the Agreement is terminated, Provider may, at the option of Participant, continue to provide Covered Services for an ongoing course of treatment for which a Participant was under Provider's care at the time of such termination so long as Participant retains eligibility under a Plan, until (a) for a Participant in the second or third trimester of pregnancy at the time of notice of the termination or pending termination, the transitional period shall extend through postpartum care related to the delivery; (b) for a period up to six (6) months in the case of a Participant who at the time of the termination has been diagnosed with a terminal illness; or (c) for a period up to ninety (90) days after the effective date of termination for all other Participants in an active course of treatment.

During the period of continued care, Provider shall continue (a) to accept as payment in full reimbursement at the rates applicable prior to the termination;
(b) to adhere to Evernorth's standards for maintaining quality health care and to provide to Evernorth necessary medical information related to the care; and (c) to adhere to Evernorth's policies and procedures, including, but not limited to, those concerning utilization review, referrals, prior authorizations and treatment plans. Provider has no obligation under the Agreement to provide services to individuals who cease to be Participants.

(3) An insurer that delivers, issues for delivery, or renews in this state a health insurance policy cannot discriminate regarding participating under a health insurance policy against a health professional who is acting within the scope of the health professional's license.

(4) An insurer is not required to contract with any health professional willing to abide by the terms and conditions for participation established by the insurer.

(5) An insurer can establish varying reimbursement rates based on quality or performance measures.

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MINNESOTA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Minnesota regarding provider contracts with providers rendering health care services in the State of Minnesota. To the extent that such Minnesota laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Minnesota laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

I

1. The definition for Emergency Services, if any, shall comply with Minnesota laws and regulations to the extent applicable.

2. In addition to the negotiated terms of the Agreement governing amendments to the Agreement, the following shall apply in accordance with applicable laws and regulations:

   A. Evernorth shall disclose to Provider at least forty-five (45) days prior to the effective date of any proposed amendment or change to the terms of the Agreement. Changes or amendments required by law or governmental regulatory authority will be disclosed when the requirement is made known to Evernorth.

   B. Evernorth shall disclose to Provider any amendment or change in the Agreement that alters the fee schedule or materially alters the written contractual policies and procedures governing the relationship between the parties not less than forty-five (45) days before the effective date of the proposed change; Provider may, in accordance with the terms of the Agreement, terminate the Agreement before the amendment or change is deemed to be in effect.

3. Evernorth may terminate this Agreement without cause upon one hundred twenty (120) days' notice to Provider.

4. In the event this Agreement contains a Most Favored Nation provision, the Most Favored Nation provision is null and void to the extent that it is prohibited under applicable Minnesota laws and regulations.

II

1. If Provider is a hospital, ambulatory surgical center, or freestanding emergency room, in addition to the negotiated terms of the Agreement establishing obligation on amendments to the Agreement, the following shall apply in accordance with applicable laws and regulations:

   A. Evernorth shall disclose to Provider any proposed amendment or change to the terms

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of the Agreement.

B. Evernorth shall disclose to Provider any amendment or change in the Agreement that alters the financial reimbursement or alters the written contractual policies and procedures governing the relationship between the parties before the amendment or change is deemed to be in effect.

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MISSISSIPPI REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Mississippi regarding provider contracts with providers rendering health care services in the State of Mississippi. To the extent that such Mississippi laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Mississippi laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

1. In addition to the negotiated terms of the Agreement establishing limits on billing Participants, the following shall apply in accordance with applicable laws and regulations:

A. Provider agrees that in no event, including but not limited to nonpayment by Evernorth, insolvency of Evernorth, or breach of this Agreement, shall Provider bill, charge, collect a deposit from, seek compensation, remuneration or reimbursement from, or have any recourse against a Participant or a person (other than Evernorth) acting on behalf of Participant for Covered Services provided pursuant to this Agreement. This provision does not prohibit Provider from collecting Coinsurance, Deductibles or Copayments, as specifically provided in the evidence of coverage, or fees for uncovered services delivered on a fee-for-service basis to Participants. Nor does this provision prohibit Provider and Participant from agreeing to continue services solely at the expense of the Participant, as long as Provider has clearly informed Participant that Evernorth may not cover or continue to cover a specific service or services. Except as provided in the Agreement, this provision does not prohibit Provider from pursuing any available legal remedy.

B. In the event of Evernorth's or Provider's insolvency or other cessation of operations, Covered Services to Participants will continue through the period for which a premium has been paid to Evernorth on behalf of the Participant or until discharge from an inpatient facility, whichever time is greater. Covered Services to Participants confined in an inpatient facility on the date of insolvency or other cessation of operations will continue until their continued confinement in an inpatient facility is no longer Medically Necessary.

C. Subsections A and B shall be construed in favor of the Participant, shall survive the termination of the contract regardless of the reason for termination, including Evernorth's insolvency, and shall supersede any oral or written contrary agreement between Provider and Participant or the representative of a Participant if the contrary agreement is inconsistent with the hold harmless and continuation of Covered Services provisions required by subsections A and B or applicable laws and regulations.

D. Provider shall ensure that all contracts entered into with Participating Providers contain a provision substantially similar to subsection A above, or substantially similar to the text required by applicable laws and regulations.

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2. In no event shall Provider collect or attempt to collect from a Participant any money owed to Provider by Evernorth.

3. Nothing in the Agreement shall be construed to offer an inducement under a managed care Plan to Provider to provide less than Medically Necessary services to Participants. Nothing in the Agreement shall be construed to prohibit Provider from discussing treatment options with Participants irrespective of Evernorth's position on the treatment options, or from advocating on behalf of Participants within the utilization review or grievance processes used by Evernorth.

4. Nothing in the Agreement shall be construed to penalize Provider because Provider, in good faith, reports to state or federal authorities any act or practice by Evernorth that jeopardizes patient health or welfare.

5. To the extent required by applicable law, including but not limited to Miss. Code Ann. §83-41-219, as may be amended from time to time, Evernorth or Payor shall have the same time limit following payment of a claim to perform any review or audit for reconsidering the validity of the claim and requesting reimbursement for payment of an invalid claim or overpayment of a claim as Provider has to submit a claim for payment under this Agreement. To the extent the Agreement does not limit the time in which Provider is required to submit a claim for payment, Payor shall not request reimbursement of offset another claim payment for reimbursement of an invalid claim or overpayment of a claim more than twelve (12) months after the payment of an invalid claim or overpaid claim. Nothing in this section shall apply to claims submitted in the context of misrepresentation, omission, concealment, or fraud, or to claims submitted by Provider for reimbursement under the Mississippi Medicaid Program.

6. Evernorth and Provider shall provide at least sixty (60) days written notice to each other before terminating the Agreement without cause. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement by Provider, such longer notification period will apply. Within five (5) working days of the date that Provider either gives or receives notice of termination, Provider shall supply a list of those Participants that are patients of the Provider.

7. To the extent permitted by applicable law, Evernorth shall retain the right to immediately terminate the Agreement upon a valid order issued by the Mississippi Department of Insurance or other lawful authority.

8. In accordance with applicable laws and regulations, Evernorth shall have the right to approve or disapprove the participation status of a participating provider in Provider's contracted network for the purpose of delivering Covered Services to Evernorth's Participants.

9. Evernorth's rights and responsibilities under the Agreement shall not be assigned or delegated by Provider without prior written consent. Nothing in the Agreement shall be construed to delegate or assign to Provider Evernorth's statutory responsibility to monitor the offering of Covered Services to Participants.

10. In accordance with applicable laws and regulations, Evernorth shall have the right, in the event of Provider's insolvency, to require the assignment to Evernorth of the provisions of Provider's contract addressing the Provider's obligation to furnish Covered Services.

11. In addition to the negotiated terms of the Agreement, Evernorth shall have access to all Provider's subcontracts, and shall have the right to make copies to facilitate regulatory review, upon twenty (20) days

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prior written notice from Evernorth.

12. In addition to the negotiated requirements of the Agreement, Provider shall maintain the books, records, financial information and documentation of services provided to Participants at its principal place of business in the state and shall preserve them in a manner that facilitates regulatory review.

13. Provider shall allow the commissioner of insurance access to Provider's books, records, financial information and any documentation of services provided to Participants, as necessary to determine compliance with applicable laws and regulations.

14. In addition to the negotiated requirements of the Agreement establishing requirements for records, Provider shall make health records available to appropriate state and federal authorities involved in assessing the quality of care or investigating the grievances or complaints of Participants, and to comply with the applicable state and federal laws related to the confidentiality of medical or health records.

15. If applicable under the terms of the Agreement, Provider shall transmit utilization documentation and claims paid documentation to Evernorth as set forth in the Agreement. Evernorth shall monitor the timeliness and appropriateness of payments made to providers and health care services received by Participants.

16. Definitions or other provisions in the Agreement shall be construed to avoid conflict with the definitions or provisions contained in the managed care plan, or in applicable laws and regulations.

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MISSOURI REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Missouri regarding provider contracts with providers rendering health care services in the State of Missouri. To the extent that such Missouri laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Missouri laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

A. (1) The definitions for Emergency Medical Condition and Emergency Services are amended to read in their entirety as follows:

Emergency Medical Condition - Means the sudden and, at the time, unexpected onset of a health condition (emergency medical condition) that manifests itself by symptoms of sufficient severity that would lead a prudent layperson, possessing an average knowledge of health and medicine, to believe that immediate medical care is required, which may include, but shall not be limited to:

1. placing the person's health in significant jeopardy;
2. serious impairment to a bodily function;
3. serious dysfunction of any bodily organ or part;
4. inadequately controlled pain; or
5. with respect to a pregnant woman who is having contractions;
   a. that there is inadequate time to effect a safe transfer to another hospital before delivery; or
   b. that transfer to another hospital may pose a threat to the health or safety of the woman or unborn child.

Emergency Services - Means health care items and services furnished or required to screen and stabilize an emergency medical condition, which may include, but shall not be limited to, health care services that are provided in a licensed hospital's emergency facility by an appropriate provider.

(2) Provider shall be bound by and comply with the provisions of all applicable State of Missouri Regulations and Statutes (including but not limited to the credentialing and recredentialing requirements of 20 CSR 400-7.180), all applicable federal laws and regulations, and Evernorth Medical Management Program. Provider shall comply with the requirements of and shall participate in quality management and utilization management.

(3) The Agreement shall be governed by applicable State of Missouri Regulations and Statutes and any applicable federal law.

(4) To the extent required by RSMo § 376.384, Provider shall file a claim for reimbursement for a health care service provided in this state for a period of up to six (6) months from the date of service, unless the Agreement specifies a different standard.

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B. With respect to Covered Services rendered to Participants covered under an HMO Plan:

(1) Evernorth shall give Provider thirty (30) days to review the Agreement.

(2) Pursuant to RSMo 354.603.1(3), Evernorth shall monitor, on an on-going basis, the ability, clinical capacity, and legal authority of Provider to furnish all Covered Services to Participants.

(3) Throughout the term of the Agreement, Provider shall maintain general and professional liability coverage in a form and amount acceptable to Evernorth ($_1 million_per occurrence and $ 3 million_in the aggregate).

(4) Payor shall not request a refund or offset against a claim more than twelve (12) months after a Payor has paid a claim except in cases of fraud or misrepresentation by Provider.

(5) Evernorth will not terminate the Agreement solely because Provider has:

a. advocated on behalf of a Participant;
b. filed a complaint against Evernorth;
c. appealed a decision made by Evernorth;
d. provided information or filed a report with the Missouri Department of Commerce & Insurance; or
e. requested a hearing or review.

(6) Evernorth and Provider shall provide at least sixty (60) days written notice to each other before terminating the Agreement with notice. The written notice shall include an explanation of why the Agreement is being terminated. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with regard to termination of the Agreement by Provider or Evernorth, such longer notification period will apply.

(7) If Provider is not a hospital, the following shall apply: Notwithstanding anything to the contrary set forth in the Agreement, the grounds for immediate termination of the Agreement or for the immediate termination of Provider are cases involving imminent harm to patients, a determination of fraud, or a final disciplinary action by a state licensing board or other governmental agency.

(8) If Provider is not a hospital or ancillary provider, the following shall apply:

a. Health Care Professionals:

Notice and Hearing: If Evernorth should choose to terminate Provider, Evernorth shall provide Provider a written explanation of the reason(s) for the proposed termination and give Provider an opportunity for a review or hearing as hereinafter set forth. Provider must notify Evernorth in writing of the request for a hearing within thirty (30) days of receipt of the notice of termination. A hearing will be held within thirty (30) days after receipt of the Provider's request by Evernorth. The hearing shall be conducted by a panel of three (3) or more people appointed by Evernorth, at least one-third of whom shall be clinical peers of Provider. The panel shall render a decision within fifteen (15) days after the hearing and shall notify Provider of the decision in writing which will include one of the following resolutions: (1) unconditional reinstatement; (2) provisional reinstatement subject to certain conditions as set forth by Evernorth; or (3) termination.

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Termination will be effective no earlier than thirty (30) days after Provider's receipt of the panel's decision. In no event shall the termination be effective earlier than sixty (60) days from Provider's receipt of the notice of termination.

Imminent harm, fraud and final disciplinary actions: The above requirements regarding notice and hearing do not apply in cases involving imminent harm to patients, a determination of fraud, or a final disciplinary action by a state licensing board or other governmental agency.

Non-renewal: Any nonrenewal of the Agreement shall not constitute a termination for the purposes of this provision.

(9) Notice to Participants: Upon termination of the Agreement, Evernorth shall give written notice within thirty (30) working days of such termination to all Participants who are seen on a regular basis by Provider. When termination involves a primary care physician, all Participants who are patients of such provider shall be notified. Within fifteen (15) working days of the date of termination Provider shall supply Evernorth with a list of Participants that must be notified of the termination.

(10) 1. Upon termination of the Agreement, Provider shall continue to provide Covered Services for specific conditions for which a Participant was under Provider's care at the time of such termination where the continuation of care is Medically Necessary and in accordance with the dictates of medical prudence, including circumstances such as disability, pregnancy or life-threatening illness, so long as Participant retains eligibility under a Plan, until the earlier of completion of such services, or the expiration of ninety (90) days. Provider shall be compensated for Covered Services provided to any such Participant in accordance with the compensation arrangements under the Agreement. Participants shall not be liable to Provider for any amounts owed for Covered Services provided during such period of continued care other than Copayments, Deductibles or Coinsurance billed in accordance with the terms of a Plan.

2. Provider has no obligation under the Agreement to provide services to individuals who cease to be Participants.

(11) In the event of Evernorth's insolvency or other cessation of operations, Covered Services to Participants shall continue through the period for which a premium has been paid to Evernorth on behalf of the Participant or until the Participant's discharge from an inpatient facility, whichever time is greater. This provision survives termination of this Agreement regardless of the cause giving rise to such termination and shall be construed to be for the benefit of Participants and supersedes any oral or written agreement to the contrary now existing or hereafter entered into between Provider and the Participant or persons acting on the Participant's behalf.

(12) If a party initiates arbitration to resolve a dispute, the arbitrator will be instructed that, to the extent that an external review organization or the Missouri Department of Commerce & Insurance has rendered a final determination as permitted by RSMo 376.1350 through RSMo 376.1399 regarding coverage of a particular service rendered to a particular Participant, the arbitrator will not have authority to revise that final determination.

(13) The Charges to Participants provision survives termination of this Agreement regardless of the cause giving rise to such termination, including Payor's insolvency.

(14) The provision of the Agreement relating to the limitation on billing Participants shall also apply in the event of nonpayment by the intermediary or the intermediary's insolvency or breach of the

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Agreement.

(15) Any modification, addition, or deletion to the provision of the Agreement relating to the limitation on billing Participants shall become effective on a date no earlier than thirty (30) days after the applicable state regulatory agency has received written notice of such proposed changes, or the date permitted by applicable law, whichever is later.

(16) a. Evernorth shall not prohibit or restrict Provider from disclosing to Participant any information that Provider deems appropriate regarding the nature of treatment, risks, or alternatives thereto, the availability of other therapy, consultation or test, the decision of any plan to authorize or deny services, or the process that the plan or any person contracting with the plan uses or proposes to use, to authorize or deny health care services. Evernorth shall not prohibit any other communication expressly protected under all applicable State of Missouri Regulations and Statutes and all applicable federal laws and regulations.

b. Provider's duties to comply with Evernorth's Participant Grievance procedure shall be subject to RSMo 376.1350 through RSMo 376.1399, including in particular Provider's option to appeal Evernorth decisions and those provisions related to determinations to the Missouri Department of Commerce & Insurance or external review organizations. Evernorth shall not penalize Provider because Provider, in good faith, reports to state or federal authorities any act or practice by Evernorth that Provider reasonably determines may jeopardize patient health or welfare.

(17) Claim Payment: Where Missouri statute applies, claims are processed in accordance with Missouri's prompt pay laws.

(18) Credentialing Requirements: For practitioners defined in MO RSMo 376.1575, Missouri requires the following be included in the credentialing process:

a. Within two (2) working days after receipt of a faxed or completed application, Evernorth shall send a notice of receipt to the practitioner.
b. Access to a provider web portal that allows the practitioner to receive notice of the status of an electronically submitted application must be made available - **https://static.evernorth.com/assets/evernorth/provider/resourceLibrary/behavioralResources/doingBusinessWithUs/cbhCredentialing.html**.
c. Within sixty (60) business days of the date of receipt of the completed application, Evernorth must make a decision as to whether to approve or deny the practitioner's credentialing application.

(19) Telehealth/Telemedicine: Missouri has minimum requirements for provision of covered services through telehealth or telemedicine.

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MONTANA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Montana regarding provider contracts with providers rendering health care services in the State of Montana. To the extent that such Montana laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of the Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Montana laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

In the event of Evernorth's insolvency or other cessation of operations, Covered Services to Participants shall continue through the period for which a premium has been paid to Evernorth on behalf of the Participant, but not to exceed thirty (30) days, or until the Participant's discharge from an acute care inpatient facility, whichever time is greater. Participants shall not be liable to Provider for any amounts owed for Covered Services provided during the period of continued care other than Copayments, Deductibles or Coinsurance billed in accordance with the terms of a Plan. Provider further agrees that this provision shall survive the termination of the Agreement regardless of the cause giving rise to such termination and shall be construed to be for the benefit of Participants, and that this provision supersedes any oral or written agreement to the contrary now existing or hereafter entered into between Provider and the Participant or persons acting on the Participant's behalf.

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NEBRASKA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Nebraska regarding provider contracts with providers rendering health care services in the State of Nebraska. To the extent that such Nebraska laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Nebraska laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

1. The definition for Emergency Services, if any, shall comply with Nebraska laws and regulations to the extent applicable.

2. Definitions or other provisions in the Agreement shall be construed to avoid conflict with the definitions or provisions contained in the Participant's Plan, or in applicable laws and regulations.

3. In addition to the negotiated terms of the Agreement establishing limits on billing Participants, the following shall apply to services rendered under Plans subject to applicable laws and regulations:

A. Provider agrees that in no event, including but not limited to, nonpayment by Evernorth or an intermediary, insolvency of Evernorth or an intermediary, or breach of this Agreement, shall Provider bill, charge, collect a deposit from, seek compensation, remuneration, or reimbursement from, or have any recourse against a Participant or a person, other than Evernorth or an intermediary, acting on behalf of Participant for Covered Services provided pursuant to this Agreement. This provision does not prohibit Provider from collecting Coinsurance, Deductibles or Copayments, as specifically provided in Participant's Plan or the evidence of coverage, or fees for non-covered services delivered on a fee-for-service basis to Participants. Nor does this provision prohibit Provider and Participant from agreeing to continue services solely at the expense of the Participant, as long as Provider has clearly informed Participant that Evernorth may not cover or continue to cover a specific service or services. Except as provided in the Agreement, this provision does not prohibit Provider from pursuing any available legal remedy.

B. In the event of Evernorth's or an intermediary's insolvency or other cessation of operations, Covered Services to Participants will continue through the period for which a premium has been paid to Evernorth on behalf of the Participant or until discharge from an inpatient facility, whichever time is greater. Covered Services to Participants confined in an inpatient facility on the date of insolvency or other cessation of operations will continue until their continued confinement in an inpatient facility is no longer Medically Necessary.

C. Subsections A and B shall be construed in favor of the Participant, shall survive the termination of this Agreement regardless of the reason for termination, including Evernorth's insolvency, and shall supersede any oral or written contrary agreement between Provider and Participant or the representative of a Participant if the contrary agreement is inconsistent with

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subsections A and B or applicable laws and regulations.

4. In no event shall Provider collect or attempt to collect from a Participant any money owed to Provider by Evernorth.

5. Evernorth and Provider shall provide at least sixty (60) days written notice to each other before terminating the Agreement without cause. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement by Provider, such longer notification period will apply.

6. The rights and responsibilities under this Agreement shall not be assigned or delegated by Provider without Evernorth's prior written consent.

7. Provider shall furnish Covered Services to Participants without regard to the Participant's enrollment in the Plan as a private purchaser or as a participant in a publicly financed program of health care services. This requirement shall not apply to circumstances when Provider cannot render health care services due to limitations arising from lack of training, experience, skill or licensing restrictions.

8. In addition to the negotiated terms of the Agreement establishing requirements for records, Provider shall make health records available to appropriate state and federal authorities involved in assessing the quality of care or investigating the grievances or complaints of Participants, and to comply with the applicable state and federal laws related to the confidentiality of medical or health records.

9. If Provider is an intermediary as defined by applicable laws and regulations, the following provisions shall apply:

A. Nothing in the Agreement shall be construed to delegate or assign Evernorth's statutory responsibility to monitor the offering of Covered Services to Participants to Provider.

B. Evernorth shall have the right to approve or disapprove of the participation status of a subcontracted provider in its own or a contracted network for the purpose of delivering Covered Services to Participants.

C. Provider shall maintain copies of all Provider health care subcontracts at its principal place of business or Provider shall allow access to all Provider health care subcontracts; Evernorth shall have the right to make copies to facilitate regulatory review upon twenty (20) days' prior written notice.

D. Evernorth shall have the right, in the event of Provider's insolvency, to require the assignment to Evernorth of the provisions of Provider's contracts addressing Provider's obligation to furnish Covered Services.

E. If applicable, Provider shall transmit utilization documentation and claims paid documentation to Evernorth. Evernorth shall monitor the timeliness and appropriateness of payments made to Provider and health care services received by Participants.

F. If applicable, Provider shall maintain the books, records, financial information, and documentation of health care services provided to Participants at its principal place of business in the state and preserve them for five (5) years in a manner that facilitates regulatory review.

G. Provider shall allow regulator access as required by applicable laws and regulations to the Provider's books, records, financial information, and any documentation of health care services provided to Participants, as necessary to determine compliance with the Managed Care Plan Network Adequacy Act.

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NEVADA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Nevada regarding provider contracts with providers rendering health care services in the State of Nevada. To the extent that such Nevada laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Nevada laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. This Addendum shall supersede any previous state mandate amendments to the Agreement. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

1. Definitions. Unless otherwise defined in the Agreement, the following terms shall have the meaning set forth below.

1.1 "Material Change" or "Material Adverse Change" shall mean a change that could reasonably be expected to have a material adverse impact on the aggregate level of payment by Evernorth or Payor to Provider for Covered Services under this Agreement, or on Provider's administration of their services.

1.2 "Timely Notice" shall mean the timeframe or timeframes established by the parties for prior written notice of an amendment to the agreement as set forth in the Amendment section, or any other provisions of the Agreement governing changes or amendments to the Agreement.

2. In addition to the obligations set forth in the Amendment provision of the Agreement, and to the extent permitted or required by applicable Nevada law:

Evernorth may amend this Agreement by providing forty-five (45) days' prior written notice to Provider of the modification or amendment of the schedule of payments, including any changes to the fee schedules applicable to Provider's practice. Failure of Provider to object in writing to any such proposed amendment within forty-five (45) days following receipt of notice shall constitute Provider's acceptance thereof. Timely written notification of rejection of such proposed amendment to Evernorth means that this Agreement shall remain in force without the proposed amendment.

Except as provided above, Amendments to this Agreement shall be agreed to in writing by Evernorth and Provider.

3. The Agreement subsection of the Effect of Termination of the Agreement or Plan listed in Exhibit C section of the Agreement is hereby deleted and replaced with the following:

Except in cases where Provider's participation under this Agreement is terminated due to medical incompetence or professional misconduct, Provider shall, if the Provider and Participant

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agree, continue to provide Covered Services for specific conditions for which a Participant was under Provider's care at the time of such termination until the later of:

a) the one hundred twentieth (120th) day after the date the contract is terminated; or

b) if the medical condition is pregnancy, in accordance with applicable federal law, the ninetieth (90th) day after:

1) the date of delivery; or

2) if the pregnancy does not end in delivery, the date of the end of the pregnancy.

During the continuation period under this section: (1) the parties shall be bound by the terms and conditions of this Agreement; (2) Provider shall be compensated for Covered Services provided to any such Participant in accordance with the compensation arrangements under this Agreement; and (3) Provider are prohibited from billing Participants for any amounts in excess of the Participant's applicable Coinsurance, Copayments or Deductibles. Provider have no obligation under this Agreement to continue to provide services to individuals who cease to be Participants.

3.1 In the event of Evernorth's insolvency or the insolvency of any applicable intermediary, or in the event of any other cessation of Evernorth's operations or the operations of any applicable intermediary, Provider must continue to deliver health care services covered by the network plan, as defined by applicable state law, to a Participant without billing Participant for any amount other than Coinsurance, Deductibles or Copayments, as specifically provided in the evidence of coverage, until the earlier of: the date of the cancellation of Participant's coverage under the network plan pursuant to applicable state law, including, without limitation, any extension of coverage provided pursuant to the terms of the contract between Participant and Evernorth, and applicable state continued medical treatment (continuity of care) laws, or any applicable federal law for Participants who are in an active course of treatment or totally disabled; or the date on which the contract between the Evernorth and Provider would have terminated if the health carrier or intermediary, as applicable, had remained in operation, including, without limitation, any extension of coverage provided pursuant to the terms of the contract between the Participant and Evernorth, and applicable state laws, or any applicable federal law for Participants who are in an active course of treatment or totally disabled.

4. If Provider is not a hospital, the Term and Termination provision of the Agreement is amended as follows:

Either party can terminate this Agreement at any time by providing at least ninety (90) days advance written notice. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement, such longer notification period will apply.

4.1. The Term and Termination provision of the Agreement is amended by adding the following:

Evernorth shall not terminate this Agreement, refuse to contract with, or refuse to compensate Provider because Provider in good faith advocates in private or in public on behalf of a Participant, assists a Participant in seeking reconsideration of a decision to deny coverage for a health care service, or reports a violation of law to an appropriate authority.

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5. Disclosure of Fee Schedules. If Provider is not a hospital:

   Evernorth shall provide the schedule of payments, including any changes to the fee schedules applicable to Provider's practice, if requested at the time the Agreement is executed and at any other time within seven (7) days upon receipt of Provider's request.

6. The following provisions shall, to the extent required by applicable law, replace and supersede the Section of the Agreement entitled Charges to Participants:

   Charges to Participants. Provider agrees that in no event, including but not limited to nonpayment by Payor, Payor's insolvency or breach of this Agreement, shall Provider bill, charge, collect a deposit from, seek compensation, remuneration or reimbursement from, or have any recourse against Participants or persons other than the applicable Payor for Covered Services or for any amounts denied or not paid under this Agreement due to Provider's failure to comply with the requirements of Evernorth's or its designee's utilization management program or Medical Management Program, or failure to file a timely claim or appeal. This provision does not prohibit collection of any applicable Copayments, Coinsurance and Deductibles, as specifically provided in the evidence of coverage. This provision survives termination of this Agreement, is intended to be for the benefit of Participants, and supersedes any oral or written agreement to the contrary now existing or hereafter entered into between Provider and a Participant or persons acting on the Participant's behalf. Modifications to this section will become effective no earlier than the date permitted by applicable law.

   This provision does not prohibit collection of fees for uncovered services delivered on a fee-for-service basis to Participants. This provision does not prohibit Provider and a Participant from agreeing to continue health care services solely at the expense of the Participant, as long as Provider has clearly informed the Participant that Evernorth may not cover or continue to cover a specific health care service or health care services. Except as provided herein, this Agreement does not prohibit Provider from pursuing any available legal remedy.

7. Provisions included in this Agreement to comply with the requirements set forth in Sections 3.1 and 6 shall be construed in favor of the covered person, shall survive the termination of the contract regardless of the reason for the termination, including, without limitation, the insolvency of the health carrier or any applicable intermediary, and shall supersede any oral or written contrary agreement between a participating provider of health care and a covered person or the representative of a covered person if the contrary agreement is inconsistent with provisions included in the contract to comply with the requirements set forth in applicable state law.

8. Evernorth will provide written notice to Provider as soon as practicable in the event that a court determines Evernorth or any applicable intermediary to be insolvent, or of any other cessation of operations of Evernorth or any applicable intermediary.

9. In addition to the requirements of the Agreement governing Participant health records, Provider shall make health records available to appropriate state and federal authorities involved in assessing the quality of care or investigating the grievances or complaints of Participants, and to comply with the applicable state and federal laws related to the confidentiality of medical and health records and Participant's right to see, obtain copies of or amend their medical and health records.

10. Neither Evernorth nor Provider may assign or delegate the rights and responsibilities of either

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party under the contract without the prior written consent of the other party.

11. In addition to the requirements of the Agreement governing standards of care, Provider is responsible for furnishing Covered Services to all Participants without regard to the participation of the Participant in a network plan (as defined by applicable law) as a private purchaser of the network plan or as a participant in a publicly financed program of health care services.

12. In addition to the requirements of the Agreement governing services upon termination, for services rendered by a non-hospital Provider subsequent to Provider's termination of the Agreement to a Participant who had obtained prior authorization for such services prior to the termination of the Agreement, or if a Participant is confined in the hospital at a time when a hospital Provider terminates, coverage for Covered Services provided by a non-hospital Provider to any such Participant and any coverage for the period of confinement for a hospital Provider will be at the rate negotiated before Provider terminated the Agreement and at no additional cost to the Participant, in accordance with the terms of the Participant's Plan.

13. Evernorth shall, in a timely manner, upon request from Provider or upon any change to the status or inclusion of Provider, inform Provider of the status as a provider of health care in a network plan and the status and inclusion of Provider on any list maintained by Evernorth.

14. The Agreement permits Evernorth to contract with another party to provide access to Evernorth's rights and responsibilities under this Agreement. Upon request, Evernorth will provide information necessary to determine whether a particular party has been authorized to access Provider's health care services and contractual discounts under this Agreement. Any party authorized to access the health care services and contractual discounts under this Agreement must comply with all applicable terms, limitations, and conditions of the Agreement.

15. Credentialing: Evernorth is part of the Council for Affordable Quality Healthcare (CAQH) for credentialing and recredentialing network physicians. Evernorth strongly encourages the use of the CAQH Universal Provider DataSource when submitting Provider's application. CAQH does comply with Nevada state application requirements. If Provider does not have access to the internet, however, Provider will need to submit Provider's credentialing information using the Nevada state application.

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NEW HAMPSHIRE REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of New Hampshire regarding provider contracts with providers rendering health care services in the State of New Hampshire. To the extent that such state laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such New Hampshire laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

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(1) Provider agrees that in no event, including but not limited to nonpayment by Payor, Payor's insolvency or breach of this Agreement, shall Provider bill, charge, collect a deposit from, seek compensation, remuneration or reimbursement from, or have any recourse against Participants or persons other than Payor for Covered Services.

(2) Evernorth shall not terminate or refuse to renew the Agreement, discriminate against or penalize Provider for participating in Participant's internal grievance procedure or external review.

(3) Provider is allowed a sixty (60) day period from the postmarked date to review any proposed Agreement and any amendment to an existing Agreement, excluding those modifications that are expressly permitted under the existing Agreement. Failure to object in writing within the sixty (60) day period shall be deemed to constitute acceptance of the proposed Agreement or amendment to the Agreement. However, if the terms, benefits and conditions of the Agreement must be changed to comply with applicable state or federal law or regulation, Provider shall continue to perform services under the Agreement as so modified.

(4) Upon termination of the Agreement, Provider, at the option of Participant, shall continue to provide Covered Services for specific conditions for which a Participant was under Provider's care at the time of such termination so long as Participant retains eligibility under a Plan, until the earlier of completion of such services, or the expiration of sixty (60) days. Provider shall be compensated for Covered Services provided to any such Participant in accordance with the compensation arrangements under the Agreement until sixty (60) days following termination. Provider has no obligation under the Agreement to provide services to individuals who cease to be Participants.

(5) Nothing in the Agreement shall be construed to require or obligate a provider who is employed by a hospital or any affiliate to refer patients to providers also employed or under contract with the hospital or any affiliate.

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NEW JERSEY REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of New Jersey regarding provider contracts with providers rendering health care services in the State of New Jersey. To the extent that such New Jersey laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such New Jersey laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

A. Compliance with New Jersey Statutory and Regulatory Requirements

(1) The following terms shall have the following defined meanings, in accordance with applicable New Jersey laws and regulations. In addition to the forgoing defined terms, all other terms used in this Agreement shall be deemed defined in compliance with New Jersey laws and regulations to the extent applicable.

a. Adverse benefit determination means a denial, reduction or termination of, or a failure to make payment (in whole or in part) for, a benefit, including a denial, reduction or termination of, or a failure to provide or make payment (in whole or in part) for, a benefit resulting from application of any utilization review, denial of a request for an in-plan exception, as well as a failure to cover an item or service for which benefits are otherwise provided because the carrier determines the item or service to be experimental or investigational, cosmetic, dental rather than medical, excluded as a pre-existing condition or because the carrier has rescinded the coverage.

b. Continuous quality improvement or CQI means an on-going and systematic effort to measure, evaluate, and improve either a carrier's process of providing quality health care services to covered persons with respect to managed care plans, or the carrier's process of performing utilization management functions with respect to health benefits plans in which utilization management has been incorporated.

c. Clean Claim. A Clean Claim is a claim that is submitted with all the information, including the substantiating documentation, requested by eviCore or Payor, and (1) The claim is for a service or supply covered by the applicable health benefits plan; (2) The claim is submitted with all the information requested by the carrier on the claim form or in other instructions distributed to the provider or covered person in accordance with N.J.S.A. 17B:30-51 and N.J.A.C. 11:22-1.4; (3) The person to whom the service or supply was provided was covered by the applicable health benefits plan on the date of service; The PROVIDER providing the service or supply is an eligible provider on the date of service (that is, a health care provider whose services or supplies are covered under the health benefits plan; Neither eviCore nor applicable Payor reasonably believes that the claim has been submitted fraudulently; and, for claims where services are provided to a person enrolled in a Medicare Advantage plan, the claim conforms to the clean claim requirements for claims processed under a Medicare Advantage Program.

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d. Credentialing/Recredentialing. Credentialing/Recredentialing is the set of policies, processes, procedures and clinical standards established, determined and utilized by eviCore or Payor to evaluate and determine whether PROVIDER has satisfied all credentialing and recredentialing requirements adopted by eviCore or Payor and to approve PROVIDER's participation in the network.

e. Independent Health Care Appeals Program means the external appeals process for a covered person or provider on behalf of the covered person with the covered person's consent, to appeal a decision of a carrier to deny, reduce or terminate services or payment of benefits resulting from a decision by a carrier with respect to the covered person which services are otherwise covered under the health benefits plan

f. Independent utilization review organization or IURO means an independent organization with which the Department contracts to provide independent reviews through the Independent Health Care Appeals Program of carrier determinations regarding medical necessity or appropriateness of services which are contested by the covered person or a provider on behalf of the covered person.

g. Medically Necessary Services. "Medically Necessary" or "Medical Necessity" means or describes a health care service that a health care provider, exercising his or her prudent clinical judgment, would provide to a person covered by a health benefits plan for the purpose of evaluating, diagnosing, or treating an illness, injury, disease, or its symptoms and that is: in accordance with the generally accepted standards of medical practice; clinically appropriate, in terms of type, frequency, extent, site, and duration, and considered effective for the covered person's illness, injury, or disease; not primarily for the convenience of the covered person or the health care provider; and not more costly than an alternative service or sequence of services at least as likely to produce equivalent therapeutic or diagnostic results as to the diagnosis or treatment of that covered person's illness, injury, or disease. Medical necessity disputes do not include claims payment disputes.

h. Emergency means a medical condition manifesting itself by acute symptoms of sufficient severity including, but not limited to, severe pain, psychiatric disturbances and/or symptoms of substance abuse such that a prudent lay person, who possesses an average knowledge of health and medicine, could reasonably expect the absence of immediate attention to result in: placing the health of the individual (or with respect to a pregnant woman, the health of the woman or her unborn child) in serious jeopardy; serious impairment to bodily functions; or serious dysfunction of a bodily organ or part. With respect to a pregnant woman who is having contractions, an emergency exists where: there is inadequate time to effect a safe transfer to another hospital before delivery; or the transfer may pose a threat to the health or safety of the woman or the unborn child.

i. Urgent care means any claim for medical care or treatment with respect to which the application of the time periods for making non-urgent care determination may seriously jeopardize the life or health of the covered person or the ability of the covered person to regain maximum function or, in the opinion of a physician with knowledge of the medical condition of the covered person, subjects the covered person to severe pain that cannot be adequately managed without the care or treatment that is subject of the claim. In determining if a claim involves urgent care, a carrier must apply the judgement of a prudent layperson who possesses an average knowledge of health and medicine. However, if a physician with knowledge of the medical condition of the covered person determines that a claim involves urgent care, the claim must be treated as an urgent care claim.

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j. Utilization management means a system for reviewing the appropriate and efficient allocation of health care services under a health benefits plan according to specified guidelines in order to recommend or determine whether, or to what extent, a health care service given or proposed to be given to a covered person should or will be reimbursed, covered, paid for, or otherwise provided under the health benefits plan. The system may include: preadmission certification, the application of practice guidelines, continued stay review, discharge planning, preauthorization of ambulatory care procedures and retrospective review.

(2) a. Providers who are physicians shall maintain appropriate hospital admission privileges at the designated hospital Participating Provider(s) throughout the terms of this Agreement. Providers that are hospitals shall follow clear procedures for granting admitting and attending privileges to physicians and shall notify Evernorth and Payors when such procedures are no longer appropriate.

b. Evernorth shall review and resolve issues raised by Provider relating to the provision of Covered Services to Participants.

c. Detailed information on Evernorth's policies on the following topics is found in Evernorth's Medical Management Program, Administrative Guidelines and Provider Manual:

Credentialing/Recredentialing
Access Standards
Provider Data Changes
Quality Management
Utilization Management
Member Rights and Responsibilities

d. If Provider is a provider group, Evernorth must be a third party beneficiary to Provider's contracts with its providers and such contracts must provide that Evernorth has privity of contract with the providers such that Evernorth has standing to enforce the providers' contracts with Provider in the absence of enforcement by Provider. If applicable, Provider shall follow clear procedures for granting admitting and attending privileges to physicians and shall notify Evernorth when such procedures are no longer appropriate.

e. Unless otherwise specifically addressed in the Agreement, Provider shall be available for the treatment of patients not less than 35 hours per week.

f. Unless otherwise specifically addressed in the Agreement, Provider shall maintain availability for routine Member appointments upon two (2) weeks' notice, and if Provider is a clinic, facility or organization which is responsible of providing Urgent Care, Provider shall maintain availability for Member appointments for urgent matters within twenty-four (24) hours of contact.

(3) a. In the event Provider does not submit a claim within the number of days specified in the Agreement, Payor, in accordance with N.J.A.C. 11:22-1.6, shall reserve the right to deny payment or dispute the claim and Provider shall be prohibited from seeking payment in whole or in part directly from Participant. Where Provider is submitting a claim under an assignment of benefits from a Participant, Provider shall, in accordance with N.J.S.A. § 45:1-10.1, file the claim within one hundred eighty (180) days of the last course of treatment.

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b. Any fee-for-service amounts owing under the Agreement shall be paid within thirty (30) calendar days following receipt of a claim or no later than the time established for the payment of claims in the Medicare program pursuant to 42 U.S.C. s.1395u(c) (2) (B), whichever is earlier, if the claim is submitted by electronic means, and no later than forty (40) calendar days following receipt if the claim is submitted by other than electronic means, if:

(i) Provider is eligible at the date of service;
(ii) the person who received the health care service was covered on the date of service;
(iii) the claim is for a service or supply covered under the Plan;
(iv) the claim is submitted with all the information requested by the Payor on the claim form or in other instructions that were distributed in advance to Provider or Participant in accordance with the provisions of section 4 of P.L. 2005, c.352 (C.17B:30-51); and
(v) Payor has no reason to believe that the claim has been submitted fraudulently.

c. If all or a portion of the claim is not paid within the time frame provided in the paragraph above because:

(i) the claim submission is incomplete because the required substantiating documentation has not been submitted to Payor;
(ii) the diagnosis coding, procedure coding, or any other required information to be submitted with the claim is incorrect;
(iii) Payor disputes the amount claimed; or
(iv) there is strong evidence of fraud and Payor has initiated an investigation into the suspected fraud,

Payor shall notify Provider, by electronic means and the Participant in writing within thirty (30) days of receiving an electronic claim, or notify the Participant and Provider in writing within forty (40) days of receiving a claim submitted by other than electronic means that:

(i) the claim is incomplete with a statement as to what substantiating documentation is required for adjudication of the claim;
(ii) the claim contains incorrect information with a statement as to what information must be corrected for adjudication of the claim;
(iii) the Payor disputes the amount claimed in whole or in part with a statement as to the basis of that dispute; or
(iv) the Payor finds there is strong evidence of fraud and has initiated an investigation into the suspected fraud in accordance with its fraud prevention plan established pursuant to section 1 of P.L.1993, c.362 (C.17:33A-15), or referred the claim, together with supporting documentation, to the Office of the Insurance Fraud Prosecutor in the Department of Law and Public Safety established pursuant to section 32 of P.L.1998, c.21 (C.17:33A-16).

d. If all or a portion of an electronically submitted claim cannot be adjudicated because the diagnosis coding, procedure coding or any other data required to be submitted with the claim was missing, Payor shall electronically notify Provider or Provider's agent within seven (7) days of that determination and request any information required to complete adjudication of the claim.

e. Any portion of a claim that meets the criteria established in the first paragraph of this subsection b. shall be paid in accordance with the time limit established in the first paragraph of this subsection b.

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f. Payor shall acknowledge receipt of a claim submitted by electronic means from Provider, no later than two (2) working days following receipt of the transmission of the claim.

g. If payment is withheld on all or a portion of a claim pursuant to the reasons stated above, the claim payment shall be overdue if not remitted to Provider or their agent on or before thirty (30) calendar days or the time limit established by the Medicare program, whichever is earlier, for claims submitted by electronic means and forty (40) calendar days for claims submitted by other than electronic means, following receipt by the Payor of the required documentation or information or modification of an initial submission.

h. If payment is withheld on all or a portion of a claim pursuant to the reasons stated above and Provider is not notified within the time frames provided, the claim shall be deemed to be overdue.

i. An overdue payment shall bear simple interest at the rate of 12% per annum. The interest shall be paid to Provider at the time the overdue payment is made.

j. With the exception of claims that were submitted fraudulently or submitted by health care providers that have a pattern of inappropriate billing or claims that were subject to coordination of benefits, Payor shall not seek reimbursement for overpayment of a claim previously paid pursuant to this section later than eighteen (18) months after the date the first payment on the claim was made. Payor shall not seek more than one reimbursement for overpayment of a particular claim. At the time the reimbursement request is submitted to Provider, Payor shall provide written documentation that identifies the error made by Payor in processing or payment of the claim that justifies the reimbursement request.

k. No Payor shall base a reimbursement request for a particular claim on extrapolation of other claims, except under the following circumstances:

(a) in judicial or quasi-judicial proceedings, including arbitration;
(b) in administrative proceedings;
(c) in which relevant records required to be maintained by Provider have been improperly altered or reconstructed, or a material number of relevant records are otherwise unavailable; or
(d) in which there is clear evidence of fraud by Provider and Payor has investigated the claim in accordance with its fraud prevention plan, and referred the claim, together with supporting documentation, to the Office of the Insurance Fraud Prosecutor.

l. Payor shall not collect or attempt to collect reimbursement for the overpayment of a claim on or before the 45th calendar day following the submission of the reimbursement request to Provider and until Provider's right to appeal, as set forth in the Dispute Resolution provision in item (11) below, is exhausted.

m. Evernorth will not retroactively deny reimbursement for a Covered Service provided to a Participant on grounds which contradict the written or oral authorization of Evernorth, or its agents, to Provider, reasonably relied upon by Provider prior to providing the service to the Participant, except in cases where there was material misrepresentation or fraud. Payor will be liable only for those specific services which are provided to a Participant and which were authorized and no additional services rendered without specific authorization.

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n. Provider shall not seek reimbursement from Payor or Participant for underpayment of a claim submitted pursuant to this section later than eighteen (18) months from the date the first payment on the claim was made, except if the claim is the subject of an appeal submitted pursuant to section 1. of the Dispute Resolution section of the Agreement set forth in item (11) below, or the claim is subject to continual claims submission. Provider shall not seek more than one reimbursement for underpayment of a particular claim.

o. Provider may bill an individual directly for any services provided following the date the individual ceases to be a Member, unless such individual was a Member at the time of inpatient admission. Evernorth has no obligation under the Agreement to pay for services rendered to individuals who no longer are Members

p. Claims will be paid in accordance with the requirements of N.J.S.A. 26:2J-8.1, N.J.S.A. 17B:26-9.1, N.J.S.A. 17B:27-44.2 and N.J.A.C. 11:4-28.7, as may be amended from time to time.

q. Evernorth shall give written notice to Provider of any amount Provider inappropriately collected from Participant and Provider shall have thirty (30) days from the date of receipt of the written notice to appeal such determination.

r. If Provider fails to reimburse Participant for any amount Provider inappropriately collected from Participant, Evernorth may elect to withhold compensation amounts from future compensation payments otherwise due to Provider. Evernorth shall provide Provider with written notice and full disclosure of the funds withheld prior to such deduction.

s. To resolve disputes relating to the payment of provider claims see the Dispute Resolution section of the Agreement set forth in item (11) below.

t. If Provider is a hospital, Provider may bill an individual directly for any services provided following the date the individual ceases to be a Participant, unless such individual was a Participant at the time of admission. Payor has no obligation under the Agreement to pay for services rendered to individuals who no longer are Participants.

(4) Provider and Evernorth shall cooperate to facilitate timely and appropriate information and record exchanges necessary for quality management, utilization management, peer review or other programs required for Evernorth's operation or in connection with the provision of health care services to Participants.

(5) In no event shall Evernorth retaliate against Participants or Provider because of complaints or appeals on behalf of a Participant.

(6) Provider shall maintain general and professional liability coverage in an amount sufficient for Provider’s anticipated risk but no less than $1,000,000 per occurrence and $3,000,000 in the aggregate per year.

(7) Notwithstanding anything to the contrary set forth in the Agreement:

a. Provider shall not be penalized or the Agreement terminated by Evernorth because Provider acts as an advocate for the Participant in seeking appropriate, medically necessary health care services. If Provider is a hospital, Provider may communicate openly with a patient

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about all appropriate diagnostic testing and treatment options.

b. Provider shall not be penalized or the Agreement terminated by Evernorth because Provider appeals a coverage determination or files a complaint against Evernorth.

(8) Any provision of the Agreement which conflicts with state or Federal law(s) or regulation(s) is hereby amended to conform to the requirements of such law(s) or regulation(s).

(8.1) In addition to the obligations set forth in the Agreement governing amendments to the Agreement:

a. Any adverse change or amendment during the term of the Agreement shall be made in accordance with the terms of the Agreement only upon ninety (90) days' notice prior to the effective date of the change or amendment or as otherwise specified by N.J.A.C. 11:24C-4.3(d) as may be amended from time to time. If Provider declines to accept the amendment, Provider may terminate by providing notice in advance of the effective date.

b. If applicable, no adverse change may be made to the terms of the Agreement upon its automatic renewal. Any such change shall be made to the Agreement as set forth above either before or after renewal. Notwithstanding any timeframe set forth in the Agreement governing amendments to the Agreement, no adverse change may be made to the Agreement without sufficient advance notice to permit Provider to terminate the Agreement in advance of the effective date of the change.

c. Adverse change or amendment shall mean any action taken by Evernorth that could reasonably be expected to have a material adverse impact on either the aggregate level of payment to a health care provider or the administrative expenses incurred by the provider in complying with the change, or as otherwise defined by N.J.A.C. 11:24C-4.2 as may be amended from time to time. Such changes attributable to a third party, including a health care provider, over which Evernorth has no control, and such changes as identified in applicable state laws or regulations, are not adverse changes.

(9) If Provider is not a hospital or ancillary provider, the following shall apply:

a. Notice and Hearing.

Evernorth may not terminate Provider's participation under the Agreement with less than ninety (90) days prior written notice except where otherwise permitted by state laws or regulations. Evernorth's written notice of this decision shall include notice of a right to a hearing. If requested by Provider Evernorth shall, within fifteen (15) days of the receipt of the request, provide the reasons for the termination in writing. Within ten (10) business days following the date of receipt of notice of termination Provider shall have the right to make a written request for a hearing. Such hearing would be held within thirty (30) days of the date of the request.

The hearing shall be conducted by a panel appointed by Evernorth, which panel shall be comprised of a minimum of three (3) persons, at least one of whom is a clinical peer in the same discipline and the same or similar specialty as Provider.

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Unless the review panel provides notice of the need for an extension for the rendering of its decision to Provider and Evernorth prior to the date the panel's decision would otherwise be due, the panel shall render a decision on the matter in writing within thirty (30) days of the close of the hearing. The review panel shall make a decision that: (1) Provider's participation shall be terminated; or (2) Provider shall be reinstated or provisionally reinstated, subject to conditions set forth by the panel. The panel's determination shall be in writing and shall be made in a timely manner. Participation in this process shall not be deemed to be an abrogation of Provider's legal rights. If Provider is unsatisfied with the determination, Provider may appeal the decision further pursuant to the Dispute Resolution provision set forth in item (11) below.

b. Expiration of Agreement, Breach of Agreement, Imminent Harm, and Fraud.

The requirements regarding notice and hearing set forth above shall not apply in the following cases: when the Agreement expires and is not renewed; when the termination is due to breach of the Agreement; when in the opinion of the Medical Director, there is an imminent danger to an individual Participant or the public health, safety or welfare; or when there is a determination of fraud.

If Evernorth finds that Provider represents an imminent danger to an individual Participant or to the public health, safety or welfare, Evernorth shall promptly notify the appropriate professional State licensing board.

c. If Provider's participation under the Agreement is terminated, Provider shall continue to provide Covered Services with regard to those Members who retain eligibility under a Benefit Plan for up to 4 months in cases where it is Medically Necessary for the Member to continue treatment with Provider, except as follows: (1) for a period not to exceed 6 months in the case of post-operative follow-up care; (2) for a period not to exceed 12 months in the case of oncological treatment and psychiatric treatment; and (3) through the duration of a pregnancy and up to 6 weeks after delivery in the case of obstetrical care. The terms, conditions and compensation arrangements of the Agreement shall apply during the applicable continuation period.

(10) A. If Provider is not a hospital or ancillary provider, the following shall apply:

a. Retainer Arrangements. Retainer Arrangements shall mean any arrangement, program or agreement that limits a physician's panel of patients to those who agree to pay a fee to a physician or practice for a set of services to be delivered for a specific period of time. Retainer Arrangements between Provider and Participants shall only apply with respect to services above and beyond Covered Services; such services shall be clearly described by Provider to Participant prior to rendering services. No Participant agreeing to such a retainer arrangement shall be charged any amounts in excess of cost-sharing amounts for Covered Services under the health benefit plan. Should Provider transition to a retainer arrangement practice, Provider shall give notice to Evernorth and Participants under Provider's care at least ninety (90) days prior to adopting such practice. Participants already receiving Covered Services shall be afforded such rights under state laws and regulations as are applicable to a patient whose provider has dropped out of the network, including rights to continuity of care, without making any retainer payments. The terms, conditions and compensation arrangements of the Agreement shall apply during the continuation period.

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B. If Provider is a hospital, the following shall apply:

a. In the event the Agreement is not renewed or is terminated by either party, Provider and Evernorth shall continue to abide by the terms and conditions of the Agreement for a period of four (4) months from a severance date mutually agreed upon by both parties as required by N.J.S.A. 26:2J-11.1.

b. With respect to a termination of the Agreement initiated by Evernorth, Evernorth shall not begin transferring Participants to other Evernorth Participating Providers during the Cure Period to the extent such transfer would conflict with the requirements of N.J.S.A. 26:2J-11.1.

C. If Provider is an ancillary provider, the following shall apply:

a. Provider and Evernorth shall abide by the terms of the Agreement, including reimbursement terms, for four (4) months following the date of the termination. Provider has no obligation to provide, and Evernorth has no obligation to reimburse at the contracted rate, services which are not Medically Necessary to be provided on and after the 31st day following the date of termination.

(11) The Agreement permits Payor to enter into agreements with third parties (PPOs, ODSs, and any such other entities to which Payor may lease its networks) that allow third parties to obtain Payor's contracting entity rights and responsibilities under the Agreement as if the third party is the contracting entity. To the extent that the terms of the Agreement allow a third party access the Agreement, the rights of the third party to access discounted rates shall cease upon termination of the Agreement.

(12) To the extent that some portion of Provider's compensation in accordance with Exhibit A to this Agreement, is tied to the occurrence of a pre-determined event, or the nonoccurrence of a pre-determined event, such event shall be clearly specified in Exhibit A, and Provider shall have a right to receive a periodic accounting (not less frequently than annually) of the funds held for, and paid to, Provider by Evernorth.

(13) Disputes Subject to State Resolution Procedures

1. (a) To resolve disputes relating to the payment of provider claims under Health Benefit Plans as defined in N.J.S.A. 17B:30-50 but not including appeals to resolve disputes pertaining to medical necessity which are eligible to be submitted to the Independent Health Care Appeals Program established pursuant to section 11 of P.L. 1997, c. 192 (C.26:2S-11):

Provider shall submit for review and resolution to the Evernorth designee identified by Evernorth in Evernorth's explanation of payment.

Any dispute regarding the determination of an internal appeal conducted pursuant to subsection (a) above may be referred to arbitration as provided inthe Administrative Guidelines. Either party may initiate arbitration following the receipt of the determination which is the basis of the appeal by providing written notice to the other party in the manner set forth in the Administrative Guidelines. No dispute shall be accepted for

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arbitration unless the payment amount in dispute is $1,000 or more, except that Provider may aggregate disputed claim amounts for the purposes of meeting such threshold requirement.

2. To resolve disputes with respect to a termination and for claim disputes with respect to ASO Participants, but not including appeals to resolve disputes pertaining to medical necessity which are eligible to be submitted to the Independent Health Care Appeals Program established pursuant to section 11 of P.L. 1997, c. 192 (C.26:2S-11), Hospital shall submit the dispute for review and resolution to the Cigna designee identified by Cigna, and in the manner set forth in Cigna's explanation of payment or termination letter, as applicable.
3. Disputes regarding a claim or a termination that are not resolved through the process described in section 2. above and any other dispute between the parties regarding the performance or interpretation of the provider agreement may be resolved by arbitration between the parties pursuant to the arbitration process set forth in the Agreement and the Administrative Guidelines. Either party may initiate arbitration by providing written notice to the other party in the manner set forth in the Administrative Guidelines.

4. In addition to the above, for disputes concerning the application of Evernorth's coding and payment rules and methodologies to patient specific factual situations, Provider should consult Evernorth's website for details regarding a billing dispute resolution process that may be applicable in lieu of arbitration.

(14) The Agreement may permit Evernorth to enter into agreements with third parties (PPOs, ODSs, and any such other entities to which Evernorth may lease its networks) that allow third parties to obtain Evernorth's contracting entity rights and responsibilities under the Agreement as if the third party is the contracting entity. To the extent that the terms of the Agreement allow a third party access the Agreement, the rights of the third party to access discounted rates shall cease upon termination of the Agreement.

B. Utilization Management and Appeals

(1) Utilization Management. Provided that Evernorth has accepted a delegation of responsibility to handle Utilization Management, Provider shall participate in Evernorth's utilization management program, as further described below, through which Evernorth shall be responsible for conducting utilization management activities, including prospective, concurrent and retrospective review on behalf of applicable Payors. Evernorth utilization management program shall be under the direction of appropriately licensed (in New Jersey) medical professionals. Provider has the right to rely upon Evernorth written or oral prior authorization of a service. Services pre-authorized by Evernorth will not be retroactively denied as not medically necessary except in cases of fraud, or in cases where there was a material misrepresentation of the facts to Evernorth the applicable Payor, or to an authorized third-party representative of the applicable Payor.

(a) Reasons for Denial. Through Evernorth's Utilization Management program, payment for or authorization for services may be denied by Evernorth on the following bases: (1) Lack of sufficient documentation to determine medical necessity; (2) The relevant services do not meet medical necessity requirements; or (3) The relevant services are not within the scope of care to be

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provided by Provider. Other bases not related to the medical necessity (and hence outside the scope of the Utilization Management program) may exist for the denial of payment of claims or authorization for treatment. Such denials of authorization or payment are handled through Evernorth's claims administration function. Upon written request to Evernorth, Provider may review and provide comment on the applicable utilization management protocols.

(b) Utilization Management Review Procedures. Evernorth's utilization management program, in which Provider is required to participate, consists of three (3) types of review. In each type of review, Evernorth will arrange for an expert reviewer, who is a duly licensed medical provider, to conduct such reviews, and will provide Provider with contact information of the expert reviewer such that Provider may review determinations with the applicable expert reviewer.

1. *Prospective review*. Prospective Review is the process by which Evernorth reviews and approves, modifies, delays, or denies prior authorization of care requests. In the case of routine prior authorization of care requests, Evernorth shall render a determination within 15 calendar days of receipt of all information reasonably necessary to render a decision. In the case of emergent expedited prior authorization requests (Requests where Provider indicates that the standard timeframes could seriously jeopardize the Member's life or health or ability to attain, maintain or regain maximum function), Evernorth shall render a determination in an appropriate timeframe as dictated by the medical exigencies of the case, but in no case longer than 72 hours following receipt of the request.

2. *Concurrent Review*. Concurrent review is review of an ongoing treatment regimen already in place. When Evernorth conducts concurrent review, care provided by Provider shall not be discontinued until the Provider has been notified of Evernorth's decision, and a care plan has been agreed upon by Provider that is appropriate for the medical needs of the Member. Evernorth will render a determination on concurrent reviews within five (5) or fewer business days or receipt of request, consistent with urgency of Member's medical condition.

3. *Post-Service/Retrospective Review*. Evernorth may conduct a retrospective review of the medical necessity of services provided by Provider. In cases where the review is retrospective, the decision shall be communicated to the Provider that performed services within 30 days of the receipt of the claim.

(2) Utilization Management Appeals. Provider has the right to receive notice of each utilization management determination made by Evernorth with regard to a Member for which Provider is providing services. Provider, with the express written consent of Member, may seek an appeal of a utilization management determination made by Evernorth on behalf of a Member by submitting an appeal according to the instructions stated on the notice of adverse benefit determination. Utilization management appeals may be handled either by Evernorth or by Payor. If Evernorth is responsible for handling utilization management appeals (which shall be the default), the below procedure shall govern the appeal process. An applicable Payor's specific procedures may vary as set forth in the applicable Client Summary.

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(a). Informal Internal Appeals: Provider shall have an opportunity to speak, regarding an adverse benefit determination, with Evernorth's medical director, or the medical director's designee who rendered the adverse benefit determination. If Provider disagrees with the medical director's or designee's denial and Provider (or a Member) pursues an appeal, Evernorth will conduct a full investigation of each internal appeal, taking into consideration all documents, clinical records, and other information submitted, without regard to whether such information was considered in the initial review of the case. In all cases, professional reviewers adhere to established clinical criteria.

1. Once Evernorth has received the clinical information required to make a decision, a licensed provider who is not subordinate to the clinical peer reviewer who conducted the original review will determine whether medical necessity is established based on the records presented and relevant clinical criteria; if so, the provider will overturn the initial denial and Evernorth will communicate the decision in accordance with applicable timeframes required by New Jersey law.

2. If the provider reviewing the appeal is unable to establish medical necessity based on the facts of the case, (s)he will uphold the original adverse determination.

3. Following Evernorth's initial review and determination of the appeal to uphold the initial adverse determination, if the applicable Member is an individual health benefit plan Member, Evernorth shall provide notice to the Member and treating Provider that Evernorth has reached a final adverse benefit determination including notice of the member and/or provider's rights to pursue an external appeal to an independent utilization review organization pursuant to the procedures set forth below. Such determination shall be rendered as soon as possible in accordance with the medical exigencies of the case, but in no event shall such time exceed seventy-two hours in the case of an appeal from an adverse benefit determination regarding urgent or emergency care, an admission, availability of care, continued stay and health care services for which the claimant received emergency services but has not been discharged from a facility; or ten calendar days in the case of all other appeals.

4. If the applicable Member is a group health benefits plan member, Evernorth shall provide notice to the Member and his or her treating provider that the member's informal internal utilization management (stage 1) appeal has been denied, and that the member or his or her treating provider may pursue a formal internal utilization management (stage 2) appeal).

(b). Formal Internal Appeals. The Stage 2 internal formal appeal process is a process where Provider (with consent of the Member) can request that the adverse determination via the Stage 1 appeal process be reviewed by a panel of licensed physicians. Following receipt of a request from a Member or Member's designee for a Stage 2 appeal, Evernorth will acknowledge the request in writing within 10 business days of receipt, and will render a determination within 20 business days of receipt of the appeal. If medical exigencies require an expedited appeal, Evernorth will render a determination on the Stage 2 appeal within seventy-two (72) hours of receipt of the appeal. If Evernorth's determination is to deny the Stage 2 appeal, Provider will be provided notice of such determination, together with notice of its right to pursue an external appeal to an independent utilization review organization, as outlined below.

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(c) External Appeals to an Independent Utilization Review Organization (IURO) / FAIR Hearing. Provider, with express written consent of the Member, may appeal a final internal adverse benefit determination to an external review. This may include an IURO and/or a Medicaid FAIR Hearing, as applicable.

1. In circumstances where Provider has provided services to a NJ FamilyCare or Medicaid beneficiary member, Provider may, with the consent and participation of the member, request a Medicaid FAIR Hearing with the New Jersey Department of Human Services. A FAIR Hearing may be requested in addition to, or in place of, and external appeal to an IURO.

2. To initiate an External Appeal to an IURO, Provider may submit the applicable form provided by the New Jersey Department of Banking and Insurance (DOBI) (available at **https://www.state.nj.us/dobi/chap352/352ihcapform.docx**) to DOBI within four months after receipt of EVERNORTH's final internal adverse benefit determination.

3. The IURO's determination shall be binding on Evernorth, the applicable Payor, Member and Provider, except to the extent that other remedies are available to either party under State or Federal law.

4. Evernorth and the applicable Payor will comply with the decision of an IURO in 10 days or less according to the exigencies of the case.

(C) With respect to Covered Services rendered to Participants covered under an HMO or Managed Care Plan:

1. Provider shall hold Participant harmless for the cost of Covered Services, whether or not Provider believes its compensation for such Covered Service from Payor is made in accordance with the reimbursement provisions of the Agreement, or is otherwise inadequate.

2. If Provider is a hospital or ancillary provider, in the event of Evernorth's insolvency, Covered Services to Participants who are confined in an inpatient facility on the date of the declaration of insolvency shall continue until the Participant's discharge from the facility or the expiration of the Participant's benefits, whichever occurs first.

(D) The following provisions will apply, in accordance with N.J.A.C. 11:24B, to any Agreement between Provider and an Evernorth entity that is an with an Organized Delivery System:

(1) Except as permitted by applicable laws and regulations, the Agreement and amendments thereto are subject to prior approval by the New Jersey Department of Banking and Insurance, and may not be effectuated without such approval.

(2) § 6.12 of the Agreement is amended to add the following:

The Agreement is governed by New Jersey law with respect to business transacted in New Jersey.

(3) Nothing in the Agreement shall be construed, contrary to N.J.A.C. 11:24B- 4.2(c)4, to limit

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Evernorth's obligation to provide remuneration, in accordance with the terms of the provider agreements, to providers under contract with an ODS for covered services rendered by those providers to covered persons in the event of the default of bankruptcy of the ODS.

(4) Nothing in the Agreement shall be construed, contrary to N.J.A.C. 11:24B- 5.2(c)6 to deny payment to a provider with respect to Medically Necessary services if the services were not pre-certified or pre-authorized; payment may be reduced for such services, as permitted by applicable state laws and regulations, in accordance with the terms of the Agreement and the Medical Management Program.

(5) Records relevant to Evernorth or Payor operations, other than medical records, are the property of Evernorth or Payor unless otherwise specified in the Agreement.

(6) Provider may not delegate any of the functions it is performing on behalf of Evernorth or Payor except as specified in the Agreement and the applicable Exhibit to the Agreement, and subject to advance notice as specified in the Agreement or applicable Exhibit; unless otherwise specified in the Agreement or the applicable Exhibit to the Agreement, Evernorth or Payor shall have the right to reject any delegation.

(7) Provider shall not require a Member to make a payment for any amount for a deductible, coinsurance payment, or a copayment for a Covered Service that is a prescription drug benefit in an amount that exceeds the amount the Member would pay for the prescription drug if the Member purchased the prescription drug without using a health benefits plan.

(8) Provider may disclose, and neither Evernorth nor any Payor shall not penalize or otherwise dis-incentivize Provider, disclosure to a Member of lower cost prescription drug options, including those that are available to the Member if the Member purchases the prescription drug without using health insurance coverage

(9) Notwithstanding any provision of this Agreement contrary, Provider shall inform each patient, at the time of taking an order from a patient for home delivery or at the time of dispensing a prescription drug to the patient at the practice site: (a) of the lowest cost option for the prescription drug; and (b) whether there is an alternative drug that is less expensive and interchangeable with the prescription drug, and if needed, that the consumer can discuss with the prescribing health care provider whether the alternative drug would be appropriate for the consumer.

(10) § 2.8 of the Agreement is amended to add the following:

Evernorth shall have continued access to Provider books and records following the termination of the Agreement for such time as needed for the purposes of collecting such data as Evernorth is required to report to the Department of Banking and Insurance for any part of the calendar year for which the Agreement is in effect. Evernorth-owned records shall, in the event of the termination of the Agreement, be transferred in a timely and orderly fashion as agreed upon by the parties at the time of the notice of termination.

(11) If Provider is a hospital or ancillary facility:

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Subrogation/Workers' Compensation Subrogation and third party recovery provisions are prohibited.
Posting payments Explanation of benefits (EOB) and explanation of payments (EOP) have been revised to include plan language and specific reasons for denials; not just the denial code. In addition, the EOB has been revised as follows: if you are covered by more than one health benefit plan, you should file all of your claims with each plan and provide each plan with information regarding the other plans under which you are covered.
Service Standards Urgent care should have access to appointments within twenty-four (24) hours of the patient's phone call.
Order of Benefit Determination When the primary plan is not subject or compliant with New Jersey regulations, the secondary plan must attempt to (i) secure all information needed to correctly determine its liability or (ii) assume the primary position if the non-complying plan is unwilling to act as primary or does not supply the needed information. Please note New Jersey has specific procedures for secondary plans/managed care coordination.
Retiree Rule The retiree rule for the state of New Jersey should be used when determining coverage for laid off or retired employees. Maintenance of Benefits (MOB), Non-duplication of Benefits (Non Dup) and Super 65 options are not permitted.
Emergency Services Emergency Services include any medical screening examination or other evaluation required by state or federal law to be provided in an emergency department of a hospital, which is necessary to determine whether an Emergency Medical Condition exists and Medically Necessary Emergency Services required for the treatment and stabilization of an Emergency Medical Condition.

(12) Referrals to Non-Participating Providers including Ambulatory Surgical Centers and Free Standing Laboratories: Patients with Evernorth coverage generally expect that when they choose to seek care from a provider who participates with Evernorth, the entire episode of care will be reimbursed using in network benefits. For this to occur, the patient needs to be referred to Evernorth participating providers including other practitioners, laboratories and facilities. When a patient is referred to non-participating provider, the patient may incur unexpected financial liability. Patients with Evernorth coverage who have out-of-network benefits are free to choose to use these benefits for covered medical services. In doing so, the patients will generally incur higher out of pocket costs. To ensure that patients are making informed choices regarding whether to seek care from participating or non-participating providers, they must have full disclosure regarding the financial effect of such referrals under their benefit plans, including the referring physician's financial interests, if any.

(13) American College of Medical Quality; Professional Policies -POLICY 32- Physician Self-Referral: The referral of patients to an additional individual provider or institution for any services in which the referring physician has a financial interest may constitute "self-referral." Physicians who make referrals

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to professional practitioners or health care entities or facilities in which they have financial interest must provide full disclosure of that interest to patients, allowing them to make informed decisions. In addition, patients should be given a list of effective alternative resources, if any, that are reasonably available, informed that they have the option to use one of the alternative resources, and assured that the physician will not treat them differently if they choose an alternative provider or entity. Physicians should make referrals to providers based only on the needs of the patient and the medical standard of care in order to provide quality health care to their patients.

(14) New Jersey law regarding self-referrals and Ambulatory Surgery:

a. Self-Referrals Generally Prohibited: New Jersey law generally prohibits health care practitioners from referring a patient (or directing an employee to refer a patient) to a health care service in which the practitioner, or the practitioner's immediate family, or the practitioner in combination with the practitioner's immediate family has a significant beneficial interest.

b. Permitted Self-Referral Exceptions On and After March 1, 2010:

The law is amended effective March 1, 2010 to make exceptions and permit self-referrals for the following:

(1) Medical treatment or a procedure that is provided at the practitioner's medical office and for which a bill is issued directly in the name of the practitioner or the practitioner's medical office;
(2) Renal Dialysis; and
(3) Ambulatory surgery or procedures requiring anesthesia performed at a surgical practice registered with the State Department of Health and Senior Services or at an ambulatory care facility licensed by the Department of Health and Senior Services to perform surgical and related services.

c. Conditions of Exception for Ambulatory Surgical Services:

The self-referral exception for ambulatory surgical services and facilities is only permitted if all the following conditions are met:

(1) the practitioner who provided the referral personally performs the procedure;
(2) the practitioner's remuneration as an owner of or investor in the practice or facility is directly proportional to his ownership interest and not to the volume of patients the practitioner refers to the practice or facility;
(3) all clinically-related decisions at a facility owned in part by non-practitioners are made by practitioners and are in the best interests of the patient; and
(4) disclosure of the referring practitioner's significant beneficial interest in the practice or facility is made to the patient in writing, at or prior to the time that the referral is made, consistent with the prescribed disclosure provisions in the law.

d. Disclosure of Practitioner’s Interest:

If a practitioner is permitted to refer a patient to a health care service pursuant to these exceptions, the practitioner shall provide the patient with a written disclosure form prepared according to the provisions in the law, and post a copy of this disclosure form in a conspicuous public place in the practitioner's office.

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The disclosure shall inform the patient whether any services or facility fees associated with the referral will be considered to be, and reimbursed at, an "out-of-network" level by the patient's insurance carrier or other third party payer.

(15) Demographic Information and Directories: Evernorth ensures that the information in the directory is based on the most recently submitted information from provider or CAQH. Evernorth confirms the participation of any provider who has not submitted a claim for a twelve (12) month period or otherwise communicated with us in a manner evidencing the intention to continue to participate in the network, and for whom no change in provider status has been reported by CAQH. If follow up contacts cannot confirm the provider has continued intention to participate, the provider will be removed from the network.

(16) Evernorth Required Form for Out-of-Network Referrals in New Jersey: To ensure that patients with Evernorth coverage have the necessary information to make an informed decision, the "Out-of-Network Referral Disclosure Form" must be completed and the choices must be explained to the patient. The use of this form is required in addition to any other state mandated requirements. A copy of the completed form should be given to the patient and the original placed in the patient's medical file. Use of this form is a condition of participation for Evernorth's Participating Providers. Evernorth may audit the use of this form by Evernorth's Participating Providers.

(17) If Provider is a hospital, with respect to indemnification of the parties, nothing in the Agreement shall waive, modify, delegate or shift the liability of either party under N.J.S.A. 2A:53A-33a.

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NEW MEXICO REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as the "Provider") to comply with legislative and regulatory requirements of the State of New Mexico regarding provider contracts with providers rendering health care services in the State of New Mexico. To the extent that such New Mexico laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of the Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such New Mexico laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) In addition to the requirements set forth in the Agreement, Provider agrees that in no event, including but not limited to non-payment by Evernorth or Payor, insolvency of Evernorth or Payor, or breach of this Agreement, shall Provider bill, charge, collect a deposit from, seek remuneration or reimbursement from, or have any recourse against, a Participant or covered person, or person acting on behalf of the covered person, for health care services provided pursuant to the Agreement. This does not prohibit Provider from collecting Coinsurance, Deductibles, or Copayments as specifically provided in the Plan, or fees for uncovered health care services delivered on a fee-for-service basis to persons referenced above, nor from any recourse against Evernorth or Payor or their successors. The requirements of this provision shall survive the termination of the Agreement regardless of the reason for the termination, including the insolvency of Evernorth or Payor.

(2) In addition to the obligations and requirements set forth in the Agreement, Provider will comply with the Administrative Guidelines including, but not limited to, policies and programs contained therein with respect to payment systems, utilization review, quality assessment and improvement programs, credentialing, confidentiality requirements, and any applicable federal or state programs.

Provider may, pursuant to 13.10.16 NMAC, and in accordance with the Administrative Guidelines, raise concerns regarding the operation of the plan, including concerns regarding quality of and access to health care services, the choice of health care providers, and the adequacy of the plan's provider network.

(3) In addition to the obligations and requirements set forth in the Agreement, and in accordance with applicable state laws and regulations, Provider will notify Evernorth not more than ten (10) days after the Provider's receipt of notice of any reduction or cancellation of professional liability and malpractice insurance.

(4) Provider shall, in accordance with applicable state laws and regulations, observe, protect, and promote the rights of Participants or covered persons as patients.

(5) In addition to the obligations and requirements set forth in the Agreement, if any, and in accordance with applicable state laws and regulations, Evernorth shall provide interpreters for limited English proficient individuals and interpretive services for patients who qualify under the Americans with Disabilities Act; such interpretive services will be made available to Provider's office at no cost to Provider.

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(6) Terms used in the Agreement that are defined by state statutes and division regulations will be used in the Agreement in a manner consistent with any state definitions.

(7) In addition to the obligations and requirements set forth in the Agreement, and in accordance with applicable state laws and regulations, should Payor fail to pay Provider for Covered Services within forty-five (45) days after receipt of a clean claim, as defined by applicable laws and regulations, Payor shall be liable for the amount due and unpaid with interest on that amount at the rate established by applicable law.

(8) "Clean Claim" means a manually or electronically submitted claim that contains all the required data elements necessary for accurate adjudication without the need for additional information from outside of Evernorth or Payor's system and contains no deficiency or impropriety, including lack of substantiating documentation currently required by Evernorth or Payor, or particular circumstances requiring special treatment that prevents timely payment from being made by Evernorth or Payor.

(9) In addition to the obligations and requirements set forth in the Agreement, and in accordance with applicable state laws and regulations, Payor shall make any retroactive adjustments for overpayment within eighteen (18) months absent Provider miscoding, claim submission error, suspected fraud and abuse, or retroactive adjustments required by federal or state agencies.

(10) In addition to the obligations and requirements set forth in the Agreement, and in accordance with applicable state laws and regulations, if the Agreement is terminated without cause, Provider shall continue to provide Covered Services for a Participant to continue an ongoing course of treatment for a transitional period as set forth in the Administrative Guidelines; the transition period for a Participant who is in the third trimester of pregnancy at the time of termination shall include the provision of postpartum care directly related to the delivery. Authorization for continuing care shall be subject to Provider’s agreement to accept reimbursement at the rates applicable prior to the commencement of continued care as payment in full, adherence to Evernorth's policies and procedures and quality assurance program, and agreement to provide Medically Necessary information related to continued care.

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NEW YORK REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred as "Provider") to comply with legislative and regulatory requirements of the State of New York regarding provider contracts with providers rendering health care services in the State of New York. To the extent that such New York laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such New York laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) To the extent permitted by Insurance Law § 3217-b, either party may seek resolution of a dispute arising pursuant to the payment terms of the Agreement through a proceeding under article seventy-five of the civil practice law and rules.

(2) To the extent required by Insurance Law § 3224-a, the parties agree to the following. Provider shall initially submit a claim within one hundred twenty (120) days after the date of service unless a longer time period is permitted under the Agreement and applicable law. Evernorth or Payor shall permit Provider to request reconsideration of a claim that is denied exclusively because it was submitted untimely if Provider can demonstrate that non-compliance was a result of an unusual occurrence and that Provider has a pattern or practice of submitting claims in compliance with timely submission requirements. Evernorth or Payor may reduce the reimbursement due for an untimely claim by an amount not to exceed twenty five percent (25%) of the amount that would have been paid had the claim been submitted in a timely manner. Evernorth or Payor may deny the claim in full for a claim submitted three hundred sixty-five (365) days after the date of service.

(3) To enable compliance with legislative and regulatory requirements of the State of New York regarding insurer provider network disclosures to insureds set forth by Insurance Law § 3217-a(a)(17) as may be amended from time to time, a physician will, in addition to the provider directory requirements found in the Medical Management Program, provide to Evernorth board certifications, languages spoken and any affiliations with participating hospitals, in writing or as specified in the Medical Management Program. Physician will also notify Evernorth in writing or as specified in the Medical Management Program in advance of any change in any affiliations with participating hospitals.

(4) For enrollees covered under and fully insured health plan and in accordance with 42 U.S.C. Section 300gg-139(b), as may be amended from time to time, If a health care provider submits a bill to an enrollee based on cost-sharing for treatment or **services** provided by the health care provider that is in excess of the normal cost-sharing applied for such treatment or **services** provided in-network, as prohibited under **section 300gg–115(b) of title 42**, **section 1185i(b) of title 29**, or **section 9820(b) of title 26**, as applicable, and the enrollee pays such bill, the provider shall reimburse the enrollee for the full amount paid by the enrollee in excess of the in-network cost-sharing amount for the treatment or **services** involved, plus interest, at an interest rate determined by the **Secretary of Health and**

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Human Services.

(5) Provider Payment Appeals: The utilization review appeal procedures available to covered individuals may be available for certain post service medical necessity denials.

(6) Provider Information and Directories: Demographic information sent to Evernorth should also be consistent with the providers' information posted in the New York Department of Education database and the New York Department of Health "Health Provider Network" (HPN) database.

(7) Patient Rights and Responsibilities – Enrollee Right to Obtain Medical Information: When it is advisable not to give diagnosis, treatment and prognosis, information to a covered individual (for example, to a minor or incapacitated dependent) the information is to be made available to an appropriate person acting on the individual's behalf.

(8) Patient Rights and Responsibilities – Non-Covered Services: Prior to initiating a non-covered service, providers are required to advise the enrollee that the service is not covered by Evernorth and to state the cost of the service.

(9) Patient Liability Collection Guidelines: For uncovered services, Provider must advise the patient, prior to initiating of service, that the services are uncovered and the cost of the service.

(10) Dispute Resolution: If a provider's license, certification or registration is revoked or suspended, by the State, the provider will be terminated from the HMO network.

(11) Referral Process:
(A) Exceptions to Referral Process:
a. A new patient whose provider is not a customer of the Healthplan's network and who:
i. Is receiving an on-going course of treatment for a life-threatening disease or condition, or a degenerative or disabling disease or condition, or
ii. Has entered the second trimester of pregnancy as of the effective date of enrollment may be eligible to receive continuity of care from that non-participating provider for a transitional period of up to sixty (60) days, or the post-partum period directly related to the delivery of the child . Such continuity of care must be approved in advance by Evernorth and the non-participating provider must agree to accept Evernorth's reimbursement rate and to abide by Evernorth policies, procedures, and quality assurance requirements.

(12) Emergency Services: An "Emergency Medical Condition" is defined as a medical or behavioral condition, the onset of which is sudden, that manifests itself by symptoms of sufficient severity, including pain, that a prudent layperson, possessing an average knowledge of medicine and health, could reasonably expect the absence of immediate medical attention to result in: (a) placing the health of the person afflicted with such condition in serious jeopardy, or in the case of a behavioral condition, placing the health of such person or others in serious jeopardy; or (b) serious impairment to such person's bodily functions; or (c) serious dysfunction of any bodily organ or part of such person; or (d) serious disfigurement of such person.

(13) Specialized Care over a Prolonged Period of Time: Extended referral to a specialty provider may be requested for a condition that requires ongoing care from a specialist. The primary care physician ("PCP") should contact Evernorth to discuss the medical necessity of the extended referral and to arrange for the authorization. For a life-threatening condition or disease, a degenerative and disabling

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condition or disease, or a condition that requires specialized medical care over a prolonged period of time, a specialty provider may be designated as the patient's coordinating care provider or the patient may be referred to a specialty care center for treatment. The PCP should contact Evernorth to determine the medical necessity of the situation and to arrange for authorization.

(14) Medical Record Reviews – Medical Record Documentation:
(A) Additional medical record guidelines:
a. Separate medical record for each enrollee;
b. Record verifies that PCP coordinates and manages care
(B) Medical record retention requirements:
a. Six (6) years after date of services to enrollees or cessation of HMO operation.
b. For minors, six (6) years from date of majority.

(15) Quality Assurance:
(A) Confidentiality of HIV-related information:
a. Each provider must develop policies and procedures to assure confidentiality of HIV information. The policies and procedures must include:
i. Initial and annual in-service education of staff, contractors;
ii. Identification of staff allowed access and limits of access;
iii. Procedure to limit access to trained staff (including contractors);
iv. Protocol for secure storage (including electronic storage);
v. Procedures for handling requests for HIV-related information;
vi. Protocols to protect persons with or suspected of having HIV infection from discrimination.

(16) Peer Review Committee: Inappropriate conduct reviewed includes fraud. In addition, actions will be taken as appropriate in a case of imminent harm to patient health.

(17) Credentialing: Healthplan will notify provider applicants within sixty (60) days of receipt of completed applications, informing the applicants whether they are credentialed or whether additional information is needed or Healthplan needs no further information. More information is available upon request from Provider Service Department at 800.926.2273.

(18) Recredentialing Process: Evernorth will provide Provider with information, profiling data and analysis used to evaluate performance during any evaluation process, and upon request. Evernorth is available to discuss the unique nature of Provider's professional patient population, which may have bearing on Provider's profile, and to work cooperatively with Provider to improve performance.

(19) Provider Performance Evaluations:
(A) Healthplan's policies and procedures to review provider performance include:
a. The information maintained to evaluate the performance/practice of providers;
b. The criteria against which the performance of providers will be evaluated;
c. The process used to perform the evaluation;
d. The procedure to providers with any information and profiling data used to evaluate the professional's performance;
e. the procedure to make available on a periodic basis and upon request of the provider the information, profiling data and analysis used to evaluate the professional's performance;
f. The opportunity to discuss the unique nature of the professional's patient population, which may have bearing on the professional's profile, and to work cooperatively with Healthplan to improve performance.

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(20) Provider Termination Appeals:

(A) If Evernorth proposes to terminate a contract, Evernorth will provide a written explanation of the reasons for the proposed contract termination and an opportunity for a review or hearing as described below. The termination notice will explain the reasons for the proposed action, include a notice that Provider has the right to requests a hearing on review at Provider's discretion before a panel appointed by Evernorth, and include the time limit for a hearing date within thirty (30) days after the date of receipt of a request for a hearing. The hearing panel will be comprised of three (3) persons appointed by Evernorth. At least one (1) person on the panel in the same discipline or same specialty as the person under review. The panel can consist of more than three (3) customers, provided the number of clinical peers constitutes one-third or more of the total membership. The hearing panel shall render a decision a decision in a timely manner. Decisions will include one of the following and will be provided in writing to the healthcare professional: reinstatement; provisional reinstatement with conditions set forth by the MCO, or termination. Decisions of termination shall be effective not less than thirty (30) days after receipt by the healthcare professional of the hearing panel's decision. In no event shall determination be effective earlier than sixty (60) days from receipt of the notice of termination. A provider terminated due to the following is not eligible for a hearing or review: a case involving imminent harm to patient care, a determination of fraud, or a final disciplinary action by a state licensing board or other governmental agency that impairs the health of the healthcare professional's ability to practice.

(B) Evernorth does not propose contract terminations solely for the following actions by a provider:

a. advocated on behalf of an enrollee
b. filed a complaint against a managed care organization
c. appealed a decision of the managed care organization
d. provided information or filed a report to PHL4406-c regarding prohibitions of plans
e. requested a hearing for review

(C) Per Evernorth, along with the Hospital shall continue abiding by all provisions of the terminated contract for a "cooling off" period of two calendar months, beginning on the termination date. During this time Evernorth must pay and the Hospital must accept the previous contracted rate. Either party may request a waiver of the cooling off period by submitting a written request to the Director of the Bureau of Certification and Surveillance no more than 5 business days after Evernorth submits the notice of termination to the Department.

(21) Provider Responsibilities:

(A) Continuity of Care with Current Providers: New enrollees whose provider is not a participating provider may request directly or through such professional to continue an ongoing course of treatment.

a. If the covered individual has a life-threatening disease or condition or a degenerative and disabling disease or condition, coverage for the ongoing treatment may be continued for up to sixty (60) days.
b. If the covered individual has entered the second trimester of pregnancy at the effective date of enrollment date, coverage for the ongoing care may be continued through the period of post-partum care related to the delivery.

(B) Transitional Care Following Contract Termination: If a patient with Evernorth or GWH-Evernorth coverage has been receiving care from a provider whose participation in the Evernorth network ends and a continued course of covered treatment is Medically Necessary, he/she may be eligible to receive "transitional care" from the non-participating provider for up to ninety (90) days. Patients may also be eligible to receive transitional care if in the second trimester of pregnancy. In this case, transitional care may continue through delivery and post-partum care. Such transitional care must be approved in advance by Evernorth and the provider, and the provider must agree to

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accept our reimbursement rate and to abide by Evernorth's policies, procedures, and quality assurance requirements. There may be circumstances where continued care by a provider no longer participating in Evernorth's network will not be available, such as when the provider loses his or her license to practice or retires.

(C) Precertification Protocol: Precertification of coverage is not required for Emergency Services. However, Emergency Services resulting in a hospital admission should be reported on the day of admission to a patient's PCP where applicable and to Evernorth if Evernorth has weekend and after-hours coverage or within twenty-four (24) hours or the next business day for remaining locations.

(D) Referrals: Child Early Intervention services do not require a referral for a specialist to provide services to assess or provided as part of an individualized family services plan (IFSP).

(22) For enrollees covered under a fully insured health plan and in accordance with Part AA of Chapter 57 of the Laws of 2024, services rendered by certain in-network clinics and facilities that provide covered outpatient mental health and substance use disorder treatment shall be reimbursed at rates that are not less than the rates that would be paid for such treatment pursuant to the medical assistance program under Social Services Law Article 5, Title 11 ("Medicaid Rate"). These reimbursement rates may vary from those listed in the Provider Agreement. In the event contract rates listed in the Provider Agreement exceed the Medicaid Rate, the contract rates will apply.

(23) **Confidentiality and Disclosure Protocols for Victims of Domestic Violence Residing in New York**: New York Insurance Law §2612 prohibits discrimination against an individual that is or has been a victim of domestic violence. This law also directs insurers that receive a valid order of protection to keep confidential and not disclose the address and telephone number of the victim of domestic violence or the address and telephone number of any child residing with the victim.

(A) **Request for Confidentiality**: Evernorth must receive a valid order of protection along with a completed Request of Use and Disclosure of Protected Health Information form** in order to request that Evernorth keep confidential and not disclose:

a. the address and telephone number of the victim of domestic violence or any child residing with the victim; and
b. the name and address, and telephone number of a person providing covered services to the victim, to:
    i. a policyholder; or
    ii. another insured covered under the policy against whom the victim has a valid order of protection.

To initiate a request, the patient must complete the form and submit to Evernorth at the address on the form or the patient can call the number on the back of their identification card. They may also contact Evernorth at 800.Evernorth24 (244.6224).

(B) **Request to Receive Claim Related Information by Alternative Means or at Alternative Locations**: An insured may also make a reasonable request to receive communications of claim related information by alternative means or at alternative locations. To make such a reasonable request an Evernorth insured or requestor must complete a Request for Confidential Communications form**.

To initiate a request, the patient must complete the form and submit to Evernorth at the address on the form or the patient can call the number on the back of their identification card. They may also contact Evernorth at 800.Evernorth24 (244.6224).

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(C) **Request to Revoke a Reasonable Request**: A requestor may request a revocation of a reasonable request by completing a Change/Revoke Request form**.

To initiate a request, the patient must complete the form and submit to Evernorth at the address on the form or the patient can call the number on the back of their identification card. They may also contact Evernorth at 800.Evernorth24 (244.6224).

(D) **To access any of the above-mentioned forms, the patient can log on to **Well.Evernorth.com** > Forms > Customer Information Privacy.

(E) **New York State Domestic and Sexual Violence Hotline**: To report an incident, contact the New York State Domestic and Sexual Violence Hotline at 800.942.6906. The Spanish language telephone number is 800.942.6908. In New York City only, dial 800.621.HOPE (4673) or dial 311. The Office for the Prevention of Domestic Violence website, **https://opdv.ny.gov/**, provides additional information and resources.

(24) Insurers may not use a contract, written policy, written procedure or practice to restrict any health care provider from disclosing to any subscriber, designated representative or, where appropriate, prospective subscriber, (collectively referred to as "subscriber" going forward) any information that the provider deems appropriate regarding:

(A) a condition or a course of treatment with a subscriber including the availability of other therapies, consultations, or tests; or

(B) the provisions, terms, or requirements of the corporation's products as they relate to the subscriber.

(25) Insurers may not use a contract, written policy, written procedure or practice to prohibit or restrict any health care provider from filing a complaint, making a report or commenting to an appropriate governmental body regarding the policies or practices of the insurer which the provider believes may negatively impact upon the quality of or access to patient care.

(26) Insurers may not take any adverse action, including but not limited to refusing to renew or execute a contract or agreement with a health care provider as retaliation against a health care provider for filing a complaint, making a report or commenting to an appropriate governmental body regarding policies or practices of such corporation which may violate New York law.

(27) Insurers may not use a contract, written policy, written procedure or practice to restrict any health care provider from advocating to the corporation on behalf of the subscriber for approval or coverage of a particular course of treatment.

(28) Contracts between insurers and a health care providers may not contain any indemnification clause transferring to the health care provider any liability relating to activities, actions or omissions of the insurer as opposed to the health care provider.

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NORTH CAROLINA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of North Carolina regarding provider contracts with providers rendering health care services in the State of North Carolina. To the extent that such North Carolina laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context indicates otherwise. For the avoidance of doubt, to the extent such North Carolina laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) The following definitions are applicable to insured Plans issued in the State of North Carolina:

(a) Emergency Medical Condition means, as set forth in applicable state laws and regulations, as may be amended from time to time, a medical condition manifesting itself by acute symptoms of sufficient severity, including, but not limited to, severe pain, or by acute symptoms developing from a chronic medical condition that would lead a prudent layperson, possessing an average knowledge of health and medicine, to reasonably expect the absence of immediate medical attention to result in any of the following: placing the health of an individual, or with respect to a pregnant woman, the health of the woman or her unborn child, in serious jeopardy; serious impairment to bodily functions; serious dysfunction of any bodily organ or part.

(b) Emergency Services means, as set forth in applicable state laws and regulations, as may be amended from time to time, health care items and services furnished or required to screen for or treat an emergency medical condition until the condition is stabilized, including prehospital care and ancillary services routinely available to the emergency department.

(c) "Medically Necessary" means those Covered Services or supplies that are:

a. Provided for the diagnosis, treatment, cure, or relief of a health condition, illness, injury, or disease; and, except as allowed under G.S. 58- 3-255, not for experimental, investigational, or cosmetic purposes.

b. Necessary for and appropriate to the diagnosis, treatment, cure, or relief of a health condition, illness, injury, disease, or its symptoms.

c. Within generally accepted standards of medical care in the community.

d. Not solely for the convenience of the Participant, the Participant's family, or the provider.

For Medically Necessary services, Evernorth is not precluded from comparing the cost-

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effectiveness of alternative services or supplies when determining which of the services or supplies will be covered.

(d) If Provider is a hospital, "Utilization Review" shall mean, as set forth in applicable state laws and regulations, as may be amended from time to time, a set of formal techniques designed to monitor the use of or evaluate the clinical necessity, appropriateness, efficacy or efficiency of health care services, procedures, providers or facilities, which may include, as specified in applicable state laws and regulations, ambulatory review, case management, certification, concurrent review, discharge planning, prospective review, retrospective review, and second opinion.

(2)a. Provider shall be duly licensed by the appropriate state licensing board in North Carolina and shall be bound by and comply with the provisions of applicable local, state and federal laws and regulations. Provider shall maintain all necessary professional credentials sufficient to meet Evernorth's credential verification, including but not limited to appropriate licenses, certifications, accreditations and other similar approvals required by applicable local, state, and federal laws and regulations. If applicable, Provider shall comply with Evernorth's Standards for Delegation of Credentialing, as amended from time to time. Evernorth's credentialing requirements may be found in the Medical Management Program. Provider shall notify Evernorth promptly of any subsequent changes in the status of any information relating to the provider's professional credentials. Provider shall comply with Evernorth's utilization management programs, credential verification programs, quality management programs, and, if applicable, provider sanction programs with the stipulation that none of these shall override the professional or ethical responsibility of Provider or interfere with Provider's ability to provide information or assistance to Provider's patients.

a.(1) If applicable pursuant to a delegation set forth in the Agreement, Provider shall comply with all applicable requirements of 11 NCAC 20.0400. Provider shall submit an updated list of its providers no less frequently than quarterly, or as otherwise specified by 11 NCAC 20.0410.

b. Provider shall provide necessary Covered Services to Participants on a twenty-four (24) hour per day, seven (7) day per week basis, or arrange to provide backup services to cover Participants.

c. Evernorth shall establish a system of Participant identification and eligibility, based on current information held by the plan, prior to Provider rendering services to Participants, communicate the Medical Management Program to Participating Providers and identify Participating Providers to Payors and Participants. With regard to Participant identification, Participants shall be required to present an Evernorth identification card at the time of service. Such identification card does not guarantee eligibility. Prior to rendering services Provider should verify Participant eligibility by calling the phone number indicated on the identification card. Mutually agreeable provisions may be made for cases where incorrect or retroactive information was submitted by employer groups.

d. Evernorth shall include the name of Provider in the provider directory distributed to its members.

e. Evernorth shall provide data and information to Provider, such as:

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(1) Performance feedback reports or information to Provider, if compensation is related to efficiency criteria.
(2) Information on benefit exclusions; administrative and utilization management requirements; credential verification programs; quality assessment programs; and, if applicable, provider sanction policies. Notification of changes in these requirements shall also be provided by Evernorth allowing Provider time to comply with such changes.

f. Provider shall:

(1) Maintain confidentiality of Participant medical records and personal information as required by G.S. 58, Article 39 and other health records as required by law.
(2) Maintain medical records and other health records according to industry and Evernorth's standards.
(3) Make copies of such records available to Evernorth and the North Carolina Department of Insurance in conjunction with its regulation of Evernorth. Provider shall provide such records to the Department of Insurance at no charge.

f.(1) Evernorth shall maintain all records, books, documents, and other business records subject to 11 N.C.A.C § 19 for at least five years from the date of a disposition or report issued or adopted pursuant to 11 N.C.A.C § 19.

g. If Provider is a hospital, Evernorth shall not deny payment for Emergency Services required for the treatment of an Emergency Medical Condition or other case where a delay in providing Emergency Services would be detrimental to a Participant's health as reasonably determined by Provider's emergency room staff.

(3) The provision in the Agreement that applies regarding coordination of benefits does not relate to subrogation.

(4) a. For pre-paid benefit plans, Provider shall not bill any network plan Participant for Covered Services, except for any applicable Copayments, Deductibles or Coinsurance. This provision shall not prohibit Provider and Participant from agreeing to continue non-covered services, at the Participant's own expense, as long as Provider has notified the Participant in advance that Evernorth may not cover or continue to cover specific services and the Participant chooses to receive the service.

b. Provider shall be responsible for collecting any applicable Participant Copayments, Deductibles or Coinsurance, and fees for non-covered services shall be specified.

c. For any Covered Services, which are reimbursed on a fee-for-service basis, Provider shall bill for Covered Services according to the following:

Claims for all Covered Services shall be submitted on the appropriate claim form within one hundred eighty (180) days after the date of service or one hundred eighty (180) days after the Participant's discharge from the facility. Unless otherwise agreed to by Evernorth and Provider, failure to submit a claim within the time required does not invalidate or reduce any claim if it was not reasonably possible for the claimant to file the claim within that time, provided that the claim is submitted as soon as reasonably possible and in no event, except in the absence of legal capacity of the Participant, later than one (1) year from the time submittal

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of the claim is otherwise required.

d. 1. Payor may recover claim overpayments made to Provider by making demands for refunds and by offsetting future payments. Any such recoveries may also include related interest payments that were made pursuant to the requirements of North Carolina law. Not less than thirty (30) calendar days before Payor seeks overpayment recovery or offsets future payments, Payor shall give written notice to Provider including adequate specific information to identify the specific claim and the specific reason for the recovery. The recovery of overpayments or offsetting of future payments shall be made within the two (2) years after the date of the original claim payment unless Payor has reasonable belief of fraud or other intentional misconduct by Provider, or Provider's agents, or the claim involves a payment for the same service from a government payor.

2. If Provider is a hospital, Provider may recover underpayments in the manner and format specified in the Agreement or the Administrative Guidelines; the recovery of underpayments shall be made within the two years after the date of the original claim adjudication, unless the claim involved Hospital receiving payment for the same service from a government payor.

e. Chargemaster. Evernorth shall provide such notice as required by applicable law, including but not limited to NCGS § 58-3-227, when making adjustments permitted by any chargemaster provisions of the Agreement, or when conducting an audit permitted by any chargemaster provisions of the Agreement.

f. Sale of Business/Change in Management. Evernorth shall provide such notice as required by applicable law, including but not limited to NCGS § 58-3-227 and 58- 3-225, in response to a notice or failure to provide a notice required under the Agreement, when exercising rights under the Agreement to retroactively adjust reimbursements or initiate an overpayment recovery process.

(5) Provider shall cooperate with Participant and Evernorth in the implementation of Evernorth's Participant appeal procedure and shall assist Participant and Evernorth in taking appropriate corrective action.

(6) Throughout the term of the Agreement, Provider shall maintain at Provider's expense general and professional liability coverage in a form and amount acceptable to Evernorth ($ 1 million per occurrence and $ 3 million in the aggregate). Provider shall give Evernorth written notice of any subsequent changes in the status of such insurance, or of any claims against Provider's liability coverage.

(7) Nothing in the Agreement shall limit: (1) Evernorth's authority or responsibility to ensure Provider's participation and compliance with Evernorth's quality management, utilization management, member grievance or other provisions of the Medical Management Program; (2) the North Carolina Department of Insurance's authority to monitor the effectiveness of Evernorth's Medical Management Program; (3) Evernorth's authority to sanction or terminate Provider for giving inadequate or poor quality care or failing to comply with Evernorth's Medical Management Program; or (4) Evernorth's obligation to comply with applicable law.

(8) Upon termination of the Agreement or upon Evernorth's or intermediary's insolvency or cessation of

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operations, Provider shall continue to provide Covered Services to Participants for the duration of the period for which premium payment has been made. For Participants who are confined in an inpatient facility, Provider shall continue to provide Covered Services until the Participant is ready for discharge. Provider shall be compensated for Covered Services provided to Participants in accordance with the compensation arrangements under the Agreement. Participants shall not be liable to Provider for any amounts owed for Covered Services provided during the period of continued care other than Copayments, Deductibles, or Coinsurance billed in accordance with the terms of the applicable Plan, or for non-covered services delivered on a fee-for-service basis to Participants. Provider shall also comply with all Evernorth's procedures in the transfer of Participants to other providers, including without limitation the transition of administrative duties and records.

This provision shall survive the termination of the Agreement, regardless of the cause giving rise to termination and shall be construed to be for the benefit of Participants. This provision supersedes any oral or written agreement to the contrary now existing or hereafter entered into between Provider and the Participant or persons acting on the Participant's behalf insofar as such contrary agreement relates to liability for payment for services provided under the terms and conditions of the Agreement.

(9) The entire Agreement includes this Addendum, any amendments to the Agreement and any exhibits to the Agreement.

(10) Provider's duties and obligations under the Agreement shall not be assigned, delegated or transferred without the prior written consent of Evernorth. Evernorth, upon prior written notice to Provider, shall be entitled, but not obligated, to assign, delegate or transfer all or any part of its rights, benefits, duties, or obligations hereunder to any of its current or future subsidiaries or affiliates, or to any entity which succeeds its business through a sale, merger, or other similar transaction.

(11) If Provider is a provider group:

a. Provider acknowledges and agrees that the form of Provider's standard agreement with its providers and any amendment thereto shall comply with and include all applicable provisions of 11 NCAC 20.0202.

b. Evernorth shall retain legal responsibility to monitor and oversee the offering of services to Participants and financial responsibility to Participants.

c. Provider may not subcontract for its services without Evernorth's written permission.

d. Evernorth may approve or disapprove individual providers for inclusion in or removal from its network.

e. Provider shall make available for review by the Department of Insurance all provider contracts and subcontracts held by Provider.

f. If Provider pays its providers on a risk basis, or is responsible for claims payment to its providers:

(i) Evernorth shall receive documentation of utilization and claims payment and

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maintain accounting systems and records.

(ii) Evernorth shall arrange for financial protection of itself and Participants through such approaches as Participant hold harmless language, retention of signatory control of the funds to be distributed, or financial reporting requirements.

(iii) To the extent provided by law, the Department of Insurance shall have access to the books, records, and financial information to examine activities performed by Provider on behalf of Evernorth. Such books and records shall be maintained in the State of North Carolina.

g. Provider shall comply with all applicable statutory and regulatory requirements that apply to the functions delegated by Evernorth and assumed by Provider.

(12) Notice Contact. Any notice required under the Agreement must be in writing and sent by United States mail, postage prepaid, to the name or title and the address provided pursuant to the Notices section of the Agreement. Evernorth may also notify Provider by sending an electronic notice with automatic receipt verification to Provider's e-mail address if mutually agreed upon and provided pursuant to the Notices section of the Agreement. Either party can change the name, title or address for notices by giving written notice of the change to the other in the manner just described.

(13) Medical Management Program. In addition to the distribution of the Medical Management Program as set forth in the Agreement, Evernorth will provide its Medical Management Program in hard copy, CD or other electronic format, or by web posting, prior to the execution or amendment of the Agreement, and annually.

(14) Amendments. In addition to the negotiated terms of the Amendment provision as set forth in the Agreement, Evernorth shall send any contract amendment to the Notice Contact identified in the Agreement proposing to change any term of this Agreement which modifies a fee schedule, including terms incorporated by reference. No such contract amendment shall be conveyed by use of an electronic medium of communication. An amendment under this section shall be dated, labeled "Amendment," signed by Evernorth, and shall include an effective date. Provider shall have at least sixty (60) days from the date of receipt to object to the proposed amendment. The amendment shall be effective upon Provider failing to object in writing within sixty (60) days. Should Provider object, the amendment will not be effective, and Evernorth shall be entitled to terminate the Agreement upon sixty (60) days written notice.

(15) Credentialing. When applying for participation in the network, North Carolina professionals will be sent the North Carolina Uniform Credentialing Form. Within sixty (60) days of receipt of a completed application and all necessary supporting documents, Evernorth shall assess and verify the applicant's qualifications and notify the applicant of its decision. In the event that the application is not complete, needed documents are missing, or verification information is unavailable, the applicant will be informed in writing of the delay, the reason for the delay, and the name and phone number of a staff participant to contact if the applicant has questions. In the event that missing information or documents have not been received within sixty (60) days of initial receipt of the application, the applicant's file will be closed or pended, and the applicant informed of this fact in writing. Included in this correspondence will be the length of time the application will be pended, if applicable, and the name and phone number of a staff person to contact with questions.

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NORTH DAKOTA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of North Dakota regarding provider contracts with providers rendering health care services in the State of North Dakota. To the extent that such North Dakota laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such North Dakota laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

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1. The definition for Emergency Services, if any, shall comply with North Dakota laws and regulations to the extent applicable.

2. In addition to the negotiated terms of the Agreement establishing limits on billing Participants, the following shall apply in accordance with applicable laws and regulations:

> Provider agrees, in the event of Evernorth's insolvency, to continue to provide Covered Services to a Participant through the period for which a premium has been paid to Evernorth and until the Participant's discharge from an inpatient facility.

3. Evernorth and Provider shall provide at least sixty (60) days written notice to each other before terminating the Agreement without cause. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement by either party, such longer notification period will apply.

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OHIO REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Ohio regarding provider contracts with providers rendering health care services in the State of Ohio. To the extent that such Ohio laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Ohio laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined herein shall have the same meaning as set forth in the Agreement.

(A) (1) Provider shall be duly licensed in the State of Ohio.

(2) Upon request, Evernorth or its designee shall notify Provider of Covered Services, including any limitations or conditions thereon, in writing and/or telephonically.

(3) (a) Pursuant to Section 3963.03 (A)(1)(a)(ii) of the Ohio Revised Code, information pertinent to the fee schedule of procedure codes reasonably expected to be billed by Provider for services provided and the associated payment or compensation for each procedure code is available at **Provider.Evernorth.com**.

(b) Pursuant to Section 3963.03 (A)(1)(a)(iii) of the Ohio Revised Code, information pertinent to the effect, if any, on payment or compensation if more than one (1) procedure code applies to the service is available at **Provider.Evernorth.com**.

(c) Pursuant to Section 3963.03 (A)(2) of the Ohio Revised Code, the identity of the contracting entity or payer responsible for the processing of Provider's compensation or payment is available at **Provider.Evernorth.com**.

(4) The Agreement may permit network rental arrangements which allow the selling, renting, or giving the contracting entity's rights to the services of the Participating Provider, to a third party (including other preferred provider organizations), and the third party accessing the Participating Provider's services is any of the following:

(a) A payer or a third-party administrator or other entity responsible for administering claims on behalf of the payer;

(b) A preferred provider organization or preferred provider network that receives access to the Participating Provider's services pursuant to an arrangement with the preferred provider organization or preferred provider network in a contract with the Participating Provider that is in compliance with Section 3963.02 (A)(1)(c) of the Ohio Revised Code, and is required to comply with all of the terms, conditions, and affirmative obligations to which the originally contracted primary participating provider network is bound under its contract with the Participating Provider, including, but not limited to, obligations concerning patient steerage and the timeliness and manner of reimbursement; or

(c) An entity that is engaged in the business of providing electronic claims transport between

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the contracting entity and the payer or third-party administrator and complies with all of the applicable terms, conditions, and affirmative obligations of the contracting entity's contract with the Participating Provider including, but not limited to, obligations concerning patient steerage and the timeliness and manner of reimbursement.

(5) (a) Evernorth or its designee may recover overpayments if the recovery process is initiated not later than two (2) years after the payment was made to Provider. Evernorth or its designee shall inform Provider of its determination of overpayment by providing notice in accordance with Section 3901.388(C) of the Ohio Revised Code. Evernorth or its designee shall give Provider an opportunity to appeal the determination. If Provider fails to respond to the notice sooner than thirty (30) days after the notice is made, elects not to appeal the determination, or appeals the determination but the appeal is not upheld, Evernorth or its designee may initiate recovery of the overpayment. Such recovery may include deducting the amount of the overpayment from other payments owed to Provider or by taking action pursuant to any other remedy available under the Ohio Revised Code. Evernorth shall permit Provider to repay the amount by making one (1) or more direct payments to Evernorth or its designee or by having the amount deducted from other payments owed to Provider.

(b) To the extent required by § 3901.388(A) of the Ohio Revised Code, if Provider believes a Covered Service has been underpaid, Provider must submit a written request for an appeal or adjustment with Evernorth or its designee within two (2) years from the date of Payor's payment or explanation of payment. The request must be submitted in accordance with the terms of the Agreement and the Medical Management Program. Requests for appeals or adjustments submitted after this date may be denied for payment, and Provider will not be permitted to bill Evernorth, the Payor or the Participant for those services.

(6) Provider shall give Evernorth ten (10) days prior written notice of cancellation, reduction or termination of general and professional liability insurance.

(7) (a) Evernorth shall provide Provider with at least ninety (90) days written notice of the effective date of a Material Amendment to the Agreement.

"Material Amendment" means an amendment to an Agreement that: a) decreases Provider's payment or compensation; b) changes the administrative procedures in a way that may reasonably be expected to
significantly increase Provider's administrative expense; or c) adds a new product.

A Material Amendment does not include: (a) a decrease in payment or compensation resulting solely from a change in a published fee schedule upon which the payment or compensation is based and the date of applicability is clearly identified in the Agreement; (b) a decrease in payment or compensation that was anticipated under the terms of the Agreement, if the amount and date of applicability of the decrease is clearly identified in the Agreement; (c) an administrative change that may significantly increase Provider's administrative expense, the specific applicability of which is clearly identified in the Agreement; (d) changes to an existing prior authorization, precertification, notification, or referral program that do not substantially increase Provider's administrative expense; (e) changes to an edit program or to specific edits, if Provider is provided notice of the changes pursuant to division (A) (1) of Section 3963.04 of the Ohio Revised Code and the notice includes information sufficient for the provider to determine the effect of the change; or (f)

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changes to the Agreement described in division (B) of Section 3963.04 of the Ohio Revised Code.

If Provider objects in writing to the Material Amendment within fifteen (15) days of receipt and there is no resolution of the objection, Evernorth or Provider may terminate the Agreement upon written notice to the other party but no later than sixty (60) days prior to the effective date of the Material Amendment.

If Provider does not object to the Material Amendment within fifteen (15) days of receipt, the change shall be effective as specified in the notice.

(b) If an amendment to the Agreement is not a Material Amendment, Evernorth shall provide Provider notice of the amendment at least fifteen (15) days prior to the effective date of the amendment.

(c) Subsections (a) and (b) above shall not apply if the delay caused by compliance with the requirements could result in imminent harm to a Participant, if the Material Amendment to the Agreement is required by state or federal law, rule, or regulation, or if Provider affirmatively accepts the Material Amendment in writing and agrees to an earlier effective date than otherwise required for such Material Amendment.

In addition, subsections (a) and (b) above shall not apply under any of the following circumstances:

(i) Provider's payment or compensation is based on the current Medicaid or Medicare physician fee schedule, and the change in payment or compensation results solely from a change in that physician fee schedule.

(ii) A routine change or update of the Agreement is made in response to any addition, deletion, or revision of any service code, procedure code, or reporting code, or a pricing change is made by any third party source.

"Service code, procedure code, or reporting code" means the current procedural terminology (CPT), current dental terminology (CDT), the healthcare common procedure coding system (HCPCS), the international classification of diseases (ICD), or the drug topics redbook average wholesale price (AWP).

"Third party source" means the American Medical Association, American Dental Association, the Centers for Medicare and Medicaid Services, the National Center for Health Statistics, the Department of Health and Human Services Office of the Inspector General, the Ohio Department of Insurance, or the Ohio Department of Medicaid.

(d) Notwithstanding anything in this section, Evernorth may amend the Agreement by operation of law as required by any applicable state or federal law, rule, or regulation.

(8) In the event of termination of the Agreement and to the extent applicable, the provisions of Section 3963.02 (E) of the Ohio Revised Code shall apply.

(9) Notwithstanding anything to the contrary set forth in the Agreement, Section 1753.09 of the Ohio Revised Code applies to the termination of a Participating

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Provider's Agreement for any of the causes described in divisions (A), (D), and (F) (1) and (2) of Section 1753.09 of the Revised Code.

(10) Notices required under this Agreement shall be in writing and shall be deemed to have been duly given a) on the date of service if served personally on the party to whom notice is to be given; b) on the date of delivery if sent via overnight courier to the party to whom notice is to be given and properly addressed as specified in the Agreement; c) with respect to notifications of termination of this Agreement, on the third day after deposit in the United States mail, if mailed via certified mail, postage prepaid, and properly addressed as specified in the Agreement; d) with respect to notifications other than notifications of termination, on the third day after deposit in the United States mail, if mailed postage prepaid and properly addressed as specified in the Agreement; or e) with respect to notifications by Evernorth other than notifications of termination, on the date Evernorth sends an electronic notice to Provider with an automatic receipt verification to Provider's e-mail address as specified in the Agreement.

(11) (a) When the arbitration issues are limited to issues that only concern the enforcement of the contract rights conferred by Section 3963.02, divisions (A) and (D) of Section 3963.03, and Section 3963.04 of the Ohio Revised Code, the arbitrator may award reasonable attorney's fees and costs for arbitration relating to the enforcement of this section to the prevailing party.

(b) A party shall not simultaneously maintain an arbitration proceeding as described in division (F)(1) of Section 3963.02 of the Ohio Revised Code and pursue a complaint with the Superintendent of Insurance to investigate the subject matter of the arbitration proceeding. However, if a complaint is filed with the Department of Insurance, the Superintendent of Insurance may choose to investigate the complaint or, after reviewing the complaint, advise the complainant to proceed with arbitration to resolve the complaint. The Superintendent of Insurance may request to receive a copy of the results of the arbitration. If the Superintendent of Insurance notifies an insurer or a health insuring corporation in writing that the superintendent has initiated a market conduct examination into the specific subject matter of the arbitration proceeding pending against that insurer or health insuring corporation, the arbitration proceeding shall be stayed at the request of the insurer or health insuring corporation pending the outcome of the market conduct investigation by the superintendent.

(B) With respect to Covered Services rendered to Participants covered under a Plan insured by a Health Insuring Corporation (as that term is defined under Ohio law), the following provisions shall apply:

(1) The definition for Emergency Services, if any, shall comply with Ohio laws and regulations to the extent applicable.

(2) In the event of Evernorth's insolvency or discontinuance of operations: (1) Provider shall continue to provide Covered Services to Participants as needed to complete any Medically Necessary procedures commenced but unfinished at the time of such insolvency or discontinuance of operations (the completion of Medically Necessary procedures shall include the rendering of all Covered Services that constitute Medically Necessary follow-up care for that procedure); and (2) if a Participant is receiving necessary inpatient care at a hospital, Provider shall continue to provide such care until the earlier of the following: (a) the day the Participant is discharged from the hospital, (b) the day the Participant's attending physician determines that such inpatient care is no longer medically indicated, (c) the day the Participant has reached the

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benefit limit under the applicable Plan, (d) the effective date of Participant's new coverage; or (d) thirty (30) days after such insolvency or discontinuance of operations.

Provider is not required to continue to provide Covered Services after the occurrence of any of the following: (a) the end of the thirty (30) day period following the entry of a liquidation order under Chapter 3903 of the Ohio Revised Code; (b) the end of the Participant's period of coverage for a contractual prepayment or premium; (c) the Participant obtains equivalent coverage with another Health Insuring Corporation or insurer, or the Participant's employer obtains such coverage for the Participant; (d) the Participant or the Participant's employer terminates coverage under the Plan; or (e) a liquidator effects a transfer of the Health Insuring Corporation's obligations under the contract under division (A) (8) of Section 3903.21 of the Ohio Revised Code.

(3) Evernorth and Provider shall observe, protect and promote the rights of Participants.

(4) Those terms used in the Agreement that are defined in Ohio Revised Code Chapter 1751 (Health Insuring Corporations) are used in the Agreement in a manner consistent with those definitions.

(C) Ohio House Bill 1:

(1) Ohio House Bill 1 requires that claims received electronically be paid electronically. In order to comply with this law, Evernorth will contact its Participating Providers three (3) times a year to ensure that they have the opportunity to provide Evernorth with the necessary information on file to issue an electronic payment.

(2) To register for direct deposit of electronic payment, log in to **Provider.Evernorth.com** > My Practice and click "Enroll" to create an account. If Provider is not yet registered for the Evernorth for Health Care Professionals website, visit **Provider.Evernorth.com** and click "Register Now." For step-by-step registration directions, go to **Provider.Evernorth.com** and click "Learn How to Register and Log In." When Provider registers for electronic payments, be certain to supply all TINs that apply to Provider and Provider's practice.

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OKLAHOMA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Oklahoma regarding provider contracts with providers rendering health care services in the State of Oklahoma. To the extent that such Oklahoma laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Oklahoma laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) Payor may only retroactively deny reimbursement to Provider during the twelve (12) month period after the date Payor paid the claim. However, this provision shall not apply: (a) if the payment was made because of fraud by Provider; or (b) if Provider has otherwise agreed to make a refund to Evernorth for overpayment of a claim.

(2) A. If Provider voluntarily chooses to terminate the Agreement, Provider shall give Evernorth ninety (90) days prior written notice of the disaffiliation. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement by Provider, the longer notification period will apply.

B. In the event Evernorth terminates the Agreement for reasons other than for cause, Provider shall continue to provide Covered Services under the terms of the Agreement for ninety (90) days from the date of notice to the Participant, for a Participant who is under Provider's care at the time of such termination and who has a degenerative and disabling condition or disease, or is terminally ill. With respect to a Participant who is under Provider's care at the time of such termination and who has entered the third trimester of pregnancy, additional coverage of services shall continue through at least six (6) weeks of postpartum evaluation. Provider shall be compensated for continued services in accordance with the compensation arrangements under the Agreement for the ninety (90) day period from the date of notice to Participants. Provider has no obligation under the Agreement to provide services to individuals who cease to be Participants.

C. In the event Provider voluntarily chooses to terminate the Agreement, Provider shall continue to provide Covered Services under the terms of the Agreement for a Participant who is under Provider's care at the time of such termination for ninety (90) days from the date of such notice of termination to Evernorth; or for a period that includes delivery and postpartum care, if the Participant has entered the third trimester of pregnancy at the time of disaffiliation. Provider shall be compensated for continued services in accordance with the compensation arrangements under the Agreement for the ninety (90) day period from the date of the termination notice to Evernorth. Provider has no obligation under the Agreement to provide services to individuals who cease to be Participants.

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OREGON REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Oregon regarding provider contracts with providers rendering health care services in the State of Oregon. To the extent that such Oregon laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Oregon laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

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(1) Evernorth, upon request by Provider, shall give Provider an annual accounting accurately summarizing the financial transactions between the parties for that year.

(2) Provider may withdraw from the care of a Participant when, in the professional judgment of Provider, it is in the best interest of the Participant to do so.

(3) Except in the case of misrepresentation, precertification determinations shall be subject to the following requirements:

(1) Precertification determinations relating to benefit coverage and medical necessity shall be binding on Evernorth if obtained no more than thirty (30) days prior to the date the service is provided.

(2) Precertification determinations relating to Participant eligibility shall be binding on Evernorth if obtained no more than five (5) business days prior to the date the service is provided.

(4) Upon request by Provider, the criteria used in the utilization management review process and the method of development of the criteria shall be made available for review.

(5) Evernorth shall employ or retain a physician licensed under O.R.S. Sec. 677.100 to 677.228 who shall be responsible for all final medical and mental health decisions relating to coverage or payment made pursuant to the Agreement.

(6) Provider will be paid for Covered Services rendered to Participants in accordance with O.R.S. Sec. 743B.450 and O.R.S. Sec. 743B.452.

(7) In the event Evernorth fails to pay for health care services covered by the Plan, Provider shall not bill or otherwise attempt to collect from Participants amounts owed by Evernorth, and Participants shall not be liable to Provider for any sums owed by Evernorth.

(8) Evernorth may not terminate or otherwise financially penalize Provider for:

(1) Providing information to or communicating with a Participant in a manner that is not slanderous,

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defamatory or intentionally inaccurate concerning:

(a) Any aspect of the Participant's medical condition;

(b) Any proposed treatment or treatment alternatives, whether covered by the Participant's Plan or not; or

(c) Provider's general financial arrangement with Evernorth.

(2) (a) Referring a Participant to another provider, whether or not that provider is under contract with Evernorth. If Provider refers Participant to another provider, Provider shall:

(i) Comply with Evernorth's written policies and procedures with respect to any such referrals; and

(ii) Inform the Participant that the referral services may not be covered by Evernorth.

(b) Allocation of costs for referral services shall be a matter of contract between Provider and Evernorth. Allocation of costs to Provider by contract shall not be considered a penalty under this section.

(9) Evernorth and Provider shall provide continuity of care to Participants as provided in O.R.S. Sec. 743B.225.

(10) Except in cases of fraud or abuse of billing, Payor may not request a refund from Provider of a payment previously made to satisfy a claim unless Payor does so in writing, specifying the reasons for the request, within eighteen (18) months after the date the payment was made. If Payor requests a refund for reasons related to coordination of benefits with another health insurer or entity responsible for payment of a claim, the request for refund must be made in writing, specifying the reasons for the request, within thirty (30) months after the date the payment was made. If Provider fails to contest the request for a refund in writing to Evernorth or Payor within thirty (30) days of receipt, the request for refund shall be deemed accepted and the refund must be paid.

(11) Except in cases of fraud, Provider may not request additional payment from Payor to satisfy a claim unless Provider does so in writing, specifying the reasons for the request, within eighteen (18) months after the date the claim was denied or the payment intended to satisfy the claim was made. If Provider requests additional payment from Payor to satisfy a claim for reasons related to coordination of benefits with another health insurer or entity responsible for payment of a claim, the request for additional payment must be made in writing, specifying the reasons for the request, within thirty (30) months after the date the claim was denied or payment intended to satisfy the claim was made.

(12) The Agreement may permit network arrangements which grant access to Evernorth's rights as a contracting entity, as defined in applicable state laws and regulations, to Provider's health care services and discounted rates to a third party, as defined in applicable state laws and regulations, provided that the third party accessing Provider's health care services and discounted rates is contractually obligated to comply with all applicable terms, limitations and conditions of the Agreement.

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(13) Confidentiality Protocols:

(1) Confidentiality protocols to redirect protected health information for customers residing in Oregon:

(a) Chapter 470, Oregon laws 2015 addresses a customer's right to have protected health information ("PHI") redirected to an alternate location. It requires health insurance companies to implement protocols to accommodate a customer's request to have protected health information sent directly to the person(s) whom the customer has identified as authorized recipients via their express consent, as permitted by law. This restriction prohibits disclosure of PHI to any other person insured under the same policy, including the person who pays for the health insurance plan (the primary account holder), unless the customer expressly consents to disclosure of the PHI.

(2) Request to have PHI redirected to an alternative person and location:

(a) The **Oregon Request for Confidential Communication** form is available on the Oregon Insurance Division website at **www.insurance.oregon.gov**. To make this request, the customer must complete, sign, and fax or email the form to Evernorth's Central Health Insurance Portability and Accountability Act (HIPAA) Unit:

Fax: 1.877.815.4827, or 1.859.410.2419
Email: **CHUSI@Evernorth.com**

To assist Provider's patients with these requests, please refer them to the **Oregon Insurance Division** > Insurance for consumers > Health insurance > Patient right to privacy > **Oregon Request for Confidential Communication**.

(14) OneHealthPort Oregon:

(1) In cooperation with the Oregon Health Leadership Council, Evernorth and OneHealthPort have launched OneHealthPort Oregon. OneHealthPort offers secure access to participating Oregon health plan and other payor administrative websites, including Evernorth for HealthCare Professionals (**Provider.Evernorth.com**), through a single login process.

(2) Evernorth providers may go to the OneHealthPort website and log in using their OneHealthPort user ID and password. Once Provider selects the Evernorth for Health Care Professionals link, Provider may proceed through security steps to ensure Provider's identity. When finished, access is granted to the Evernorth website. There are a few Evernorth for Health Care Professionals functions that still require direct login to **Provider.Evernorth.com**. These services include fee schedule inquiries.

(15) Credentialing: Effective June 1, 2020, Evernorth will make a determination approving or denying a credentialing application submitted to Evernorth no later than ninety (90) days after receiving a complete application from a health care provider. All determinations made by Evernorth in approving or denying credentialing applications will average no more than sixty (60) days.

(16) Coordination of Benefits: When applicable, Evernorth coordination of benefits activities are subject to Oregon Administrative Rules 836-020-0770 – 836-020-0801, which provides that:

(1) If an agreement cannot be reached, Evernorth will share equally the allowable expenses between the plans according to 836-020-0785.

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(2) If the plans cannot agree on the order of benefits within thirty (30) calendar days after the plans have received all of the information needed to pay the claim, the plans shall immediately pay the claim in equal shares and determine their relative liabilities following payment, except that no plan shall be required to pay more than it would have paid had it been the primary plan.

(17) Precertification: In accordance with Oregon state law O.R.S. 836-053-1200 (6), a prior authorization determination shall be binding on the insurer for the lesser of the following periods: (a) five (5) business days, or (b) the period during which the enrollee's coverage remains in effect, provided that when the insurer issues the prior authorization, the insurer has specific knowledge that the enrollee's coverage will terminate sooner than five (5) business days following the day the authorization is issued and the insurer specifies the termination date in the authorization; and for the period during which the enrollee's coverage remains in effect beyond the time period established pursuant to subsection (a) of this section, up to a maximum of thirty (30) calendar days. For purposes of counting days, day 1 occurs on the first business or calendar day, as applicable, following the day on which Evernorth issues a prior authorization determination. Evernorth will not impose a restriction or condition on its prior authorization determinations that limits, restricts or effectively eliminates the binding force established for such determinations.

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EVERNORTH HEALTH SERVICES

PENNSYLVANIA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Pennsylvania regarding provider contracts with providers rendering health care services in the State of Pennsylvania. To the extent that such Pennsylvania laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Pennsylvania laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) Evernorth's administrative guidelines applicable to the Agreement are found in the provider reference manual entitled "Behavioral Medical Management Program."

(2) (A) The following definition of Emergency Services is applicable to the Agreement:

Emergency Services means any health care service provided to a Participant after the sudden onset of a medical condition that manifests itself by acute symptoms of sufficient severity or severe pain, such that a prudent layperson, who possesses an average knowledge of health and medicine, could reasonably expect the absence of immediate medical attention to result in one or more of the following:

(a) placing the health of the Participant, or, with respect to a pregnant woman, the health of the woman or her unborn child, in serious jeopardy;
(b) serious impairment to bodily functions; or
(c) serious dysfunction of any bodily organ or part.

Coverage for Emergency Services provided during the period of the emergency shall include evaluation, testing, and if necessary, stabilization of the condition, emergency transportation and related emergency care provided by a licensed ambulance service.

(B) If the patient is admitted to a hospital or other health care facility, the emergency provider shall notify Evernorth of the emergency services delivered within forty-eight (48) hours or on the next business day, whichever is later. An exception to this requirement will be made where the medical condition of the patient precludes the provider from accurately determining the patient's benefit plan.

(3) Evernorth acknowledges and agrees that practitioners or other individuals conducting utilization management are not compensated for approvals or denials of Covered Services.

(4) The "Charges to Participants" provision survives termination of this Agreement, regardless of the reason for termination, is intended to be for the benefit of Participants, and supersedes any oral or written agreement to the contrary now existing or hereafter entered into between Provider and a Participant or persons acting on the Participant's behalf. Provider hereby agrees that in no event, including, but not limited to non-payment by Payor, Payor's insolvency or breach of this Agreement,

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shall Provider bill, charge, collect a deposit from, seek compensation, remuneration or reimbursement from, or have any recourse against Participants or persons acting on the Participant's behalf (other than Payor) for Covered Services provided pursuant to this Agreement. This provision shall not prohibit collection of any applicable Copayments, Deductibles or Coinsurance billed in accordance with the terms of a Plan or fees for non-Covered Services.

(5) Termination of the Agreement With Notice: Neither party to the Agreement is permitted to terminate the Agreement with notice upon less than sixty (60) days' prior written notice to the other party. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement with notice, such longer notification period will apply.

(6) Notwithstanding anything to the contrary set forth in the Agreement:

(A) Evernorth shall not penalize or restrict a health care provider from discussing:
(1) the process that Evernorth, Plan, or any entity contracting with Evernorth or Plan uses or proposes to use to deny payment for a health care service;
(2) Medically Necessary and appropriate care with or on behalf of a Participant, including information regarding the nature of treatment; risks of treatment; alternative treatments; or the availability of alternate therapies, consultation or tests; or
(3) Evernorth's or Plan's decision to deny payment for a health care service.

(B) A provision to prohibit or restrict disclosure of Medically Necessary and appropriate health care information contained in a contract with a health care provider is contrary to public policy and shall be void and unenforceable.

(C) Evernorth shall not terminate a contract with a health care provider for any of the following:
(1) advocating for Medically Necessary and appropriate health care consistent with the degree of learning and skill ordinarily possessed by a reputable health care provider practicing according to the applicable legal standard of care;
(2) filing a grievance pursuant to the procedures set forth in Pennsylvania law; or
(3) protesting a decision, policy or practice that the health care provider, consistent with the degree of learning and skill ordinarily possessed by a reputable health care provider practicing according to the applicable legal standard of care, reasonably believes interferes with the health care provider's ability to provide Medically Necessary and appropriate health care.

(D) Nothing in this provision shall:
(1) prohibit Evernorth or Plan from making a determination not to pay for a particular medical treatment, supply or service, enforcing reasonable peer review or utilization review protocols or making a determination that a health care provider has or has not complied with appropriate protocols;
(2) be construed as requiring Evernorth or Plan to provide, reimburse for or cover counseling, referral, or other health care services if Evernorth or Plan: (i) objects to the provision of that service on moral or religious grounds; and (ii) makes available information on its policies regarding such health care services to Participants and prospective Participants.

(E) Nothing in this provision shall be construed to permit Evernorth to sanction, terminate or fail to renew a health care provider's participation for any of the following reasons:
(1) advocating for Medically Necessary and appropriate health care services for a Participant;

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(2) filing a grievance on behalf of and with the written consent of a Participant, or helping a Participant to file a grievance;
(3) protesting an Evernorth decision, policy or practice the health care provider believes interferes with its ability to provide Medically Necessary and appropriate health care;
(4) the health care provider has a practice that includes a substantial number of patients with expensive medical conditions;
(5) the health care provider objects to the provision of or refuses to provide a health care service on moral or religious grounds; or
(6) taking another action specifically permitted by sections 2113, 2121, and 2171 of the act (40 P.S. sections 991.2113, 991.2121 and 991.2171).

(7) Except in the event of Evernorth's termination of the Agreement for cause, including breach of the Agreement, fraud, criminal activity or posing a danger to Participants or the health, safety or welfare of the public as determined by Evernorth, if Evernorth initiates termination of the Agreement with Provider, a Participant may continue an ongoing course of treatment with Provider, at the Participant's option, for a transition period of up to sixty (60) days from the date the Participant was notified by Evernorth of the termination or pending termination. Evernorth in consultation with Participant and Provider, may extend the transitional period if determined to be clinically appropriate. In the case of a Participant in the second or third trimester of pregnancy at the time of notice of the termination or pending termination, the transitional period shall extend through postpartum care related to the delivery. Any health care service provided under this provision shall be covered by Evernorth under the same terms and conditions applicable prior to the termination of the Agreement. Nothing in this provision shall require Evernorth to cover health care services that are not otherwise covered under the terms and conditions of the Plan.

(8) Evernorth has three (3) types of internal resolution processes to resolve the various disputes which may arise between Evernorth and Pennsylvania providers.

(A) Overview of Dispute Resolution Process:
a. Termination Dispute Resolution Process. Disputes pertaining to the termination of the Agreement (including termination resulting from Provider's failure to provide Evernorth with requested recredentialing information on a timely basis and Evernorth's termination of the Agreement for business reasons) will be resolved through this process.
b. Informal Dispute Resolution Process. If Provider does not have the Participant's consent to appeal a payment denial, or if the Participant is covered by a service agreement which is not subject to Act 68 (for example, the Participant is covered by a self-funded plan) Provider may appeal a denial of payment for a health care service (on the grounds of medical necessity or for administrative reasons) in accordance with this process.

(B) Details of Dispute Resolution Process:
a. Termination Disputes Resolution Process. For all disputes pertaining to the termination of the Agreement (including termination resulting from Provider's failure to provide Evernorth with requested recredentialing information on a timely basis and Evernorth's termination of the Agreement for business reasons), Evernorth has a two (2) level internal appeal process and an arbitration of disputes process:
i. Internal Review of Disputes:
1. First Level Review. In the event that Provider has a dispute with respect to termination of the Agreement, the dispute shall be submitted for review and resolution to the Evernorth designee identified by Evernorth in Evernorth's termination letter, as applicable (the "First Level Review"). Provider must submit a request for a First Level Review of a termination dispute within

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thirty (30) days of the date of the termination letter. The appeal will be scheduled within thirty (30) days of receipt of request for the appeal. A panel of at least three (3) providers who were not involved in the original decision will decide the appeal of the termination. Provider may bring third parties with them to the appeal and may present information to the panel. The decision shall be communicated to Provider within five (5) days. There is no cost to Provider for requesting a First Level Review.

2. Second Level Review. If Provider is not satisfied with the resolution at the First Level Review, Provider may submit the matter for a second level review to the Evernorth designee identified in the First Level Review decision letter (the "Second Level Review"). Provider must submit a request for a Second Level Review within sixty (60) days of the date of the letter communicating the First Level Review decision. The Second Level Review is sent to the National Medical Director or his designee and is reviewed by a committee of nurses and providers employed by Evernorth, at the next available meeting of such committee. The decision of the committee shall be communicated to Provider within five (5) days of the decision. There is no cost to Provider for requesting a Second Level Review. The Second Level Review decision will be binding on Evernorth and Provider if the resolution is accepted by Provider.

ii. Arbitration of Disputes. In the event that Provider's termination dispute as described above is not resolved through the aforementioned process, Provider may initiate arbitration by providing written notice to Evernorth. Written acknowledgment of Provider's request for arbitration shall be made within fifteen (15) days after the receipt of the request for arbitration. If Provider initiates arbitration as provided above, the proceeding shall be held in the jurisdiction of the provider's domicile. The parties will jointly appoint a mutually acceptable arbitrator. If the parties are unable to agree upon such an arbitrator within thirty (30) days after a party has given the other party written notice of its desire to submit a dispute for arbitration, then the parties shall prepare a Request for a Dispute Resolution List and submit it to the American Health Lawyers Association Alternative Dispute Resolution Service ("AHLA ADR Service") along with the appropriate administration fee. In accordance with the Codes of Ethics and Rules of Procedure developed by the AHLA ADR Service, the parties will be sent a list of ten (10) arbitrators along with a background and experience description, references and fee schedule for each. The ten (10) will be chosen by the AHLA ADR Service on the basis of their experience in the area of the dispute, geographic location and other criteria as indicated on the request form. The parties to the dispute will review the qualifications of the ten (10) suggested arbitrators and rank them in order of preference from 1 to 9. Each party has the right to strike one (1) of the names from the list the person with the lowest total will be appointed to resolve the case. Each party shall assume its own costs, but the compensation and expenses of the arbitrator(s) and any administrative fees or costs shall be borne equally by the parties. Arbitration shall be the exclusive remedy for the settlement of disputes arising under this Agreement. The decision of the arbitrator(s) shall be final, conclusive, and binding, and no action at law or in equity may be instituted by either party other than to enforce the award of the arbitrator(s). Judgment upon the award rendered by the arbitrator(s) may be entered in any court of competent jurisdiction. The Agreement will remain in full force and effect during any such period of arbitration unless otherwise terminated pursuant to the terms of this

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Agreement. If Provider fails to request a First Level Review, Second Level Review, or Arbitration of Provider's termination dispute within the applicable timeframes, Evernorth's last determination regarding the dispute shall be binding on Provider.

b. Informal Dispute Resolution ("IDR") Process. If Provider does not have the Participant's consent to appeal a payment denial, or if the Participant is covered by a service agreement which is not subject to Act 68 (for example, Participant is covered by a service agreement between Evernorth or Evernorth affiliate and a self-funded payor), Provider may appeal a denial of payment for a health care service in accordance with this process. Examples of disputes that may be resolved through this process include:
    1. Denied inpatient stays/length of stay or level of care challenges where Provider has NOT indicated that Provider is appealing on behalf of the Participant
    2. ClaimCheck challenges
    3. Fee/discount discrepancies
    4. Untimely claim filing denials
    5. Denials based upon Provider's failure to contact Evernorth to secure any required prior authorization
    6. Denials based on violation of any "Delayed Treatment Days" provision

    The IDR Process includes two internal levels of review and an arbitration of disputes process.

    i. First Level IDR Review. If Provider wishes to appeal pursuant to the IDR process, Provider submits the appeal of the payment dispute to the Evernorth designee identified by Evernorth in Evernorth's explanation of payment (the "First Level IDR Review"). Provider must submit a request for a First Level IDR Review within one hundred eighty (180) days of the date of the initial explanation of payment, or if the appeal relates to a claim that was adjusted by Evernorth, within one hundred eighty (180) days from the date of the last payment adjustment. The First Level IDR Review shall be completed within thirty (30) days after receipt of the request for such review. The decision shall be communicated to Provider within fifteen (15) days of completion of the First Level IDR Review. An appeal processor or a nurse reviews administrative appeals (i.e. untimely claim filing, claim check challenges). A nurse initially reviews medical-necessity appeals. If the nurse cannot overturn the appeal, it is reviewed and decided by a provider. There is no cost to Provider for requesting a First Level IDR Review.

    ii. Second Level IDR Review. If Provider is not satisfied with the resolution at the First Level IDR Review, Provider may submit the matter for a second level review to the Evernorth designee identified in the First Level Review decision letter (the "Second Level IDR Review"). Provider must submit a request for a Second Level IDR Review within sixty (60) days of the date of the letter communicating the First Level IDR Review decision. The Second Level IDR Review shall be considered within thirty (30) days after the receipt of the request for such review. Administrative appeals (i.e., untimely claim filing, claim check challenges) are reviewed by an appeal processor or a nurse. A nurse initially reviews medical necessity appeals. If the nurse cannot overturn the appeal, it is reviewed and decided by a provider. The decision shall be communicated to Provider within fifteen (15) days of completion of the Second Level IDR Review. The Second Level IDR Review decision will be binding on Evernorth and Provider if the resolution is accepted by Provider. There is no cost to Provider for requesting a Second Level IDR Review.

    iii. External Review/Arbitration of Disputes. In the event that Provider is not satisfied with the Second Level IDR Review decision payment dispute Provider may initiate

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arbitration by providing written notice to Evernorth. With respect to a provider payment dispute, Provider must submit a request for arbitration within one (1) year of the date of the letter communicating the Second Level IDR Review decision. Written acknowledgment of Provider's request for arbitration shall be made within fifteen (15) days after the receipt of the request for arbitration. If Provider initiates arbitration as provided above, the proceeding shall be shall be held in the jurisdiction of Provider's domicile. The parties will jointly appoint a mutually acceptable arbitrator. If the parties are unable to agree upon such an arbitrator within thirty (30) days after a party has given the other party written notice of its desire to submit a dispute for arbitration, then the parties shall prepare a Request for a Dispute Resolution List and submit it to the American Health Lawyers Association Alternative Dispute Resolution Service ("AHLA ADR Service") along with the appropriate administration fee. In accordance with the Codes of Ethics and Rules of Procedure developed by the AHLA ADR Service, the parties will be sent a list of ten (10) arbitrators along with a background and experience description, references and fee schedule for each. The ten (10) will be chosen by the AHLA ADR Service on the basis of their experience in the area of the dispute, geographic location and other criteria as indicated on the request form.

The parties to the dispute will review the qualifications of the ten (10) suggested arbitrators and rank them in order of preference from 1 to 9. Each party has the right to strike one (1) of the names from the list. The person with the lowest total will be appointed to resolve the case. Each party shall assume its own costs, but the compensation and expenses of the arbitrator(s) and any administrative fees or costs shall be borne equally by the parties. Arbitration shall be the exclusive remedy for the settlement of disputes arising under this Agreement. The decision of the arbitrator(s) shall be final, conclusive, and binding, and no action at law or in equity may be instituted by either party other than to enforce the award of the arbitrator(s). Judgment upon the award rendered by the arbitrator(s) may be entered in any court of competent jurisdiction. The Agreement will remain in full force and effect during any such period of arbitration unless otherwise terminated pursuant to the terms of this Agreement.

iv. If Provider fails to request a First Level IDR Review, Second Level IDR Review, or Arbitration of Provider's payment dispute within the applicable timeframes, Evernorth's last determination regarding the dispute shall be binding on Provider. Provider shall not bill the Participant for payments that are denied on the basis that Provider failed to submit the request for review or arbitration within the required timeframes.

(9) For amendments other than those required by changes to state or federal laws or regulations, Evernorth shall provide Group with thirty (30) days advance written notice of the amendment.

(10) Specific Pennsylvania requirements relating to Provider's agreements with each of its providers are found in Section 9.725 of Title 28 of the Pennsylvania Code.

(11) With respect to quality management:

(A) Evernorth shall evaluate its quality management program annually and include a report to the board of directors, or the quality assurance or quality improvement committee, which addresses the appropriateness of clinical criteria; the consistency of decision making through the conduct of reliability studies of staff application of utilization criteria; staff

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resources and training; and the timeliness of decisions.

(12) With respect to the credentialing process:

(A) At a minimum, for primary care physicians ("PCPs") and specialists, the following credentialing elements shall be verified:

a. Current licensure
b. Education and training
c. Board certification status
d. Drug enforcement administration certification status
e. Current and adequate malpractice coverage
f. Malpractice claims history
g. Work history
h. Hospital privileges if the provider provides services at hospitals
i. Any other information the Department of Health may require

(B) At a minimum, for non-PCPs and non-specialists, current licensure and malpractice coverage shall be verified, to the extent licensure and coverage is required by Pennsylvania or federal law.

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RHODE ISLAND REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Rhode Island regarding provider contracts with providers rendering health care services in the State of Rhode Island. To the extent that such Rhode Island laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Rhode Island laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

I

(1) The definition for Emergency Services, if any, shall comply with Rhode Island laws and regulations to the extent applicable. Emergent life threatening cases must be seen immediately, 24 hours a day 7 days a week.

(2) Urgent Care shall have the same meaning as the term "Urgent Health Care Services" contained in the rules and regulations promulgated pursuant to Chapter 12.3 of Title 42, as may be amended from time to time, and shall include those resources necessary to treat a symptomatic medical, mental health or substance abuse or other health care condition requiring treatment within a 24 hour period of the onset of such a condition in order that the patient's health status not decline as a consequence. This does not include those conditions considered to require Emergency Services.

(3) The following item is added to the list of for cause reasons for which Evernorth may terminate the Agreement:

Lack of need by Evernorth due to economic considerations.

(4) In the event of termination of the Agreement by either party, the written notice of termination must include the reason(s) for such termination.

(5) (A) Right to a Hearing. If Evernorth proposes to terminate the Agreement, Evernorth will notify Provider of this decision in writing including the reason(s) for the proposed termination and a notice of Provider's right to request a hearing or review. The proposed termination will not be effective until the appeal process has been completed. The above rights regarding notice and hearing may be waived, in writing, by Provider. Evernorth shall not require Provider to waive notice and hearing rights as a condition of the Agreement.

(B) Immediate Harm to Participant's Health or Safety. When Evernorth has reason to suspect there is immediate danger to Participants as a result of conduct by Provider, Evernorth shall notify the Director of Health of the State of Rhode Island Department of Health immediately and shall take appropriate action to protect Participants.

(C) Notice to Participants. In the event the Agreement is terminated by Provider, Provider shall

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give reasonable advance notice of such termination to those Participants whom Provider is currently treating and who are affected by the termination.

(6) In addition to the terms of the Agreement governing Provider responsibilities regarding referrals, Participants shall be held harmless from any liability attributable to the failure of a referring provider to adhere to Evernorth's referral process including by failing to submit any required referral documents to Evernorth when there is evidence that the Participant sought and received a referral from Provider.

(7) In addition to the conditions and requirements of the Agreement governing limitations on billing Participants, in no event shall Provider bill, charge, collect a deposit from, or seek compensation, remuneration or reimbursement from Participant including but not limited to facility or administrative fees added to a Participant for Covered Services rendered by Provider.

(8) In the event of termination of the Agreement, Participant shall be held harmless for Covered Services except for Copayments, Coinsurance, and Deductibles for the duration of an active course of treatment, as defined by applicable laws or regulations, or up to one (1) year, whichever is earlier, subject to the terms and conditions of the Agreement, unless Provider is able to safely transition Participant to a network provider. For a period of active treatment, Participant shall only be responsible for in-network cost share under the Participant's Plan and not otherwise prohibited by applicable laws and regulations.

II

(1) Evernorth shall reimburse Provider for Covered Services rendered by Provider to Participants following the first day after Evernorth's acceptance of Provider's application for participation as a Participating Provider, provided that Provider is a health care provider as defined under applicable state laws and regulations, and returns a signed participating provider Agreement within fifteen (15) business days of receipt from Evernorth.

(2) Evernorth may amend the Agreement with Provider subject to the requirements of this section if Provider is a professional provider as defined under applicable state laws and regulations. These changes shall include, but not be limited to, effects upon utilization review and management activities or payment or coverage policies. Any such proposed contractual changes must include the following: (a) an explanation of the contractual changes, including the impact of the proposed changes in non-technical terms; and (b) notice must be sent to Provider in writing by mail.

Provider shall have an opportunity to amend or terminate the Agreement as a result of the proposed changes within sixty (60) calendar days of receipt of the notice of the changes. Any decision to terminate the Agreement by Provider shall be effective fifteen (15) calendar days from the mailing of the notice of the termination. Notice of termination by Provider must be made in writing by mail to Evernorth.

In the event that state or federal law or regulation, or an arbitration or judicial interpretation of same, should require that the terms, benefits and conditions of the Agreement must be changed accordingly, then upon notice from Evernorth, Provider shall continue to perform services under the Agreement as modified.

(3) Evernorth shall not require Provider, if Provider is a physician as defined under applicable state laws

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and regulations, to participate in any financial or reimbursement incentive program, commonly referred to as pay-for-performance programs, unless such program meets the principles and guidelines for pay-for-performance programs endorsed by the national quality forum and adopted by the AQA Alliance or the hospital quality alliance, or similar principles and guidelines for pay-for-performance programs approved by the Office of the Health Insurance Commissioner of the State of Rhode Island.

III

(1) Covered Practitioners in Rhode Island

a. Evernorth shall:

i. provide coverage for the services of a Certified Registered Nurse Practitioner, practicing collaboratively, or in the employ of a physician licensed and psychiatric and mental health nurse clinical specialists, to Participants, if the services are within the Certified Registered Nurse Practitioner's or psychiatric and mental health nurse clinical specialist's area of professional competence and are currently reimbursed when rendered by any other licensed health care provider.

ii. provide Participants with an opportunity to select a Certified Registered Nurse Practitioner, who is a participating provider, as a primary care provider to provide primary care, intermediate, home, long-term and inpatient care, when said Certified Registered Nurse Practitioner is a participating provider and practicing within the scope of his/her professional license.

iii. ensure that all participating primary care provider Certified Registered Nurse Practitioners are included on any publicly accessible list of participating providers for the respective organization.

IV

(1) Evernorth shall prohibit from engaging in retaliation or retribution, directly or indirectly, or terminating or modifying the terms of a provider contract because the physician or provider reports or is about to report, verbally or in writing, a violation by the insurer of a provider contract, law, rule or regulation to any of the following: a public body, a regulatory agency, an insured or member, the family or heirs or personal representatives of the insured or member, or to any other person or public or private agency. Any carrier that violates this law is subject to fines up to $1,000, imprisonment for up to one year, or both. A carrier that willfully or negligently violates this law may also be subject a civil penalty to a maximum of $5,000, imprisonment for up to one year, or both. A physician or other provider may bring a civil action for appropriate injunctive relief, actual and punitive damages and cost including reasonable attorney fees.

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SOUTH CAROLINA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as the "Provider") to comply with legislative and regulatory requirements of the State of South Carolina regarding provider contracts with providers rendering health care services in the State of South Carolina. To the extent that such South Carolina laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such South Carolina laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

1. With respect to claim payments made on or after June 11, 2009:

Evernorth may not initiate overpayment recovery efforts more than eighteen (18) months after the initial claim payment was received by Provider; however, this time limit does not apply to the initiation of overpayment recovery efforts:

(1) based upon a reasonable belief of fraud or other intentional misconduct;

(2) required by a self-insured plan; or

(3) required by a state or federal government program.

2. In addition to the requirements of the Agreement and to the extent required by S.C. Code § 38-71-243, upon termination of the Agreement and upon written attestation by the treating physician, on a form prescribed by the South Carolina Department of Insurance, that a health condition or illness exists that requires medical attention where failure to provide the current course of treatment through Provider would place a Participant's health in serious jeopardy, Provider shall, if requested by a Participant, continue to provide Covered Services for ninety (90) days or until the termination of the benefit period, whichever is greater. Provider shall accept the negotiated rate under the Agreement as payment in full for such services rendered. Except for applicable Deductible, Copayment or Coinsurance, Provider shall not bill or otherwise hold a Participant financially responsible for Covered Services rendered by Provider in the continuation of care, unless Provider has not received payment pursuant to the Agreement and in accordance with applicable law.

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SOUTH DAKOTA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of South Dakota regarding provider contracts with providers rendering health care services in the State of South Dakota. To the extent that such South Dakota laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such South Dakota laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

I

1. The definition for Emergency Services, if any, shall comply with South Dakota laws and regulations to the extent applicable.

2. In addition to the negotiated terms of the Agreement establishing obligations related to services after termination of the Agreement, the following shall apply in accordance with applicable laws and regulations:

   Evernorth shall, upon request from Provider who has chosen to leave the network or is terminated without cause or from a Participant, permit Participant to continue, upon agreement by Provider follow all applicable network requirements, an ongoing course of treatment from Provider for ninety (90) days following the effective date of the termination of the Agreement. Continuation of network coverage through Provider for Participant that has entered a second trimester of pregnancy at the time the Agreement is terminated shall extend to the provision of postpartum care directly related to the delivery.

3. In no event shall Provider collect or attempt to collect from a Participant any money owed to Provider by Evernorth, nor may Provider have any recourse against a Participant for any Covered Services charges in excess of the Copayment, Coinsurance, or Deductible amounts specified in the coverage, including Participants who have a health savings account.

4. Evernorth and Provider shall provide at least sixty (60) days written notice to each other before terminating the Agreement without cause. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement by Provider, such longer notification period will apply.

5. In addition to the negotiated terms of the Agreement establishing requirements for records:

   Provider shall make health records available to Evernorth upon request. Such requests shall be limited to those health records necessary to process claims, perform necessary quality assurance or quality improvement programs, or to comply with any lawful request for information from appropriate state authorities. Evernorth shall maintain the confidentiality of such records and may

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not make such records available to any other person who is not legally entitled to such records.

II

This section shall apply only if Provider is an intermediary as defined by applicable South Dakota laws and regulations including but not limited to SDCL § 58-17F-1(13), as may be amended from time to time.

1. Nothing in the Agreement shall be construed to delegate or assign to Provider Evernorth's statutory responsibility to monitor the offering of Covered Services to Participants.

2. In accordance with applicable laws and regulations, Evernorth shall have the right to approve or disapprove the participation status of a subcontracted provider in Evernorth's or Provider's contracted network for the purpose of delivering Covered Services to Evernorth's Participants.

3. In accordance with applicable laws and regulations, Evernorth shall have the right, in the event of Provider's insolvency, to require the assignment to Evernorth of the provisions of Provider's contract addressing Provider's obligation to furnish Covered Services.

4. In addition to the negotiated terms of the Agreement, Evernorth shall have access to all Provider's subcontracts, and shall have the right to make copies to facilitate regulatory review, upon twenty (20) days prior written notice from Evernorth.

5. In addition to the negotiated requirements of the Agreement, Provider shall maintain the books, records, financial information and documentation of services provided to Participants and shall preserve them for examination pursuant to applicable law.

6. Provider shall allow the director of the division of insurance access to Provider's books, records, financial information and any documentation of services provided to Participants, as necessary to determine compliance with applicable laws and regulations.

7. If applicable under the terms of the Agreement, Provider shall transmit utilization documentation and claims paid documentation to Evernorth as set forth in the Agreement. Evernorth shall monitor the timeliness and appropriateness of payments made to Provider and health care services received by Participants.

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TENNESSEE REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Tennessee regarding provider contracts with providers rendering health care services in the State of Tennessee. To the extent that such Tennessee laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Tennessee laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

1. The definition for Emergency Services, if any, shall comply with Tennessee laws and regulations to the extent applicable.

2. Based on guidance provided by the State of Tennessee, the duties of Clinics/PHO and its Represented Providers shall comply with Tennessee HMO laws and regulations to the extent applicable. Clinic/PHO represents and warrants that is employs and is authorized to act on behalf of its Represented Providers. Represented Providers shall comply with the terms and conditions of the Agreement to the extent applicable and shall look to the clinic/PHO for compensation for Covered Services.

3. (a) Within thirty (30) calendar days after Payor's receipt of Provider's claim, if submitted in paper form, Payor shall: (i) if the claim is a clean claim as defined below, pay for any fee-for-service amounts owing under the Agreement for such health care services provided; (ii) pay the portion of the claim that is clean and not in dispute and notify Provider in writing of the reason or reasons why the remaining portion of the claim will not be paid; or (iii) notify Provider in writing of all reasons why the claim is not a clean claim and will not be paid and what substantiating documentation and information is required to adjudicate the claim as a clean claim.

   (b) Within twenty-one (21) calendar days after Payor's receipt of an electronic submission of Provider's claim Payor shall: (i) if the claim is a clean claim as defined below, pay for any fee-for-service amounts owing under the Agreement for such health care services provided; (ii) pay the portion of the claim that is clean and not in dispute and notify Provider in writing of the reason or reasons why the remaining portion of the claim will not be paid; or (iii) notify Provider in writing of all reasons why the claim is not a clean claim and will not be paid and what substantiating documentation and information is required to adjudicate the claim as a clean claim.

   (c) If Payor fails to comply with the applicable requirements of subsection (a) or (b) above, Payor shall pay 1% interest per month, accruing from the day after the day payment was due, on that amount of the claim that remains unpaid.

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(d) As used herein clean claim means a claim received by Payor which requires no further information, adjustment or alteration by the provider of services in order to be processed and paid by Payor. A claim is clean if it has no defect or impropriety (including any lack of any required substantiating documentation) or particular circumstances requiring special treatment that prevents timely payment from being made on Provider's claim. A clean claim does not include a duplicate claim. A duplicate claim means an original claim and its duplicate when the duplicate is filed within thirty (30) days of the original claim. A clean claim does not include any claim submitted more than ninety (90) days after the date of service. The definition of clean claim includes resubmitted paper form claims with previously identified deficiencies corrected.

(e) Provider shall file a claim for reimbursement for a health care service within one hundred twenty (120) days of the date of service. Evernorth or Payor may deny the claim in full for a claim submitted more than one hundred twenty (120) days after the date of service.

4. Pursuant to the requirements of Tennessee Code Annotated Section 56-7-110:

(a) Payor shall not be required to correct a payment error to Provider, if Provider's request for a payment correction is filed more than fifteen (15) months after the date that Provider received payment for the claim from Payor.

(b) Except in cases of fraud or suspected fraud committed by Provider, Payor may only recoup reimbursements to Provider during the fifteen (15) month period after the date that Payor paid the claim submitted by Provider.

(c) Except in cases of fraud or suspected fraud, if Payor intends to recoup reimbursement to Provider under this section, Payor shall give Provider a written or electronic notice specifying the basis for the recoupment and the notice must contain, at a minimum, the information required by applicable state law. Should provider initiate an appeal within thirty (30) days of the date of notice of recoupment, then payment will not be withheld from provider unital all appeals are exhausted.

5. Pursuant to the requirements of Tennessee Code Annotated Section 56-7-110:

(a) Payor shall not be required to correct a payment error to Provider, if Provider's request for a payment correction is filed more than fifteen (15) months after the date that Provider received payment for the claim from Payor.

(b) Except in cases of fraud or suspected fraud committed by Provider, Payor may only recoup reimbursements to Provider during the fifteen (15) month period after the date that Payor paid the claim submitted by Provider.

(c) Except in cases of fraud or suspected fraud, if Payor intends to recoup reimbursement to Provider under this section, Payor shall give Provider a written or electronic notice specifying the basis for the recoupment and the notice must contain, at a minimum, the information required by applicable state law. Should Provider initiate an appeal within thirty (30) days of the date of a notice of recoupment, then payment will not be withheld from Provider until all appeals are exhausted.

6. If the Agreement is terminated by Provider or Evernorth without cause, then Provider and Evernorth shall allow a Participant who retains eligibility under a Plan and who is:

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(a) Under active treatment for a particular injury or sickness, to continue to receive Covered Services from Provider for such injury or sickness for a period of one hundred twenty (120) days from the date of notice of termination,

(b) In the second trimester of pregnancy to continue care with Provider until completion of postpartum care,

(c) Being treated at an inpatient facility to remain at the facility until Participant is discharged.

The terms, conditions and compensation arrangement of the Agreement shall apply during the period of continued care.

7. Any change to the fee schedule of the Agreement shall be made available to Provider at least ninety (90) days prior to the effective date of the amendment. However, this requirement shall not apply to changes in standard codes and guidelines developed by the American Medical Association or a similar organization.

8. If required by the Agreement, where precertification is not required for a hospital admission, Provider must notify Evernorth or its designee within twenty-four (24) hours after the admission; if such notification or admission occurs on a weekend or federal holiday, Provider will notify Evernorth or its designee within one (1) business day of the conclusion of the weekend or federal holiday.

9. The Agreement may permit network rental arrangements which allow the selling, renting, or otherwise grant access to Evernorth's rights to Provider's services to a third party provided that the third party accessing Provider's services is contractually obligated to comply with all applicable terms, limitations and conditions of the Agreement, and the third party is any of the following:

   (a) A payer, a third-party administrator, or another entity that administers or processes claims on behalf of the payer;

   (b) A preferred provider organization or preferred provider network, including a physician organization or physician hospital organization; or

   (c) An entity engaged in the electronic claims transport between the Evernorth and the payer that does not provide access to the provider's services and a discount to any other covered entity.

10. Unless otherwise set forth in the Agreement as permitted by TCA § 56-32-104(a)(3)(E)(iii), for agreements for the provision of healthcare services subject to TCA § 56-32-104(a)(e)(E), Provider shall look solely to Evernorth for payment for Covered Services except for Copayments, Coinsurance and Deductibles.

11. If Provider is a hospital, then the following provisions shall apply:

    (a) With respect to the process for the appointment of an arbitrator from the list of suggested arbitrators chosen by the American Arbitration Association (section 6.4 of the Agreement):

    If Provider wants to use three (3) arbitrators instead of one (1) and Provider agrees to bear the costs for such additional arbitrators, then the two (2) arbitrators with the next two (2) lowest total scores will also be appointed to help resolve the case. Each party shall assume its own costs, but

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the compensation and the expenses of one (1) arbitrator and any administrative fees or costs shall be borne equally by the parties.

(b) Evernorth will provide Provider with at least sixty (60) days advance notice of material changes made to the Medical Management Program.

(c) Provider may terminate the Agreement if Evernorth has materially changed the Medical Management Program and such change is unacceptable to Provider provided that Provider gives Evernorth notice of rejection of such change within sixty (60) days of receipt by Provider of Evernorth's notice concerning the change and Evernorth does not withdraw the change to the Medical Management Program or the parties do not reach an agreement with respect to a mutually acceptable change to the Medical Management Program within the timeframe set forth in the Agreement.

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TEXAS REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Texas regarding provider contracts with providers rendering health care services in the State of Texas. To the extent that such Texas laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Texas laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) The definition for Emergency Services, if any, shall comply with Texas laws and regulations to the extent applicable.

(2) Appointment Wait Time Standards

(A) Providers should ensure Participants have access to timely appointments and scheduling. The following guidelines represent both the Plan's and the state of Texas' requirements for behavioral health appointment accessibility
(1) Routine care must be available within two weeks of the appointment request.
(2) Urgent care must be available within 24 hours of the appointment request
(3) Emergency care must be available and accessible 24 hours a day, 7 days a week.

(3) To the extent required by applicable Texas statutes and rules pertaining to the prompt payment of Clean Claims, the following provisions shall apply:

(A) Effect of Filing a Clean Claim.

(1) The Statutory Claims Payment Period begins to run upon receipt by Evernorth of a Clean Claim from Provider as determined under Texas law. The date of claim payment is as determined under Texas law.

(2) After the receipt of a Clean Claim from Provider at the address designated by Evernorth and prior to the expiration of the applicable Statutory Claims Payment Period (subject to any extensions of time permitted under Texas law):

(a) Payor shall pay the total amount of the Clean Claim in accordance with the terms of the Agreement;

(b) The Clean Claim shall be denied in its entirety after a determination that Payor is not liable for the Clean Claim and
Provider shall be notified in writing why the Clean Claim will not be paid;

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(c) Provider shall be notified in writing that the entire Clean Claim will be audited and Payor shall pay 100% of the Contracted Rate on the claim to Provider; or

(d) Payor shall pay the portion of the Clean Claim for which liability is acknowledged in accordance with the terms of the Agreement, and;

(i) the remainder of the Clean Claim shall be denied after a determination that Payor is not liable for the remainder of the Clean Claim and Provider shall be notified in writing why the remainder of the Clean Claim will not be paid; or

(ii) Provider shall be notified in writing that the remainder of the Clean Claim will be audited and Payor shall pay 100% of the Contracted Rate on the unpaid portion of the Clean Claim to Provider.

(3) Requests for Additional Information From Treating Provider. If necessary to determine whether a claim is payable, Evernorth may, within thirty (30) days of receipt of a Clean Claim, request additional information from the treating provider. The time period to request additional information may be extended as allowed under Texas law. In the event that Evernorth requests information under this section, Evernorth shall determine whether the Clean Claim is payable and Payor shall pay or Evernorth will deny the Clean Claim or audit the Clean Claim on or before the later of:

(a) the 15th day after the date Evernorth receives the requested information from the treating provider along with a copy of Evernorth's written request for information or with the name of the patient, patient identification number, the claim number as provided by Evernorth, the date of service and the name of the treating provider (If Evernorth submitted the request for additional information electronically in accordance with federal requirements concerning electronic transactions, the treating provider must submit the response in accordance with those requirements);

(b) the 15th day after the date Evernorth receives a written response from the treating provider that the treating provider does not possess the requested information; or

(c) the latest date for determining whether the claim is payable under subsections (1) and (2) above.

(4) Requests for Additional Information From Other Sources. If Evernorth requests additional information from a person other than Provider, Evernorth will provide Provider with a notice containing the name of the physician, provider or other entity from whom Evernorth is requesting information. Payor may not withhold payment beyond the applicable Statutory Claims Payment Period pending receipt of information under this section. If on receiving information requested under this section Evernorth determines that there was an error in payment of the claim, the overpayment may be recovered pursuant to section F. below.

(5) To the extent applicable, Evernorth will not refuse to process or pay an electronically submitted Clean Claim because the claim is submitted with or in a batch submission with a Clean Claim that is deficient. A "batch submission" is a group of electronic claims submitted for processing at the same time within HIPAA standard ASC X12N 837 Transaction Set and identified by a batch control number.

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(B) Effect of Filing a Deficient Claim.

If a submitted claim is determined by Evernorth to be deficient, Provider shall be notified that the claim is deficient within forty-five (45) calendar days of Evernorth's receipt of the claim at the address designated by Evernorth or within thirty (30) days of receipt by Evernorth of an electronic claim. If the deficient claim is a claim for a prescription benefit, Evernorth will notify Provider that the claim is deficient within twenty-one (21) calendar days of receipt of the nonelectronic claim by Evernorth, or within eighteen (18) days of receipt of an electronic claim. The failure to notify Provider that a claim is deficient within the timelines specified in this section shall not render a deficient claim a Clean Claim.

(C) Audit Procedures.

If Payor is unable to pay or deny a Clean Claim, in whole or in part, within the applicable Statutory Claims Payment period and intends to audit the Clean Claim to determine whether it is payable, Evernorth will notify Provider that the claim is being audited and Payor shall pay 100% of the Contracted Rate within the applicable Statutory Claims Payment Period. Payment of 100% of the Contracted Rate is not an admission that liability is acknowledged on that claim. Evernorth will complete the audit within one hundred eighty (180) calendar days from receipt of the Clean Claim. Upon completion of any audit of a Clean Claim, Evernorth will notify Provider of the results of the audit and:

(1) If Evernorth determines that additional payment is due to Provider, such additional payment shall be paid by Payor within thirty (30) calendar days after the completion of the audit;

(2) If Evernorth determines that a refund is due from Provider, such refund shall be made by Provider within thirty (30) calendar days of the later of notification to Provider of the results of the audit or exhaustion of any Participant appeal rights, if a Participant appeal is filed before the thirty (30) calendar day refund period has expired, and may be made by any method, including chargeback against Provider.

(D) Failure to Meet Statutory Claims Payment Period.

(1) If Evernorth determines that a Clean Claim is payable and Payor fails to pay any amount due and owing on the Clean Claim within the statutory time frames, Payor shall pay to Provider, in addition to the Contracted Rate owed, a penalty as follows:

(a) if the claim is paid on or before the 45th day after the end of the applicable Statutory Claims Payment Period, the lesser of:

(i) 50% of the difference between the Billed Charge and the Contracted Rate:
or

(ii) $100,000.

(b) if the claim is paid on or after the 46th day and before the 91st day after the end of the applicable Statutory Claims Payment Period, the lesser of:

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(i) 100% of the difference between the Billed Charge and the Contracted Rate: or

(ii) $200,000.

(c) if the claim is paid on or after the 91st day after the end of the applicable Statutory Claims Payment Period, a penalty computed under subsection (1)(b) above plus 18% annual interest on the penalty amount. Interest under this subsection accrues beginning on the date the claim was required to be paid and ending on the date the claim and the penalty are paid in full.

(d) Notwithstanding any other provision of section (2), this subsection governs the payment of a penalty under section (2)(D).

(i) For a penalty relating to a clean claim submitted by a physician or other provider other than an institutional provider, Evernorth shall pay the entire penalty to the physician or provider, except any interest computed under subsection (2)(D)(1)(c) above which Evernorth shall pay to the Texas Department of Insurance or as otherwise required by applicable laws or regulations.
(ii) For a penalty relating to a clean claim submitted by an institutional provider, Evernorth shall pay 50 percent of the total penalty amount computed under section (2)(D) above, including interest, to the institutional provider and the remaining 50 percent of that amount to the Texas Department of Insurance or as otherwise required by applicable laws or regulations. Institutional Provider means a hospital or other medical or health-related service facility that provides car for the sick or injured or other care that may be covered in an evidence of coverage.

(2) If Evernorth determines that a Clean Claim is payable and Payor pays only a portion of the amount of the Clean Claim on or before the applicable Statutory Claims Payment Period and pays the balance of the Contracted Rate owed for the Clean Claim after that date, Payor shall pay to Provider, in addition to the Contracted Rate owed, a penalty as follows:

(a) if the balance of the Clean Claim is paid on or before the 45th day after the applicable Statutory Claims Payment Period, the lesser of:

(i) 50% of the underpaid amount; or
(ii) $100,000.

(b) if the balance of the Clean Claim is paid on or after the 46th day and before the 91st day after the end of the applicable Statutory Claims Payment Period, the lesser of:

(i) 100% of the underpaid amount; or (ii)

$200,000.

(c) if the balance of the Clean Claim is paid on or after the 91st date after the end of the applicable Statutory Claims Payment Period, a penalty computed under subsection (2)(b) above plus 18% annual interest on the penalty amount. Interest under this subsection accrues beginning on the date the claim was required to be paid and ending on the date the claim and the penalty are paid in full.

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(d) For purposes of this subsection 2, the underpaid amount is calculated on the ratio of the amount underpaid on the Contracted Rate as applied to an amount equal to the Billed Charge submitted minus the Contracted Rate.

(3) No penalty shall be owed:

(a) if the failure to pay the claim in accordance with the applicable Statutory Claims Payment Period is a result of a catastrophic event that Evernorth certified in accordance with Texas law; or

(b) if the claim was paid in accordance with statutory time frames but for less than the Contracted Rate, and

(i) Provider notifies Evernorth of the underpayment after the 270th day after the date the underpayment was received; and

(ii) Payor pays the balance of the claim on or before the 30th day after the date Evernorth received notice of the underpayment.

(4) Subsection 3 above does not relieve Payor of any obligation to pay the remaining unpaid Contracted Rate owed.

(E) Claims Filing Deadline.

Provider must submit a claim to Evernorth not later than the 95th day after the date of discharge with regard to the medical care or health care services for which the claim is made. For a claim for which coordination of benefits applies, the 95 day period does not begin for submission of the claim to the secondary payor until Provider receives notice of the payment or denial from the primary payor. If Provider fails to submit a claim in compliance with this section, Provider forfeits the right to payment unless Provider has certified in accordance with Texas law that the failure to timely submit is a result of a catastrophic event. The date of receipt of a claim and whether the method of submission of a claim is appropriate shall be determined in accordance with Texas law. Provider may not submit a duplicate claim prior to the date that the applicable Statutory Claims Payment Period has passed. If Evernorth receives a duplicate claim prior to such date, such claim shall not be subject to the requirements set forth above relating to the effect of filing a Clean Claim and failure to meet the Statutory Claims Payment Period.

(F) Overpayment of Claims.

(1) A refund due to overpayment or completion of audit may be recovered if:

(a) Evernorth notifies Provider of the overpayment not later than the 180th day after the date of receipt of the overpayment; or

(b) Evernorth notifies Provider of the completion of an audit in accordance with section (C) above.

(2) Notification under this provision shall:

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(a) be in written form and include the specific claims and amounts for which a refund is due and for each claim the basis and specific reasons for the request for refund;

(b) include notice of the Provider's right to appeal; and

(c) describe the methods by which Evernorth intends to recover the refund.

(3) Provider may appeal a request for refund by providing written notice of disagreement with the refund request not later than forty-five (45) days after receipt of notice under subsection (2) above. Upon receipt of a written notice under this subsection, Evernorth shall begin Evernorth's internal appeal process as provided in the Medical Management Program to the Agreement.

(4) A refund may not be recovered under this section until:

(a) for overpayments, the later of the 45th day after notification under subsection (1) (a) of this section or the exhaustion of any Provider appeal rights under subsection (3) of this section where Provider has not made arrangements for payment with Evernorth; or

(b) for audits, the later of the 30th day after notification under subsection (1) (b) of this section or the exhaustion of any Provider appeal rights under subsection (3) of this section where the Provider has not made arrangements for payment with Evernorth.

(5) If Payor is a secondary payor and pays a portion of a claim that should have been paid by the primary payor and that was paid to Provider by the primary payor, Payor may recover the amount of overpayment from Provider pursuant to this section (F).

(6) This section (F) does not affect Payor's ability to recover any overpayment in the case of fraud or a material misrepresentation by Provider.

(G) Terms.

The terms Clean Claim, Statutory Claims Payment Period, Billed Charge and Contracted Rate shall have the same meaning as defined under applicable Texas law.

(4) Evernorth's claims submission processes are set forth in Evernorth's Medical Management Program, as amended from time to time.

(5) Upon request and to the extent required by Texas law, Evernorth will provide Provider with the information necessary to determine that Provider is being compensated in accordance with the Agreement.

(6) If Provider is compensated on a discounted fee basis, the Participant's financial obligation for Deductibles or Coinsurance shall be determined based upon the discounted fee and not upon the full billed charge.

(7) If Provider is not a hospital or ancillary provider, the following shall apply:

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(A) If Represented Provider is a primary care physician and is reimbursed on a capitated basis, Payor shall begin payment of capitated amounts calculated from the date of a Participant's enrollment no later than the 60th day following the date the Participant has selected or has been assigned to the primary care physician. If selection or assignment does not occur at the time of enrollment, capitation which would otherwise have been paid to a selected primary care physician had a selection been made shall be reserved as a capitation payable until such time as Participant makes a selection or Evernorth assigns a primary care physician.

(B) If a Participant does not select a primary care physician at the time of application or enrollment, Evernorth may assign the Participant to a primary care physician. If Evernorth elects to assign a Participant to a primary care physician, the assignment shall be made to a primary care physician located within the zip code nearest the Participant's residence or place of employment and, to the extent practicable given the zip code limitation, shall be done in a manner that results in a fair and equal distribution of Participants among Evernorth's primary care groups in the applicable Evernorth network. Evernorth shall inform a Participant of the name, address and telephone number of the primary care physician to whom the Participant has been assigned and of the Participant's right to select a different primary care physician. A Participant shall have the right at any time to reject the primary care physician assigned and to select another physician from the list of primary care groups for the applicable Evernorth network. An election by a Participant to reject an assigned physician shall not be counted as a change in providers under the Texas Insurance Code. Evernorth shall notify Provider or the primary care physician of the selection of such primary care physician by a Participant within thirty (30) working days of the selection or assignment of the Participant to a primary care physician by Evernorth.

(8) Provider acknowledges and agrees that the Agreement does not contain any financial incentive or make any payment that acts directly or indirectly as an inducement to limit Medically Necessary services. This provision shall not prohibit the savings from cost effective utilization of health services by contracting physicians or health care providers from being shared with physicians or health care providers in the aggregate.

(9) Provider shall post a notice to Participants at the Provider's location on the process for resolving complaints with Evernorth. The notice must include the Texas Department of Insurance's toll-free telephone number for filing complaints.

(10) Evernorth shall not engage in any retaliatory action, including termination of or refusal to renew the Agreement, because Provider, on behalf of a Participant, reasonably filed a complaint against Evernorth or has appealed a decision of Evernorth.

(11) (A) Evernorth will not as a condition of the Agreement or in any other manner prohibit, attempt to prohibit or discourage Provider from discussing with or communicating in good faith to a Participant who is a current, prospective or former patient or a party designated by such Participant, with respect to: a)
information or opinions regarding the Participant's health care including the Participant's medical condition or treatment options; b) information or opinions regarding the provisions, terms, requirements or services of the Participant's health benefit plan as they relate to the medical needs of the Participant; c) the fact that a Provider's contract with Evernorth has terminated or that a Provider will otherwise no longer be providing care for Evernorth Participants; or d) the fact that, if Medically Necessary Covered Services are not available through Participating Providers, Evernorth must, upon the request of the Provider, and within

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time appropriate to the circumstances relating to the delivery of the services and the condition of the Participant, but in no event to exceed five (5) business days after receipt of reasonably requested documentation, allow referral to a non-Participating Provider.

(B) Evernorth will not in any way penalize or terminate Provider or refuse to compensate Provider for Covered Services for communicating with a Participant who is a current, prospective or former patient, or a party designated by Participant, in any manner protected by this provision.

(12) (A) If Evernorth terminates the Agreement, Evernorth shall give Provider not less than ninety (90) days' prior written notice of the termination, except in the case of imminent harm to patient health, action against license to practice, or fraud, in which case termination may be immediate. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement by Evernorth, such longer notification period will apply.

(B) Notice and Hearing. If Evernorth should choose to terminate the Agreement, Evernorth will notify Provider of this decision in writing and the reason(s) for termination. In the case of termination of a Provider the notice will include the right to request a hearing or review as specified in applicable laws and regulations. On request and before the effective date of the termination, but within a period not to exceed sixty (60) days, Provider shall be entitled to a review of the proposed termination by an advisory review panel. The requirements regarding notice and hearing shall not apply in the case of imminent harm to patient health, action against license to practice, or fraud.

(C)
The decision of the advisory review panel must be considered but is not binding on Evernorth. On request, a copy of the recommendation of the advisory review panel and Evernorth's determination shall be given to Provider.

(D) (1) In the event the Agreement is terminated by Provider, Evernorth, in accordance with Insurance Code § 843.309, shall give reasonable advance notice of such termination to those Participants whom Provider is currently treating and who are affected by the termination.

(2) Evernorth will provide assistance to Provider in the event the Agreement is voluntarily terminated by Provider, and Provider, in accordance with Insurance Code § 1301.160, must give reasonable advance notice of such termination to those Participants whom Provider is currently treating and who are affected by the termination.

(3) In the event the Agreement is terminated by Evernorth, Evernorth will notify those Participants whom Provider is currently treating and are affected by the termination. Evernorth may notify such Participants of Provider's termination immediately if the Agreement is terminated for reasons related to imminent harm.

(E) (1) If the Agreement is terminated for reasons other than medical competence or professional conduct, Provider shall continue to provide Covered Services for those Participants who retain eligibility under a Plan and whom 1) Provider has identified to Evernorth as having special circumstances (i.e. persons with a disability, acute condition, life-threatening illness, past the 24th week of pregnancy or a condition such that Provider reasonably believes that discontinuing care could cause harm to the Participant); and 2) Provider has requested to continue treatment. Provider shall be compensated for Covered Services provided pursuant to this provision in accordance with the compensation arrangements under the Agreement for a period of nine (9) months for those Participants diagnosed with a terminal illness at the time of termination of the

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Agreement, through delivery, immediate post- partum care and the follow-up checkup within the first six (6) weeks of delivery for Participants past the 24th week of pregnancy at the time of termination, and for a period of ninety (90) days following termination for all others.

(2) Provider shall not seek payment from the Participant with respect to services rendered pursuant to this provision of amounts for which the Participant would not be responsible if Provider were still a Participating Provider.

(13) Nothing in the Agreement shall be construed to require Provider to indemnify Evernorth for any tort liability resulting from acts or omissions of Evernorth.

(14) Provider shall hold Participants harmless for payment of the cost of Covered Services in the event Payor fails to pay Provider for such Covered Services.

(15) Nothing in the Agreement shall be construed to require a referring Provider to bear the expenses of a referral for specialty care in or out of Evernorth's provider panel. Savings from cost-effective utilization of health services may, however, be shared with physician and health care providers in the aggregate.

(16) To the extent that Evernorth conducts, uses or relies upon economic profiling to terminate the Agreement, Evernorth shall make available to Provider on request the economic profile of Provider, including the written criteria by which Provider's performance was measured. An economic profile will be adjusted to recognize the characteristics of Provider's practice that may account for variations from expected costs.

(17) Quality assessment (as that term is defined under Texas law) shall be conducted through a panel of not less than three (3) physicians selected by Evernorth from among a list of participating physicians which list is to be provided by participating physicians in the applicable service area.

(18) Provider may be required pursuant to procedures contained in the Medical Management Program, when referring a Participant to another provider, to disclose to Participant that the physician, provider or facility to whom Participant is referred might not be a Participating Provider; and, if applicable, that the referring Provider has an ownership interest in the facility to which Participant is referred. Such disclosure shall not be required when referring for emergency care, and as necessary to avoid interruption or delay of Medically Necessary care. Nothing in this section or in the Medical Management Program shall be construed to limit access to non- Participating Providers.

(19) Provider shall, except for instances of emergency care as defined under state law, when referring a Participant to a facility for surgery: notify Participant that out-of-network providers may provide treatment and that Participant can contact Evernorth for more information; notify Evernorth that surgery has been recommended; and, notify Evernorth of the facility that has been recommended for the surgery.

(20) Provider shall, except for instances of emergency care as defined under state law, when scheduling a Participant for surgery: notify Participant that out-of- network providers may provide treatment and that Participant can contact Evernorth for more information; and, notify Evernorth that surgery has been scheduled.

(21) Provider shall comply with all applicable requirements of Insurance Code § 1661.005. Provider must

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refund the amount of an overpayment to a Participant no later than the 30th day after the date Provider determines that an overpayment has been made.

(22) If Provider is a hospital, Provider shall, in accordance with the notice requirements of the Agreement, give notice as soon as reasonably practicable but not later than the 5th business day following the termination of a contract between Provider and a hospital based physician group that is an Evernorth Participating Provider.

(23) To the extent applicable, Provider may request, pursuant to procedures contained in the Medical Management Program, a waiver of any requirement for the use of information technology established or required by Chapter 1661 of the Texas Insurance Code as may be changed from time to time. A waiver granted under this section will expire September 1, 2013, or as otherwise permitted by applicable laws and regulations. If Provider has fewer than five full-time- equivalent employees, Provider is not required, pursuant to Texas Insurance Code § 1661.0055, to use information technology as required by Chapter 1661 of the Texas Insurance Code.

(24) To the extent applicable, Provider may request, pursuant to procedures contained in the Medical Management Program, a waiver of any requirement for electronic submission established or required by Chapter 1213 of the Texas Insurance Code as may be changed from time to time.

(25) To the extent applicable, nothing in this Agreement shall be construed to permit Evernorth or Payor to directly or indirectly charge or hold Provider responsible for a fee for the adjudication of a claim.

(26) The Agreement permits Evernorth to contract with another party to provide access to Evernorth's rights and responsibilities under this Agreement. Upon request, Evernorth will provide information necessary to determine whether a particular party has been authorized to access Provider's health care services and contractual discounts under this Agreement. Any party authorized to access the health care services and contractual discounts under this Agreement must comply with all applicable terms, limitations, and conditions of the Agreement.

(27) The fee schedule for each type of Plan is included in the exhibits to this Agreement. Notwithstanding the foregoing, Evernorth may at its option provide the fee schedule for any Plan to Provider electronically.

(28) Upon the request of Provider, Evernorth will provide Provider with such information as is necessary to allow Provider to determine that a Payor is authorized to access the reimbursement rates under this Agreement.

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U.S. VIRGIN ISLANDS REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the U.S. Virgin Islands regarding provider contracts with providers rendering health care services in the U.S. Virgin Islands. To the extent that such laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

Participant Billing and Liability Collection Guidelines:

a. Copayments: Copayment Plans require Participants to pay a fixed dollar amount (Copayment) per service. Copayment amounts are indicated on the patient's Evernorth ID card. Provider shall collect applicable Copayment amounts indicated on the ID card at the time of service. Deductibles may apply to these types of Plans.
b. Coinsurance & Deductibles:
    i. For Participants with Plans that have Deductibles or require Participants to pay a percentage of the covered charges (Coinsurance) after satisfying any Deductible amount, Provider shall submit claims to Evernorth or its designee and receive an explanation of payment (EOP) indicating the Participants' responsibility before billing patients. Coinsurance and Deductibles may also be collected at the time of service. Accessing the Evernorth Cost of Care Estimator described below will give Provider an estimate of the Deductible and Coinsurance obligations of the Participant and Provider may print a copy of the estimate to give to the Participant at the time of service.
    ii. The Evernorth Cost of Care Estimator tool, available on the Evernorth for Health Care Professionals website (**Provider.Evernorth.com**) helps eliminate financial surprises by estimating the cost of the medical service, highlighting the Participant's anticipated payment responsibility, and providing Provider and Provider's patients with an itemized, printable Explanation of Estimate. It is fast to use, easy for Provider's patients to understand, and can be used anytime during Provider's patient's visit: prior to care, at check in, or at checkout.
    iii. By entering the CPT code(s) or identifying information about the procedure along with the Plan Participant's Evernorth identification number and date of birth, Provider will receive a personalized Explanation of Estimate that contains the following information:
        1. Total cost of the service
        2. Plan Participant's Deductible/Coinsurance/Copayment responsibility
        3. Plan Participant's anticipated payment from their health account (HSA, HRA, FSA) when automatic claim forwarding is enabled
        4. Plan Participant's estimated amount owed out of pocket
    iv. The Estimator is available to participating providers in the Evernorth network. To use it, Provider may log in to **Provider.Evernorth.com** > Patients > Search Patients > Select a Patient > Estimate Costs.
    v. The estimate Provider receives represents Provider's patient's anticipated out-of-pocket expense if the services billed are covered under their medical Plan. It does not guarantee coverage or payment, but allows Provider to have a financial discussion with Provider's

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patient and set realistic financial obligations for them.

vi. Provider cannot bill Participants for Covered Services or for services for which payment was denied due to Provider's failure to comply with the Agreement or Medical Management Program, including Evernorth utilization management requirements and timely filing requirements.

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UTAH REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Utah regarding provider contracts with providers rendering health care services in the State of Utah. To the extent that such Utah laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Utah laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) The definition for Emergency Services, if any, shall comply with Utah laws and regulations to the extent applicable.

(2) In accordance with Utah state regulation, UCA 31A-26-301.6:

- Except in cases of fraud, Evernorth and Provider shall request payment adjustments for incorrectly processed claims (overpayments or underpayments) within the following timeframes: (1) within twenty-four (24) months of the amount improperly paid for a coordination of benefits error; or (2) within twelve (12) months of the amount improperly paid for any other reason; or (3) within thirty-six (36) months of the amount improperly paid when the improper payment was due to a recovery by Medicaid, Medicare, the Children's Health Insurance Program, or any other state or federal health care program.
- Provider should submit requests for review of incorrectly processed claims as per the Dispute Resolution guidelines outlined in the Agreement.

(3) (a) Upon Evernorth's insolvency, the rehabilitator or liquidator may require Provider to continue to provide Covered Services under the Agreement between Provider and Evernorth until the earlier of: (1) ninety (90) days after the date of the filing of a petition for rehabilitation or the petition for liquidation; or (2) the date the term of the Agreement ends.

(b) The rehabilitator or liquidator may reduce the fees Provider is otherwise entitled to receive under the Agreement during the time period described above. Provider shall accept the reduced payment as payment in full and relinquish the right to collect additional amounts from the Participant. However, the rehabilitator or liquidator may not reduce a fee to less than seventy-five percent (75%) of the regular fee set forth in the Agreement and the Participant shall continue to pay the same Copayments, Deductibles, and other payments for services received from Provider that the Participant was required to pay before the filing of the petition for reorganization or petition for liquidation.

(4) Nothing in the Agreement shall be construed to require Provider to notify Evernorth of a hospital inpatient emergency admission within a period of time that is less than one (1) business day of the hospital inpatient admission, if compliance with the notification requirement would result in notification by Provider on a weekend or a federal holiday.

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VERMONT REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as the "Provider") to comply with legislative and regulatory requirements of the State of Vermont regarding provider contracts with providers rendering health care services in the State of Vermont. To the extent that such Vermont laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all provider governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Vermont laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) The definition for Emergency Services, if any, shall comply with Vermont laws and regulations to the extent applicable.

(1.1) "Material Change" or "Material Adverse Change" shall mean a change that could reasonably be expected to have a material adverse impact on the aggregate level of payment by Evernorth or Payor to Provider for Covered Services under this Agreement, or on Provider's administration of their services.

(1.2) "Timely Notice" shall mean the timeframe or timeframes established by the parties for prior written notice of an amendment to the Agreement as set forth in the Amendment section, or any other provisions of the Agreement governing changes or amendments to the Agreement.

(2) Evernorth shall not prohibit Provider from, or penalize Provider for discussing treatment options with Participants regardless of Evernorth's position on the treatment options, or advocating on behalf of Participants within the utilization review or grievance process established by Evernorth, nor shall it penalize Provider because Provider in good faith reports to state or federal authorities any act or practice by Evernorth that jeopardizes Participant health or welfare.

(3) Provider will comply with the Medical Management Program including but not limited to the rules, policies and procedures established by Evernorth and required by state laws and regulations, including but not limited to the Consumer Protection and Quality Requirements for Managed Care Plans, as may be amended from time to time. Provider will comply with the Medical Management Program governing grievance procedures and the credentialing process. Provider may give feedback to Evernorth, on an ongoing basis, for Evernorth's use in assessing and enhancing Evernorth's quality management program, utilization management program, Participant appeal procedures, and dispute resolution process. In addition, Provider will be invited to give input annually in the form of a written survey. For Administrative Guidelines subject to 18 VSA § 9418c(5), Evernorth shall provide not fewer than 60 days notice for any new guideline or for any change to an existing guideline, as specified in applicable law.

(4) A. Payor shall not retrospectively deny a previously paid claim or any part of a previously paid claim, unless:

(a) Payor has provided at least thirty (30) days' notice of any retrospective denial or overpayment recovery or both in writing to Provider. The notice must include:

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(i) the patient's name;
(ii) the service date;
(iii) the payment amount;
(iv) the proposed adjustment; and
(v) a reasonably specific explanation of the proposed adjustment.

(b) the time that has elapsed since the date of payment of the previously paid claim does not exceed twelve (12) months.

B. The retrospective denial of a previously paid claim shall be permitted beyond twelve (12) months from the date of payment for any of the following reasons:

(a) Evernorth has a reasonable belief that fraud or other intentional misconduct has occurred;
(b) the claim payment was incorrect because Provider was already paid for the health services identified in the claim;
(c) the health care services identified in the claim were not delivered by Provider;
(d) the claim payment is the subject of adjustment with another health plan; or
(e) the claim is the subject of legal action.

C. For routine recoveries as described below, retrospective denials or overpayment recovery of any or all a previously paid claim shall not require thirty (30) days' notice before recovery may be made. A recovery shall be considered routine only if one of the following situations applies:

(a) duplicate payment to Provider for the same service;
(b) payment with respect to an individual who was not a Plan Participant as of the date the service was provided;
(c) payment for a noncovered, not to include services denied as not Medically Necessary, experimental, or investigational in nature, or services denied through a utilization review mechanism;
(d) erroneous payment for services due to a plan administrator error;
(e) erroneous payment for services where the claim was processed in a manner inconsistent with the data submitted by Provider;
(f) payment where Provider provides Evernorth with new or additional information demonstrating an overpayment;
(g) payment to Provider at an incorrect rate or using an incorrect fee schedule;
(h) payment of claims for the same Plan Participant that are received by Evernorth out of the chronological order in which services were performed;
(i) payment where Provider has received payment for the same services from another payer whose obligation is primary; or
(j) payments made in coordination with a payment by a government payer that require adjustment based on an adjustment in the government-paid portion of the claim.

D. Recoveries which, in Evernorth's reasonable judgment, would be likely to affect a significant volume of claims or accumulate to a significant dollar amount shall not be deemed routine, regardless of whether one or more situations above apply.

(5) Pursuant to Vermont laws, Evernorth shall, to the extent applicable, be bound by and comply with the Consumer Protection and Quality Requirements for Managed Care Plans, as may be amended from time to time.

(6) A. In the event of Evernorth's insolvency or other cessation of operations, Covered Services to Participants shall continue through the contract period for which premiums have been paid

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on behalf of the Participant or until the Participant's discharge from an inpatient facility, whichever period is greater. Covered Services to Participants confined in an inpatient facility on the date of insolvency or other cessation of operations will continue until the Participant's continued confinement in the facility is no longer Medically Necessary. This provision shall be construed in favor of the Participant, shall survive the termination of the Agreement regardless of the reason for termination, including the insolvency of Evernorth, and shall supersede any oral or written contrary agreement between Provider and Participant or Participant's representative.

B. Upon termination of the Agreement without cause, Provider shall continue to provide Covered Services for specific conditions for which a Participant was under Provider's care at the time of such termination as follows: (a) Participants with life-threatening, disabling or degenerative conditions shall be allowed to continue undergoing a course of treatment for sixty (60) days from the date of termination or until Evernorth's provision for the assumption of such treatment by another provider, whichever is shorter; and (b) Participants who are in the second or third trimester of a pregnancy shall be permitted to continue to receive Medically Necessary Covered Services from Provider until the completion of postpartum care. The terms and conditions of the Agreement shall continue to apply. Provider shall be compensated for such continued care in accordance with the compensation arrangements that were in effect under the Agreement prior to termination. Participants shall not be liable to Provider for any amounts owed for Covered Services provided during the period of continued care other than Copayments, Deductibles or Coinsurance. Provider has no obligation under the Agreement to provide services to individuals who cease to be Participants.

C. Notwithstanding any provision in the Agreement to the contrary, within five (5) business days of the date Provider either gives or receives notice of termination of the Agreement, either with or without cause, Provider shall, in accordance with applicable laws and regulations, supply to Evernorth a list of Participants seen by Provider.

D. Provider shall notify Evernorth of any changes that would impact Provider's credentialing status or ongoing availability to Participants.

(7) The Agreement may permit network rental arrangements which allow the selling, renting, or otherwise grant access to Evernorth's rights to Provider's services to a third party provided that the third party accessing Provider's services is contractually obligated to comply with all applicable terms, limitations and conditions of the Agreement, and the third party is any of the following:
(a) a payer, a third-party administrator, or another entity that administers or processes claims on behalf of the payer;
(b) a preferred provider organization or preferred provider network, including a physician organization or physician hospital organization; or
(c) an entity engaged in the electronic claims transport between the Evernorth and the payer that does not provide access to the provider's services and a discount to any other covered entity.

(8) A. The Agreement may be amended by mutual agreement of the parties. Absent mutual agreement, Evernorth shall provide Provider with a written notice of a proposed amendment and the amendment in writing not later than sixty (60) days prior to the effective date of the amendment. The written notice shall be conspicuously entitled "Notice of Amendment to Contract," and shall contain a summary of the amendment as required by applicable law. The notice period may be extended by mutual agreement of the parties.

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B. Provider shall have sixty (60) days after receipt of the notice and amendment to object in writing to the proposed amendment. If Provider objects in writing and there is no resolution of the objection within sixty (60) days of Evernorth's receipt of the objection, Evernorth or Provider may terminate the Agreement upon written notice to the other party. The terms of the Agreement shall remain in effect through the termination period and shall be unaffected by the proposed amendment.

C. If Provider does not object to the proposed amendment as specified in subsection B, the amendment shall be effective as specified in the notice.

D. Subsections A and B shall not apply under the following circumstances:

1. the delay caused by compliance with the requirements could result in imminent harm to a Participant;
2. the amendment is required by a state or federal law, rule, or regulation that includes an effective date for the amendment;
3. Provider affirmatively accepts the amendment in writing and agrees to an earlier effective date than specified in the notice;
4. Provider's payment or compensation is based on the current Medicaid or Medicare reimbursement schedule, and the change in payment or compensation results solely from a change in that reimbursement schedule;
5. the amendment is a routine change or update of the Agreement made in response to any addition, deletion, or revision of any service code, procedure code, or reporting code, or a pricing change made by a third party source.
   (a) For purposes of this subsection, "service code, procedure code, or reporting code" means the American Medical Association's Current Procedural Terminology, the American Dental Association's Current Dental Terminology, the Centers for Medicare and Medicaid Services' Healthcare Common Procedure Coding System, the World Health Organization's International Classification of Diseases, or the Drug Topic Red Book average wholesale price. For purposes of this subsection, "third party source" means the American Medical Association, the American Society of Anesthesiologists, the American Dental Association, the Centers for Medicare and Medicaid Services, the National Center for Health Statistics, the U.S. Department of Health and Human Services Office of the Inspector General, the Vermont Department of Financial Regulation (DFR), or the Vermont Agency of Human Services.

E. Notwithstanding anything in this section, Evernorth may modify the Agreement by operation of law as required by any applicable state or federal law, rule, or regulation.

(9) A. Evernorth shall provide such information sufficient for Provider to determine compensation or payment terms for Covered Services. Such information shall include: the manner of payment; on request, the fee-for-service dollar amount allowable for each CPT code for those CPT codes that Provider typically uses or actually bills.

B. Evernorth shall provide a readily available mechanism that includes information on the commercially available claims editing software used, standards used for claims edits, payment percentages for modifiers, and any significant edits to the claims software.

C. Evernorth shall provide any policies for prepayment or post payment audits, or both, including whether the policies include limits on the number of medical records Evernorth may

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request for audit in any calendar year.

(10) If included in the Agreement, a "Most Favored Nation" provision or any other provision prohibited by 18 V.S.A. § 9418e is hereby deleted in its entirety.

(11) Participant Access to Care:

A. The Vermont Department of Financial Regulation requires Evernorth to maintain Vermont specific policies and procedures outlining travel and wait times for Participants accessing care. Evernorth maintains the following standards in Vermont:
   a. Travel times for Participants under normal conditions from their residence or place of business generally should not exceed the following:
      i. Thirty (30) minutes to routine, office-based mental health and substance abuse services;
      ii. Thirty (30) minutes to an outpatient facility for mental health or chemical dependency; and
      iii. Sixty (60) minutes to intensive outpatient, partial hospitalization, residential or inpatient mental health and substance abuse services.
   b. Waiting times for appointments should generally not exceed the following:
      i. Immediate access to emergency care for conditions that meet the definition of emergency medical condition;
      ii. In Vermont, Evernorth shall use the following definition of "Emergency": a condition or illness of a participant, if not treated immediately, presents a serious risk of harm to himself or herself, others or property.
      iii. Twenty-four (24) hours for urgent care;
      iv. Two (2) weeks for the initial treatment of non-emergency or non-urgent care, with prompt follow-up care as necessary, including referrals for specialty services.
   c. Annually, Evernorth analyzes data on where plan participants live and compares that to the location of our contracted network. Evernorth evaluates the availability of psychiatrists, non-physician therapists and facilities against geographic standards for rural and urban/suburban market areas. Evernorth also reviews the number of professionals available per covered Participant.
   d. Evernorth measures geographic convenience by drive time for the State of Vermont. The time standards include: one (1) professional within thirty (30) minutes, one (1) outpatient facility within thirty (30) minutes and one (1) inpatient facility within sixty (60) minutes.

B. Providing Clinical Information:
   a. Evernorth strives to obtain all necessary clinical information from the Provider in order to make a utilization review decision. If there is insufficient information on which to make a determination, Evernorth will ensure there are at least three (3) attempts to obtain needed information over no more than two (2) business days. When additional information is not available, the determination must be made based on available information.
   b. In cases where the Provider or Participant will not release the necessary information, Evernorth may not authorize coverage. In no event will Evernorth penalize Provider for failing to provide a Participant's medical records when the Participant has not authorized release of the records and the Provider is not otherwise obligated by law or regulation to disclose the records.

C. Facility Standards:
   a. Evernorth-credentialed facilities must have one of the following in order to be considered for network participation: Accreditation from a nationally recognized and Evernorth-approved accreditation organization (e.g. JCAHO, CARF), a state survey certification, or designation as a rural location as identified by the U.S. Census Bureau. Evernorth actively

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monitors the accreditation status of its network facilities.

(12) Healthplan Quality Committee for Vermont: As a provider in Vermont, Provider has the opportunity to provide input on Evernorth policies, serve on Evernorth's Healthplan quality committee and volunteer to participate in focus groups and surveys. Should Provider wish to provide feedback to the Quality Management Department about Evernorth's published provider satisfaction and preventive health reports or receive more information about the Evernorth Quality Management Program, the annual program evaluation or other chronic case or preventive health measures, Provider should call 800.591.9407.

(13) Emergency Room Results: Specialty and/or emergency room providers are responsible for ensuring that ER results/records will be forwarded to the PCP for all ER visits to ensure appropriate follow-up care.

(14) Covered Practitioners for Vermont: As required by the State of Vermont, Evernorth recognizes athletic trainers as covered practitioners and covers services provided within the scope of their practice. Naturopathic physicians may serve as PCPs in Vermont and should contact Evernorth at 800.88Evernorth (882.4462) to request PCP status in the Evernorth network. Evernorth accepts claims submitted directly by physician assistants or nurse practitioners who are billing within the scope of their licensure or certification. Additionally, a health insurance plan or health benefit plan providing maternity benefits must provide coverage for services rendered by a licensed midwife or an licensed advanced practice registered nurse licensed who is certified as a nurse midwife for services within their scope of practice and provided in a hospital or other health care facility or at home. Evernorth will continue to accept qualified licensed midwives or licensed advanced practice registered nurses who wish to contract and be credentialed with Evernorth. To request an application or for further information, contact Evernorth at 800.88Evernorth (882.4462).

(15) Medical Records: Provider must allow patients with Evernorth ID cards access to their medical records and make them available at no charge beyond the cost of copying.

(16) Credentialing Requirements: As required by Vermont statute, Evernorth has implemented the use of the CAQH credentialing application form for the initial application, and for applications for re-credentialing.

(17) Web Based Prior Approval: As of January 1, 2011, the State of Vermont Rule H-2008-04 requires that providers use the web-based system to request prior authorization. Evernorth has a web-based tool on website and if Provider does not have the ability to request online, Provider should contact Customer Service at 800.88Evernorth (882.4462).

(18) Vermont's Uniform Prior Authorization Form for Medical Services: According to Vermont statute, each insurer must accept either a national standard transaction information for prior authorizations electronically via online portal or accept the uniform prior authorization form approved by DFR for requests for medical procedures (including mental health services) and medical tests when prior authorization is required beginning on or after March 1, 2014. To review the statute, click the following link: **https://legislature.vermont.gov/statutes/section/18/221/09418b**. The statute also requires that the Vermont Uniform Prior Authorization Form(s) must be available on DFR's website and the websites of each health insurer. *Providers requesting prior authorization must send or fax a completed form to the Participant's Plan in advance of the proposed services. Please refer to information provided on each Plan's website for submission instructions and contact information.* Provider may access the standard prior authorization process or obtain a uniform medical or prior authorization form

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from Evernorth, including submission instructions and contact information at **Provider.Evernorth.com**.

If Provider has any questions, please contact: 800.926.2273.

(19) Communication of Health Care Professional termination: Per the State of Vermont Rule H-2009-03: A managed care organization and its contracted health care professionals shall provide at least sixty (60) days final written notice to each other before terminating a contract without cause. Such notices shall not issue unless and until negotiations have concluded and a final decision on termination has been reached. Within five (5) working days of the date that the health care professional either gives or receives final notice of termination, either for or without cause, the health care professional shall supply the managed care organization with a list of his or her patients that are customers of the managed care organization.

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VIRGINIA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Virginia regarding provider contracts with providers rendering health care services in the State of Virginia. To the extent that such state laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Virginia laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

I

1. The definition for Emergency Services, if any, shall comply with Virginia laws and regulations to the extent applicable.

2. Pursuant to Code of Virginia 38.2-3407.15, to the extent applicable and/or not otherwise preempted by federal law, Evernorth shall comply with the following minimum fair business standards in the processing and payment of claims for Covered Services:

   a. Payor shall pay any claim within forty (40) days of receipt of the claim except where the obligation of the Payor to pay a claim is not reasonably clear due to the existence of a reasonable basis supported by specific information available for review by the person submitting the claim that:

      1. The claim is determined by Evernorth not to be a clean claim due to a good faith determination or dispute regarding: (a) the manner in which the claim form was completed or submitted, (b) the eligibility of a person for coverage, (c) the responsibility of another payor for all or part of the claim, (d) the amount of the claim or the amount currently due under the claim, (e) the benefits covered, or (f) the manner in which Covered Services were accessed or provided; or

      2. The claim was submitted fraudulently.

      Evernorth shall maintain a written or electronic record of the date of receipt of a claim. Provider shall be entitled to inspect such record on request and to rely on that record or on any other admissible evidence as proof of the fact of receipt of the claim, including without limitation electronic or facsimile confirmation of receipt of a claim.

   b. Evernorth shall, within thirty (30) days after receipt of a claim, notify Provider of any defect or impropriety that prevents Cigna from deeming the claim a clean claim, as defined by applicable law, and request the information that believes will be required to process and pay the claim. Upon

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receipt of the additional information necessary to make the original claim a clean claim, Payor shall make the payment of the claim in compliance with applicable law. Payor may not refuse to pay a claim for services which are Covered Services rendered pursuant to the Agreement if Evernorth fails to timely notify or attempt to notify Provider of the matters identified above unless such failure was caused in material part by Provider. However, nothing herein shall preclude a Payor from imposing a retroactive denial of payment of such a claim unless such retroactive denial of payment of the claim would violate subsection f. below. Beginning no later than January 1, 2026, all notifications and information required under this section shall be delivered electronically.

c. Any interest owing or accruing on a claim under Code of Virginia 38.2-3407.1 or 38.2-4306.1, under the Agreement or under any other applicable law shall, if not sooner paid or required to be paid, be paid, without the necessity of demand, at the time the claim is paid or within sixty (60) days thereafter.

1. Evernorth shall establish and implement reasonable policies to permit Provider: (a) to confirm in advance during normal business hours by free telephone or electronic means, if available, whether the health care services to be provided are Medically Necessary and Covered Services; and (b) to determine Evernorth's requirements applicable to Provider (or to the type of health care services which Provider has contracted to deliver under the Agreement) for: (i) precertification or authorization of coverage decisions, (ii) retroactive reconsideration of a certification or authorization of coverage decision or retroactive denial of a previously paid claim, (iii) provider-specific payment and reimbursement methodology, coding levels and methodology, down-coding, and bundling of claims, and; (iv) other provider- specific, applicable claims processing and payment matters necessary to meet the terms and conditions of the Agreement, including determining whether a claim is a clean claim.

2. With respect to Agreements entered into, amended, extended or renewed on or after January 1, 2006, if Evernorth routinely, as a matter of policy, bundles or down-codes claims submitted by Provider, Evernorth shall clearly disclose that practice in the Agreement. Further, Evernorth shall either: (i) disclose in the Agreement or on the Evernorth website the specific bundling and down-coding policies that Evernorth reasonably expects to be applied to Provider's services on a routine basis as a matter of policy; or (ii) disclose in the Agreement a telephone or facsimile number or e-mail address that Provider can use to request the specific bundling and down-coding policies that Evernorth reasonably expects to be applied to Provider's services on a routine basis as a matter of policy. If such request is made by or on behalf of Provider, Evernorth shall provide Provider with such policies within ten (10) business days following the date the request is received. The paragraph below contains the disclosure of information concerning Evernorth's claim bundling and down-coding practice and policies.

Payments for Covered Services under the Agreement are subject to Evernorth's payment policies. Payment policies are the guidelines adopted by Evernorth for calculating payment of claims under the Agreement. Such guidelines include Evernorth's standard claim coding and bundling methodology and claims processing policies and procedures. Evernorth's payment policies may change from time to time. Provider may obtain up-to-date information regarding payment policies by visiting the Evernorth website (**Evernorth.com**). Providers should contact Provider Services at 800.926.2273 to request a copy of their fee schedule. (Evernorth reserves the right to rename these website addresses and such change shall not require a modification to this addendum so long as Evernorth communicates the name of the new website address to

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Provider). Provider may access this secure website tool twenty-four (24) hours a day, seven (7) days a week, by logging on through the Evernorth website. Provider may also use this website tool to e-mail Evernorth with specific questions about claim coding, fee schedules, Covered Services and coverage criteria. Provider may appeal a payment policy issue in accordance with the dispute resolution process described in the Agreement and the Medical Management Program. The Medical Management Program contains the rules, policies and procedures adopted by Evernorth or a Payor to be followed by Provider in providing services and doing business with Evernorth and Payors under the Agreement.

3. Evernorth shall make available to Provider within ten (10) business days of receipt of a request, copies of or reasonable electronic access to all such policies which are applicable to Provider and to the health care services identified by Provider which Provider has contracted to deliver under the Agreement. In the event that the provision of the entire policy would violate any applicable copyright law, Evernorth may instead comply with this subsection by timely delivering to Provider a clear explanation of the policy as it applies to Provider and to those health care services identified by Provider which Provider has contracted to deliver under the Agreement.

d. Payor shall pay a claim if Evernorth has previously authorized the health care service or has advised Provider or Participant in advance of the provision of health care services that the health care services are Medically Necessary and a Covered Service, unless:

1. The documentation for the claim provided by Provider clearly fails to support the claim as originally authorized; or

2. The Payor's refusal is because: (a) another payor is responsible for the payment, (b) Provider has already been paid for the health care services identified on the claim, (c) the claim was submitted fraudulently or the authorization was based in whole or material part on erroneous information provided to Evernorth by Provider, Participant, or other person not related to Evernorth, or (d) the individual receiving the health care services was not eligible to receive the health care services on the date the services were provided and Evernorth did not know, and with the exercise of reasonable care could not have known, of the Participant's eligibility status; or

3. During the post-service claims process, it is determined that the claim was submitted fraudulently.

e. In the case of an invasive or surgical procedure, if Evernorth has previously authorized a health care service as Medically Necessary and during the procedure Provider discovers clinical evidence prompting Provider to perform a less or more extensive or complicated procedure than was previously authorized, then Evernorth shall pay the claim, provided that the additional procedures were (i) not investigative in nature, but Medically Necessary as a Covered Service under the Participant's Plan; (ii) appropriately coded consistent with the procedure actually performed; and (iii) compliant with Evernorth's post-service claims process, including required timing for submission to Evernorth.

1. Payor shall not impose any retroactive denial of a previously paid claim or in any other way seek recovery or refund of a previously paid claim unless Evernorth specifies in writing the specific claim or claims for which the retroactive denial is to be imposed or the recovery or

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refund is sought, Evernorth has provided a written explanation of why the claims is being retroactively adjusted, and: (1) the original claim was submitted fraudulently, (2) the original claim payment was incorrect because Provider was already paid for the health care services identified on the claim or the health care services identified on the claim were not delivered by Provider, or (3) the time which has elapsed since the date of the payment of the original challenged claim does not exceed twelve (12) months, Notwithstanding the provisions of clause (3), Provider and Cigna may agree in writing that recoupment of overpayments by withholding or offsetting against future payments may occur after such 12-month limit for the imposition of the retroactive denial. Evernorth shall notify Provider at least thirty (30) days in advance of any retroactive denial or recovery refund of a previously paid claim. Beginning no later than January 1, 2026, all written communications, explanations, notifications, and related provider responses applicable to this section shall be delivered electronically. The electronic method and location for delivery shall be agreed upon by Cigna and Provider and included in the provider contract.

Evernorth requires that claims subject to Code of Virginia Section 8.2-3407.15 must be submitted within twelve (12) months of the date of service and that claims received after that date may be denied for payment.

c. The Agreement shall include or attach at the time it is presented for execution: (1) the fee schedule, reimbursement policy or statement as to the manner in which claims will be calculated and paid which is applicable to Provider or to the range of health care services reasonably expected to be delivered by Provider on a routine basis under the Agreement, and (2) all material addenda, schedules and exhibits thereto and any policies (including those referred to in subsection d. above) applicable to Provider or to the range of health care services reasonably expected to be delivered by Provider under the Agreement.

d. 1. No amendment to the Agreement or to any addenda, schedule, exhibit or policy thereto (or new addenda, schedule, exhibit, or policy) applicable to Provider (or to the range of health care services reasonably expected to be delivered by Provider under the Agreement) shall be effective as to Provider, unless Provider has been provided with the applicable portion of the proposed amendment (or of the proposed new addenda, schedule, exhibit, or policy) and has failed to notify Evernorth within fifteen (15) business days of receipt of the documentation of Provider's intention to terminate the Agreement at the earliest date thereafter permitted under the Agreement.

2. No amendment to the Agreement or to any addenda, schedule, exhibit or policy thereto (or new addenda, schedule, exhibit, or policy) applicable to Provider (or to the range of health care services reasonably expected to be delivered by Provider under the Agreement) shall be effective as to Provider, unless Provider has been provided with the applicable portion of the proposed amendment (or of the proposed new addenda, schedule, exhibit, or policy) at least sixty (60) calendar days before the effective date and Provider has failed to notify Evernorth within thirty (30) calendar days of receipt of the documentation of Provider's intention to terminate the Agreement at the earliest date thereafter permitted under the Agreement.

e. In the event that Evernorth's provision of a policy required to be provided under subsections h. or i. above would violate any applicable copyright law, Evernorth may instead comply with this section by providing a clear, written explanation of the policy as it applies to Provider.

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I. Evernorth shall establish, in writing, its claims payment dispute mechanism and shall make this information available to Provider. If Evernorth claim denial is overturned following the completion of a dispute review, Evernorth shall, on the day the decision to overturn is made, consider the claims impacted by such decision as clean claims. All applicable laws related to the payment of a clean claim shall apply to the payments due.

a. Evernorth shall not be in violation of the above provisions if its failure to comply is caused in material part by Provider or if Evernorth's compliance is rendered impossible due to matters beyond Evernorth's reasonable control (such as an act of God, insurrection, strike, fire or power outages) which are not caused in material part by Evernorth.

b. Evernorth shall not terminate or fail to renew the Agreement or otherwise penalize Provider for invoking any of Provider's rights under this section of the Agreement.

c. Beginning July 1, 2025, Cigna shall make available through electronic means a way for Provider to determine whether a Participant is covered by a health plan that is subject to the Commissioner's jurisdiction.

3. Nothing in the Agreement shall be construed to:

a. Require Provider to refuse to provide treatment which the Provider believes to be Medically Necessary and appropriate, and which is provided with respect to others with similar conditions.

b. Require that Provider indemnify Evernorth for Evernorth's negligence, willful misconduct or breach of contract, if any.

c. Require Provider, as a condition of participation on Evernorth's panel, to waive any right to seek legal redress against Evernorth.

d. Prohibit, impede or interfere in the discussion of medical treatment options between Provider and Participants. The Agreement expressly permits and requires Provider to discuss medical treatment options with Participants.

4. Evernorth must notify Provider or entity completing a transaction on behalf of the carrier, if the carrier uses a payment method that imposes a transaction or processing fee or similar charge on the provider. The carrier must offer the provider an alternative payment method that does not impose such a fee or similar charge. If the provider chooses the alternative payment method and has provided all required information to the carrier to enroll in the alternative method, the carrier must pay the claim using the alternative payment method.

5. a. Evernorth shall give Provider notice at least ninety (90) days prior to the date of termination of Provider, except when Provider is terminated for cause. To the extent that the Agreement provides for a notification period longer than ninety (90) days, such longer notification period will apply, except when Provider is terminated for cause.

b. Provider shall give Evernorth at least sixty (60) days' prior notice of termination of the Agreement. Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the Agreement by Provider, such longer notification period

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will apply.

c. Provider shall be permitted to render Covered Services for a period of at least 90 days from the date of Provider's termination from a provider panel, except when the termination is for cause. Provider shall continue to render Covered Services to Participants who have an existing provider-patient relationship with Provider for a period of at least 90 days from the date of Provider's termination from a provider panel, except when the termination is for cause. Existing provider-patient relationship shall mean that Provider has rendered Covered Services to Participant or admitted or discharged Participant in the previous 12 months, or as otherwise defined by applicable law as may be amended from time to time.

(c)(i). Notwithstanding Section c above, Evernorth shall permit Provider to continue rendering and Provider shall continue rendering Covered Services to any Participant who has an existing provider-patient relationship with Provider and who has been medically confirmed to be pregnant at the time of a Provider's termination of participation, except when Provider is terminated for cause. Such treatment shall, at the Participant's option, continue through the provision of postpartum care directly related to the delivery.

(c)(ii). Notwithstanding Section c above, Evernorth shall permit Provider to continue rendering and Provider shall continue rendering Covered Services to any Participant who has an existing provider-patient relationship with Provider and who is determined to be terminally ill (as defined under § 1861 (dd)(3)(A) of the Social Security Act) at the time of a Provider's termination of participation, except when Provider is terminated for cause. Such treatment shall, at the Participant's option, continue for the remainder of the Participants' life for care directly related to the treatment of the terminal illness.

(c)(iii). Notwithstanding Section c above, Evernorth shall permit Provider to continue rendering and Provider shall continue rendering Covered Services to any Participant who has an existing provider-patient relationship with Provider and who has been determined by a medical professional to have a life-threatening condition at the time of Provider's termination of participation. Such treatment shall, at the Participant's option, continue for up to 180 days for care directly related to the life-threatening condition.

(c)(iv). Notwithstanding Section c above, Evernorth shall permit Provider to continue rendering and Provider shall continue rendering Covered Services to any Participant who has an existing provider-patient relationship with Provider and who is admitted to and receiving treatment in any inpatient facility at the time of Provider's termination of participation. Such admission and treatment shall continue until the Participant is discharged from the inpatient facility.

II

Pursuant to the prior authorization requirements for a health care provider with prescriptive authority (hereinafter "prescriber") of Code of Virginia § 38.2-3407.15:2, as may be amended from time to time, to the extent applicable and/or not otherwise preempted by federal law, Evernorth shall:

1. In a method of Evernorth's choosing, accept telephonic, facsimile, or electronic submission of prior authorization requests that are delivered from e-prescribing systems, electronic health record systems, and health information exchange platforms that utilize the National Council for Prescription Drug Programs' SCRIPT standards;

2. Communicate to the prescriber or his designee within twenty-four (24) hours, including weekend hours,

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of submission of an urgent prior authorization request, if submitted telephonically or in an alternate method directed by Evernorth, that the request is approved, denied, or requires supplementation;

3. Communicate electronically, telephonically, or by facsimile to the prescriber or his designee, within two (2) business days of submission of a fully completed prior authorization request, that the request is approved, denied, or requires supplementation;

4. Communicate electronically, telephonically, or by facsimile to the prescriber or his designee, within two (2) business days of submission of a properly completed supplementation from the prescriber or his designee, that the request is approved or denied;

5. Communicate electronically, telephonically, or by facsimile to the prescriber or his designee, within the timeframes established by section 3 or 4 above, as applicable, for any prior authorization request denied, the reasons for the denial;

5.1 Not revoke, limit, condition, modify, or restrict a prior authorization request that is approved for prescription drugs and such prescription drugs have been scheduled, provided, or delivered to Participant consistent with the authorization, unless (i) there is evidence that the authorization was obtained based on fraud or misrepresentation; (ii) final actions by the U.S. Food and Drug Administration, other regulatory agencies,or the manufacturer remove the drug from the market, limit its use in a manner that affects the authorization, or communicate a patient safety issue that would affect the authorization alone or in combination with other authorizations; (iii) a combination of drugs prescribed would cause a drug interaction; or (iv) a generic or biosimilar is added to the prescription drug formulary. (Nothing in Code of Virginia § 38.2-3407.15:2 B(5), as may be amended from time to time, shall require Cigna to cover any benefit not otherwise covered or cover a prescription drug if Participant is no longer covered by a health plan on the date the prescription drug was scheduled, provided, or delivered.)

6. Effective January 1, 2027 Evenorth shall communicate electronically or telephonically to the provider or his designee within 72 hours, including weekend hours, of submission of an expedited prior authorization request to Evernorth that the request is approved, denied, or requires supplementation

7. Effective January 1, 2027 Evernorth shall communicate electronically or telephonically to the provider or his designee within seven calendar days of submission of a standard prior authorization request to Evernorth that the request is approved, denied, or requires supplementation

8. Where supplementation is required, Evernorth shall specify to the provider or his designee the supplementation necessary for Evernorth to make a final determination that the request is approved or denied, and following properly completed supplementation from the provider or his designee, Evernorth shall approve or deny the request within the timeframes specified above.

9. If a prior authorization request is approved for health care services and such health care services have been scheduled or provided to the enrollee consistent with the authorization, Evernorth shall not revoke, limit, condition, modify, or restrict that authorization unless (i) the provider requests a change, (ii) there is evidence that the authorization was obtained based on fraud or misrepresentation, or (iii) a final action by a federal regulatory agency or the manufacturer removes an approved health care service from the market, limits its use in a manner impacting the prior authorization, or communicates a patient safety issue that

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would impact the prior authorization. Nothing in this section shall require Evernorth to authorize any health care service if the enrollee is no longer enrolled in the health plan; and

10. If the prior authorization request is denied, Evernorth shall communicate electronically or telephonically to the provider or his designee within the timeframes established above, as applicable, the reasons for the denial.

11. Honor a prior authorization approved by another carrier, upon Evernorth's receipt from the prescriber or his designee of a record demonstrating the previous carrier's prior authorization approval or any written or electronic evidence of the previous carrier's coverage of such drug, at least for the initial 90 days of a Participant's prescription drug benefit coverage under a new health plan, subject to the provisions of Evernorth's evidence of coverage and any exception listed in Section 5.1 above;

12.Use a tracking system for all prior authorization requests; the identification information shall be provided electronically, telephonically, or by facsimile to the prescriber or his designee, upon Evernorth's response to the prior authorization request;

13.Make available through one (1) central location on Evernorth's website prescription drug formularies, all drug benefits subject to prior authorization, all prior authorization procedures, and all prior authorization request forms; such information will be updated within seven (7) days of approved changes;

14.Honor a prior authorization issued by Evernorth for a drug, other than an opioid, regardless of changes in dosages of such drug, provided such drug is prescribed consistent with FDA labeled dosages;

15. Honor a prior authorization issued by Evernorth for a drug regardless if the covered person changes plans with the same carrier and the drug is a covered benefit with the current health plan;

16. Identify the specific information required when requiring a prescriber to provide supplemental information that is in the Participant's health record or electronic health record; and

17. Require that no prior authorization be required for at least one (1) drug prescribed for substance abuse medication-assisted treatment, provided that (i) the drug is a covered benefit,
(ii) the prescription does not exceed the FDA labeled dosages, and (iii) the drug is prescribed consistent with the regulations of the Board of Medicine.

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WASHINGTON REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Washington regarding provider contracts with providers rendering health care services in the State of Washington. To the extent that such Washington laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. For the avoidance of doubt, to the extent such Washington laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

The parties hereby agree to the following modifications to the terms of the Agreement:

1. The following definition is added to the Section of the Agreement entitled Definitions:

   Clean Claim - Means a claim that has no defect or impropriety, including any lack of any required substantiating documentation, or particular circumstances requiring special treatment that prevents timely payments from being made on such claim.

2. The following provision is added to Section 4 of the Agreement entitled Compensation:

   a. Ninety-five percent (95%) of the monthly volume of Clean Claims shall be paid within thirty (30) days of receipt by the responsible carrier or agent of the carrier; and

   b. Ninety-five percent (95%) of the monthly volume of all claims shall be paid or denied within sixty (60) days of receipt by the responsible carrier or agent of the carrier, except as agreed to in writing by Evernorth and Provider on a claim-by-claim basis.

   c. Should Evernorth fail to pay claims within the above standards, Evernorth shall apply interest on undenied and unpaid clean claims more than sixty-one (61) days old at the rate of one percent (1%) per month.

   d. Evernorth shall not deny payment to Provider for a Covered Service that Evernorth has prior-authorized under Evernorth's written policies and procedures, except if the claim for the Covered Service was submitted fraudulently or negligently or Provider was already paid for the Covered Service.

3. The following shall replace and supersede the section of the Agreement entitled Charges to Participants:

   a. Provider hereby agrees that in no event, including, but not limited to nonpayment by Evernorth, Evernorth's insolvency, or breach of this Agreement shall Provider bill, charge, collect a deposit from, seek compensation, remuneration, or reimbursement from, or have any recourse against a Participant or person acting on their behalf, other than Evernorth, for services provided pursuant to this Agreement. This provision shall not prohibit collection of Deductibles, Copayments, Coinsurance and/or non-Covered Services, which have not otherwise been paid by a primary or

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secondary carrier in accordance with regulatory standards for coordination of benefits, from Participants in accordance with the terms of the Participant's health plan.

b. Provider agrees, in the event of Evernorth's insolvency, to continue to provide the services promised in this Agreement to Participants of Evernorth for the duration of the period for which premiums on behalf of the Participant were paid to Evernorth or until the Participant's discharge from inpatient facilities, whichever time is greater.

c. Notwithstanding any other provision of this Agreement, nothing in this Agreement shall be construed to modify the rights and benefits contained in the Participant's health plan.

d. Provider may not bill the Participant for Covered Services (except for Deductibles, Copayments, or Coinsurance) where Evernorth denies payments because Provider have failed to comply with the terms or conditions of this Agreement.

e. Provider further agrees (i) that the provisions of WAC 284- 170-421(3) (a), (b), (c), and (d) shall survive termination of this Agreement, regardless of the cause of termination and shall be construed to be for the benefit of Evernorth's Participants, and (ii) that this provision supersedes any oral or written contrary agreement now existing or hereafter entered into between Provider and Participants or persons acting on their behalf.

f. If Provider contracts with other providers or facilities who agree to provide Covered Services to Participants of Evernorth with the expectation of receiving payment directly or indirectly from Evernorth or Payor, such providers or facilities must agree to abide by the provisions of WAC 284-170-421(3) (a), (b), (c), (d) and (e) and all requirements of this Agreement.

Provider acknowledges that willfully collecting or attempting to collect an amount from a Participant knowing that collection to be in violation of the Agreement constitutes a class C felony under the Revised Code of Washington (RCW) Section 48.80.030(5).

Effective July 25. 2021, if Provider intends to bill a covered person's health plan for an audio-only telemedicine service, Provider must obtain patient consent from the covered person for the billing in advance of the service being delivered.

4. The following provision is added to Section 7 of the Agreement entitled Access to Books and Records:

The parties agree that any provisions in the Agreement relating to one party's right to audit the records of the other party shall (i) be limited to health information and other similar records relating only to Evernorth Participants, (ii) be limited only to those records that are necessary to perform the audit, and (iii) apply mutually to both parties.

5. a. The following is added to Section 4 of the Agreement entitled Compensation:

When applicable, Evernorth overpayment recovery activities are subject to the Revised Code of Washington Section 48.43.600:

Except in the case of fraud, Payor may not: (a) request a refund from Provider of a payment previously made to satisfy a claim unless it does so in writing to Provider within twenty-four (24) months after the date that the payment was made; or (b) request that a contested refund be paid any sooner than six (6) months after receipt of the request. Any such request must specify why Payor believes Provider owes a refund. If Provider fails to contest the request in writing to the carrier

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within thirty (30) days of its receipt, the request is deemed accepted and the refund must be paid.

Payor may not, if doing so for reasons related to coordination of benefits with another carrier or entity responsible for payment of a claim: (a) request a refund from Provider of a payment previously made to satisfy a claim unless it does so in writing to Provider within thirty (30) months after the date that the payment was made; or (b) request that a contested refund be paid any sooner than six (6) months after receipt of the request. Any such request must specify why Payor believes Provider owes the refund, and include the name and mailing address of the entity that has primary responsibility for payment of the claim. If Provider fails to contest the request in writing to Payor within thirty (30) days of its receipt, the request is deemed accepted and the refund must be paid.

Payor may at any time request a refund from Provider of a payment previously made to satisfy a claim if: (a) a third party, including a government entity, is found responsible for satisfaction of the claim as a consequence of liability imposed by law, such as tort liability; and (b) Payor is unable to recover directly from the third party because the third party has either already paid or will pay Provider for the health services covered by the claim.

b. The following is added to Section 4 of the Agreement entitled Compensation:

Except in cases of fraud, Provider may not request additional payment from Payor, unless it does so in writing during the twenty-four (24) month period after the date Payor denied or paid the claim submitted by Provider. If Provider request an additional payment for reasons related to coordination of benefits with another carrier or entity responsible for payment of a claim, the request for an additional payment must be made in writing within thirty (30) months of the payment.

6. The parties agree that those provisions entitled Dispute Resolution Procedure in Section 12 of the Agreement shall not apply to the resolution of disputes between the parties. Such disputes shall be submitted to the internal dispute resolution process described in Paragraph 7 below. No alternative resolution process may be required by Evernorth to the exclusion of judicial remedies.

7. The parties agree to resolve disputes arising during the term of this Agreement in accordance with the following procedures:

Provider Payment Appeals Process
To initiate a first-level review, provider must submit the following information in writing within one hundred eighty (180) calendar days of the date of the initial payment or denial notice:

- A completed request for provider payment appeal (form available at **Evernorth.com**) or a provider payment appeal letter indicating the reason for the appeal;
- A copy of the original claim and EOP or EOB, if applicable;
- For reviews with a clinical component, such as services denied for no prior authorization for not being medically necessary, supporting documentation should include a narrative describing the situation, an operative report and medical records, as applicable;

After preparing the necessary documentation, providers should contact their local customer service representative for specific appeal-designee mailing information. Providers may also visit the Evernorth website at **Evernorth.com** for specific address information.

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Second-Level Review
Providers who are not satisfied with the resolution of the first-level review may submit the appeal to a second-level review within sixty (60) calendar days of the date of the first-level review determination as follows:

- Repeat steps above, indicating a second-level request on the appeal form or letter.
- Include any additional or pertinent supporting documentation.

Providers must return the completed documentation as directed in the first-level determination notice. Providers may also contact their local customer service representative for assistance or visit the Evernorth website at **Evernorth.com.**

Providers should submit one (1) appeal per form.
Evernorth must render a decision on provider payment appeals within sixty (60) days of the appeal.

Provider Termination Appeals Process
First-Level Review
To initiate a first-level review of a provider termination, Provider must submit the following information in writing within thirty (30) calendar days of the date of the provider termination notice:

- A completed provider termination appeal form or provider termination appeal letter indicating the reason for the appeal;
- A copy of the original termination notice;
- Supporting documentation for reconsideration.

To safeguard the sensitive nature of Provider's information, Provider must follow the contact instructions in the termination notice. Returning the completed documentation as directed will help insure confidentiality.

Second-Level Appeal
Providers who are not satisfied with the resolution of the first-level review may submit the appeal to the second-level review within sixty (60) calendar days of the date of the first-level review determinations follows:

- Repeat steps above, indicating a second-level request on the form or letter.
- Include any additional or pertinent supporting documentation.

Providers must follow contact instructions in the first-level review decision letter. Returning the completed documentation as directed in the first-level review decision letter will help ensure confidentiality.

If Evernorth fails to grant or reject a request within thirty (30) days after it is made, the complaining provider may proceed as if the complaint had been rejected. A complaint that has been rejected by Evernorth may be submitted to nonbinding mediation. Mediation shall be conducted under the Uniform Mediation Act, Title 7, Chapter 7.07 of the Revised Code of Washington (RCW), or any other rules of mediation agreed to by the parties. This section is solely for resolution of provider complaints.

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Complaints by, on behalf of, a Participant are subject to the grievance processes in RCW 48.43.530.

The disclosures set forth in (1) through (3) below are being provided pursuant to directions received from the Washington State Office of the Insurance Commissioner beginning in April 2021, requiring Evernorth to include within an Agreement a statement with regard to RCW 48.43.420(Chapter 171, Laws of 2019; eff. July 28, 2019).

(1) Evernorth acknowledges its obligation under RCW 48.43.420 to provide access to a clear, readily accessible, and convenient process to request an exception to the prescription drug management process.
(2) Evernorth acknowledges its obligation under RCW 48.43.420 to provide sixty (60) days' notice for any new policies or procedures applicable to prescription drug utilization management protocols.
(3) Program Requirements or updates may be posted on the Evernorth for Health Care Professionals website at **Evernorth.com** or communicated through notifications delivered by mail, email, phone, fax, or in person.

The disclosures set forth in (1) through (5) below are being provided pursuant to regulations adopted by the Washington State Office of the Insurance Commissioner November 22, 2021, to implement the health plan reimbursement requirements for telemedicine services found in RCW 48.43.735 as amended in July 2021.

(1) Payor shall reimburse Hospital for a Covered Service provided to a Covered Person through telemedicine, as defined in applicable law, or store and forward technology as set forth in applicable law including, but not limited to RCW 48.43.735 and WAC 284-170-433, as may be amended from time to time.

(2) Where required by applicable law, including, but not limited to RCW 48.43.735 and WAC 284-170-433, as may be amended from time to time, Payor shall reimburse Hospital for a Covered Service provided to a Covered Person through telemedicine, as defined in applicable law, the same amount of compensation as Payor would pay Hospital if the Covered Services was provided in person by Hospital, unless Hospital is eligible under applicable law to elect to negotiate, and has negotiated, an amount of compensation that differs from the amount of compensation for in-person services.

(3) To the extent required by applicable law and regulations, as may be amended from time to time, an originating site for a telemedicine health care service shall include a: hospital; rural health clinic; federally qualified health center; physician's or other health care provider's office; licensed or certified behavioral health agency; skilled nursing facility; home, or any location determined by the individual receiving the service including, but not limited to a pharmacy or a school-based health center; or a renal dialysis center.

(4) Hospital shall, consistent with applicable state and federal laws, obtain patient consent from a Covered Person for the billing of an audio-only telemedicine service in advance of the service being delivered should Hospital intend to bill a Covered Person or a Covered Person's health plan.

(5) Evernorth acknowledges its obligations under applicable state regulations, as may be amended from time to time, to ensure that access to telemedicine services is inclusive for those patients who may have disabilities or limited-English proficiency for whom the use of telemedicine technology may be more challenging.

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The disclosures set forth below are being provided pursuant to directions received from the Washington State Office of the Insurance Commissioner in November 2022, requiring Evernorth to include the following additional provisions to support the implementation of the health plan reimbursement requirements for telemedicine services found in RCW 48.43.735.

(A) Effective January 1, 2023 for health plans subject to RCW 48.43.735, Payor shall reimburse Hospital for audio-only telemedicine if the Covered Person has an established relationship with Hospital. An established relationship shall be defined as set forth in RCW 48.43.735 and WAC 284-170-130, as the laws and regulations are amended or revised from time to time.

(B) Unless otherwise set forth in applicable laws and regulations, established relationship means the provider providing audio-only telemedicine has access to sufficient health records to ensure safe, effective, and appropriate care services and: (i) For health care services included in the essential health benefits category of mental health and substance use disorder services, including behavioral health treatment: (A) The covered person has had, within the past three years, at least one in-person appointment, or at least one real-time interactive appointment using both audio and video technology, with the provider providing audio-only telemedicine or with a provider employed at the same medical group, at the same clinic, or by the same integrated delivery system operated by a carrier licensed under chapter 48.44 or 48.46 RCW as the provider providing audio-only telemedicine; or (B) The covered person was referred to the provider providing audio-only telemedicine by another provider who has had, within the past three years, at least one in-person appointment, or at least one real-time interactive appointment using both audio and video technology, with the covered person and has provided relevant medical information to the provider providing audio-only telemedicine; (ii) For any other health care service: (A) The covered person has had, within the past two years, at least one in-person appointment, or, until January 1, 2024, at least one real-time interactive appointment using both audio and video technology, with the provider providing audio-only telemedicine or with a provider employed at the same medical group, at the same clinic, or by the same integrated delivery system operated by a carrier licensed under chapter 48.44 or 48.46 RCW as the provider providing audio-only telemedicine; or (B) The covered person was referred to the provider providing audio-only telemedicine by another provider who has had, within the past two years, at least one in-person appointment, or, until January 1, 2024, at least one real-time interactive appointment using both audio and video technology, with the covered person and has provided

8. Enrollee/Patient Notice requirements for expiring or terminating contracts

(A) Provider agree to follow all applicable requirements of RCW 48.43.732 and corresponding regulations at WAC 284-170-441, 284-170-445, and 284-170-447.

(B) If this provider contract expires by its own terms or one party gives notice to the other party of an intended termination without cause in accordance with the terms of this provider contract, provider/facility and carrier may not make or cause to be made public statements regarding such expiration or termination until 45 days prior to the termination date of the provider contract.

(C) Except for notifications to enrollees/patients that the contract termination is no longer applicable, such as in the case of a rescission of the contract termination, all notices sent to enrollees and patients under this contract termination provision must either:

(a) Solely utilize the standard template language posted on the website of the office of the insurance commissioner under WAC 284-170-445, with no modifications to the text of the template other

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than to insert the specific information requested in the bracketed sections of the template. Notices complying with this subsection may be sent to enrollees and patients without the commissioner's review or prior approval; or

(b) Receive the commissioner's prior approval if the notice does not solely utilize the template language. Notices described in this subsection must be reviewed and approved by the commissioner prior to being used.

(c) Provider/facility and carrier may send enrollees and patients the notices described in this subsection electronically rather than by physical mail if the enrollee or patient has consented to receive electronic communications.

(D) Carrier acknowledges that, for violations of RCW 48.43.732 and WAC 284-170-441 and 284-170-445 by carriers, the commissioner may pursue enforcement actions under RCW 48.02.080 or impose a civil monetary penalty upon carriers of up to $100 per noncompliant notice, per day, per enrollee to whom the notice has been sent in advance of the 45-day period established under WAC 284-170-441.

(E) Provider/facility acknowledges that, for potential violations of RCW 48.43.732 and WAC 284-170-441 and 284-170-445 by health care providers and health care facilities (as those terms are defined in RCW 48.43.005), if the commissioner has cause to believe that any health care provider or health care facility has violated the requirements of RCW 48.43.732 or WAC 284-170-441 or 284-170-445, the commissioner may submit information to the department of health or the appropriate health care facility or provider licensing or disciplining entity.

9. The following is added to Section 13 of the Agreement:

To the extent applicable, pursuant to WAC 284-170-401, Provider shall maintain agreements with each subcontracted provider or facility requiring subcontracted provider or facility to comply with all of the terms and conditions of this Agreement.

10. The following is added to Section 13.b. of the Agreement entitled Amendment:

Evernorth may amend this Agreement by providing at least sixty (60) days prior written notice for changes that affect Provider compensation or health care service delivery unless changes to federal or state law or regulations make such advance notice impossible, in which case notice shall be provided as soon as possible.

11. A. If Provider is not a hospital, the following shall replace and supersede the section of the Agreement entitled Coordination of Benefits:

Certain claims for Covered Services are claims for which another payor may be primarily responsible under coordination of benefit rules. Provider may pursue those claims in accordance with the process set out in the Medical Management Program. When a Participant's coverage under a Benefit Plan is secondary, Payor will pay an amount which, together with the payment made by the primary plan, cannot be less than the same allowable expense as the secondary plan would have paid if it was the primary plan. However, in no event shall the Payor be required to pay an amount in excess of its maximum benefit plus accrued savings. When Medicare, Part A, Part B, Part C, or Part D is primary, Medicare's allowable amount is the allowable expense.
B. When applicable, Evernorth coordination of benefits activities are subject to Washington Administrative Code 284-51-215, which provides that:

- When Provider notifies Evernorth that more than one (1) plan covers an enrollee who has

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submitted a claim, Evernorth will resolve with the other plan within thirty (30) days which plan is primary. This deadline may be extended in situations involving court orders for dependent coverage, if the court order contains information needed to determine which plan is primary and has not been provided to the issuer. If an agreement cannot be reached, Evernorth will share equally the allowable expenses between the plans according to Washington Administrative Code 284-51-205(4) (f).

- If Evernorth is the secondary plan and receives a claim without the primary plan's explanation of benefits or other necessary payment details, Evernorth will notify Provider within thirty (30) days that the claim is incomplete.
- If the primary plan has not adjudicated a claim within sixty (60) days of receipt, then Provider may submit the claim to Evernorth and Evernorth will process the claim as primary within thirty (30) days.

12. If Provider is not a hospital or ancillary provider, the following sentence is added to Section 10.a of the Agreement:

    If Provider is a primary care physician whose contract is being terminated without cause, Evernorth will continue to reimburse the primary care provider for Covered Services rendered to Participants (whether or not they are suffering from a chronic condition) for at least sixty (60) days following the termination, or until the next open enrollment period of the Participants requesting continuity of care.

13. The following provision is added to Section 8 of the Agreement entitled Relationship of Parties:

    Evernorth shall not in any way preclude or discourage Provider from informing patients of the care they require, including various treatment options, and whether in their view such care is consistent with medical necessity, medical appropriateness, or otherwise covered by the Participant's health benefit plan. Evernorth shall not prohibit, discourage, or penalize Provider otherwise practicing in compliance with the law from advocating on behalf of a Participant with Evernorth. Nothing in this section shall be construed to authorize Provider to bind Evernorth or Payor to pay for any service. Evernorth may not preclude or discourage Participants or those paying for their coverage from discussing the comparative merits of different health carriers with their Provider. This prohibition specifically includes prohibiting or limiting Provider from participating in those discussions even if critical of a carrier.

14. The following provision is added to the Agreement as specified below:

    (A) The following provision is added to Provider duties under Section 3 of the Agreement:
    (1) Under the terms of the Agreement and subject to WAC 284-170-470, as may be amended from time to time, Provider shall have the right to make a prior authorization request.

    (B) The following provision is added to the Agreement:
    (1) Emergency Fill, as defined by WAC 284-43-0160, as may be amended from time to time, shall mean a limited dispensed amount of medication that allows time for the processing of a preauthorization request, and only applies to those circumstances where a patient presents at a contracted pharmacy with an immediate therapeutic need for a prescribed medication that requires preauthorization.
    (2) Evernorth, in accordance with the requirements of WAC 284-170-470, as may be amended from time to time, will authorize an Emergency Fill by a dispensing pharmacist and approve the claim payment. Emergency Fill is only applicable when: the dispensing pharmacy cannot reach Evernorth's prior authorization department by phone as it is outside the department's business hours; or, Evernorth is available to respond to phone calls from the dispensing pharmacy regarding a covered benefit, but Evernorth cannot reach the prescriber for full consultation.
    (3) Evernorth will provide an authorization number to Provider after approval of a prior

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authorization request and upon receipt of a claim for that approved medication.

15. If Provider is an independent practice association, the following shall be added to Section 3 of the Agreement:

    Provider represents and warrants that it is authorized to act on behalf of its providers and will provide evidence of such authority upon request. To the extent that such providers are not employed by Provider but are independently contracted with Provider, Provider shall maintain agreements with such providers requiring the providers to comply with the terms and conditions of this Agreement to the extent applicable. The form of Provider's standard agreement(s) and all applicable compensation schedules or exhibits with such providers shall conform to the requirements of and shall contain all provisions required by applicable law, including but not limited to chapter 284-170 WAC, subchapter B, and shall include a Participant and Payor hold harmless provision acceptable to Evernorth, consistent with applicable law, and which provides, among other things, that such providers shall look solely to Provider for compensation for Covered Services if payments for Covered Services under this Agreement are directed to Provider. Such forms and all applicable compensation schedules or exhibits and any material amendments thereto must be approved in advance by the Office of the Insurance Commissioner pursuant to RCW 48-43-730, WAC 284-170-480, and any other applicable laws and regulations. Provider shall make available to Evernorth and to any applicable regulatory authority for review and approval by Evernorth and any applicable regulatory authority a copy of each such provider agreement, all applicable compensation schedules or exhibits, and any subsequent changes or amendments thereto prior to use. Provider shall require that each such provider execute an election to participate form. Upon request, Provider shall provide Evernorth with copies of all executed elections to participate.

16. If Provider is an independent practice association, the following shall replace and supersede the section of the Agreement entitled Assignment and Subcontracting:

    Neither Evernorth nor Provider may assign any rights or delegate any obligations under this Agreement without the written consent of the other party; provided, however, that any reference to Evernorth includes any successor in interest and Evernorth may assign its duties, rights and interests under this Agreement in whole or in part to an Evernorth affiliate or may delegate any and all of its duties to a third party in the ordinary course of business. Provider shall, prior to requesting Evernorth's consent to delegate any function performed under the Agreement, include the requirements of chapter 284-170 WAC, subchapter B in its contracting documents with the subcontractor in accordance with WAC 284-170-240 as may be amended from time to time; Evernorth shall retain the right to approve, suspend or terminate the arrangement. Provider's contracting documents shall be subject to review and approval by the Office of the Insurance Commissioner if required by applicable laws and regulations.

17. The section of the Agreement entitled Compensation is modified as follows; the underscored language reflects the changes made to the Agreement to conform to applicable state laws:

    Notwithstanding any provision to the contrary set forth in the Compensation section of the Agreement, or any similar provision in the Agreement, or a rate exhibit, the rates in the Agreement will be payment in full for all Covered Services furnished to Participants under the Agreement.

18. The section of the Agreement entitled Compensation is modified as follows; the underscored language reflects the changes made to the Agreement to conform to applicable state laws:

    Notwithstanding any provision to the contrary set forth in the Compensation section of the Agreement,

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or any similar provision in the Agreement, or a rate exhibit, the rates in this Agreement apply to all Covered Services provided to Participants in the Plan types covered by this Agreement, including services covered under a Participant's in or out-of- network benefits and whether the Payor or Participant is financially responsible for payment.

19. Credentialing: In accordance with Washington Administrative Code Section 284-170-411(1), Evernorth shall not establish selection criteria (a) that would allow Evernorth to avoid risk by excluding providers or facilities because they are located in geographic areas that contain populations presenting a risk of higher than average claims, losses, or health services utilization; or (b) that would exclude providers or facilities because they treat or specialize in treating persons presenting a risk of higher than average claims, losses, or health services utilization or because they treat or specialize in treating minority or special populations; or (c) discriminate regarding participation in the network solely based on the provider or facility type or category if the provider is acting within the scope of their license.

    Evernorth will make a determination approving or denying a credentialing application submitted to Evernorth no later than ninety (90) days after receiving a complete application from a health care provider.

    Effective June 1, 2020, Evernorth will make a determination approving or denying a credentialing application submitted to Evernorth no later than ninety (90) days after receiving a complete application from a health care provider. All determinations made by Evernorth in approving or denying credentialing applications will average no more than sixty (60) days.

20. Medical Management:

    a. Coverage Determinations: In accordance with Washington Administrative Code Section 284-43-2000(7), Evernorth will not penalize Provider by reducing future payments or terminating Provider's contract simply because Provider disputes the determination of coverage or payment for health care services.

    b. Precertification: In accordance with Washington Administrative Code Section 284-43-2000(4), Evernorth (the "issuer"), when conducting utilization review, shall:

        i. Accept information from any reasonably reliable source that will assist in the certification process;
        ii. Collect only the information necessary to certify the admission, procedure or treatment, length of stay, or frequency or duration of services;
        iii. Not routinely require providers or facilities to numerically code diagnoses or procedures to be considered for certification, but may request such codes, if available;
        iv. Not routinely request copies of medical records on all enrollees reviewed;
        v. Require only the section(s) of the medical record during concurrent review necessary in that specific case to certify medical necessity or appropriateness of the admission or extension of stay, frequency or duration of service;
        vi. For concurrent review, base review determinations solely on the medical information obtained by the carrier at the time of the review determination;
        vii. For retrospective review, base review determinations solely on the medical information available to the provider or facility at the time the health service was provided;
        viii. Not retrospectively deny coverage for emergency and nonemergency care that

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had prior authorization under the plan's written policies at the time the care was rendered unless the prior authorization was based upon a material misrepresentation by the provider or facility;

ix. Not retrospectively deny coverage or payment for care based upon standards or protocols not communicated to the provider or facility within a sufficient time period for the provider or facility to modify care in accordance with such standard or protocol; and

x. Reverse its certification determination only when information provided to the issuer is materially different from that which was reasonably available at the time of the original determination.

c. Medical Necessity Services: In accordance with Washington Administrative Code Section 284-170-270, Evernorth will cover the services of every category of heath care provider as long as the service is covered and is Medically Necessary, clinically efficacious and cost effective and the provider is operating within the scope of his/her license. However, Evernorth may determine that particular services for particular conditions by particular categories of providers are not cost-effective or clinically efficacious, and may exclude such services from coverage or reimbursement under a health plan. Services for acupuncture/acupressure and massage therapy will be reviewed for consistency with these requirements.

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WEST VIRGINIA REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred as "Provider") to comply with legislative and regulatory requirements of the State of West Virginia regarding provider contracts with providers rendering health care services in the State of West Virginia. To the extent that such West Virginia laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such West Virginia laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

A. (1) The definition for Emergency Services, if any, shall comply with West Virginia laws and regulations to the extent applicable.

(2) To the extent required, both parties agree to comply with applicable law with respect to the administration of claims, including but not limited to, W. Va. Code § 33-45-2. Provider shall accept as full payment the compensation as set forth in the Agreement.

(3) Any retroactive denial of claims, including any recoupment process utilized by Cigna, shall be administered in accordance with applicable law, including but not limited to, W. Va. Code § 33-45-2.

(4) No amendment to the Agreement or to any exhibit, addenda, schedule or policy that relates to payment, delivery of care, or the range of health care services provided by Provider shall be effective as to Provider unless Provider fails to notify Cigna in writing within twenty (20) business days of receipt of such amendment of Provider's intention to terminate the Agreement at the earliest date permitted under the Agreement.

(5) Cigna shall establish policies and procedures applicable to Provider in accordance with applicable law. Provider shall comply with and be bound by all such policies and procedures, including but not limited to, Cigna's policies relating to administrative requirements, utilization management, credentialing, quality management, payment and reimbursement, precertification and authorization of coverage decisions and claims processing.

(6) Nothing in the Agreement shall be construed to permit Cigna to terminate or fail to renew the Agreement or otherwise penalize Provider for invoking any of its rights under the Agreement or applicable law.

(7) "Experimental Medical Care" means medical, surgical or other health care procedures and treatments which are experimental or investigational, as determined by the Healthplan Medical Director in accordance with consensus derived from peer review medical and scientific literature and the practice of the national medical community, including (i) any procedures or treatments which are not recognized as conforming to accepted medical practice; (ii) any procedures or treatments in which the scientific assessment of the technique, or its application for a particular condition, has not been completed or its effectiveness has not been established; and (iii) any procedures or treatments for which the required approval of a governmental agency has not been granted at the time the services are rendered. Covered Services do not include Experimental Medical Care.

However, to the extent applicable, Covered Services include the patient cost to a Participant

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in a clinical trial, as a result of:

(a) treatment provided for a life-threatening condition; or
(b) prevention, early detection, and treatment studies on cancer.

Coverage shall be required if:

(1) The treatment is being provided or the studies are being conducted in a Phase I, Phase II, Phase III, or Phase IV clinical trial for cancer; or the treatment is being provided in a Phase I, Phase II, Phase III, or Phase IV clinical trial for any other life-threatening condition.
(2) The treatment is being provided in a clinical trial approved by:
    (i) one of the National Institutes of Health;
    (ii) a National Institutes of Health cooperative group or a National Institutes of Health center;
    (iii) the FDA in the form of an investigational new drug application;
    (iv) the federal Department of Veterans Affairs; or
    (v) an institutional review board of an institution in the state which has a multiple project assurance contract approved by the Office of Protection from Research Risks of the National Institutions of Health.
(3) The facility and personnel providing the treatment are capable of doing so by virtue of their experience, training, and volume of patients treated to maintain expertise.
(4) There is no clearly superior noninvestigational treatment alternative.
(5) The available clinical or preclinical data provide a reasonable expectation that the treatment will be at least as effective as the noninvestigational alternative.

Covered Services include the patient cost incurred for drugs and devices that have been approved for sale by the FDA whether or not the FDA has approved the drug or device for use in treating the patient's particular condition, to the extent that the drugs or devices are not paid for by the manufacturer, distributor, or provider of that drug or device.

An entity seeking coverage for treatment in a clinical trial approved by an institutional review board under subsection (2)(v) above shall post electronically and keep up-to-date a list of the clinical trials meeting the requirements of this section. The list shall include for each clinical trial: (a) the phase for which the trial is approved; (b) the entity approving the trial; (c) whether the trial is for treatment of cancer or another life- threatening disease and, if not cancer, the particular disease; and the estimated number of patients in the trial.

B. If Provider is an Intermediary as defined by W.Va. Code § 33-53-1, the following provisions shall apply to the Agreement as required by W.Va. Code § 33-53-5, as may be amended from time to time.

(1) Nothing in the Agreement shall be construed to delegate or assign to Provider Evernorth's statutory responsibility to monitor the offering of Covered Services to Participants.
(2) In accordance with applicable laws and regulations, Evernorth shall have the right to approve or disapprove the participation status of a Represented Provider in Provider's contracted network for the purpose of delivering Covered Services to Evernorth's Participants.
(3) In addition to the negotiated terms of the Agreement, Evernorth shall have access to all Provider's subcontracts, and shall have the right to make copies to facilitate regulatory review, upon twenty (20) days prior written notice from Evernorth.
(4) If applicable under the terms of the Agreement, Provider shall transmit utilization documentation and

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claims paid documentation to Evernorth as set forth in the Agreement. Evernorth shall monitor the timeliness and appropriateness of payments made to providers and health care services received by Participants.

(5) In addition to the negotiated requirements of the Agreement, Provider shall maintain the books, records, financial information and documentation of services provided to Participants at its principal place of business in the state and shall preserve them for two years in a manner that facilitates regulatory review.

(6) Provider shall allow the commissioner of insurance access to Provider's books, records, financial information and any documentation of services provided to Participants, as necessary to determine compliance with applicable laws and regulations.

(7) In accordance with applicable laws and regulations, Evernorth shall have the right, in the event of Provider's insolvency, to require the assignment to Evernorth of the provisions of a Represented Provider's contract addressing the provider's obligation to furnish Covered Services. Should Evernorth require such assignment, Evernorth shall remain obligated to compensate such provider for furnishing Covered Services under the same terms and conditions as Provider prior to the insolvency.

(8) Notwithstanding any other provision of this Section IV, to the extent that Evernorth has delegated any of its responsibilities to Provider, Evernorth shall retain full responsibility for Provider's compliance with the requirements of this section.

C. With respect to Covered Services rendered to Participants covered under an HMO Plan:

If Provider elects to terminate the Agreement for any reason, Provider must give sixty (60) days' advance written notice to Evernorth and the State of West Virginia Commissioner of Insurance before terminating the Agreement. Nonpayment for goods or services rendered by Provider is not a valid reason for avoiding the sixty (60) day notice period.

Notwithstanding the foregoing, to the extent that the Agreement provides for a longer notification period with respect to termination of the agreement by Provider, the longer notification period will apply.

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WISCONSIN REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as "Provider") to comply with legislative and regulatory requirements of the State of Wisconsin regarding provider contracts with providers rendering health care services in the State of Wisconsin. To the extent that such Wisconsin laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Wisconsin laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) The definition for Emergency Services, if any, shall comply with Wisconsin laws and regulations to the extent applicable.

(2) Upon termination of the Agreement, unless Provider no longer practices in the Plan's service area, or the Agreement is terminated for misconduct on the part of Provider, Provider shall continue to provide Covered Services for Participants undergoing a course of treatment for the following period of time: (i) if maternity care is the course of treatment and Participant is in the second or third trimester of pregnancy when the Agreement terminates, until the completion of postpartum care for Participant and infant; or (ii) for all other courses of treatment, for the remainder of the course of treatment or for ninety (90) days after the Agreement is terminated, whichever is shorter, except that Provider is not required to provide Covered Services beyond: (i) the end of the current plan year, for a Participant of a Plan with no open enrollment period; or (ii) the end of the plan year for which it was represented that Provider was, or would be, a Participating Provider, for a Participant of a Plan with an open enrollment period.

Provider shall be compensated for Covered Services provided to any such Participant in accordance with the compensation arrangements under the Agreement until ninety (90) days following termination, and compensation thereafter for continued Covered Services authorized by Evernorth shall be as mutually agreed.

Provider has no obligation under the Agreement to provide services to individuals who cease to be Participants.

(3) Upon termination of the Agreement, Provider is required to post a notification of termination with the plan in the Provider's office within 30 days prior to the termination or 15 days following receipt of the provider's termination notice by the Plan.

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EVERNORTH HEALTH SERVICES

WYOMING REGULATORY ADDENDUM FOR PROVIDERS

The provisions set forth in this Addendum are being added to the Agreement between Evernorth Behavioral Health, Inc. (hereafter referred to as "Evernorth") and the provider named in the Agreement (hereafter referred to as the "Provider") to comply with legislative and regulatory requirements of the State of Wyoming regarding provider contracts with providers rendering health care services in the State of Wyoming. To the extent that such Wyoming laws and regulations are applicable and/or not otherwise preempted by federal law, the provisions set forth in this Addendum shall apply and, to the extent of a conflict with a provision in the Agreement, shall control. The provisions of this Addendum shall apply to all providers governed by the Agreement, unless the context dictates otherwise. For the avoidance of doubt, to the extent such Wyoming laws and regulations are not applicable, the terms of the Agreement shall control. The provisions set forth in this Addendum do not apply with regard to Covered Services rendered to Participants covered under self-funded plans. Unless otherwise defined in this Addendum or applicable laws and regulations, capitalized terms used but not defined in this Addendum shall have the same meaning as set forth in the Agreement.

(1) In accordance with Wyo. Stat. § 26-22-504, Evernorth shall not refuse to re-contract with, or compensate Provider for Covered Services solely because Provider in good faith communicated with a Participant regarding the provisions, terms, or requirements of the Payor's products as they relate to the needs of that Participant.

(2) To the extent required by applicable law, Evernorth, Payor and Provider shall comply with Wyo. Stat. § 26-15-124 and any successor provisions regarding payment of claims.

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